# Aged Care Rules 2025

made under the

*Aged Care Act 2024*

## Compilation No. 10

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This compilation is in 2 volumes

| | |
|---|---|
| **Volume 1:** | **Chapters 1 to 5**<br>**sections 1-5 to 177-20** |
| Volume 2: | Chapters 6 to 15<br>sections 187-1 to 610-1B<br>Endnotes |

Each volume has its own contents

Prepared by the Office of Parliamentary Counsel, Canberra

# About this compilation

### This compilation

This is a compilation of the *Aged Care Rules 2025* that shows the text of the law as amended and in force on 1 August 2026 (the ***compilation date***).

The notes at the end of this compilation (the ***endnotes***) include information about amending laws and the amendment history of provisions of the compiled law.

### Uncommenced amendments

The effect of uncommenced amendments is not shown in the text of the compiled law. The details of amendments made up to, but not commenced at, the compilation date are underlined in the endnotes. Any uncommenced amendments affecting the law are accessible on the Register (www.legislation.gov.au).

### Application, saving and transitional provisions

If the operation of a provision or amendment of the compiled law is affected by an application, saving or transitional provision that is not included in this compilation, details are included in the endnotes.

### Editorial changes

For more information about any editorial changes made in this compilation, see the endnotes.

### Presentational changes

The *Legislation Act 2003* provides for First Parliamentary Counsel to make presentational changes to a compilation. Presentational changes are applied to give a more consistent look and feel to legislation published on the Register, and enable the user to more easily navigate those documents.

### Modifications

If the compiled law is modified by another law, the compiled law operates as modified but the modification does not amend the text of the law. Accordingly, this compilation does not show the text of the compiled law as modified. Any modifications affecting the law are accessible on the Register.

### Self-repealing provisions

If a provision of the compiled law has been repealed in accordance with a provision of the law, details are included in the endnotes.

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# Chapter 1—Introduction

## Part 1—Preliminary

### 1-5 Name

This instrument is the *Aged Care Rules 2025*.

### 3-5 Authority

This instrument is made under the *Aged Care Act 2024*.

### 4-5 Simplified outline of this instrument

This instrument provides for matters for the purposes of the Act that relate to the delivery of funded aged care services to individuals under the Commonwealth aged care system.

Chapter 2 provides for matters relating to entry to the Commonwealth aged care system.

Chapter 3 provides for matters relating to provider registration.

Chapter 4 provides for matters relating to conditions on provider registration.

Chapter 5 provides for matters relating to registered provider, responsible person and aged care worker obligations.

Chapter 6 provides for matters relating to obligations of operators of aged care digital platforms.

Chapter 7 provides for matters relating to Commonwealth contributions for funding of aged care services.

Chapter 8 provides for matters relating to individual fees and contributions for funding of aged care services.

Chapter 9 provides for matters relating to accommodation payments and accommodation contributions for funding of aged care services.

Chapter 10 provides for matters relating to means testing for funding of aged care services.

Chapter 11 provides for matters relating to governance of the aged care system.

Chapter 12 provides for matters relating to regulatory mechanisms.

Chapter 13 provides for matters relating to information management.



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Section 4-5

Chapter 14 provides for the review and reconsideration of certain decisions made under this instrument.



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# Part 2—Definitions

## Division 1—Definitions—general

### 5-5 Definitions

Note: The following expressions used in this instrument are defined in the Act:
(a) associated provider;
(b) care and services plan;
(c) enrolled nurse;
(d) health service;
(e) means testing category;
(f) Multi-Purpose Service Program;
(g) National Law;
(h) nursing;
(i) nursing assistant;
(j) old Act;
(k) registered nurse;
(l) service agreement;
(m) specialist aged care program;
(n) subsidy basis;
(o) Transition Care Program;
(p) transition time.

In this instrument:

***2023 MM category*** means a category for an area provided for by the Modified Monash Model, as the model existed on 13 April 2025, and known as MM 1, MM 2, MM 3, MM 4, MM 5, MM 6 or MM 7.

***Aboriginal or Torres Strait Islander Health Practitioner*** means a person who is registered under the National Law in the Aboriginal and Torres Strait Islander health practice profession.

***Aboriginal or Torres Strait Islander Health Worker*** means a person who:
(a) is:
(i) a full member of the National Association of Aboriginal and Torres Strait Islander Health Workers and Practitioners; and
(ii) not an Aboriginal or Torres Strait Islander Health Practitioner; or
(b) holds a Certificate II or higher in Aboriginal or Torres Strait Islander Primary Health Care Practice from a registered training organisation.

***accepted mental health condition*** means a mental health condition for which:
(a) the Repatriation Commission has accepted liability to pay a pension under the Veterans' Entitlements Act; or
(b) the Military Rehabilitation and Compensation Commission has accepted liability to pay compensation under the MRC Act or the *Safety, Rehabilitation and Compensation Act 1988*.



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***accommodation bond agreement*** means an agreement between an individual in the pre-2014 accommodation class and a registered provider in relation to the delivery of ongoing funded aged care services to the individual through the service group residential care in an approved residential care home of the provider that meets the requirements set out in section 287-25.

***accommodation bond retention amount*** means an amount that a registered provider may deduct from an accommodation bond balance in accordance with section 287-95.

***accommodation charge agreement*** means an agreement between an individual in the pre-2014 accommodation class and a registered provider in relation to the delivery of ongoing funded aged care services to the individual through the service group residential care in an approved residential care home of the provider that meets the requirements set out in section 287-115.

***accommodation wing***, of an approved residential care home, includes any of the following:

(a) a building;
(b) a floor or level of a building;
(c) an annex to a building;

that is used to provide accommodation for an individual to whom funded aged care services are being delivered in the home.

***ACN*** has the same meaning as in the *Corporations Act 2001*.

***Act*** means the *Aged Care Act 2024*.

***additional service fee*** means an additional service fee under section 146 of the *Aged Care (Consequential and Transitional Provisions) Rules 2025*.

***ad hoc higher everyday living agreement***: see section 284-2.

***AFM assessment item*** means the Australian Modified Functional Independence Measure assessment item of the AN-ACC Assessment Tool.

***AFM cognition score***, for an individual, means the individual's total score for communication and social cognition on the AFM assessment item.

***AFM communication score***, for an individual, means the individual's total score for communication on the AFM assessment item.

***AFM eating score***, for an individual, means the individual's score for eating on the AFM assessment item.

***AFM motor score***, for an individual, means the individual's total score for self-care, sphincter control, transfers and locomotion on the AFM assessment item.

***AFM social cognition score***, for an individual, means the individual's total score for social cognition on the AFM assessment item.



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***AFM transfers score***, for an individual, means the individual's total score for transfers on the AFM assessment item.

***Aged Care Assessment Manual*** means the Aged Care Assessment Manual, published by the Department, as existing on 29 June 2026.

Note: The Aged Care Assessment Manual could in 2026 be viewed on the Department's website (https://www.health.gov.au).

***aged care financial report***, for a registered provider, means the report required by section 166-310.

***age pension*** has the same meaning as in Part 2.2 of the Social Security Act.

***agitation score***, for an individual, means the individual's score for physically aggressive or inappropriate behaviour on the Behaviour Resource Utilisation Assessment assessment item of the AN-ACC Assessment Tool.

***AKPS assessment item*** means the Australia-modified Karnofsky Performance Status assessment item of the AN-ACC Assessment Tool.

***AKPS score***, for an individual, means the individual's score on the AKPS assessment item.

***allied health assistant*** means a person who holds a Certificate IV in Allied Health Assistance from a registered training organisation.

***allied health profession*** means a health profession other than the following:

(a) dental (including the profession of a dentist, dental therapist, dental hygienist, dental prosthetist and oral health therapist);
(b) medical;
(c) midwifery;
(d) nursing.

***allied health professional*** means a person who is any of the following:

(a) a registered health practitioner who:
    (i) is registered under the National Law to practise an allied health profession, other than as a student (within the meaning of the National Law); or
    (ii) holds non-practising registration under the National Law in an allied health profession;
(b) an Aboriginal or Torres Strait Islander Health Worker;
(c) an art therapist;
(d) an audiologist;
(e) a certified practicing nutritionist;
(f) a counsellor;
(g) a dietitian;
(h) an exercise physiologist;
(i) a genetic counsellor;
(j) a music therapist;



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(k) an orthoptist;
(l) an orthotist;
(m) a pedorthist;
(n) a prosthetist;
(o) a recreational therapist;
(p) a rehabilitation counsellor;
(q) a social worker;
(r) a sonographer;
(s) a speech pathologist.

***AN-ACC Assessment Tool*** means the Australian National Aged Care Classification Assessment Tool, published by the Department, as existing on 1 April 2021.

Note: The AN-ACC Assessment Tool could in 2025 be viewed on the Department's website (https://www.health.gov.au).

***AN-ACC Reference Manual*** means the Australian National Aged Care Classification Reference Manual, published by the Department, as existing on 1 April 2021.

Note: The AN-ACC Reference Manual could in 2025 be viewed on the Department's website (https://www.health.gov.au).

***annual accountability report***, for a registered provider, means the report required under section 166-745.

***annual activity report***, for a registered provider, means the report required under section 166-725.

***annual financial declaration statement***, for a registered provider, means the statement required by section 166-605.

***annual prudential compliance statement***, for a registered provider, means the statement required by section 166-360.

***annual statement of financial compliance and income and expenditure***, for a registered provider, means the statement required by section 166-730.

***annual wellness and reablement report***, for a registered provider, means the report required under section 166-615.

***antipsychotic medication***, in relation to medication management, means the prescription of medications to an individual for the purposes of the treatment of a diagnosed condition of psychosis.

***approval year***, for an approved higher maximum accommodation payment amount, means the period of 1 year beginning on:
(a) the day the approval takes effect under section 290-25 or 290-30 (as applicable); or
(b) any later anniversary of that day.



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***approved health practitioner*** means a medical practitioner, nurse practitioner or registered nurse.

***approved higher maximum accommodation payment amount*** means a higher maximum accommodation payment amount (expressed as a refundable accommodation deposit amount) approved by the Pricing Authority under subsection 290(6) of the Act.

***ARIA value***, for a location, means the value given to that location in accordance with the methodology set out in the document titled *Measuring Remoteness: Accessibility/Remoteness Index of Australia (ARIA)*, Occasional Papers: New Series Number 14, published by the Department in October 2001, as the document existed on 1 July 2013.

Note: The document titled *Measuring Remoteness: Accessibility/Remoteness Index of Australia (ARIA)* could in 2025 be viewed on the Department's website (https://www.health.gov.au).

***art therapist*** means a person who is registered with the Australian, New Zealand and Asian Creative Arts Therapies Association as a Creative Art Therapist.

***assistance dog*** means a dog that is an assistance animal within the meaning of the *Disability Discrimination Act 1992*.

***AT-HM List*** means the Assistive Technology and Home Modifications list published by the Department, as existing on 1 November 2025.

Note: The AT-HM List could in 2025 be viewed on the Department's website (https://www.health.gov.au).

***audiologist*** means a person who:

(a) is both:
  (i) a full member of Audiology Australia; and
  (ii) certified by Audiology Australia as an accredited audiologist; or
(b) is a full or ordinary member, or fellow audiologist, of the Australian College of Audiology incorporating the Hearing Aid Audiology Society of Australia with a certificate of recognition of competency, issued by that College, as both a hearing rehabilitation specialist and a diagnostic rehabilitation specialist.

***audited income and expenditure report***, for a registered provider, means the report required under section 166-640.

***Australian accounting standards*** means the accounting standards in force under section 334 of the *Corporations Act 2001*.

***Australian Commission on Safety and Quality in Health Care*** means the Commission established by subsection 8(1) of the *National Health Reform Act 2011*.

***Australia New Zealand Food Standards Code*** has the same meaning as in the *Food Standards Australia New Zealand Act 1991*.

***Australian Privacy Principle*** has the same meaning as in the *Privacy Act 1988*.



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***banning orders register*** means the register of banning orders established and maintained under section 507 of the Act.

***Barthel Index*** means the questions used in the Barthel Index of Activities of Daily Living set out in Collin C, Wade DT, Davies S, Horne V. The Barthel ADL Index: a reliability study. Int Disabil Stud. 1988;10(2):61-3. doi: 10.3109/09638288809164103. PMID: 3403500.

Note: The Barthel Index could in 2026 be viewed in the Quality Indicator Program Manual on the Department's website (https://www.health.gov.au).

***base interest rate***, on a day in a month, means the rate (expressed as a percentage) that is the sum of the following:

(a) the below threshold rate on the first day of the month;
(b) 2%.

***below threshold rate***, on a day, means the below threshold rate under subsection 1082(1) of the Social Security Act on the day.

***Braden activity score***, for an individual, means the individual's score for activity on the Braden Scale assessment item of the AN-ACC Assessment Tool.

***Braden total score***, for an individual, means the individual's total score on the Braden Scale assessment item of the AN-ACC Assessment Tool.

***building status amount***, for an individual for a day: see subsection 230-15(1).

***calculation day***, for a quarter, means the 15th day of the calendar month before the calendar month in which the quarter begins.

***care minute***: a minute is a ***care minute*** if it is spent delivering direct care, other than the following:

(a) the planning or delivery of activities to a group of individuals;
(b) the provision or maintenance of aids, appliances or equipment.

***care minutes performance statement***, for a registered provider, means the statement required by section 166-335.

***Category A residential care home*** means an approved residential care home that, immediately before the transition time, was a multi-purpose service (within the meaning of section 104 of the *Subsidy Principles 2014*) that was a Category A service within the meaning of section 88 of the *Aged Care (Subsidy, Fees and Payments) Determination 2014*.

***Category B residential care home*** means an approved residential care home that, immediately before the transition time, was a multi-purpose service (within the meaning of section 104 of the *Subsidy Principles 2014*) that was a Category B service within the meaning of section 89 of the *Aged Care (Subsidy, Fees and Payments) Determination 2014*.

***Category C residential care home*** means an approved residential care home that, immediately before the transition time, was a multi-purpose service (within the meaning of section 104 of the *Subsidy Principles 2014*) that was a Category C



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service within the meaning of section 90 of the *Aged Care (Subsidy, Fees and Payments) Determination 2014*.

***Category D residential care home*** means an approved residential care home that is not a Category A residential care home, Category B residential care home or Category C residential care home.

***certified practicing nutritionist*** means a person who is registered with the Australasian Association and Register of Practicing Nutritionists as a Certified Practicing Nutritionist.

***chemical restraint***: see subsection 17-5(2).

***child safety compliance statement***, for a registered provider, means the statement required by section 166-628.

***CHSP*** is short for the program known as the Commonwealth Home Support Program.

***CHSP contribution***: see subsection 286-15(1).

***compensation*** has the same meaning as in the *Health and Other Services (Compensation) Act 1995*.

***compensation payer*** has the same meaning as in the *Health and Other Services (Compensation) Act 1995*.

***complainant***: see subsection 361-10(1).

***complaint determination*** has the same meaning as in paragraph 361(1A)(d) of the Act.

***compliance report***, for a registered provider, means the report required under section 166-620.

***conditionally included AT-HM item*** means a product, item of equipment or home modification:

(a) that is listed in the part of the AT-HM List headed "Conditional inclusions"; and
(b) that is not an excluded AT-HM item.

***conditionally included communication and information management item*** means a product or item of equipment:

(a) that is listed in the part of the AT-HM List headed "Conditional inclusions" under "Communication and information management"; and
(b) that is not an excluded AT-HM item.

***conditionally included domestic life item*** means a product or item of equipment:

(a) that is listed in the part of the AT-HM List headed "Conditional inclusions" under "Domestic life"; and
(b) that is not an excluded AT-HM item.



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***consecutive unplanned weight loss***, in relation to unplanned weight loss, means a decrease in the weight of an individual of any amount, as determined by comparing the weight of an individual from the finishing weight of the previous quarter against the starting weight, middle weight and finishing weight for the current quarter.

***Consumer Experience Assessment*** means an assessment using the Quality-of-Care Experience questions.

***contact*** includes physical contact, face-to-face contact, oral communication, written communication and electronic communication.

***continuing service delivery branch***: see subsection 263A-5(1).

***continuity of care plan***: see section 149-75.

***Co-operatives National Law*** means the Law set out in the appendix to the *Co-operatives (Adoption of National Law) Act 2012* (NSW), and applying in a State or Territory under the following:

(a) *Co-operatives (Adoption of National Law) Act 2012* (NSW);
(b) *Co-operatives National Law Application Act 2013* (Vic.);
(c) *Co-operatives National Law Act 2020* (Qld);
(d) *Co-operatives National Law (South Australia) Act 2013* (SA);
(e) *Co-operatives National Law (Tasmania) Act 2015* (Tas.);
(f) *Co-operatives National Law (ACT) Act 2017* (ACT);
(g) *Co-operatives (National Uniform Legislation) Act 2015* (NT).

***counsellor*** means a person who:

(a) is registered as a registered counsellor with:
  (i) the Australian Counselling Association; or
  (ii) the Psychotherapy and Counselling Federation of Australia; and
(b) has a qualification covered by level 7, 8, 9 or 10 of the Australian Qualifications Framework (within the meaning of the *Higher Education Support Act 2003*).

***counted mainstream individual***: see subsection 176-15(5).

***counted NATSIFACP individual***: see subsection 176-35(5).

***current wait time***, for the allocation of a place for a classification type for a service group to an individual, on a day (the ***current day***) before the place is allocated to the individual, means the time starting at the time the individual's access approval for the classification type for the service group was given and ending at the end of the day before the current day.

***daily remaining income amount***, for an individual for a day: see subsection 231-17(5).

***daily total essential expenses***, for an individual for a day: see subsection 231-17(4).



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***DAP index number***, for an indexation day: see section 302-15.

***day of eligible residential funded aged care services***: see subsection 239-15(4).

***day of recognised residential care***: see subsections 176-20(4) and (5).

***DEMMI score***, for an individual, means the individual's total score on the De Morton Mobility Index assessment item.

***De Morton Mobility Index assessment item*** means the De Morton Mobility Index assessment item of the AN-ACC Assessment Tool.

***dietitian*** means a person who is accredited by Dietitians Australia as an Accredited Practising Dietitian.

***disruptiveness score***, for an individual, means the individual's score for verbally disruptive or noisy behaviour on the Behaviour Resource Utilisation Assessment assessment item of the AN-ACC Assessment Tool.

***diverse cultural activities*** includes cultural activities for the following:

(a) Aboriginal or Torres Strait Islander persons;
(b) individuals from culturally, ethnically and linguistically diverse backgrounds;
(c) individuals who are lesbian, gay, bisexual, trans/transgender or intersex or other sexual orientations, gender diverse or bodily diverse.

***diverse individual***, means an individual who is:

(a) an Aboriginal or Torres Strait Islander person, including an Aboriginal or Torres Strait Islander person from the stolen generations; or
(b) a veteran or war widow; or
(c) from a culturally, ethnically and linguistically diverse background; or
(d) experiencing homelessness or at risk of experiencing homelessness; or
(e) a parent or child who is or was separated by forced adoption or removal; or
(f) an adult survivor of institutional child sexual abuse; or
(g) a care-leaver, including a Forgotten Australian or former child migrant placed in out of home care; or
(h) lesbian, gay, bisexual, trans/transgender or intersex or other sexual orientations or is gender diverse or bodily diverse; or
(i) an individual with disability or mental ill-health; or
(j) neurodivergent; or
(k) deaf, deafblind, vision impaired or hard of hearing.

***diversional therapist*** means a person who holds any of the following qualifications from a registered training organisation:

(a) Certificate IV in Leisure and Health;
(b) Diploma in Leisure and Health;
(c) Bachelor of Health Science;
(d) Bachelor of Applied Science with a major in:
    (i) Leisure and Health; or



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(ii) Therapeutic Recreation.

***emergency department presentation*** means when an individual presents to an emergency department or an urgent care centre including where that presentation is in person or via a technology enabled platform.

***end-of-life care*** means the care provided in what is sometimes referred to as the terminal phase of life, where death is imminent and likely to occur within 3 months.

***environmental restraint***: see subsection 17-5(3).

***excluded AT-HM item*** means a product, item of equipment or home modification that is listed in the part of the AT-HM List headed "Exclusions".

***exercise physiologist*** means a person who is accredited by Exercise & Sports Science Australia as an Accredited Exercise Physiologist.

***extra service fee*** means an extra service fee under section 145 of the *Aged Care (Consequential and Transitional Provisions) Rules 2025*.

***fall*** means an event that results in an individual coming to rest inadvertently on the ground, floor or other lower level.

***fall resulting in major injury*** means a fall that results in one or more of the following:

(a) bone fracture;
(b) joint dislocation;
(c) closed head injury with altered consciousness;
(d) closed head injury with subdural haematoma.

***fee reduction supplement daily threshold amount*** for a day means the amount obtained by rounding down to the nearest cent the amount equal to 15% of the basic age pension amount (worked out on a per day basis) for the day.

***fee reduction supplement fortnightly threshold amount*** for a fortnight means the amount obtained by rounding down to the nearest cent the amount equal to 15% of the basic age pension amount (worked out on a per fortnight basis) for the fortnight.

***financial support statement***, for a registered provider, means the statement required by section 166-315.

***finishing weight***, in relation to unplanned weight loss, means the weight of an individual recorded in the third month of a quarter.

***first asset threshold***: see subsection 319-5(6).

***first income threshold***: see subsection 319-5(2).

***first subsidy year***, for an approved residential care home and a registered provider, means the period of 12 months after the day the registered provider is first eligible for subsidy for the delivery (other than under a specialist aged care



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program) of a funded aged care service through the service group residential care to any individual in the approved residential care home.

***formal agreement*** has the same meaning as in the old Act.

***fortnightly base AT-HM individual amount***, for an individual for a fortnight: see subsection 197-10(10).

***fortnightly base home support individual amount***, for an individual for a fortnight: see subsection 197-10(9).

***fortnightly base individual amount***, for an individual for a fortnight: see subsection 197-10(8).

***fortnightly individual contribution cap***, for an individual for a fortnight: see subsection 197-10(7).

***fortnightly remaining income amount***, for an individual for a fortnight: see subsection 197-10(6).

***fortnightly total essential expenses***, for an individual for a fortnight: see subsection 197-10(4).

***fortnightly total income amount***, for an individual for a fortnight: see subsection 197-10(5).

***fourth asset threshold***: see subsection 319-5(9).

***fourth income threshold***: see subsection 319-5(5).

***general purpose financial report***, for a registered provider, means the report required by section 166-345.

***genetic counsellor*** means a person who is registered with the Human Genetics Society of Australasia as a Certified Genetic Councillor.

***Ghent Global Incontinence Associated Dermatitis Categorisation Tool*** means the *Ghent Global IAD Categorisation Tool (GLOBIAD)*, version 1.0 (June 2017), developed by Skin Integrity Research Group, at Ghent University.

Note: The Ghent Global Incontinence Associated Dermatitis Categorisation Tool could in 2025 be viewed in the Quality Indicator Program Manual on the Department's website (https://www.health.gov.au).

***giving day***, for a financial support statement: see subsection 166-320(3).

***grant agreement*** means one or more grants of financial assistance to a registered provider for the delivery of funded aged care services entered into by the System Governor on behalf of the Commonwealth under section 264 of the Act.

***group A residential care home***, for a payment period: see subsection 239-15(2).

***group B residential care home***, for a payment period: see subsection 239-15(3).

***has specialised Aboriginal or Torres Strait Islander status***: an approved residential care home ***has specialised Aboriginal or Torres Strait Islander status***



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on a day if a determination that the home has specialised Aboriginal or Torres Strait Islander status under subsection 243(3) of the Act is in effect on the day.

***has specialised homeless status***: an approved residential care home ***has specialised homeless status*** on a day if a determination that the home has specialised homeless status under subsection 243(3) of the Act is in effect on the day.

***health profession*** has the same meaning as in the National Law.

***health professional*** means a person who is registered under the National Law in a health profession.

***health service standards assessment***: see subsection 109-10(4).

***higher cognitive ability***: an individual who is mobile only with assistance has ***higher cognitive ability*** if the individual's AFM cognition score is 22 or more.

***higher everyday living agreement***: see section 284 of the Act.

***higher function***: an individual who is not mobile has ***higher function*** if the individual's RUG total score is 16 or less.

***higher pressure sore risk***: an individual who is not mobile has ***higher pressure sore risk*** if the individual's Braden total score is 13 or less.

***home or community fee reduction supplement determination***, for an individual, means a determination made under section 197-10 in relation to the individual (including as varied).

***home or community place***, for an approved residential care home of a registered provider in or from which the provider delivers funded aged care services through the service group home support, assistive technology or home modifications under the MPSP, means a place allocated to the registered provider for delivering those services in or from that home.

***home support compounding factors***: see section 81-7.

***home support functional independence score***, for an individual, means the individual's total score for the questions in the Integrated Assessment Tool with the following headings:

(a) "Health literacy difficulties";
(b) "Get to places out of walking distance";
(c) "Does the client drive?";
(d) "Undertake housework (heavy/moderate)";
(e) "Go shopping (assuming transportation)";
(f) "Prepare meals";
(g) "Take medicine";
(h) "Handle money";
(i) "Use the telephone";
(j) "Use other communication device";



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(k) "Use online services";
(l) "Walk";
(m) "Wheelchair mobility";
(n) "Climb stairs";
(o) "Take a bath or shower";
(p) "Dressing";
(q) "Grooming";
(r) "Eating";
(s) "Transfers";
(t) "Upper body strength";
(u) "Toilet use";
(v) "Toileting – bladder";
(w) "Toileting – bowels".

***home support needs met score***, for an individual, means the individual's total score for the questions in the Integrated Assessment Tool in the form "Is the need being met?".

***hospital admission*** means when an individual is accepted by a hospital inpatient speciality service for ongoing management, whether planned or unplanned and including an admission of any length and occurring in any location.

***ICD-10 Australian Modified Pressure Injury Classification System*** means the classification system contained in the *International Statistical Classification of Diseases and Related Health Problems, Tenth Revision, Australian Modification (ICD-10-AM)*, as published by the Independent Health and Aged Care Pricing Authority, and as existing on 1 November 2025.

Note: The ICD-10-AM could in 2025 be viewed on the website of the Independent Health and Aged Care Pricing Authority (https://www.ihacpa.gov.au).

***illness separated couple*** has the same meaning as in the Social Security Act.

***in a service group***: a funded aged care service is ***in a service group*** if the service is in a service type that is in the service group.

Note: See Part 3.

***included AT-HM item*** means a product, item of equipment or home modification:

(a) that is listed in the part of the AT-HM List headed "Inclusions"; and
(b) that is not an excluded AT-HM item.

***included communication and information management item*** means a product or item of equipment:

(a) that is listed in the part of the AT-HM List headed "Inclusions" under "Communication and information management"; and
(b) that is not an excluded AT-HM item.

***included domestic life item*** means a product or item of equipment:



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(a) that is listed in the part of the AT-HM List headed "Inclusions" under "Domestic life"; and
(b) that is not an excluded AT-HM item.

***included home modifications item*** means a product, item of equipment or home modification:
(a) that is listed in the part of the AT-HM List headed "Inclusions" under "Home modifications"; and
(b) that is not an excluded AT-HM item.

***included managing body functions item*** means a product or item of equipment:
(a) that is listed in the part of the AT-HM List headed "Inclusions" under "Managing body functions"; and
(b) that is not an excluded AT-HM item.

***included mobility item*** means a product or item of equipment:
(a) that is listed in the part of the AT-HM List headed "Inclusions" under "Mobility"; and
(b) that is not an excluded AT-HM item.

***included self-care item*** means a product or item of equipment:
(a) that is listed in the part of the AT-HM List headed "Inclusions" under "Self-care"; and
(b) that is not an excluded AT-HM item.

***income tested fee***, for an individual in the pre-2014 residential contribution class for a day: see subsection 285A-13(1).

Note: For an individual in the pre-2014 residential contribution class, the calculation of the daily means tested amount involves the individual's income but not their assets (see section 319-15).

***Incontinence Associated Dermatitis*** means a type of irritant contact dermatitis characterised by erythema and oedema of the perianal or genital skin, and may be accompanied by bullae, erosion or secondary cutaneous infection.

Note: This definition is consistent with the Ghent Global Incontinence Associated Dermatitis Categorisation Tool.

***independently mobile***: an individual is ***independently mobile*** if the individual's DEMMI score is 13 or more.

***index number***, for a quarter, means the All Groups Consumer Price Index number (being the weighted average of the 8 capital cities) published by the Australian Statistician for that quarter.

***individual nominee***: see subsection 6-15(2).

***individual's room***, in an approved residential care home:
(a) means a room, or a part of a room, in the home that:
(i) is intended to be occupied as personal space by an individual to whom funded aged care services are delivered in the home; and



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(ii) contains a bed to be used by the individual; and

(b) includes:

(i) the areas that are in the immediate vicinity of the bed in the room or the part of the room; and

(ii) the contents of the room or the part of the room; and

(iii) an ensuite, or a shared bathroom and toilet, that is for the use of the individual.

***Injune*** means the Injune Multi-Purpose Service, 7 Fifth Avenue, Injune Queensland 4454.

***Integrated Assessment Tool*** means the Integrated Assessment Tool published by the Department, as existing on 29 June 2026.

Note: The Integrated Assessment Tool could in 2026 be viewed on the Department's website (https://www.health.gov.au).

***Integrated Health and Aged Care Services Module*** means the Integrated Health and Aged Care Services Module published by the Australian Commission on Safety and Quality in Health Care, as existing on 30 June 2025.

Note: The Integrated Health and Aged Care Services Module could in 2025 be viewed on the website of the Australian Commission on Safety and Quality in Health Care (https://www.safetyandquality.gov.au).

***judgment*** has the same meaning as in the *Health and Other Services (Compensation) Act 1995*.

***key personnel*** of a person or body to which the Grantee Code of Conduct applies means:

(a) if the person or body is a registered provider—a responsible person of the registered provider;

(b) if the body is not a registered provider—any person who:

(i) is responsible for the executive decisions of the person or body; or

(ii) has authority or responsibility for (or significant influence over) planning, directing, operating or controlling the activities of the person or body.

Note: For ***responsible person*** of a registered provider, see section 12 of the Act.

***local region*** means an aged care planning region specified in the document titled *2018 Aged Care Planning Regions by Statistical Areas Level 2 (SA2) 2016 - Australia* published by the Department, as existing on the day this instrument commences.

Note: The document titled *2018 Aged Care Planning Regions by Statistical Areas Level 2 (SA2) 2016 - Australia* could in 2025 be viewed on the Department's website (https://www.health.gov.au).

***lower cognitive ability***: an individual who is mobile only with assistance has ***lower cognitive ability*** if the individual's AFM cognition score is 10 or less.

***lower function***: an individual who is not mobile has ***lower function*** if the individual's RUG total score is 17 or more.



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***lower pressure sore risk***: an individual who is not mobile has ***lower pressure sore risk*** if the individual's Braden total score is 14 or more.

***low means individual***: an individual to whom a registered provider is delivering funded aged care services for a classification type for the service group residential care is a ***low means individual*** if, on the start day for the individual for the classification type, the individual's means tested amount was less than the maximum accommodation supplement amount for that day.

***low means resident***: see section 230-13.

***low means resident percentage***: see 230-13.

***maximum daily amount of the transitional resident contribution***: see subsection 285A-10(1).

***maximum permissible interest rate*** for a day, means the maximum rate for the day worked out in accordance with section 301-5.

***maximum possible daily accommodation payment amount*** for an individual for a day: see step 3 of the method statement in subsection 296-5(3).

***means tested care fee***, for an individual in the post-2014 residential contribution class: see subsection 285A-14(1).

***means testing class***: each of the following is a ***means testing class***:

(a) full-pensioner;
(b) part-pensioner;
(c) seniors health card holder;
(d) self-funded retiree.

***mechanical restraint***: see subsection 17-5(4).

***medical or psychological treatment***, in relation to a priority 1 reportable incident under section 165A-25, means treatment that may only be provided by a medical practitioner, nurse practitioner, registered nurse, psychologist or social worker.

***medical practitioner*** has the same meaning as in the National Law.

***medical treatment authority***, for an individual (the ***individual concerned***), means an individual or body that, under an appointment in writing that is in effect under the law of the State or Territory in which the individual concerned accesses funded aged care services, can give informed consent to the provision of medical treatment (however described) to the individual concerned if the individual concerned lacks capacity to give that consent.

***medication***, in relation to medication management, means a chemical substance given with the intention of preventing, diagnosing, curing, controlling or alleviating disease or otherwise enhancing the physical or mental health of an individual.



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***medium cognitive ability***: an individual who is mobile only with assistance has ***medium cognitive ability*** if the individual's AFM cognition score is between 11 and 21 (inclusive).

***member of a couple*** has the same meaning as in the Social Security Act.

***merging service delivery branch***: see subsection 263A-5(1).

***middle weight***, in relation to unplanned weight loss, means the weight of an individual recorded in the second month of a quarter.

***minimum monetary spend amount***, for an approved residential care home that has been, or is proposed to be, significantly refurbished, means the amount worked out by multiplying $25,000 by 40% of the lower of:

(a) the total number of individual's rooms in the home before the commencement of the refurbishment; and

(b) the total number of individual's rooms in the home after the completion of the refurbishment.

***mobile only with assistance***: an individual is ***mobile only with assistance*** if the individual's DEMMI score is between 4 and 12 (inclusive).

***Modified Monash Model*** means the model known as the Modified Monash Model developed by the Department for categorising metropolitan, regional, rural and remote locations according to both geographical remoteness and population size, based on population data published by the Australian Bureau of Statistics, as the model exists from time to time.

Note: In 2025, the Modified Monash Model categorisation for a location could be viewed on the Department's website (https://www.health.gov.au).

***monthly performance report***, for a registered provider, means the report required under section 166-610.

***MPSP*** is short for Multi-Purpose Service Program.

***MRC Act*** means the *Military Rehabilitation and Compensation Act 2004*.

***music therapist*** means a person who is registered with the Australian Music Therapy Association as a Registered Music Therapist.

***national efficient price***: the ***national efficient price*** for residential care activity is $295.64.

***National Law*** has the same meaning as in the *Health Insurance Act 1973*.

***National Safety and Quality Health Service Standards*** means the standards of that name developed by the Australian Commission on Safety and Quality in Health Care under paragraph 9(1)(e) of the *National Health Reform Act 2011*, as existing on 31 May 2021.

Note: The National Safety and Quality Health Service Standards could in 2025 be viewed on the website of the Australian Commission on Safety and Quality in Health Care (https://www.safetyandquality.gov.au).



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***NATSIFACP*** is short for the program known as the National Aboriginal and Torres Strait Islander Flexible Aged Care Program.

***NDIS screening application*** means a screening application within the meaning of paragraph 181Y(5)(a) of the NDIS Act.

***newly built home***: see section 230-20.

***nominated entity***: see subsection 95-5(1).

***non-subscription service*** means a funded aged care service that is not in a subscription service type.

***non-subscription service type*** means a service type that is not a subscription service type.

***no-show circumstances***: see subsections 11A-5(3) and (4).

***not mobile***: an individual is ***not mobile*** if the individual's DEMMI score is 3 or less.

***nurse practitioner*** has the same meaning as in the *Health Insurance Act 1973*.

***NWAU*** (short for National Weighted Activity Unit) means a measure of residential care activity, expressed as a common unit, against which the national efficient price is set.

***occupational therapist*** means a person who is registered under the National Law in the occupational therapy profession.

***occupied bed***, for an approved residential care home on a day, means an operational bed for the home that is occupied by an individual to whom funded aged care services are delivered on the day.

***offline bed***, for an approved residential care home, means a bed covered by the approval of the home that is covered by a notice under section 167 of the Act given in accordance with section 167-70 of this instrument.

***old Act wait time***, for an individual who, immediately before the transition time, was approved under section 22-1 of the old Act as a recipient of home care but was not a prioritised home care recipient (within the meaning of the old Act), means the time starting at the time the individual's approval as a recipient of home care was given and ending immediately before the transition time.

***old Principles*** means Principles made under section 96-1 of the old Act.

***operational bed***, for an approved residential care home, means a bed covered by the approval of the home that is not an offline bed for the home.

***orthoptist*** means a person who is registered with the Australian Orthoptic Board.

***orthotist*** means a person who:

(a) is a certified member of the Australian Orthotic Prosthetic Association; and
(b) is certified by that Association as a qualified orthotist.



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***pedorthist*** means a person who is registered with the Pedorthic Association of Australia as a Certified Pedorthist.

***Pension Rate Calculator A*** means the Rate Calculator at the end of section 1064 of the Social Security Act.

***permitted uses reconciliation*** has the meaning given by section 166-325.

***physical restraint***: see subsection 17-5(5).

***physiotherapist*** means a person who is registered under the National Law in the physiotherapy profession.

***police certificate***, for a person, means:

(a) a report about the person's criminal conviction record prepared by:
  (i) the Australian Federal Police; or
  (ii) the Australian Criminal Intelligence Commission; or
  (iii) an agency accredited by the Australian Criminal Intelligence Commission; or
  (iv) the police force or police service of a State or Territory; or
(b) for a responsible person of a registered provider that delivers funded aged care services in South Australia, or an aged care worker delivering funded aged care services in South Australia—the screening check known as the Aged Care Sector Employment Check issued to the person by the Department administered by the Minister administering the *Child Safety (Prohibited Persons) Act 2016* (SA).

***polypharmacy***, in relation to medication management, means the prescription of 9 or more medications to an individual.

***post-2014 flexible accommodation class***: an individual is in the ***post-2014 flexible accommodation class*** if:

(a) the individual entered a flexible care service (within the meaning of the old Act) before the transition time; and
(b) the approved provider of the service charged the individual an accommodation payment (within the meaning of the old Act); and
(c) if, at the transition time, the individual is not accessing funded aged care services through the residential care service group under a specialist aged care program:
  (i) the individual has a transition break period; and
  (ii) the transition break period is not more than 28 days; and
(d) since the transition time, the individual has not ceased accessing funded aged care services through the residential care service group under a specialist aged care program for a continuous period of more than 28 days.

***post-2014 home contribution class***: an individual is in the ***post-2014 home contribution class*** if, on 12 September 2024, the individual was approved as a recipient of home care (within the meaning of the old Act).



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***post-2014 residential accommodation class***: an individual is in the ***post-2014 residential accommodation class*** if:

(a) at the transition time, the individual is in the post-2014 residential contribution class; and

(b) since the transition time, the individual has not:

(i) both:

(A) elected, in the approved form, to cease being a member of the post-2014 residential contribution class; and

(B) ceased accessing funded aged care services in an approved residential care home and started accessing funded aged care services in another approved residential care home; or

(ii) ceased accessing funded aged care services in an approved residential care home for a continuous period of more than 28 days.

***post-2014 residential contribution class***: an individual is in the ***post-2014 residential contribution class*** if the individual has not elected, in the approved form, to cease being a member of the class, and:

(a) all of the following apply to the individual:

(i) before the transition time, the individual entered residential care (within the meaning of the old Act) other than as a recipient of respite care (within the meaning of that Act) and other than residential care provided through a multi-purpose service (within the meaning of the *Subsidy Principles 2014*);

(ii) immediately before the transition time, the individual was not a continuing residential care recipient (within the meaning of the old Act);

(iii) if, at the transition time, the individual is not accessing funded aged care services in an approved residential care home:

(A) the individual has a transition break period; and

(B) the transition break period is not more than 28 days; or

(b) all of the following apply to the individual:

(i) the individual is a member of the post-2014 home contribution class at the transition time;

(ii) the individual has entered residential care after the transition time;

(iii) the individual has not elected, in the approved form, to cease being a member of the post-2014 home contribution class.

***pre-2014 accommodation class***: an individual is in the ***pre-2014 accommodation class*** at a particular time (the ***test time***) that is on or after the transition time if:

(a) immediately before the transition time, any of the following agreements were in effect for the individual:

(i) a formal agreement (within the meaning of the old Act);

(ii) an accommodation bond agreement (within the meaning of the *Aged Care (Transitional Provisions) Act 1997*);

(iii) an accommodation charge agreement (within the meaning of the *Aged Care (Transitional Provisions) Act 1997*); or



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(b) all of the following apply to the individual:
  (i) immediately before the transition time, an agreement of a kind mentioned in paragraph (a) was not in effect for the individual;
  (ii) at the transition time, the individual was not accessing funded aged care services in an approved residential care home, but had previously received residential care through a residential care service (within the meaning of the old Act), and the most recent agreement of any kind under the old Act that had been in effect for the individual was an agreement of a kind mentioned in paragraph (a);
  (iii) at the test time, the individual has a transition break period and the transition break period is not more than 28 days; or

(c) both of the following apply to the individual:
  (i) immediately before the transition time, an agreement of a kind mentioned in subparagraph (a)(ii) or (iii) was not in effect for the individual, but the individual was eligible under the old Act to enter into such an agreement;
  (ii) at the transition time, the individual was accessing funded aged care services in an approved residential care home;

unless, before the test time, the individual:

(d) had ceased accessing funded aged care services in an approved residential care home (the ***first home***); and

(e) had started accessing funded aged care services in another approved residential care home (the ***second home***); and

(f) had elected in the approved form, before starting to access funded aged care services in the second home, to cease being a member of the pre-2014 residential contribution class.

***pre-2014 maximum permissible interest rate***: see section 287-165.

***pre-2014 minimum permissible asset value***: see subsection 287-45(3).

***pre-2014 residential contribution class***: an individual is in the ***pre-2014 residential contribution class*** if:

(a) immediately before the transition time, the individual was a continuing residential care recipient (within the meaning of the old Act); and

(b) if, at the transition time, the individual is not accessing funded aged care services in an approved residential care home:
  (i) the individual has a transition break period; and
  (ii) the transition break period is not more than 28 days; and

(c) the individual has not elected, in the approved form, to cease being a member of the class.

***pressure injury*** means a localised injury to the skin or underlying tissue, or both, usually over a bony prominence as a result of pressure, shear or a combination of these factors.

***previous weight***, in relation to unplanned weight loss by an individual in a quarter, means the individual's finishing weight for the previous quarter.



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***price agreement day***, for an individual and an approved residential care home, means:

(a) the day on which the registered provider for the approved residential care home and the individual enter into an accommodation agreement for the approved residential care home in accordance with section 293 of the Act; or

(b) if the accommodation agreement between the registered provider for the approved residential care home and the individual is varied because the individual proposes to change the individual's room and the proposed move is voluntary—the day on which the accommodation agreement is varied in relation to the individual's room; or

(c) if the individual is notified by the registered provider for the approved residential care home that the individual's room is to be changed for 28 days or longer, and the proposed move is not voluntary—the day on which the notice is given.

***Pricing Authority advice activity*** means an activity mentioned in paragraph 131A(1)(c) of the *National Health Reform Act 2011* conducted for the purpose of performing a function mentioned in paragraph 131A(1)(a) of that Act.

***principal home*** has the meaning given by section 11A of the Social Security Act other than subsections 11A(8) and (9) of that Act (which deal with the effect of absences from the principal home).

Note: An individual's principal home may be in a retirement village (see section 12 of the Social Security Act).

***priority 1 notice***: see section 165A-25.

***priority 1 reportable incident***: see section 165A-25.

***priority 2 notice***: see section 165A-30.

***priority 2 reportable incident***: see section 165A-30.

***priority category waiting proportion***, for a priority category for a classification type for the service group home support on a day, means the number of waiting individuals who have that priority category for the classification type for the service group divided by the total number of waiting individuals for all priority categories for the classification type for the service group on the day.

***prosthetist*** means a person who:

(a) is a certified member of the Australian Orthotic Prosthetic Association; and

(b) is certified by that Association as a qualified prosthetist.

***qualifying residential care home***, for a payment period: see subsection 239-15(1).

***quality indicators report***, for a registered provider, means the report required by Subdivision B of Division 3 of Part 2 of Chapter 5.



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***Quality-of-Care Experience questions*** means the questions in the Quality of Care Experience Aged Care Consumers (QCE-ACC) Tool (2020) developed by Flinders University, as existing on 20 March 2026.

Note: The Quality-of-Care Experience questions could in 2026 be viewed in the Quality Indicator Program Manual on the Department's website (https://www.health.gov.au).

***Quality of Life Assessment*** means an assessment using the Quality of Life questions.

***Quality of Life questions*** means the questions in the Quality of Life Aged Care Consumers (QOL-ACC) Tool (2021) developed by Flinders University, as existing on 20 March 2026.

Note: The Quality of Life questions could in 2026 be viewed in the Quality Indicator Program Manual on the Department's website (https://www.health.gov.au).

***quarterly financial report***, for a registered provider, means the report required by section 166-340.

***queue rate*** means:

(a) for the priority category high for the classification type ongoing for the service group home support—0.25; or

(b) for the priority category medium for the classification type ongoing for the service group home support—1.5; or

(c) for the priority category standard for the classification type ongoing for the service group home support—2.

***recreational therapist*** means a person who is a certified member of the Australian Recreational Therapy Association.

***reduced daily amount of the resident contribution***, for an individual for a day: see subsection 231-17(7).

***reference period***, for a quarter, has the meaning given by subsection 176-20(6).

***refunding event*** means:

(a) in relation to the refund of a refundable deposit balance—an event referred to in paragraph 311(1)(a) or (b) of the Act; or

(b) in relation to the refund of an accommodation bond balance—an event referred to in paragraph 287-102(1)(a) or (b) of this instrument.

***refund period*** means:

(a) in relation to the refund of a refundable deposit balance—the period within which the refundable deposit balance must be refunded in accordance with section 311 of the Act; or

(b) in relation to the refund of an accommodation bond balance—the period within which the accommodation bond balance must be refunded in accordance with section 287-102 of this instrument.

***refurbishment cost***, for an approved residential care home that has been, or is proposed to be, significantly refurbished, means:



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(a) unless paragraph (b) applies—the total cost of the refurbishment or proposed refurbishment of the home; or

(b) if the refurbishment or proposed refurbishment includes fire safety improvements, and the cost of the fire safety improvements is more than 25% of the minimum monetary spend amount for the home—the total cost of the refurbishment or proposed refurbishment, reduced by the amount by which the cost of the fire safety improvements exceeds 25% of the minimum monetary spend amount for the home.

***registered health practitioner*** has the same meaning as in the National Law.

***registered nurse staff member*** means a staff member of a registered provider who is a registered nurse.

***registered training organisation*** has the same meaning as in the *National Vocational Education and Training Regulator Act 2011*.

***rehabilitation counsellor*** means a person who is a member of the Australian Society of Rehabilitation Counsellors.

***reimbursement arrangement*** has the same meaning as in the *Health and Other Services (Compensation) Act 1995*.

***remedial massage therapist*** means a person who holds either of the following qualifications from a registered training organisation:

(a) a Diploma of Remedial Massage Therapy;

(b) a Certificate IV in Massage Therapy.

***required combined staff average number of care minutes***, per individual per day in respect of an approved residential care home for a quarter: see subsection 176-20(1).

***required registered nurse average number of care minutes***, per individual per day in respect of an approved residential care home for a quarter: see subsection 176-20(2).

***residential care compounding factors***: see section 81-8.

***residential care fee reduction supplement determination***, for an individual, means a determination made under subsection 231-16(2) in relation to the individual (including as varied).

***residential care place***, for an approved residential care home of a registered provider in which the provider delivers funded aged care services through the service group residential care under the MPSP, means a place allocated to the registered provider for delivering those services in that home.

***restorative care*** means an intensive short-term period of care after an illness or injury to help an individual maintain or regain independence.

***restorative care partner***: see subsection 148-40(4).

***restrictive practices nominee***: see subsection 6-15(1).



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***restrictive practices substitute decision-maker***: see section 6-20.

***Rockwood frailty score***, for an individual, means the individual's score on the Rockwood Frailty Score assessment item of the AN-ACC Assessment Tool.

***RRMA Classification*** means the document titled *Rural, Remote and Metropolitan Area Classification*, as existing at the transition time, setting out certain categories of areas in Australia that have been determined by the Department by reference to population size and remoteness of locality on the basis of 1991 census data published by the Australian Bureau of Statistics in 1994.

Note: The RRMA Classification for a location could in 2025 be viewed on the Department's website (https://www.health.gov.au).

***RUG total score***, for an individual, means the individual's total score on the Resource Utilisation Group - Activities of Daily Living assessment item of the AN-ACC Assessment Tool.

***seclusion***: see subsection 17-5(6).

***second asset threshold***:

(a) for an individual in the post-2014 residential contribution class: see paragraph 319-5(7)(a); or
(b) in any other case: see paragraph 319-5(7)(b).

***second income threshold***: see subsection 319-5(3).

***seniors health card*** has the same meaning as in the Social Security Act.

***serious offence conviction***: a person has a ***serious offence conviction*** if the person has been:

(a) convicted of murder or sexual assault; or
(b) convicted of, and sentenced to imprisonment for, any other form of assault.

***service activity report***, for a registered provider, means the report required under section 166-645.

***service delivery report***, for a registered provider, means the report required under section 166-625.

***service demographics report***, for a registered provider, means the report required under section 166-735.

***settlement*** has the same meaning as in the *Health and Other Services (Compensation) Act 1995*.

***significant compounding factors***:

(a) for a classification assessment for an individual for a classification type for the service group home support—the individual has ***significant compounding factors*** if the home support compounding factors for the individual, considered together, indicate that the individual has significantly higher care needs relative to the needs of other individuals



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with similar home support functional independence scores and home support needs met scores; and

(b) for a classification assessment for an individual for the classification type ongoing for the service group residential care—an individual mentioned in an item of the table in section 81-8 has ***significant compounding factors*** if the residential care compounding factors for the individual, considered together, indicate that the individual has significantly higher care needs relative to the needs of other individuals mentioned in that item.

***significantly refurbished home*** means an approved residential care home in relation to which a determination under subsection 230-30(1) or 230-35(1) is in effect.

***significant unplanned weight loss***, in relation to unplanned weight loss, means a decrease in the weight of an individual that is equal to or greater than 5%, as determined by comparing the finishing weight of the previous quarter against the finishing weight for the current quarter.

***Social Security Act*** means the *Social Security Act 1991*.

***social worker*** means a person who is certified by the Australian Association of Social Workers as an Accredited Social Worker.

***sonographer*** means a person who is registered with the Australian Sonographer Accreditation Registry as an Accredited Medical Sonographer.

***specialist Aboriginal or Torres Strait Islander programs*** means specialist programs for Aboriginal or Torres Strait Islander persons and includes, but is not limited to, the following:

(a) programs to deliver care and services that are culturally safe for, and tailored to meet the particular needs of, the Aboriginal or Torres Strait Islander persons to whom funded aged care services are being delivered in the approved residential care home in question;

(b) programs to promote social and cultural engagement and participation of Aboriginal or Torres Strait Islander persons.

***specialist aged care program fee***: see subsection 286-10(1).

***specialist homeless programs*** means specialist programs for persons with a background as a homeless person and includes, but is not limited to, the following:

(a) programs and interventions to manage complex behavioural needs of persons with that background;

(b) programs to promote social engagement and participation of persons with that background.

***speech pathologist*** means a person who is certified by Speech Pathology Australia as a Certified Practising Speech Pathologist.

***standing higher everyday living agreement***: see section 284-2.



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***starting weight***, in relation to unplanned weight loss, means the weight of an individual recorded in the first month of a quarter.

***Statement of Accounting Concepts SAC 1*** means Statement of Accounting Concepts SAC 1 "Definition of the Reporting Entity" published by the Australian Accounting Standards Board, as existing on 6 March 2020.

Note: The Statement of Accounting Concepts SAC 1 could in 2025 be viewed on the website of the Australian Accounting Standards Board (https://www.aasb.gov.au).

***subscription service*** means a funded aged care service in a subscription service type.

***subscription service type*** means any of the following service types:

(a) subscription allied health and therapy;
(b) subscription domestic assistance;
(c) subscription home maintenance and repairs;
(d) subscription home or community general respite;
(e) subscription meals;
(f) subscription nursing care;
(g) subscription personal care;
(h) subscription social support and community engagement;
(i) subscription therapeutic services for independent living;
(j) subscription transport.

***supported individual***: an individual is a ***supported individual*** if, immediately before the transition time, the individual was a supported resident within the meaning of the *Aged Care (Transitional Provisions) Act 1997*.

***surplus and uncommitted funds***, for a registered provider and a financial year, means financial assistance the System Governor, on behalf of the Commonwealth, has granted the registered provider in the financial year relating to the same or similar activities and that are:

(a) surplus and uncommitted; and
(b) confirmed by provided financial statements.

***target classification type wait time***: see section 93-14.

***target priority category wait time***, for a priority category for a classification type for the service group home support on a day, means:

(a) for the priority category urgent—1 month; or
(b) for the priority category high, medium or standard—the result of multiplying the wait time factor on the day by the queue rate for the priority category.

***TCP*** is short for Transition Care Program.

***third asset threshold***: see subsection 319-5(8).

***third income threshold***: see subsection 319-5(4).



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***transitional agreed amount***: see subsection 285A-25(2).

***transitional basic daily fee***: see subsection 285A-11(1).

***transitional bed reservation fee***: see subsection 285A-20(1).

***transitional compensation payment fee***: see subsection 285A-10(2).

***transitional higher everyday living agreement***: see subsection 285A-25(2).

***transitional higher everyday living fee***: see subsection 285A-25(1).

***transitional pre-entry fee***: see subsection 285A-15(1).

***transitional pre-entry period***: see subsection 285A-15(1).

***transitional resident contribution***: see subsection 285A-5(1).

***transition break period***, for an individual, means the sum of:

(a) the number of days in a continuous period that ended immediately before the transition time during which the individual was not being provided with residential care through a residential care service (within the meaning of the old Act) other than because the person was on leave (within the meaning of that Act); and

(b) the number of days in a continuous period beginning at the transition time during which the individual was not accessing funded aged care services in an approved residential care home.

The period mentioned in paragraph (a) and the period mentioned in paragraph (b) must each be a period of at least one day.

***unavoidable service cost***: see subsections 284-11(16) and (17).

***unplanned weight loss*** means:

(a) significant unplanned weight loss; or

(b) consecutive unplanned weight loss.

***unrealisable asset*** has the meaning given by subsections 11(12) and (13) of the Social Security Act.

***unreduced daily amount of the resident contribution***, for an individual for a day: see subsection 231-17(6).

***unreduced individual contribution***, for the delivery of a funded aged care service to an individual on a day, means the amount that would be the individual contribution for the delivery of the service to the individual on the day if step 4 in the method statement in subsection 273(2) of the Act were disregarded.

***veteran*** means a person:

(a) who is taken to have rendered eligible war service under section 7 of the Veterans' Entitlements Act; or

(b) in respect of whom a pension is payable under subsection 13(6) of that Act; or



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(c) who is:
  (i) a member of the Forces within the meaning of subsection 68(1) of that Act; or
  (ii) a member of a Peacekeeping Force within the meaning of that subsection; or
(d) who is:
  (i) a member within the meaning of the MRC Act; or
  (ii) a former member within the meaning of that Act; or
(e) who is an employee within the meaning of the *Safety, Rehabilitation and Compensation Act 1988*.

Note: The Acts mentioned in paragraphs (d) and (e) provide that, in some cases:
(a) a member of the Forces, or a member of a Peacekeeping Force, includes a person who is no longer serving; and
(b) an employee includes a person who has ceased to be an employee.

***Veterans' Entitlements Act*** means the *Veterans' Entitlements Act 1986*.

***waiting individual***, for a priority category for a classification type for a service group on a day, means an individual who:
(a) has an access approval in effect for the classification type for the service group; and
(b) has been assigned the priority category for the classification type for the service group; and
(c) has not been allocated a place for the classification type for the service group.

***wait time factor***: see section 93-13.

## 6-5 References to actions taken or statements given

In this instrument, a reference to an action taken or a statement given by one of the following persons is a reference to such actions or statements that are within the person's professional scope of practice:
(a) an allied health assistant;
(b) an allied health professional;
(c) a diversional therapist;
(d) a registered health practitioner;
(e) a remedial massage therapist.

## 6-15 Nominating restrictive practices nominees

(1) ***Restrictive practices nominee***, for a restrictive practice in relation to an individual, means:
(a) if there is only a single individual nominee for the restrictive practice in relation to the individual—that individual nominee; or
(b) if there is only a nominee group for the restrictive practice in relation to the individual—that nominee group; or



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(c) if there is more than one individual nominee, or a nominee group and one or more individual nominees, for the restrictive practice in relation to the individual—the individual nominee or nominee group (as applicable) that takes precedence (see paragraph (9)(a)).

(2) ***Individual nominee***, for a restrictive practice in relation to an individual (the ***individual concerned***), means an individual:

(a) who has been nominated by the individual concerned, in accordance with this section, as an individual who can give informed consent to the use of the restrictive practice in relation to the individual concerned if the individual concerned lacks capacity to give that consent; and

(b) who has agreed, in writing, to the nomination (and has not withdrawn that agreement); and

(c) who has capacity to give the informed consent mentioned in paragraph (a).

(3) ***Nominee group***, for a restrictive practice in relation to an individual (the ***individual concerned***), means a group of individuals:

(a) who have been nominated by the individual concerned, in accordance with this section, as a group of individuals who can jointly give informed consent to the use of the restrictive practice in relation to the individual concerned if the individual concerned lacks capacity to give that consent; and

(b) each of whom has agreed, in writing, to the nomination (and has not withdrawn that agreement); and

(c) each of whom has capacity to give the informed consent mentioned in paragraph (a).

(4) An individual may make, vary or revoke a nomination only if the individual concerned has capacity to do so.

(5) A nomination, or a variation or revocation of a nomination, must be made in writing.

(6) A nomination (or varied nomination) of a group may nominate not more than 3 individuals as members of the group.

(7) A nomination (or varied nomination) may include only one nomination of a group.

(8) An individual may be nominated as an individual, or as a member of a group, but not both.

(9) If a nomination (or a varied nomination) nominates more than one individual nominee, or both one or more individual nominees and a nominee group, the nomination (or varied nomination) must:

(a) state the order of precedence in which the individual nominees and nominee group (as applicable) are nominated; and

(b) if a nominee group is nominated—state the rules that will apply if the members of the group cannot agree on whether to give informed consent as mentioned in paragraph (3)(a) in a particular case.



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(10) An individual (the ***individual concerned***) may nominate, as an individual or a member of a group, an aged care worker of a registered provider that is delivering funded aged care services to the individual concerned only if the aged care worker is the partner or a relative of the individual concerned.

## 6-20 Meaning of *restrictive practices substitute decision-maker*

(1) An individual or body is the ***restrictive practices substitute decision-maker*** for a restrictive practice in relation to an individual (the ***individual concerned***) if:

(a) the individual or body is appointed by the law of the State or Territory in which the individual concerned accesses funded aged care services as an individual or body that can give informed consent to the use of the restrictive practice in relation to the individual concerned if the individual concerned lacks capacity to give that consent; or

(b) under an appointment in writing that is in effect under the law of the State or Territory in which the individual concerned accesses funded aged care services, the individual or body can give informed consent to the use of the restrictive practice in relation to the individual concerned if the individual concerned lacks capacity to give that consent.

(2) The following table has effect if:

(a) there is no such individual or body appointed for the restrictive practice in relation to the individual concerned under the law of the State or Territory in which the individual concerned accesses funded aged care services; and

(b) either:

(i) there is no clear mechanism for appointing such an individual or body under the law of the State or Territory; or

(ii) an application has been made for an appointment under the law of the State or Territory in relation to the use of the restrictive practice in relation to the individual concerned, but there is a significant delay in deciding the application.

**Meaning of *restrictive practices substitute decision-maker***

| Item | Column 1<br>For a restrictive practice in relation to the individual concerned, if … | Column 2<br>the *restrictive practices substitute decision-maker* for that restrictive practice in relation to the individual concerned is … |
|---|---|---|
| 1 | there is a restrictive practices nominee for the restrictive practice in relation to the individual concerned | that restrictive practices nominee. |
| 2 | item 1 does not apply to the restrictive practice in relation to the individual concerned, but the individual concerned has a partner:<br>(a) with whom the individual concerned has a close continuing relationship; and<br>(b) who has agreed, in writing, to act as a | that partner. |



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**Meaning of *restrictive practices substitute decision-maker***

| Item | Column 1<br>For a restrictive practice in relation to the individual concerned, if … | Column 2<br>the *restrictive practices substitute decision-maker* for that restrictive practice in relation to the individual concerned is … |
|---|---|---|
| | restrictive practices substitute decision-maker for the restrictive practice in relation to the individual concerned (and has not withdrawn that agreement); and<br>(c) who has capacity to act as a restrictive practices substitute decision-maker for the restrictive practice in relation to the individual concerned | |
| 3 | items 1 and 2 do not apply to the restrictive practice in relation to the individual concerned, but the individual concerned has a relative or friend:<br>(a) who, immediately before the individual concerned entered an approved residential care home, was an unpaid carer for the individual; and<br>(b) who has a personal interest in the welfare of the individual concerned on an unpaid basis; and<br>(c) with whom the individual concerned has a close continuing relationship; and<br>(d) who has agreed, in writing, to act as a restrictive practices substitute decision-maker for the restrictive practice in relation to the individual concerned (and has not withdrawn that agreement); and<br>(e) who has capacity to act as a restrictive practices substitute decision-maker for the restrictive practice in relation to the individual concerned | (a) if there is one such relative or friend—that relative or friend; or<br>(b) if there are 2 or more such relatives or friends—the eldest of those relatives or friends. |
| 4 | items 1, 2 and 3 do not apply to the restrictive practice in relation to the individual concerned, but the individual concerned has a relative or friend:<br>(a) who has a personal interest in the welfare of the individual concerned on an unpaid basis; and<br>(b) with whom the individual concerned has a close continuing relationship; and<br>(c) who has agreed, in writing, to act as a restrictive practices substitute | (a) if there is one such relative or friend—that relative or friend; or<br>(b) if there are 2 or more such relatives or friends—the eldest of those relatives or friends. |



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**Meaning of *restrictive practices substitute decision-maker***

| Item | Column 1<br>For a restrictive practice in relation to the individual concerned, if … | Column 2<br>the *restrictive practices substitute decision-maker* for that restrictive practice in relation to the individual concerned is … |
|---|---|---|
| | decision-maker for the restrictive practice in relation to the individual concerned (and has not withdrawn that agreement); and<br>(d) who has capacity to act as a restrictive practices substitute decision-maker for the restrictive practice in relation to the individual concerned | |
| 5 | items 1, 2, 3 and 4 do not apply to the restrictive practice in relation to the individual concerned, but there is a medical treatment authority for the individual concerned | (a) if there is one such medical treatment authority—that medical treatment authority; or<br>(b) if there are 2 or more such medical treatment authorities and the law of the State or Territory in which the individual concerned accesses funded aged care services provides for the order of precedence of the medical treatment authorities—the medical treatment authority that takes precedence under that law; or<br>(c) if:<br>(i) there are 2 or more medical treatment authorities; and<br>(ii) the law of the State or Territory in which the individual concerned accesses funded aged care services does not provide for the order of precedence of the medical treatment authorities; and<br>(iii) one of the medical treatment authorities is an individual;<br>that individual; or<br>(d) if:<br>(i) there are 2 or more medical treatment authorities; and<br>(ii) the law of the State or Territory in which the individual concerned accesses funded aged care services does not provide for the order of precedence of the medical treatment authorities; and<br>(iii) one or more of the medical treatment authorities are individuals;<br>the eldest of those individuals. |



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(3) For the purposes of paragraph (a) of column 1 of item 3 of the table in subsection (2), a person was an unpaid carer for the individual concerned if:

(a) the person was not employed, hired, retained or contracted (whether directly or through an employment or recruiting agency) as a carer for the individual concerned; and

(b) no payment or benefit other than one or more of the following was or will be made or given to the person for being a carer for the individual concerned:

(i) a carer payment or equivalent benefit;

(ii) payment in kind;

(iii) a payment or benefit as a beneficiary under the will of the individual concerned.

(4) For the purposes of paragraph (b) of column 1 of item 3 of the table and paragraph (a) of column 1 of item 4 of the table in subsection (2), a person has a personal interest in the welfare of the individual concerned on an unpaid basis if:

(a) the person is not employed, hired, retained or contracted (whether directly or through an employment or recruiting agency) to have that interest; and

(b) no payment or benefit other than one or more of the following is or will be made or given to the person for having that interest:

(i) a carer payment or equivalent benefit;

(ii) payment in kind;

(iii) a payment or benefit as a beneficiary under the individual's will.



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# Division 2—Matters prescribed for definitions in section 7 of the Act

## 7-1 Accommodation bond

For the purposes of the definition of ***accommodation bond*** in section 7 of the Act, an ***accommodation bond*** means an amount that does not accrue daily and that is paid or payable by an individual in the pre-2014 accommodation class in accordance with an accommodation bond agreement.

## 7-2 Accommodation bond balance

For the purposes of the definition of ***accommodation bond balance*** in section 7 of the Act, an ***accommodation bond balance*** is, at a particular time, an amount equal to the difference between:

(a) an accommodation bond, other than:
  (i) an accommodation bond that is to be paid by periodic payment; or
  (ii) the part of an accommodation bond that is to be paid by periodic payment; and

(b) the sum of any amounts that have been, or are permitted to be, deducted from the accommodation bond in accordance with Subdivision D of Division 3 of Part 2 of Chapter 9 of this instrument as at that time.

## 7-3 Accommodation charge

For the purposes of the definition of ***accommodation charge*** in section 7 of the Act, an ***accommodation charge*** means an amount that accrues daily and that is paid or payable by an individual in the pre-2014 accommodation class in accordance with an accommodation charge agreement.

## 7-8 Approved needs assessors

*Approving entities*

(1) For the purposes of paragraph (a) of the definition of ***approved needs assessor*** in section 7 of the Act, the kind of entity is an entity specified in the document titled *Single Assessment System assessment organisations by service area, region, state and territory*, published by the Department, as existing on the day this instrument commences.

Note: The document titled *Single Assessment System assessment organisations by service area, region, state and territory* could in 2025 be viewed on the Department's website (https://www.health.gov.au).

*Criteria*

(2) For the purposes of paragraph (b) of the definition of ***approved needs assessor*** in section 7 of the Act, the criteria for a person are that the person is an individual who is employed or otherwise engaged by:



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(a) an entity (a ***prescribed entity***) prescribed by subsection (1) of this section; or

(b) an entity engaged by a prescribed entity.

Note: An individual engaged by an entity includes an independent contractor.

## 7-11 Cost

For the purposes of the definition of ***cost*** in section 7 of the Act, the cost for the delivery by a registered provider of a funded aged care service for which the subsidy basis is cost means the amount charged by the provider for the delivery of the service.

## 7-12 Direct care

For the purposes of the definition of ***direct care*** in section 7 of the Act, the following funded aged care services are prescribed:

(a) the funded aged care services listed and described in items 2, 3, 4, 5, 6 and 7 of the table in section 8-150 of this instrument;

(b) the funded aged care services listed and described in items 3, 4, 5 and 6 of the table in section 8-155 of this instrument.

## 7-13 Entry contribution

For the purposes of the definition of ***entry contribution*** in section 7 of the Act, an ***entry contribution*** means a payment that was made before 1 October 1997 by an individual in the pre-2014 accommodation class in accordance with a formal agreement.

## 7-14 Entry contribution balance

For the purposes of the definition of ***entry contribution balance*** in section 7 of the Act, an ***entry contribution balance*** is, at a particular time, an amount equal to the difference between:

(a) the amount of an entry contribution; and

(b) the sum of any amounts that have been, or are permitted to be, deducted from the entry contribution in accordance with a formal agreement as at that time.

## 7-17 Entry day

(1) This section is made for the purposes of the definition of ***entry day*** in section 7 of the Act.

*Classification type ongoing*

(2) The first day an individual accesses a funded aged care service for the classification type ongoing through a service group is the entry day for the individual for that classification type for that service group.



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*Classification type short-term for the service groups home support, assistive technology and home modifications*

(3) The first day an individual accesses a funded aged care service for a classification level for the classification type short-term for the service group home support, assistive technology or home modifications is the entry day for the individual for the period of effect for that classification level.

*Classification type short-term for the service group residential care*

(4) The first day an individual accesses a funded aged care service for the classification type short-term for the service group residential care is the entry day for the individual for that classification type for that service group.

*Classification type hospital transition*

(5) The first day an individual accesses a funded aged care service for a classification level for the classification type hospital transition for a service group is the entry day for the individual for the period of effect for that classification level.

## 7-19 Homeowner

For the purposes of the definition of ***homeowner*** in section 7 of the Act:

(a) an individual who is not a member of a couple is a ***homeowner*** if:
  (i) the individual has a right or interest in the individual's principal home; and
  (ii) the individual's right or interest in the individual's principal home gives the individual reasonable security of tenure in the home; and

(b) an individual who is a member of a couple is a ***homeowner*** if:
  (i) the individual, or the individual's partner, has a right or interest in one residence that is the individual's principal home, or the partner's principal home, or the principal home of both of them; and
  (ii) the individual's right or interest, or the partner's right or interest, in the home gives the individual, or the individual's partner, reasonable security of tenure in the home.

## 7-20 Lifetime cap

For the purposes of the definition of ***lifetime cap*** in section 7 of the Act, the amount is:

(a) unless paragraph (b) applies—$137,917.01; or

(b) for an individual in the post-2014 home contribution class or the post-2014 residential contribution class—$86,185.23.



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## 7-21 Price charged

For the purposes of the definition of ***price charged*** in section 7 of the Act, the price charged for the delivery by a registered provider of a funded aged care service means the amount charged by the provider for the delivery of the service.

## 7-22 Serious injury or illness

For the purposes of the definition of ***serious injury or illness*** in section 7 of the Act, each of the following is a serious injury or illness:

(a) malnutrition;

(b) dehydration.

## 7-23 Service delivery branch

For the purposes of the definition of ***service delivery branch*** in section 7 of the Act, a ***service delivery branch*** of a registered provider means a place of business of the registered provider through which the provider delivers funded aged care services to an individual through the service group home support, assistive technology or home modifications.



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# Part 3—Aged care service list

## Division 1—Preliminary

### 8-5 Aged care service list

For the purposes of subsection 8(1) of the Act:

(a) each service listed and described in an item of a table in a section of Division 2, 4, 6 or 8 of this Part is a service for which funding may be payable under the Act; and

(b) this Part lists and describes each service, and specifies the service type that the service is in; and

(c) this Part specifies, for each service type:

(i) the service group the service type is in; and

(ii) any specialist aged care program under which the service type can be delivered; and

(iii) whether the service type can only be delivered under a specialist aged care program; and

(iv) each provider registration category under which the service type can be delivered; and

(d) this Part specifies, for each service mentioned in paragraph 8(1)(g) of the Act:

(i) the means testing category for the service; and

(ii) the subsidy basis for the service; and

(e) this Part specifies, for the service groups home support, assistive technology and home modifications, that all service types in those service groups must be delivered in a home or community setting; and

(f) this Part specifies, for the service group residential care, that all service types in the service group must be delivered in a residential care home.

### 8-10 Subsidy basis

The subsidy basis for each funded aged care service referred to in paragraph 8(1)(g) of the Act is cost.

### 8-12 Service types

For the purposes of subsection 8(2) of the Act, the following service types are prescribed:

(a) allied health and therapy;

(ab) subscription allied health and therapy;

(b) assistance with transition care;

(c) care management;

(d) community cottage respite;

(e) domestic assistance;



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(ea) subscription domestic assistance;
(f) equipment and products;
(g) hoarding and squalor assistance;
(h) home adjustments;
(i) home maintenance and repairs;
(ia) subscription home maintenance and repairs;
(j) home or community general respite;
(ja) subscription home or community general respite;
(k) meals;
(ka) subscription meals;
(l) nursing care;
(la) subscription nursing care;
(m) nutrition;
(n) personal care;
(na) subscription personal care;
(o) residential accommodation;
(p) residential clinical care;
(q) residential everyday living;
(r) residential non-clinical care;
(s) restorative care management;
(t) social support and community engagement;
(ta) subscription social support and community engagement;
(u) therapeutic services for independent living;
(ua) subscription therapeutic services for independent living;
(v) transport;
(w) subscription transport.



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# Division 2—Home support service types

## 8-15 Allied health and therapy

(1) A service listed and described in an item of the following table is in the service type allied health and therapy.

(2) The service requirements for a service listed and described in an item of the following table are that:
  (a) the service is for the individual to regain or maintain physical, functional or cognitive abilities that support the individual to remain safe and independent at home; and
  (b) the service is within the parameters specified in subsection (3); and
  (c) the service is for the management of conditions related to age-related disability or decline.

(3) For the purposes of paragraph (2)(b), the parameters for a service are the following:
  (a) the service may include clinical intervention, expertise, care and treatment, review, education (including techniques for self-management), and advice and supervision to improve capacity;
  (b) the service aims to give the individual the skills and knowledge to manage their own condition and promote independent recovery where appropriate;
  (c) the service may be delivered in person or via telehealth, as appropriate;
  (d) the service may be delivered individually or in a group-based format (such as clinically supervised group exercise classes), as appropriate;
  (e) for a service other than the services listed and described in items 6 and 7 of the following table—the service may be delivered:
    (i) directly by a registered health practitioner or allied health professional (as applicable); or
    (ii) by an allied health assistant or aged care worker, under the supervision of a registered health practitioner or allied health professional where safe and appropriate to do so;
  (f) for the service listed and described in item 6 of the following table—the service may be delivered:
    (i) directly by an Aboriginal or Torres Strait Islander Health Practitioner; or
    (ii) by an allied health assistant or aged care worker, under the supervision of an Aboriginal or Torres Strait Islander Health Practitioner, where safe and appropriate to do so;
  (g) for a service listed and described in item 7 of the following table—the service may be delivered:
    (i) directly by an Aboriginal or Torres Strait Islander Health Worker; or
    (ii) by an allied health assistant or aged care worker, under the supervision of an Aboriginal or Torres Strait Islander Health Worker, where safe and appropriate to do so.



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| Services in the service type allied health and therapy | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Allied health assistance | Allied health therapy assistance that meets the service requirements specified in subsection (2) |
| 2 | Podiatry | Podiatry that meets the service requirements specified in subsection (2) |
| 3 | Social work | Social work activities that meet the service requirements specified in subsection (2) |
| 4 | Speech pathology | Speech pathology that meets the service requirements specified in subsection (2) |
| 5 | Diet or nutrition | Assistance with diet or nutrition that meets the service requirements specified in subsection (2) |
| 6 | Aboriginal or Torres Strait Islander Health Practitioner assistance | Assistance provided by an Aboriginal or Torres Strait Islander Health Practitioner that meets the service requirements specified in subsection (2) |
| 7 | Aboriginal or Torres Strait Islander Health Worker assistance | Assistance provided by an Aboriginal or Torres Strait Islander Health Worker that meets the service requirements specified in subsection (2) |
| 8 | Physiotherapy | Physiotherapy that meets the service requirements specified in subsection (2) |
| 9 | Psychology | Psychology that meets the service requirements specified in subsection (2) |
| 10 | Exercise physiology | Exercise physiology that meets the service requirements specified in subsection (2) |
| 11 | Occupational therapy | Occupational therapy that meets the service requirements specified in subsection (2) |
| 12 | Counselling or psychotherapy | Counselling or psychotherapy that meets the service requirements specified in subsection (2) |
| 13 | Music therapy | Music therapy that meets the service requirements specified in subsection (2) |

## 8-16 Subscription allied health and therapy

A service listed and described in an item of the following table is in the service type subscription allied health and therapy.

| Services in the service type subscription allied health and therapy | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to allied health assistance | Access to the service listed and described in item 1 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |



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**Services in the service type subscription allied health and therapy**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 2 | Subscription access to podiatry | Access to the service listed and described in item 2 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 3 | Subscription access to social work | Access to the service listed and described in item 3 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 4 | Subscription access to speech pathology | Access to the service listed and described in item 4 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 5 | Subscription access to diet or nutrition | Access to the service listed and described in item 5 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 6 | Subscription access to Aboriginal or Torres Strait Islander Health Practitioner assistance | Access to the service listed and described in item 6 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 7 | Subscription access to Aboriginal or Torres Strait Islander Health Worker assistance | Access to the service listed and described in item 7 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 8 | Subscription access to physiotherapy | Access to the service listed and described in item 8 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 9 | Subscription access to psychology | Access to the service listed and described in item 9 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 10 | Subscription access to exercise physiology | Access to the service listed and described in item 10 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 11 | Subscription access to occupational therapy | Access to the service listed and described in item 11 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 12 | Subscription access to counselling or psychotherapy | Access to the service listed and described in item 12 of the table in section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 13 | Subscription | Access to the service listed and described in item 13 of the table in |



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| Services in the service type subscription allied health and therapy | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | access to music therapy | section 8-15 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-20 Assistance with transition care

A service listed and described in the following table is in the service type assistance with transition care.

| Services in the service type assistance with transition care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Transition care management | Initial and ongoing assessment, planning and management, and coordination and monitoring, of the individual's movement from hospital, through the TCP and back into the community or into a residential care home, including the following:<br>(a) ensuring that:<br>(i) the individual's care and services plan is carried out; and<br>(ii) progress against the care and services plan goals is monitored;<br>(b) acting as a central point of contact;<br>(c) liaising with and organising all care requirements provided by external service providers (including registered health practitioners and allied health professionals);<br>(d) administration and operation of the TCP, including documentation relating to the individual;<br>(e) arranging for another aged care assessment if needed prior to the completion of the individual's transition care episode;<br>(f) managing the individual's transition into their post transition care arrangements, including a comprehensive written and verbal handover<br>Note: For requirements for care and services plans, see paragraph 148(e) of the Act and Subdivisions A and D of Division 3 of Part 4 of Chapter 4 of this instrument. For Aged Care Quality Standards for care and services plans, see subsections 15-20(1) to (3) of this instrument. |
| 2 | Assistance to access medical practitioner | Transport for the individual to visit a medical practitioner, or assistance in arranging a home visit by a medical practitioner |
| 3 | Transition care medication management | The following:<br>(a) implementation of a safe and efficient system to manage prescribing, procuring, dispensing, supplying, packaging, storing and administering of both prescription and over-the-counter medicines;<br>(b) administration of, and monitoring the effects of, medication (including injections), including supervision and physical assistance with taking both prescription and over-the-counter medication under the delegation and clinical supervision of a registered nurse or other appropriate registered health practitioner |



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**Services in the service type assistance with transition care**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 4 | Transition care emergency or after hours assistance | Having at least one suitably skilled employee of the registered provider or an appropriate agency continuously on call to give emergency assistance when needed |
| 5 | Transition care continence management | The following:<br>(a) assisting the individual to:<br>(i) maintain continence or manage incontinence; and<br>(ii) use aids and appliances designed to assist continence management;<br>(b) the supply of aids and appliances designed to assist continence management to meet the individual's needs, including the following:<br>(i) commode chairs, over-toilet chairs, bed-pans, uridomes, and catheter and urinary drainage appliances;<br>(ii) as many continence aids (such as disposable urinal covers, pants, pads, chair pads and enemas) as are needed to meet the individual's needs |
| 6 | Waste disposal | Safe disposal of transition care related organic and inorganic waste material |

## 8-25 Care management

A service listed and described in an item of the following table is in the service type care management.

**Services in the service type care management**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Home support care management | Activities that ensure funded aged care services contribute to the overall wellbeing of the individual, including implementing the care and services plan for the individual, service coordination, monitoring, review and evaluation, advocacy, support and education, but not including administrative costs funded through prices on services<br>Note: For requirements for care and services plans, see paragraph 148(e) of the Act and Subdivisions A and D of Division 3 of Part 4 of Chapter 4 of this instrument. For Aged Care Quality Standards for care and services plans, see subsections 15-20(1) to (3) of this instrument. |

## 8-30 Community cottage respite

A service listed and described in the following table is in the service type community cottage respite.



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| Services in the service type community cottage respite | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Cottage respite | Overnight care for the individual that is provided:<br>(a) in a cottage-style respite facility; and<br>(b) in a community setting (other than in the individual's home, the home of a carer of the individual or the home of a host family); and<br>(c) to provide respite for a carer of the individual |

## 8-35 Domestic assistance

A service listed and described in an item of the following table is in the service type domestic assistance.

| Services in the service type domestic assistance | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | General house cleaning | The following:<br>(a) the provision of, or assistance with, light household cleaning, including mopping, vacuuming, washing dishes, and general tidying of surface areas, that ensure the individual remains safe at home;<br>(b) the supply of equipment or consumables required for cleaning mentioned in paragraph (a);<br>but not including professional cleaning that would usually be paid for by an individual (such as carpet cleaning, pest control, dry cleaning or pet care) |
| 2 | Laundry services | The following:<br>(a) the provision of, or assistance with, laundry activities including but not limited to the laundering of clothing and bedding and the ironing of clothing;<br>(b) the supply of consumables required for laundry activities mentioned in paragraph (a);<br>but not including dry cleaning |
| 3 | Shopping assistance | The provision of shopping, or assistance with shopping activities, including developing a shopping list, online shopping, driving to a shop and assisting with the collection of shopping, but not including the cost of the shopping |

## 8-36 Subscription domestic assistance

A service listed and described in an item of the following table is in the service type subscription domestic assistance.

| Services in the service type subscription domestic assistance | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription | Access to the service listed and described in item 1 of the table in |



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| Services in the service type subscription domestic assistance | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | access to general house cleaning | section 8-35 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to laundry services | Access to the service listed and described in item 2 of the table in section 8-35 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 3 | Subscription access to shopping assistance | Access to the service listed and described in item 3 of the table in section 8-35 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-40 Hoarding and squalor assistance

A service listed and described in the following table is in the service type hoarding and squalor assistance.

| Services in the service type hoarding and squalor assistance | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Hoarding and squalor supports | Services for an individual who is experiencing symptoms of hoarding disorder or who is living in severe domestic squalor, including, for example, the following:<br>(a) implementing the care and services plan for the individual;<br>(b) a one-off clean-up;<br>(c) review and evaluation;<br>(d) linking the individual to specialist support services<br>Note: For requirements for care and services plans, see paragraph 148(e) of the Act and Subdivisions A and D of Division 3 of Part 4 of Chapter 4 of this instrument. For Aged Care Quality Standards for care and services plans, see subsections 15-20(1) to (3) of this instrument. |

## 8-45 Home maintenance and repairs

A service listed and described in an item of the following table is in the service type home maintenance and repairs.

| Services in the service type home maintenance and repairs | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Gardening | The provision of, or assistance with, maintenance of a residential garden, including essential light gardening such as mowing lawns, pruning shrubs and clearing yards that contribute to maintaining the individual's home in a safe and habitable condition, but not including the following: |



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| Services in the service type home maintenance and repairs | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | | (a) professional gardening services that would usually be paid for by an individual (such as tree removal, landscaping or farm or water-feature maintenance);<br>(b) gardening services that relate to visual appeal rather than safety or accessibility (such as installing and maintaining plants, garden beds and compost);<br>(c) services that are the responsibility of other parties (such as landlords or government housing authorities) |
| 2 | Assistance with home maintenance and repairs | Essential minor repairs and maintenance:<br>(a) that the individual used to be able to do themselves, or that are required to maintain safety (such as cleaning gutters, replacing lightbulbs and repairing broken door handles); or<br>(b) that are required to address an imminent age-related safety risk (such as repairing uneven flooring that poses a falls risk or a section of carpet damaged by a wheelchair);<br>but not including the following:<br>(c) professional maintenance and repair services that would usually be paid for by an individual (such as professional pest extermination, installing cabinetry or replacing carpets due to usual wear and tear);<br>(d) services that are the responsibility of other parties (such as landlords or government housing authorities) |
| 3 | Expenses for home maintenance and repairs | The supply of equipment or consumables required for that service |

## 8-46 Subscription home maintenance and repairs

A service listed and described in an item of the following table is in the service type subscription home maintenance and repairs.

| Services in the service type subscription home maintenance and repairs | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to gardening | Access to the service listed and described in item 1 of the table in section 8-45 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to assistance with home maintenance and repairs | Access to the service listed and described in item 2 of the table in section 8-45 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |



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## 8-50 Home or community general respite

A service listed and described in an item of the following table is in the service type home or community general respite.

**Services in the service type home or community general respite**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Flexible respite | Support and assistance for the individual that is provided:<br>(a) during the day or overnight; and<br>(b) in the individual's home; and<br>(c) to provide respite for a carer of the individual |
| 2 | Community and centre-based respite | Small day outings or structured group activities for the individual that are provided:<br>(a) to enable the individual to develop, maintain or support independent living and social interaction; and<br>(b) in a community setting; and<br>(c) to provide respite for a carer of the individual |

## 8-51 Subscription home or community general respite

A service listed and described in an item of the following table is in the service type subscription home or community general respite.

**Services in the service type subscription home or community general respite**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Subscription access to flexible respite | Access to the service listed and described in item 1 of the table in section 8-50 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to community and centre-based respite | Access to the service listed and described in item 2 of the table in section 8-50 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-55 Meals

A service listed and described in an item of the following table is in the service type meals.

**Services in the service type meals**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Meal | Preparation, packaging and delivery of pre-prepared meals, but not including |



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| Services in the service type meals | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | delivery | the following:<br>(a) the cost of ingredients;<br>(b) takeaway food delivery;<br>(c) meal delivery for other members of the household |
| 2 | Meal preparation | Support to prepare meals in the home or community, but not including the cost of ingredients |

## 8-56 Subscription meals

A service listed and described in an item of the following table is in the service type subscription meals.

| Services in the service type subscription meals | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to meal delivery | Access to the service listed and described in item 1 of the table in section 8-55 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to meal preparation | Access to the service listed and described in item 2 of the table in section 8-55 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-60 Nursing care

(1) A service listed and described in an item of the following table is in the service type nursing care.

*Clinical care matters*

(2) For items 1 to 3 of the following table, the clinical care matters are the following:
- (a) the assessment, treatment and monitoring of clinical conditions;
- (b) administration of medications;
- (c) wound care;
- (d) clinical continence management;
- (e) management of skin integrity;
- (f) education;
- (g) specialist service linkage.

| Services in the service type nursing care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Registered | Clinical care provided by a registered nurse, including but not limited to the |



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| Services in the service type nursing care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | nurse clinical care | clinical care matters specified in subsection (2) |
| 2 | Enrolled nurse clinical care | Clinical care provided by an enrolled nurse, including but not limited to the clinical care matters specified in subsection (2) |
| 3 | Nursing assistant clinical care | Clinical care provided by a nursing assistant, including but not limited to the clinical care matters specified in subsection (2) |
| 4 | Nursing care consumables | The supply of consumables used in delivering the clinical care mentioned in items 1 to 3, including oxygen and specialised products for wound care, continence management and skin integrity |

## 8-61 Subscription nursing care

A service listed and described in an item of the following table is in the service type subscription nursing care.

| Services in the service type subscription nursing care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to registered nurse clinical care | Access to the service listed and described in item 1 of the table in section 8-60 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to enrolled nurse clinical care | Access to the service listed and described in item 2 of the table in section 8-60 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 3 | Subscription access to nursing assistant clinical care | Access to the service listed and described in item 3 of the table in section 8-60 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-65 Nutrition

A service listed and described in an item of the following table is in the service type nutrition.

| Services in the service type nutrition | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Nutrition supports | The supply of:<br>(a) supplementary dietary products (enteral and oral); and |



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| Services in the service type nutrition | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | | (b) aids;<br>that are:<br>(c) required for conditions related to age-related functional decline or impairment; and<br>(d) prescribed by a dietitian or registered health practitioner |

## 8-70 Personal care

A service listed and described in an item of the following table is in the service type personal care.

| Services in the service type personal care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Assistance with self-care and activities of daily living | Attendant care to meet essential and ongoing needs (such as mobility, eating and hygiene), but not including professional services that would usually be paid for by an individual (such as waxing or hairdressing) |
| 2 | Assistance with self-administration of medications | Assistance with self-administration of medications, including arranging for medications to be dispensed by a pharmacist, but not including prescribing or administering medications |
| 3 | Continence management (non-clinical) | Attendant non-clinical care to manage continence needs (such as support to access advice or funding, or assistance changing aids) |

## 8-71 Subscription personal care

A service listed and described in an item of the following table is in the service type subscription personal care.

| Services in the service type subscription personal care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to assistance with self-care and activities of daily living | Access to the service listed and described in item 1 of the table in section 8-70 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to assistance with self-administration | Access to the service listed and described in item 2 of the table in section 8-70 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |



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| Services in the service type subscription personal care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | of medications | |
| 3 | Subscription access to continence management (non-clinical) | Access to the service listed and described in item 3 of the table in section 8-70 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-75 Restorative care management

A service listed and described in an item of the following table is in the service type restorative care management.

| Services in the service type restorative care management | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Home support restorative care management | The provision of specialist coordination services for individuals with the classification level SAH restorative care pathway for the classification type short-term for the service group home support, but not including administrative costs funded through prices on services |

## 8-80 Social support and community engagement

(1) A service listed and described in an item of the following table is in the service type social support and community engagement.

*Excluded costs*

(2) For items 2, 3 and 4 of the following table, the excluded costs are fees associated with participation in an activity (such as tickets, accommodation and membership fees).

| Services in the service type social support and community engagement | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Group social support | Support in a group setting to facilitate meeting the individual's need for social contact or company or participation in community life (including diverse cultural activities), including service and activity identification and linkage |
| 2 | Individual social support | Support on an individual basis to facilitate meeting the individual's need for social contact or company or participation in community life (including diverse cultural activities), including the following:<br>(a) service and activity identification and linkage;<br>(b) assistance to participate in social interactions (in-person or online); |



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| Services in the service type social support and community engagement | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | | (c) assistance to visit services or participate in telephone and web-based check-in services;<br>but not including the excluded costs specified in subsection (2) |
| 3 | Accompanied activities | Accompanying the individual to facilitate:<br>(a) meeting the individual's need for social contact or company or participation in community life (including diverse cultural activities), but not including the excluded costs specified in subsection (2); or<br>(b) the individual's attendance at medical or other appointments, but not including the cost of the appointments |
| 4 | Cultural support | Culturally focused support to facilitate meeting the individual's need for social contact or company or participation in community life (including diverse cultural activities), including the following:<br>(a) service and activity identification and linkage;<br>(b) assistance to access translating and interpreting services and translation of information into the individual's chosen language;<br>(c) referral pathways to advocacy or community organisations;<br>(d) assistance in attending cultural and community events;<br>but not including the excluded costs specified in subsection (2) |
| 5 | Digital education and support | Technology-focused support to facilitate meeting the individual's need for social contact or company or participation in community life (including diverse cultural activities), including provision of or assistance with access to training, or direct assistance in the use of technologies to improve digital literacy to aid independence and participation (such as paying bills online, accessing telehealth services and connecting with digital social programs), but not including the purchase of smart devices for the purpose of online engagement |
| 6 | Assistance to maintain personal affairs | Support for maintaining the individual's financial and legal affairs to facilitate meeting the individual's need for social contact or company or participation in community life (including diverse cultural activities), but not including service fees (such as funeral plans or accountant's fees) |
| 7 | Expenses to maintain personal affairs | An internet or phone service (or both) for an individual who is homeless or is at risk of homelessness, and who needs support to maintain connection to funded aged care services |

## 8-81 Subscription social support and community engagement

A service listed and described in an item of the following table is in the service type subscription social support and community engagement.



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| Services in the service type subscription social support and community engagement | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to group social support | Access to the service listed and described in item 1 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to individual social support | Access to the service listed and described in item 2 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 3 | Subscription access to accompanied activities | Access to the service listed and described in item 3 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 4 | Subscription access to cultural support | Access to the service listed and described in item 4 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 5 | Subscription access to digital education and support | Access to the service listed and described in item 5 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 6 | Subscription access to assistance to maintain personal affairs | Access to the service listed and described in item 6 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 7 | Subscription access to expenses to maintain personal affairs | Access to the service listed and described in item 7 of the table in section 8-80 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-85 Therapeutic services for independent living

(1) A service listed and described in an item of the following table is in the service type therapeutic services for independent living.

(2) For a service listed and described in the following table, the service requirements are that:

(a) the service uses evidence-based techniques to manage social, mental and physical wellbeing in support of the individual remaining safe and independent at home; and

(b) the service is within the parameters specified in subsection (3); and

(c) the service is for the management of conditions related to age-related disability or decline; and

(d) the service does not include relaxation massage or fees associated with participation in an activity (such as tickets, accommodation, membership fees, or supplies for an activity such as craft materials).



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(3) For the purposes of paragraph (2)(b), the parameters for a service are the following:

(a) the service may include treatment, education and advice;

(b) the service aims to give the individual the skills and knowledge to manage their own condition and promote independent recovery where appropriate;

(c) the service may be delivered in person or via telehealth, as appropriate;

(d) the service may be delivered individually or in a group-based format, as appropriate;

(e) the service may be delivered:

(i) directly by a registered health practitioner or allied health professional (as applicable); or

(ii) by an allied health assistant or aged care worker, under the supervision of a registered health practitioner or allied health professional, where safe and appropriate to do so.

**Services in the service type therapeutic services for independent living**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Acupuncture | Acupuncture:<br>(a) provided by a registered health practitioner; and<br>(b) that meets the service requirements specified in subsection (2);<br>but not including herbal medicine |
| 2 | Chiropractics | Chiropractic therapy:<br>(a) provided by a registered health practitioner; and<br>(b) that meets the service requirements specified in subsection (2) |
| 3 | Diversional therapy | Diversional therapy:<br>(a) provided by an allied health professional or a diversional therapist; and<br>(b) that meets the service requirements specified in subsection (2) |
| 4 | Remedial massage | Remedial massage:<br>(a) prescribed by an allied health professional; and<br>(b) provided by an allied health professional or a remedial massage therapist; and<br>(c) that meets the service requirements specified in subsection (2) |
| 5 | Art therapy | Art therapy:<br>(a) provided by an allied health professional; and<br>(b) that meets the service requirements specified in subsection (2) |
| 6 | Osteopathy | Osteopathy:<br>(a) provided by a registered health practitioner; and<br>(b) that meets the service requirements specified in subsection (2) |

## 8-86 Subscription therapeutic services for independent living

A service listed and described in an item of the following table is in the service type subscription therapeutic services for independent living.



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| Services in the service type subscription therapeutic services for independent living | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to acupuncture | Access to the service listed and described in item 1 of the table in section 8-85 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 2 | Subscription access to chiropractics | Access to the service listed and described in item 2 of the table in section 8-85 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 3 | Subscription access to diversional therapy | Access to the service listed and described in item 3 of the table in section 8-85 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 4 | Subscription access to remedial massage | Access to the service listed and described in item 4 of the table in section 8-85 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 5 | Subscription access to art therapy | Access to the service listed and described in item 5 of the table in section 8-85 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |
| 6 | Subscription access to osteopathy | Access to the service listed and described in item 6 of the table in section 8-85 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |

## 8-90 Transport

(1) A service listed and described in an item of the following table is in the service type transport.

*Excluded things*

(2) For the items of the following table, the excluded things are the following:
  (a) an individual's purchase of a vehicle;
  (b) an individual's vehicle running costs;
  (c) licence costs;
  (d) professional transit services (such as public transport, flights or ferries);
  (e) travel for holidays.

| Services in the service type transport | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Direct transport | Provision of a car and driver for group and individual transport services to connect the individual with their usual activities, and including fuel, but not including the excluded things specified in subsection (2) |
| 2 | Indirect | Supply of a service voucher for taxi or rideshare services for group and |



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| Services in the service type transport | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | transport | individual transport services to connect the individual with their usual activities, but not including the excluded things specified in subsection (2) |

## 8-91 Subscription transport

A service listed and described in an item of the following table is in the service type subscription transport.

| Services in the service type subscription transport | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Subscription access to direct transport | Access to the service listed and described in item 1 of the table in section 8-90 by way of a subscription arrangement under which the service is made available to the individual during a particular period on a day |



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# Division 3—Other specified matters for home support service types

## 8-95 All service types must be delivered in a home or community setting

All service types in the service group home support must be delivered in a home or community setting.

## 8-100 Other specified matters—non-subscription service types that can only be delivered under specialist aged care programs

A service type mentioned in column 1 of an item of the following table:

(a) is in the service group home support; and

(b) can only be delivered under a specialist aged care program mentioned in column 2 of the item; and

(c) can be delivered under a provider registration category mentioned in column 3 of the item.

**Other specified matters**

| Item | Column 1<br>Service type | Column 2<br>Specialist aged care programs | Column 3<br>Provider registration categories |
|---|---|---|---|
| 1 | Assistance with transition care | TCP | Nursing and transition care |
| 2 | Community cottage respite | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP | Personal and care support in the home or community |
| 3 | Hoarding and squalor assistance | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP | Advisory and support services |

## 8-105 Other specified matters—other non-subscription service types

*Service group, specialist aged care programs and provider registration categories*

(1) A service type mentioned in column 1 of an item of the following table:

(a) is in the service group home support; and

(b) can be delivered under a specialist aged care program mentioned in column 2 of the item; and

(c) can be delivered under a provider registration category mentioned in column 3 of the item.

*Means testing categories*

(2) The means testing category for a service in a service type mentioned in column 1 of an item of the following table is the means testing category mentioned in column 4 of the item.



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**Other specified matters**

| Item | Column 1<br>Service type | Column 2<br>Specialist aged care programs | Column 3<br>Provider registration categories | Column 4<br>Means testing category |
|---|---|---|---|---|
| 1 | Allied health and therapy | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP;<br>(d) TCP | Personal and care support in the home or community | Clinical supports |
| 2 | Care management | (a) MPSP;<br>(b) NATSIFACP | Personal and care support in the home or community | Clinical supports |
| 3 | Domestic assistance | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP;<br>(d) TCP | Home and community services | Everyday living |
| 4 | Home maintenance and repairs | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP;<br>(d) TCP | Home and community services | Everyday living |
| 5 | Home or community general respite | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP | Personal and care support in the home or community | Independence |
| 6 | Meals | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP;<br>(d) TCP | Home and community services | Everyday living |
| 7 | Nursing care | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP;<br>(d) TCP | Nursing and transition care | Clinical supports |
| 8 | Nutrition | (a) MPSP;<br>(b) NATSIFACP;<br>(c) TCP | Personal and care support in the home or community | Clinical supports |
| 9 | Personal care | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP;<br>(d) TCP | Personal and care support in the home or community | Independence |
| 10 | Restorative care management | (a) MPSP;<br>(b) NATSIFACP | Personal and care support in the home or community | Clinical supports |
| 11 | Social support and community engagement | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP | Advisory and support services | Independence |
| 12 | Therapeutic | (a) CHSP; | Personal and care support | Independence |



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| Other specified matters | | | | |
|---|---|---|---|---|
| **Item** | **Column 1**<br>**Service type** | **Column 2**<br>**Specialist aged care programs** | **Column 3**<br>**Provider registration categories** | **Column 4**<br>**Means testing category** |
| | services for independent living | (b) MPSP;<br>(c) NATSIFACP | in the home or community | |
| 13 | Transport | (a) CHSP;<br>(b) MPSP;<br>(c) NATSIFACP | Home and community services | Independence |

## 8-106 Other specified matters—subscription service types

*Service group and provider registration category*

(1) A service type mentioned in column 1 of an item of the following table:
  (a) is in the service group home support; and
  (b) can be delivered under the provider registration category subscription trial.

*Means testing categories*

(2) The means testing category for a service in a service type mentioned in column 1 of an item of the following table is the means testing category mentioned in column 2 of the item.

| Other specified matters | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service type** | **Column 2**<br>**Means testing category** |
| 1 | Subscription allied health and therapy | Clinical supports |
| 2 | Subscription domestic assistance | Everyday living |
| 3 | Subscription home maintenance and repairs | Everyday living |
| 4 | Subscription home or community general respite | Independence |
| 5 | Subscription meals | Everyday living |
| 6 | Subscription nursing care | Clinical supports |
| 7 | Subscription personal care | Independence |
| 8 | Subscription social support and community engagement | Independence |
| 9 | Subscription therapeutic services for independent living | Independence |
| 10 | Subscription transport | Independence |



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# Division 4—Assistive technology service types

## 8-110 Equipment and products

(1) A service listed and described in an item of the following table is in the service type equipment and products.

*Means testing category*

(2) For a service listed and described in any of items 1 to 10 of the following table, the means testing category is independence.

(3) For the service listed and described in item 11 of the following table, the means testing category is clinical supports.

**Services in the service type equipment and products**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Managing body functions items (non-loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, other than on loan, of included managing body functions items; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 2 | Managing body functions items (loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, on loan, of included managing body functions items; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 3 | Self-care items (non-loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, other than on loan, of included self-care items; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 4 | Self-care items (loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, on loan, of included self-care items; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 5 | Mobility items (non-loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, other than on loan, of included mobility items; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 6 | Mobility items (loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, on loan, of included mobility items; and |



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**Services in the service type equipment and products**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| | | (b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 7 | Domestic life items (non-loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, other than on loan, of either or both of the following:<br>(i) included domestic life items;<br>(ii) conditionally included domestic life items, if the conditions specified for the items in the AT-HM List are satisfied; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 8 | Domestic life items (loan) | A service:<br>(a) that consists of the sourcing, supply and provision to the individual, on loan, of either or both of the following:<br>(i) included domestic life items;<br>(ii) conditionally included domestic life items, if the conditions specified for the items in the AT-HM List are satisfied; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 9 | Communication and information management items (non-loan) | A service that:<br>(a) consists of the sourcing, supply and provision to the individual, other than on loan, of either or both of the following:<br>(i) included communication and information management items, other than items that the individual is able to access under the Hearing Services Program administered by the Department;<br>(ii) conditionally included communication and information management items, if the conditions specified for the items in the AT-HM List are satisfied and the individual is not able to access the items under the Hearing Services Program administered by the Department; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 10 | Communication and information management items (loan) | A service that:<br>(a) consists of the sourcing, supply and provision to the individual, on loan, of either or both of the following:<br>(i) included communication and information management items, other than items that the individual is able to access under the Hearing Services Program administered by the Department;<br>(ii) conditionally included communication and information management items, if the conditions specified for the items in the AT-HM List are satisfied and the individual is not able to access the items under the Hearing Services Program administered by the Department; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 11 | Assistive | Either: |



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**Services in the service type equipment and products**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| | technology prescription and clinical support | (a) the following, delivered by a registered health practitioner or an allied health professional:<br>(i) identifying an issue or problem that restricts the individual's physical, functional or cognitive ability;<br>(ii) assessing the level of assistive technology needed for the individual to regain or maintain physical, functional or cognitive ability;<br>(iii) identifying included AT-HM items and conditionally included AT-HM items (other than included home modifications items) that will assist the individual to regain or maintain physical, functional or cognitive ability; or<br>(b) any of the following wraparound activities for the supply to an individual of an included AT-HM item or a conditionally included AT-HM item under a service listed and described in any of items 1 to 10 of this table:<br>(i) setting up, fitting or providing training on the use of the item;<br>(ii) providing support and troubleshooting to minimise abandonment of the item;<br>(iii) maintenance and follow up for the item (including evaluating the effects on the individual's functioning);<br>(iv) if the supply of the item to the individual is covered by a service listed and described in any of item 2, 4, 6, 8 or 10 of this table—maintenance, repair and refurbishment of the item or, at the end of the item's safe working life, disposal of the item;<br>(v) administrative activities relating to the supply of the item |



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# Division 5—Other specified matters for assistive technology service types

## 8-115 All service types must be delivered in a home or community setting

All service types in the service group assistive technology must be delivered in a home or community setting.

## 8-120 Other specified matters for assistive technology service types

The service type equipment and products:

(a) is in the service group assistive technology; and

(b) can be delivered under any of the following specialist aged care programs:

(i) CHSP;

(ii) MPSP;

(iii) NATSIFACP;

(iv) TCP; and

(c) can be delivered under the provider registration category assistive technology and home modifications.



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# Division 6—Home modifications service types

## 8-125 Home adjustments

(1) Each service listed and described in the following table is in the service type home adjustments.

*Means testing category*

(2) For the service listed and described in item 1 of the following table, the means testing category is independence.

(3) For the service listed and described in item 2 of the following table, the means testing category is clinical supports.

**Services in the service type home adjustments**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Home modifications items | A service:<br>(a) that consists of the sourcing, supply and provision to the individual of included home modifications items; and<br>(b) to which at least one of the subparagraphs in paragraph 67(1)(b) of the Act applies |
| 2 | Home modifications prescription and clinical support | Either:<br>(a) the following, delivered by a registered health practitioner or an allied health professional:<br>(i) identifying an issue or problem that restricts the individual's physical, functional or cognitive ability;<br>(ii) assessing the level of home modification needed for the individual to regain or maintain physical, functional or cognitive ability;<br>(iii) identifying included home modifications items that will assist the individual to regain or maintain physical, functional or cognitive ability; or<br>(b) any of the following wraparound activities for the supply to an individual of an included home modifications item under the service listed and described in item 1 of this table:<br>(i) setting up, fitting or providing training on the use of the item;<br>(ii) coordination activities relating to the supply of the item |



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# Division 7—Other specified matters for home modifications service types

## 8-130 All service types must be delivered in a home or community setting

All service types in the service group home modifications must be delivered in a home or community setting.

## 8-135 Other specified matters for home modifications service types

The service type home adjustments:

(a) is in the service group home modifications; and

(b) can be delivered under any of the following specialist aged care programs:

(i) CHSP;

(ii) MPSP;

(iii) NATSIFACP; and

(c) can be delivered under the provider registration category assistive technology and home modifications.



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# Division 8—Residential care service types

## 8-140 Residential accommodation

Each service listed and described in the following table is in the service type residential accommodation.

**Services in the service type residential accommodation**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Accommodation | The following:<br>(a) capital infrastructure costs and depreciation of buildings and grounds used by individuals;<br>(b) communal areas for living, dining and recreation, as well as personal accommodation in either individual or shared rooms;<br>(c) refurbishments and replacements of fixtures, fittings and infrastructure;<br>(d) maintenance, of buildings and grounds used by individuals, to address normal wear and tear |
| 2 | Accommodation administration | Administration relating to the general operation of the residential care home, including accommodation agreements, accommodation bond agreements and accommodation charge agreements |

## 8-145 Residential everyday living

Each service listed and described in the following table is in the service type residential everyday living.

**Services in the service type residential everyday living**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Operational administration and emergency assistance | The following:<br>(a) administration relating to:<br>(i) the delivery of the other services listed and described in this table; and<br>(ii) service agreements;<br>(b) emergency assistance, including the following:<br>(i) at all times, having at least one suitable employee of the registered provider onsite and able to take action in an emergency;<br>(ii) if an individual is in need of urgent medical attention—providing emergency assistance in accordance with the registered provider's protocol for providing such assistance;<br>(iii) activation of emergency plans in the case of fire, floods or other emergency;<br>(iv) contingency planning for emergencies; |



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**Services in the service type residential everyday living**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| | | (v) staff training for emergencies |
| 2 | Communication services | Access for individuals to an external telecommunications mechanism in the residential care home (and in individuals' rooms if requested), such as telephone, internet or wi-fi services, but not including any usage charges or device costs |
| 3 | Utilities | The following:<br>(a) utility running costs for the residential care home (such as electricity, water and gas);<br>(b) heating and cooling for bedrooms and common areas to a comfortable temperature;<br>(c) testing and tagging of all electrical equipment provided by the registered provider;<br>but not including electrical equipment brought into the residential care home by individuals |
| 4 | Cleaning services and waste disposal | The following:<br>(a) cleanliness and tidiness of the entire residential care home, including the individual's personal area unless the individual chooses to and is able to maintain their personal area themselves;<br>(b) safe disposal of organic and inorganic waste material |
| 5 | Communal furnishings | Fit-for-purpose communal lounge and dining furniture, including the following:<br>(a) televisions;<br>(b) if the residential care home has a communal outdoor space—outdoor furniture |
| 6 | Bedroom and bathroom furnishings | The following (other than bedroom and bathroom furnishings that are customised or that the individual chooses to provide):<br>(a) a bed and a mattress that meet the individual's care, safety and comfort needs, including, if required, a bed that is adjustable to cater for the individual's needs and accommodates the individual's height and weight;<br>(b) equipment or technologies used to ensure the safety of the individual in bed and to avoid injury to the individual and to aged care workers;<br>(c) pillows (including, if required, pressure cushions, tri pillows and wedge pillows);<br>(d) a bedside table, bedside locker or bedside chest of drawers, wardrobe space, draw screens (for shared rooms), a visitor chair (if required) and an over bed table (if required);<br>(e) a fixture or item of furniture where the individual can safely lock and store valuables, if this is not provided by the furniture items mentioned in paragraph (d);<br>(f) a chair, with arms, that meets the individual's care, safety and comfort needs, including, if required, a chair with particular features, such as an air, water or gel chair;<br>(g) a shower chair (if required), containers for personal laundry, and |



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**Services in the service type residential everyday living**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| | | waste collection containers or bins for bedrooms and bathrooms;<br>(h) bed linen, blankets or doonas, air or ripple mattresses (if required), absorbent or waterproof covers, sheeting and bed pads (if required), bath towels, hand towels and face washers;<br>(i) laundering of all products mentioned in paragraph (h) |
| 7 | Toiletry goods | The supply of the following goods (or substitutes if needed to meet the individual's medical needs, including specialist products for conditions such as dermatitis) but not including alternative items requested on the basis of the individual's personal preferences:<br>(a) facial cleanser (or alternatives such as facial wipes), shower gel or soap, shower caps, shampoo and conditioner;<br>(b) toothpaste, toothbrushes and mouthwash;<br>(c) hairbrush or comb, shaving cream and disposable razors;<br>(d) tissues and toilet paper;<br>(e) moisturiser and deodorant;<br>(f) cleaning products for dentures, hearing aids, glasses and artificial limbs (and their storage containers) |
| 8 | Personal laundry | The following:<br>(a) laundering (other than by a special cleaning process such as dry cleaning or hand washing) items that can be machine washed, using laundry detergents that meet the individual's medical needs, such as skin sensitivities;<br>(b) if requested, ironing of machine washed clothes (other than underwear and socks);<br>(c) a labelling system for the individual's clothing, but not including alternate labelling systems requested on the basis of the individual's personal preferences;<br>(d) return of personal laundry to the individual's clothing storage space |
| 9 | Meals and refreshments | The following:<br>(a) at least 3 meals served each day (including the option of dessert with either lunch or dinner) plus morning tea, afternoon tea and supper, of adequate variety, quality and quantity to meet the individual's nutritional and hydration needs;<br>(b) special diets where required to meet the individual's medical, cultural or religious needs, including but not limited to enteral feeding, nutritional supplements, texture modified meals and thickened fluids, diets to address food allergies and intolerances, and vegetarian, vegan, kosher and halal diets (but not for meeting the individual's social preferences on food source such as non-genetically modified and organic);<br>(c) reasonable flexibility in mealtimes, if requested, so the individual can exercise choice;<br>(d) a variety of non-alcoholic beverages available at all times (such as water, milk, fruit juice, tea and coffee); |



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| Services in the service type residential everyday living | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | | (e) eating and drinking utensils and eating aids if needed;<br>(f) snack foods of adequate variety, including fruit and options suitable for texture modified diets, available at all times in the residential care home |

## 8-150 Residential non-clinical care

Each service listed and described in the following table is in the service type residential non-clinical care.

| Services in the service type residential non-clinical care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| 1 | Care and services administration | Administration related to:<br>(a) the delivery of the other services listed and described in the other items of this table; and<br>(b) the delivery of the services in the service type residential clinical care |
| 2 | Personal care assistance | Personal assistance, including individual attention, individual supervision and physical assistance, with the following:<br>(a) bathing, showering, personal hygiene and grooming (other than hairdressing);<br>(b) dressing, undressing and using dressing aids;<br>(c) eating and drinking, and using utensils and eating aids (including actual feeding if necessary);<br>(d) cleaning of personal items (and their storage containers) needed for daily living, including dentures, hearing aids, glasses, mobility aids and artificial limbs |
| 3 | Communication | Assistance with daily communication, including the following:<br>(a) assistance to address difficulties arising from impaired hearing, sight or speech, cognitive impairment, or lack of common language (for example, visual aids such as cue cards, paper-based photo or alphabet spelling communication boards or books, photo based easy language written information, and menu and activity choice boards or learning of key phrases);<br>(b) fitting sensory communication aids and checking hearing aid batteries |
| 4 | Emotional support | The following:<br>(a) if the individual is experiencing social isolation, loneliness or emotional distress—ongoing emotional support to, and supervision of, the individual (including pastoral support);<br>(b) if the individual is new to the residential care home—assisting the individual to adjust to their new living environment; |



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| Services in the service type residential non-clinical care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | | (c) provision of culturally safe supports that have been determined in consultation with the individual and their supporters (if required) |
| 5 | Mobility and movement needs | The following (other than the provision of motorised wheelchairs, electric mobility scooters, customised aids, or mobility aids requested on the basis of the individual's personal preferences):<br>(a) assisting the individual with moving, walking and wheelchair use;<br>(b) assisting the individual with using devices and appliances designed to aid mobility;<br>(c) the fitting of artificial limbs and other personal mobility aids;<br>(d) supply and maintenance of crutches, quadruped walkers, walking frames, wheeled walkers, standing walkers, walking sticks, wheelchairs and tilt-in-space chairs;<br>(e) aids and equipment used by aged care workers to move the individual, including for individuals with bariatric needs;<br>taking into account:<br>(f) the individual's care, safety and comfort needs; and<br>(g) the individual's ability to use aids, appliances, devices and equipment; and<br>(h) the safety of other individuals and of aged care workers and visitors to the residential care home |
| 6 | Continence management | The following:<br>(a) assisting the individual to:<br>(i) maintain continence or manage incontinence; and<br>(ii) use aids and appliances designed to assist continence management;<br>(b) the supply of aids and appliances designed to assist continence management to meet the individual's needs, including the following:<br>(i) commode chairs, over-toilet chairs, bed-pans, uridomes, and catheter and urinary drainage appliances;<br>(ii) as many continence aids (such as disposable urinal covers, pants, pads, chair pads and enemas) as are needed to meet the individual's needs |
| 7 | Recreational and social activities | Tailored recreational programs and leisure activities (including communal recreational equipment and products) aimed at preventing loneliness and boredom, creating an enjoyable and interesting environment, and maintaining and improving the social interaction of the individual. These programs and activities must include the option of:<br>(a) at least one recreational or social activity each day that is not screen-based, television-based or meal-based; and<br>(b) regular outings into the community (but not including the cost of entry tickets, transport or purchased food and beverages associated with the outings) |



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## 8-155 Residential clinical care

Each service listed and described in the following table is in the service type residential clinical care.

**Services in the service type residential clinical care**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 1 | Care and services plan oversight | Ensuring that:<br>(a) the individual's care and services plan is carried out; and<br>(b) progress against the care and services plan goals is monitored<br>Note: For requirements for care and services plans, see paragraph 148(e) of the Act and Subdivisions A and D of Division 3 of Part 4 of Chapter 4 of this instrument. For Aged Care Quality Standards for care and services plans, see subsections 15-20(1) to (3) of this instrument. |
| 2 | Allied health, rehabilitation and therapeutic exercise therapy programs | Allied health, rehabilitation and therapeutic exercise therapy programs that are:<br>(a) designed by:<br>(i) appropriate registered health practitioners; or<br>(ii) appropriate allied health professionals; or<br>(iii) appropriate registered health practitioners and appropriate allied health professionals; and<br>(b) designed in consultation with the individual and their supporters (if required); and<br>(c) delivered in individual or group settings; and<br>(d) delivered by, or under the supervision, direction or appropriate delegation of:<br>(i) registered health practitioners; or<br>(ii) allied health professionals; or<br>(iii) registered health practitioners and allied health professionals; and<br>(e) aimed at maintaining and restoring the individual's physical, functional and communication abilities to perform daily tasks for themselves, including through:<br>(i) maintenance therapy that is designed to provide ongoing therapy services to prevent reasonably avoidable physical and functional decline and maintain and improve levels of independence in everyday living; and<br>(ii) if required, more focused restorative care therapy on a time-limited basis that is designed to allow the individual to reach a level of independence at which maintenance therapy will meet their needs;<br>but not including the following:<br>(f) intensive, long-term rehabilitation services required following (for example) serious illness or injury, surgery or trauma;<br>(g) allied health services and appointments made for or by the individual or their supporters that are in addition to those required to meet the individual's care needs under programs covered by paragraphs (a) to (e) |



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**Services in the service type residential clinical care**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| 3 | Medication management | The following:<br>(a) implementation of a safe and efficient system to manage prescribing, procuring, dispensing, supplying, packaging, storing and administering of both prescription and over-the-counter medicines;<br>(b) administration and monitoring of the effects of medication (via all routes (including injections)), including supervision and physical assistance with taking both prescription and over-the-counter medication, under the delegation and clinical supervision of a registered nurse or other appropriate registered health practitioner;<br>(c) reviewing the appropriateness of medications as needed under the delegation and clinical supervision of a registered nurse, or other appropriate registered health practitioner;<br>but not including the cost of prescription and over-the-counter medications |
| 4 | Nursing | Services provided by or under the supervision of a registered nurse, including but not limited to the following:<br>(a) initial comprehensive clinical assessment for input to the care and services plan for the individual, carried out:<br>(i) in line with the individual's needs, goals and preferences; and<br>(ii) by a registered nurse; and<br>(iii) if required, in consultation with other appropriate registered health practitioners, appropriate allied health professionals, or appropriate registered health practitioners and appropriate allied health professionals;<br>(b) ongoing regular comprehensive clinical assessment of the individual, including identifying and responding appropriately to change or deterioration in function, behaviour, condition or risk, carried out:<br>(i) in line with the individual's needs, goals and preferences; and<br>(ii) by a registered nurse, or an enrolled nurse under appropriate delegation by a registered nurse; and<br>(iii) if required, in consultation with other appropriate registered health practitioners, appropriate allied health professionals, or appropriate registered health practitioners and appropriate allied health professionals;<br>(c) all other nursing services, carried out:<br>(i) by a registered nurse, or an enrolled nurse under appropriate delegation by a registered nurse; and<br>(ii) if required, in consultation with other appropriate registered health practitioners, appropriate allied health professionals, or appropriate registered health practitioners and appropriate allied health professionals<br>Note 1: Examples of services include (but are not limited to) the following:<br>(a) ongoing monitoring and evaluation of the individual, and identification where care may need to be escalated or altered due to the changing health or needs of the individual;<br>(b) maintaining accurate, comprehensive, and up-to-date clinical documentation of the individual's care;<br>(c) assistance with, or provision of support for, personal hygiene, |



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**Services in the service type residential clinical care**

| Item | Column 1<br>Service | Column 2<br>Description |
|---|---|---|
| | | including oral health management and considerations for bariatric care needs;<br>(d) chronic disease management, including blood glucose monitoring;<br>(e) if the individual is living with cognitive decline—support and supervision of the individual;<br>(f) if the individual is living with mental health decline—support and supervision of the individual;<br>(g) establishment and supervision of a pain management plan, including the management and monitoring of chronic pain;<br>(h) medication management (as listed and described in item 3 of this table);<br>(i) insertion, maintenance, monitoring and removal of devices, including intravenous lines, naso-gastric tubes, catheters and negative pressure devices;<br>(j) if the individual has identified feeding and swallowing needs—support for the individual;<br>(k) skin assessment and the prevention and management of pressure injury wounds;<br>(l) establishment and supervision of a continence management plan;<br>(m) stoma care;<br>(n) wound management, including of complex and chronic wounds;<br>(o) provision of bandages, dressings, swabs, saline, drips, catheters, tubes and other medical items required as a part of nursing services;<br>(p) assistance with, and ongoing supervision of, breathing, including oxygen therapy, suctioning of airways and tracheostomy care;<br>(q) required support and observations for peritoneal dialysis treatment;<br>(r) assisting or supporting an individual to use appropriate healthcare technology in support of their care, including telehealth;<br>(s) risk management relating to infection prevention and control;<br>(t) advance care planning, palliative care and end-of-life care.<br>Note 2: For requirements for care and services plans, see paragraph 148(e) of the Act and Subdivisions A and D of Division 3 of Part 4 of Chapter 4 of this instrument. For Aged Care Quality Standards for care and services plans, see subsections 15-20(1) to (3) of this instrument. |
| 5 | Dementia and cognition management | If the individual has dementia or other cognitive impairments:<br>(a) development of an individual therapy and support program designed and carried out to:<br>(i) prevent or manage a particular condition or behaviour; and<br>(ii) enhance the individual's quality of life; and<br>(iii) enhance care for the individual; and<br>(b) ongoing support (including specific encouragement) to motivate or enable the individual to take part in general activities of the residential care home (if appropriate) |
| 6 | General access to medical and allied health services | The following:<br>(a) making arrangements for registered health practitioners to visit the individual for any necessary registered health practitioner appointments (but not the cost of the appointments or any gap payments charged for the appointments);<br>(b) making arrangements for the individual to attend any necessary |



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| Services in the service type residential clinical care | | |
|---|---|---|
| **Item** | **Column 1**<br>**Service** | **Column 2**<br>**Description** |
| | | registered health practitioner appointments (but not the cost of the appointments or any gap payments charged for the appointments, or transport or escort costs);<br>(c) if required, making arrangements for allied health professionals to visit the individual, or for the individual to visit an allied health professional, for any services or appointments mentioned in paragraph (f) of item 2 of this table (but not the cost of the appointments or any gap payments charged for the appointments, or transport or escort costs);<br>(d) if required, provision of audio-visual equipment for use with telehealth appointments;<br>(e) arranging for an ambulance in emergency situations |



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# Division 9—Other specified matters for residential care service types

## 8-160 Service types must be delivered in a residential care home

Each service type in the service group residential care must be delivered in a residential care home.

## 8-165 Other specified matters for residential care service types

Each service type set out in Division 8:

(a) is in the service group residential care; and

(b) can be delivered under the following specialist aged care programs:

(i) NATSIFACP;

(ii) MPSP;

(iii) TCP; and

(c) can be delivered under the provider registration category residential care.



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# Part 4—Other key concepts

## 11-5 Provider registration categories

For the purposes of paragraph 11(3)(b) of the Act, the following categories are prescribed:

(a) home and community services;
(b) assistive technology and home modifications;
(c) advisory and support services;
(d) personal and care support in the home or community;
(e) nursing and transition care;
(f) subscription trial.

Note: The category residential care is a provider registration category (see paragraph 11(3)(a) of the Act).

## 11A-5 Delivery of funded aged care services

*Late cancellations*

(1) For the purposes of subparagraph 11A(3)(a)(ii) of the Act, the period of 2 business days is prescribed.

*Prescribed circumstances—no-show circumstances*

(2) For the purposes of paragraph 11A(3)(b) of the Act, the prescribed circumstances are that no-show circumstances apply in relation to the service.

(3) ***No-show circumstances*** apply in relation to the service if:
(a) the service was to be delivered by the registered provider to the individual through the service group home support, assistive technology or home modifications; and
(b) the service was scheduled to be delivered by the registered provider to the individual on the day referred to in paragraph 11A(2)(b) of the Act at an agreed time and place; and
(c) the only reason that the service was not delivered was:
(i) the individual was required to be present (whether physically, by video link or other means) for the delivery of the service; and
(ii) the individual was not present as agreed for the delivery of the service on that day and at that time and place; and
(iii) the individual did not cancel the delivery of the service before the agreed time.

(4) ***No-show circumstances*** also apply in relation to the service if:
(a) the service was to be delivered by the registered provider to the individual through the service group home support, assistive technology or home modifications; and



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(b) the service was scheduled to be delivered by the registered provider to the individual on the day referred to in paragraph 11A(2)(b) of the Act at an agreed time and place; and

(c) the only reason that the service was not delivered was because the individual refused, or refused or failed to facilitate, access to the agreed place necessary for the delivery of the service.

*Circumstances in which subsection 11A(2) of the Act does not apply to a service*

(5) For the purposes of paragraph 11A(5)(a) of the Act, the circumstances in which subsection 11A(2) of the Act does not apply to a service is that the service was to have been delivered under a specialist aged care program.



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# Part 5—Aged Care Code of Conduct

## 14-1 Purpose of this Part

For the purposes of section 14 of the Act, this Part prescribes requirements relating to the conduct of the following:

(a) registered providers;
(b) aged care workers of registered providers;
(c) responsible persons of registered providers.

Note: These requirements are the ***Aged Care Code of Conduct***: see section 7 of the Act.

## 14-5 Requirements

When delivering funded aged care services to individuals, I must:

(a) act with respect for individuals' rights to freedom of expression, self-determination and decision-making in accordance with applicable laws and conventions; and
(b) act in a way that treats individuals with dignity and respect, and values their diversity; and
(c) act with respect for the privacy of individuals; and
(d) deliver funded aged care services in a safe and competent manner, with care and skill; and
(e) act with integrity, honesty and transparency; and
(f) promptly take steps to raise and act on concerns about matters that may impact the quality and safety of funded aged care services; and
(g) deliver funded aged care services free from:
    (i) all forms of violence, discrimination, exploitation, neglect and abuse; and
    (ii) sexual misconduct; and
(h) take all reasonable steps to prevent and respond to:
    (i) all forms of violence, discrimination, exploitation, neglect and abuse; and
    (ii) sexual misconduct.

## 14-10 Application of requirements

(1) The requirements apply to the following in accordance with this section:
    (a) registered providers;
    (b) aged care workers of registered providers;
    (c) responsible persons of registered providers.

(2) The requirements apply as if the reference to "I" in section 14-5 was a reference to all of the following:
    (a) a registered provider;



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(b) an aged care worker of a registered provider;

(c) a responsible person of a registered provider.

(3) For the purposes of the application of requirements to the delivery of funded aged care services to individuals by a responsible person of a registered provider, the requirements apply to the performance by the responsible person of the responsibilities and functions of the responsible person.

Note: Provisions relevant to compliance with the Aged Care Code of Conduct include the following:

(a) section 142 of the Act (civil penalties for non-compliance with conditions of registration of registered providers);

(b) section 145 of the Act (condition of registration of registered providers);

(c) section 173 of the Act (civil penalties for aged care workers);

(d) section 174 of the Act (civil penalties for responsible persons);

(e) subparagraph 498(2)(d)(i) of the Act (grounds for banning orders against individuals who are or were aged care workers or responsible persons of a registered provider).



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# Part 6—Aged Care Quality Standards

## Division 1—Purpose and application

### 15-1 Purpose of this Division

For the purposes of subsection 15(1) of the Act, this Division:

(a) prescribes standards relating to the quality of funded aged care services delivered by a registered provider; and

(b) provides for the application of the standards to registered providers in specified provider registration categories.

### 15-5 Application of standards

*Standards 1 to 4*

(1) Standards 1 to 4 apply to a registered provider in any of the following provider registration categories:

(a) personal and care support in the home or community;

(b) nursing and transition care;

(c) residential care.

(1A) Standards 1 to 4 apply to a registered provider in the provider registration category subscription trial if the provider is also in a provider registration category referred to in paragraph (1)(a) or (b).

*Standard 5*

(2) Standard 5 applies to a registered provider in either of the following provider registration categories:

(a) nursing and transition care;

(b) residential care.

(2A) Standard 5 applies to a registered provider in the provider registration category subscription trial if the provider is also in the provider registration category referred to in paragraph (2)(a).

(3) Subsections 15-30(1) and (2) (Standard 5—*Outcome 5.1—Clinical governance*) apply to a registered provider in the provider registration category personal and care support in the home or community that delivers, or intends to deliver, either of the following service types:

(a) care management;

(b) restorative care management.

*Standards 6 and 7*

(4) Standards 6 and 7 apply to a registered provider in the provider registration category residential care.



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# Division 2—The standards

## 15-10 Standard 1—The individual

*Outcome 1.1—Person-centred care*

(1) The registered provider demonstrates that the provider understands that the safety, health, wellbeing and quality of life of individuals is the primary consideration in the delivery of funded aged care services.

Note: See also subsection 144(2) of the Act (rights and principles).

(2) The provider demonstrates that the provider understands and values individuals, including their identity, culture, ability, diversity, beliefs and life experiences.

Note: See also section 23 of the Act (Statement of Rights) and section 144 of the Act (rights and principles).

(3) The provider demonstrates that the provider develops funded aged care services with, and tailored to, individuals, taking into account their needs, goals and preferences.

*Outcome 1.2—Dignity, respect and privacy*

(4) The provider must deliver funded aged care services to individuals in a way that:
  (a) is free from all forms of discrimination, abuse and neglect; and
  (b) treats individuals with dignity and respect; and
  (c) respects the personal privacy of individuals.

Note: See section 23 of the Act (Statement of Rights) and section 144 of the Act (rights and principles).

(5) The provider:
  (a) demonstrates that the provider understands the rights of individuals under the Statement of Rights; and
  (b) must have practices in place to ensure that the provider acts compatibly with the Statement of Rights, in accordance with subsection 24(2) of the Act (acting compatibly with the Statement of Rights).

Note: See also section 144 of the Act (rights and principles).

*Outcome 1.3—Choice, independence and quality of life*

(6) The provider must support individuals to exercise choice and make decisions about their funded aged care services, and provide them with support to exercise choice and make decisions when they want or need it.

(7) The provider must provide individuals with timely, accurate, tailored and sufficient information about their funded aged care services, in a way they understand.

Note: See also section 29 of the Act (giving information and documents to supporters).



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(8) The provider must support individuals to exercise dignity of risk to achieve their goals and maintain independence and quality of life.

*Outcome 1.4—Transparency and agreements*

(9) Before entering into any agreements with individuals about the delivery of funded aged care services, the provider must provide individuals with:
  (a) the opportunity to exercise autonomy; and
  (b) the time they need to consider the agreement; and
  (c) an opportunity to seek advice.

Note: Refer to sections 148-65 and 148-70 of this instrument in relation to the requirements for entering into a service agreement, details and contents of such an agreement.

(10) The provider must support individuals to understand and make informed decisions about their agreements, fees and invoices.

## 15-15 Standard 2—The organisation

*Outcome 2.1—Partnering with individuals*

(1) The registered provider must engage in meaningful and active partnerships with individuals to inform organisational priorities and continuous improvement.

Note: See also section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).

*Outcome 2.2a—Quality, safety and inclusion culture to support aged care workers to deliver quality care*

(2) The governing body must lead a culture of quality, safety and inclusion that supports aged care workers to provide quality funded aged care services by:
  (a) focussing on continuous improvement; and
  (b) embracing diversity; and
  (c) prioritising the safety, health and wellbeing of aged care workers.

Note: See also section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).

*Outcome 2.2b—Quality, safety and inclusion culture to support individuals*

(3) The governing body must lead a culture of quality, safety and inclusion that supports individuals accessing quality funded aged care services by:
  (a) focussing on continuous improvement; and
  (b) embracing diversity; and
  (c) prioritising the safety, health and wellbeing of individuals.

Note: See also section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).



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*Outcome 2.3—Accountability, quality system and policies and procedures*

(4) The governing body is accountable for the delivery of quality funded aged care services and must maintain oversight of all aspects of the provider's operations.

(5) The provider must use a quality system to enable and drive continuous improvement of the provider's delivery of funded aged care services.

Note: See also section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).

(6) The provider must:
(a) maintain current policies and procedures that guide the way aged care workers undertake their roles; and
(b) require aged care workers to follow the policies and procedures.

*Outcome 2.4—Risk management*

(7) The provider must use a risk management system to identify, manage and continuously review risks to individuals, aged care workers and the provider's operations.

Note: See also section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).

*Outcome 2.5—Incident management*

(8) The provider must use an incident management system to:
(a) safeguard individuals; and
(b) acknowledge, respond to, effectively manage and learn from incidents.

Note: See also section 164 of the Act and Division 1 of Part 10 of Chapter 4 of this instrument (conditions of registration relating to incident management).

*Outcome 2.6a—Complaints and feedback management—for aged care workers*

(9) The provider must encourage and support aged care workers to make complaints and give feedback about the provider's delivery of funded aged care services without reprisal.

Note: See also section 165 of the Act and Division 2 of Part 10 of Chapter 4 of this instrument (conditions of registration relating to complaints, feedback and whistleblowers).

*Outcome 2.6b—Complaints and feedback management—for individuals*

(10) The provider must encourage and support individuals and others to make complaints and give feedback about the provider's delivery of funded aged care services without reprisal.

Note: See also section 165 of the Act and Division 2 of Part 10 of Chapter 4 of this instrument (conditions of registration relating to complaints, feedback and whistleblowers).



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*Outcomes 2.6a and 2.6b—Complaints and feedback management*

(11) The provider must:
  (a) acknowledge and transparently manage all complaints and feedback; and
  (b) use complaints and feedback to contribute to the continuous improvement of funded aged care services.

Note: See also section 165 of the Act and Division 2 of Part 10 of Chapter 4 of this instrument (conditions of registration relating to complaints, feedback and whistleblowers), and section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).

*Outcome 2.7—Information management*

(12) The provider must ensure that information recorded about an individual:
  (a) is accurate and current; and
  (b) is able to be accessed and understood by the individual, supporters of the individual, aged care workers, registered health practitioners, allied health professionals, allied health assistants and others involved in the individual's care.

Note: See also section 155 of the Act (provision of information to individuals) which sets out conditions applicable to registered providers to allow access to records and information to individuals and explain those records and information to those individuals. Subsection 156(3) of the Act in specified circumstances allows independent aged care advocates access to records and information held by a registered provider about an individual.

(13) The provider must ensure that the information of individuals is kept confidential and is managed appropriately, in line with their informed consent.

Note: See also section 154 of the Act (personal information and record keeping) which requires registered providers as a condition of their registration to keep and retain kinds of records in accordance with requirements set out in this instrument, and section 168 of the Act (protection of personal information) which requires registered providers to ensure the protection of personal information.

*Outcome 2.8—Workforce planning*

(14) The provider must demonstrate that the provider understands and manages their workforce needs and plans for the future.

*Outcome 2.9—Human resource management*

(15) The provider must deliver funded aged care services to individuals by aged care workers who:
  (a) are skilled and competent in their roles; and
  (b) hold relevant qualifications for their roles; and
  (c) have expertise and experience relevant to delivering quality funded aged care services.

Note: See also section 152 of the Act (workforce and aged care worker requirements) which requires registered providers as a condition of their registration to comply with workforce screening and other requirements in relation to the employment of aged care workers.



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(16) The provider must provide aged care workers with training and supervision to enable them to effectively perform their roles.

*Outcome 2.10—Emergency and disaster management*

(17) The provider must demonstrate that emergency and disaster management planning considers and manages risks to the health, safety and wellbeing of individuals and aged care workers.

## 15-20 Standard 3—The care and services

*Outcome 3.1—Assessment and planning*

(1) The registered provider must actively engage with:
  (a) individuals to whom the provider delivers funded aged care services; and
  (b) supporters of individuals (if any); and
  (c) any other persons involved in the care of individuals;
in developing and reviewing the individual's care and services plans through ongoing communication.

(2) Care and services plans must:
  (a) describe the current care needs, goals and preferences of individuals; and
  (b) include strategies for risk management and preventative care.

(3) The provider must ensure that care and services plans are regularly reviewed and are used by aged care workers to guide the delivery of funded aged care services.

Note: See also section 148-80 of this instrument (requirements for care and services plans-general) which provides details in relation to the development of care and services plans for an individual.

*Outcome 3.2—Delivery of funded aged care services*

(4) The provider must ensure that individuals receive quality funded aged care services that:
  (a) meet their care needs, goals and preferences; and
  (b) optimise their quality of life, reablement and maintenance of function.

(5) The provider must ensure that funded aged care services are delivered in a way that is culturally safe and culturally appropriate for individuals with specific needs and diverse backgrounds.

*Outcome 3.3—Communicating for safety and quality*

(6) The provider must ensure that critical information relevant to the delivery of funded aged care services to individuals is communicated effectively:
  (a) to the individuals; and
  (b) between aged care workers delivering the services; and
  (c) to supporters of the individuals and other persons supporting the individuals; and



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(d) to registered health practitioners, allied health professionals, allied health assistants and others involved with the individual's care.

(7) The provider must ensure that risks to individuals, and changes and deterioration in the condition of individuals, are escalated and communicated as appropriate.

*Outcome 3.4—Planning and coordination of funded aged care services*

(8) The provider must ensure that individuals receive funded aged care services that are planned and coordinated, including where multiple health providers and registered providers, supporters of individuals and other persons supporting individuals are involved.

## 15-25 Standard 4—The environment

*Outcome 4.1a—Environment—services delivered in the individual's home*

(1) When delivering funded aged care services to individuals in their homes, the registered provider must support the individuals to mitigate environmental risks relevant to the services.

*Outcome 4.1b—Environment—services delivered other than in the individual's home*

(2) Where the provider delivers funded aged care services to individuals other than in their homes, the provider must ensure that individuals are able to access funded aged care services in a clean, safe and comfortable environment that optimises their sense of belonging, interaction and function.

*Outcomes 4.1a and 4.1b—Equipment*

(3) Where the provider uses equipment in the delivery of any funded aged care services to individuals, or provides equipment to individuals, the equipment must be safe and must meet the needs of the individuals.

*Outcome 4.2—Infection prevention and control*

(4) The provider must have an appropriate infection prevention and control system.

(5) The provider must ensure that aged care workers use hygienic practices and take appropriate infection prevention and control precautions when delivering funded aged care services.

## 15-30 Standard 5—Clinical care

*Outcome 5.1—Clinical governance*

(1) The governing body must ensure:

(a) that the governing body continuously improves the safety and quality of clinical care services delivered to individuals; and



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(b) that the provider delivers safe and quality clinical care services to individuals.

Note: See also section 147 of the Act and Division 2 of Part 4 of Chapter 4 of this instrument (conditions of registration relating to continuous improvement).

(2) The registered provider must integrate clinical governance into corporate governance to actively manage and improve the safety and quality of clinical care services delivered to individuals.

*Outcome 5.2—Preventing and controlling infections in delivering clinical care services*

(3) The provider must ensure that individuals, aged care workers, registered health practitioners and others are encouraged and supported to use antimicrobials appropriately to reduce risks of increasing resistance.

(4) The provider must ensure that infection risks are minimised and, if they occur, are controlled effectively.

*Outcome 5.3—Safe and quality use of medicines*

(5) The provider must encourage and support individuals, aged care workers, registered health practitioners and allied health professionals to use medicines in a way that maximises benefits and minimises the risks of harm.

(6) The provider must ensure that:
(a) before administering medicine to an individual, the medicine has been prescribed for the individual; and
(b) medicines are appropriately and safely administered, monitored and reviewed by registered health practitioners, considering the clinical needs and informed decisions of the individual.

(7) The provider must ensure that medicine-related adverse events are monitored and reported, and are used to inform safety and quality improvement.

*Outcome 5.4—Comprehensive care*

(8) The provider must ensure that individuals receive comprehensive, safe and quality clinical care services that are evidence-based, person-centred and delivered by registered health practitioners, allied health professionals, allied health assistants or nursing assistants.

(9) Clinical care delivered by the provider must encompass clinical assessment, prevention, planning, treatment, management and review to minimise harm and optimise quality of life, reablement and maintenance of function.

(10) The provider must have systems and processes that support coordinated and multidisciplinary clinical care services:
(a) that are delivered to individuals, in partnership with individuals, supporters of individuals and other persons supporting individuals; and
(b) that are aligned with the individuals' needs, goals and preferences.



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(11) The provider must support early identification of, and response to, changing clinical needs.

*Outcome 5.5—Safety of clinical care services*

(12) The provider must identify, monitor and manage high impact and high prevalence risks in the delivery of clinical care services:
  (a) to ensure the delivery of safe and quality clinical care services; and
  (b) to reduce the risk of harm to individuals.

*Outcome 5.6—Cognitive impairment*

(13) The provider must ensure that individuals who experience cognitive impairment (whether acute, chronic or transitory) receive comprehensive funded aged care services that:
  (a) optimise clinical outcomes; and
  (b) are aligned with the individuals' needs, goals and preferences.

(14) The provider must identify situations and events that may lead to changes in behaviours.

*Outcome 5.7—Palliative care and end-of-life care*

(15) The provider must recognise and address the needs, goals and preferences of individuals for palliative care and end-of-life care, and must preserve the dignity of individuals in those circumstances.

(16) The provider must ensure that the pain and symptoms of individuals are actively managed, with access to specialist palliative and end-of-life care when required.

(17) The provider must ensure that supporters of individuals and other persons supporting individuals are informed and supported, including during the last days of life.

Note: See also section 29 of the Act (giving information and documents to supporters).

## 15-35 Standard 6—Food and nutrition

*Outcome 6.1—Partnering with individuals on food and drinks*

(1) The registered provider must partner with individuals to deliver a quality food and drinks service that includes appetising and varied food and drinks and an enjoyable dining experience.

Note: See also item 9 of the table in section 8-145 of this instrument (residential everyday living—meals and refreshments).

*Outcome 6.2—Assessment of nutritional needs and preferences*

(2) The provider must demonstrate that the provider understands the specific nutritional needs of individuals and assesses the current needs, abilities and preferences of individuals in relation to what and how they eat and drink.



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*Outcome 6.3—Provision of food and drinks*

(3) The provider must provide individuals with:
  (a) food and drinks that meet their nutritional needs and are appetising and flavoursome; and
  (b) variation and choice about what they eat and drink; and
  (c) choice about how much they eat and drink.

*Outcome 6.4—Dining experience*

(4) The provider must support individuals to eat and drink.

(5) The provider must ensure that the dining experience meets the needs and preferences of individuals to support social engagement, function and quality of life.

## 15-40 Standard 7—The residential community

*Outcome 7.1—Daily living*

(1) The registered provider must ensure that individuals receive funded aged care services that optimise their quality of life, promote use of their skills and strengths and enable them to do the things they want to do.

(2) The provider must ensure that individuals feel safe in their residential care home.

*Outcome 7.2—Transitions*

(3) The provider must ensure that individuals experience a well-coordinated transition, whether planned or unplanned, to or from a provider.

Note: See also section 149 of the Act (starting and ceasing the provision of funded aged care services and continuity of those services).

(4) The provider must set out clear responsibility and accountability for the delivery of funded aged care services to individuals between aged care workers, registered health practitioners, allied health professionals and allied health assistants, and across organisations.



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# Part 7—Reportable incidents and restrictive practices

## Division 1—Reportable incidents

### 16-5 Defining and clarifying expressions relating to reportable incidents

(1) For the purposes of subsection 16(2) of the Act, this section prescribes definitions or clarifications of expressions used in paragraphs 16(1)(a), (b), (c), (d), (e), (f) and (h) of the Act (which deal with incidents that have occurred, are alleged to have occurred, or are suspected of having occurred, in connection with the delivery of funded aged care services to an individual by a registered provider).

*Unreasonable use of force*

(2) In paragraph 16(1)(a) of the Act, the expression "unreasonable use of force against the individual" includes conduct ranging from a deliberate and violent physical attack to use of unwarranted physical force.

(3) To avoid doubt, that expression does not cover gently touching the individual:
  (a) for the purposes of providing funded aged care services; or
  (b) to attract the individual's attention; or
  (c) to guide the individual; or
  (d) to comfort the individual when the individual is distressed.

*Unlawful sexual contact, or inappropriate sexual conduct*

(4) In paragraph 16(1)(b) of the Act, the expression "unlawful sexual contact, or inappropriate sexual conduct, inflicted on the individual" includes the following:
  (a) if the contact or conduct is inflicted by a person who is an aged care worker of the registered provider—the following:
    (i) any conduct or contact of a sexual nature inflicted on the individual, including (without limitation) sexual assault, an act of indecency and the sharing of an intimate image of the individual;
    (ii) any touching of the individual's genital area, anal area or breast in circumstances where this is not necessary to deliver funded aged care services to the individual;
  (b) any non-consensual contact or conduct of a sexual nature, including (without limitation) sexual assault, an act of indecency and the sharing of an intimate image of the individual;
  (c) engaging in conduct relating to the individual with the intention of making it easier to procure the individual to engage in sexual contact or conduct.

(5) However, that expression does not include consensual contact or conduct of a sexual nature between the individual and a person who is not an aged care worker of the registered provider, including another individual to whom the registered provider delivers funded aged care services.



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*Psychological or emotional abuse*

(6) In paragraph 16(1)(c) of the Act the expression "psychological or emotional abuse of the individual" includes conduct that:
  (a) has caused the individual psychological or emotional distress; or
  (b) could reasonably have been expected to have caused an individual psychological or emotional distress.

(7) Conduct covered by subsection (6) includes (without limitation) the following:
  (a) taunting, bullying, harassment or intimidation;
  (b) threats of maltreatment;
  (c) humiliation;
  (d) unreasonable refusal to interact with the individual or acknowledge the individual's presence;
  (e) unreasonable restriction of the individual's ability to engage socially or otherwise interact with people;
  (f) repetitive conduct or contact which does not constitute unreasonable use of force but the repetition of which:
    (i) has caused the individual psychological or emotional distress; or
    (ii) could reasonably have been expected to have caused an individual psychological or emotional distress.

*Unexpected death*

(8) In paragraph 16(1)(d) of the Act the expression "unexpected death of the individual" includes death in circumstances where:
  (a) the individual was accessing funded aged care services in an approved residential care home of a registered provider and reasonable steps were not taken by the registered provider to prevent the death; or
  (b) the death was a result of:
    (i) funded aged care services delivered by the registered provider; or
    (ii) a failure of the registered provider to deliver funded aged care services.

*Stealing or financial coercion*

(9) In paragraph 16(1)(e) of the Act the expression "stealing from, or financial coercion of, the individual by an aged care worker of the provider" includes the following:
  (a) stealing from the individual by an aged care worker of the registered provider;
  (b) conduct by an aged care worker of the registered provider that:
    (i) is coercive or deceptive in relation to the individual's financial affairs; or
    (ii) unreasonably controls the individual's financial affairs.



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*Neglect*

(10) In paragraph 16(1)(f) of the Act the expression "neglect of the individual" means circumstances in which a registered provider, aged care worker, or responsible person of the registered provider:

(a) has delivered funded aged care services to the individual that exposes the individual to the risk of serious injury or illness; or

(b) has caused or contributed (including through reckless or intentional behaviour) to:

(i) a significant failure to deliver funded aged care services to the individual; or

(ii) a systematic pattern of conduct; or

(c) has delivered a grossly inadequate funded aged care services to the individual; or

(d) has been reckless or intentionally negligent in delivering funded aged care services to the individual.

*Unexplained absence*

(11) In paragraph 16(1)(h) of the Act the expression "unexplained absence of the individual in the course of the delivery of funded aged care services to the individual" means:

(a) for an individual accessing funded aged care services in an approved residential care home—an absence of the individual from the home in circumstances where there are reasonable grounds to report the absence to police; or

(b) for an individual accessing funded aged care services in a home or community setting—an absence of the individual from the setting during the delivery of funded aged care services to the individual in circumstances where there are reasonable grounds to report the absence to police.

## 16-10 Acts, omissions and events that are not reportable incidents—general

(1) For the purposes of paragraph 16(3)(b) of the Act, an act, omission or event covered by one of the following subsections is not a ***reportable incident***.

*Approved residential care homes—incidents resulting from refusal by individuals of delivery of services*

(2) This subsection covers an incident if the incident results from an individual accessing funded aged care services in an approved residential care home deciding to refuse the delivery of funded aged care services.

*Home or community settings—incidents resulting from choices made by individuals about services to be delivered*

(3) This subsection covers an incident if:



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(a) the incident occurred, is alleged to have occurred, or is suspected of having occurred, in connection with the delivery of funded aged care services in a home or community setting to an individual by a registered provider; and
(b) apart from this subsection, the incident would be a reportable incident under paragraph 16(1)(f) of the Act (neglect of the individual), but would not otherwise be a reportable incident; and
(c) the incident results from a choice made by the individual about the funded aged care services the registered provider is to deliver to the individual, or how the services are to be delivered by the provider; and
(d) before the incident occurred, is alleged to have occurred, or is suspected of having occurred, the individual had communicated the individual's choice to the provider, and the provider had recorded the individual's choice in writing.

## 16-15 Acts, omissions and events that are not reportable incidents—use of restrictive practices in a home or community setting

For the purposes of paragraph 16(3)(b) of the Act, the use of a restrictive practice in relation to an individual is not a ***reportable incident*** if:

(a) the restrictive practice is used in connection with the delivery of funded aged care services to the individual in a home or community setting; and
(b) before the restrictive practice is used, the following matters were set out in the care and services plan for the individual:
  (i) the circumstances in which the restrictive practice may be used in relation to the individual, including the individual's behaviours of concern that are relevant to the need for the use;
  (ii) the manner in which the restrictive practice is to be used, including its duration, frequency and intended outcome; and
(c) the restrictive practice is used:
  (i) in the circumstances set out in the plan; and
  (ii) in the manner set out in the plan; and
  (iii) in accordance with any other provisions of the plan that relate to the use; and
(d) details about the use of the restrictive practice are documented as soon as practicable after the restrictive practice is used.



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# Division 2—Restrictive practices

## 17-5 Practices and interventions that are restrictive practices in relation to individuals

(1) For the purposes of subsection 17(2) of the Act, each of the following is a restrictive practice in relation to an individual:
  (a) chemical restraint;
  (b) environmental restraint;
  (c) mechanical restraint;
  (d) physical restraint;
  (e) seclusion.

(2) ***Chemical restraint*** is a practice or intervention that is, or that involves, the use of medication or a chemical substance for the primary purpose of influencing an individual's behaviour, but does not include the use of medication prescribed for:
  (a) the treatment of, or to enable treatment of, the individual for:
    (i) a diagnosed mental disorder; or
    (ii) a physical illness; or
    (iii) a physical condition; or
  (b) end-of-life care for the individual.

(3) ***Environmental restraint*** is a practice or intervention that restricts, or that involves restricting, an individual's free access to all parts of the individual's environment (including items and activities) for the primary purpose of influencing the individual's behaviour.

(4) ***Mechanical restraint*** is a practice or intervention that is, or that involves, the use of a device to prevent, restrict or subdue an individual's movement for the primary purpose of influencing the individual's behaviour, but does not include the use of a device for therapeutic or non-behavioural purposes in relation to the individual.

(5) ***Physical restraint*** is a practice or intervention that:
  (a) is or involves the use of physical force to prevent, restrict or subdue movement of an individual's body, or part of an individual's body, for the primary purpose of influencing the individual's behaviour; but
  (b) does not include the use of a hands-on technique in a reflexive way to guide or redirect the individual away from potential harm or injury if it is consistent with what could reasonably be considered to be the exercise of care towards the individual.

(6) ***Seclusion*** is a practice or intervention that is, or that involves, the sole confinement of an individual in a room or a physical space at any hour of the day or night where:
  (a) voluntary exit is prevented or not facilitated; or
  (b) it is implied that voluntary exit is not permitted;



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for the primary purpose of influencing the individual's behaviour.



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# Part 8—Supporters

## Division 1—Actions and duties of supporters

### 28-5 Role of guardians etc—prescribed classes of persons

For the purposes of paragraph 28(2)(d) of the Act, a prescribed class of persons is restrictive practices substitute decision-makers.



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## Division 2—Registration of supporters

### 37-5 Registration of supporters—matters the System Governor must take into consideration

(1) For the purposes of paragraph 37(6)(c) of the Act, this section prescribes other matters the System Governor must take into consideration in deciding whether to register a person under subsection 37(1) of the Act as a supporter of an individual.

(2) Another matter is, for a person who is a person covered by subsection 28(2) of the Act, the nature of the person's decision-making authority as such a person (including the extent to which it extends to making decisions relevant to the delivery of funded aged care services to the individual).



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# Chapter 2—Entry to the Commonwealth aged care system

# Part 1—Introduction

### 55-5 Simplified outline of this Chapter

This Chapter provides for matters relating to entry to the Commonwealth aged care system under Chapter 2 of the Act.

Part 2 of this Chapter relates to eligibility of individuals for entry to the Commonwealth aged care system, and deals with:

(a) applying for access to funded aged care services; and
(b) the assessment tool for aged care needs assessments and circumstances in which reassessments are required;
(c) approval of access to funded aged care services.

Part 3 of this Chapter relates to classification, and deals with:

(a) requirements for classification assessments; and
(b) the period for making classification decisions and the period of effect of classification levels for classification types for service groups for individuals; and
(c) prescribing classification levels for classification types for service groups, including criteria used to establish certain classification levels and methods or procedures for deciding certain classification levels.

Part 4 of this Chapter relates to prioritisation, and deals with:

(a) priority category decisions; and
(b) priority categories for classification types for service groups and eligibility criteria for those priority categories; and
(c) circumstances in which urgency ratings will apply to individuals in relation to the classification type ongoing or short-term for the service group residential care.

Part 5 of this Chapter relates to place allocation, and deals with:

(a) allocation of places to individuals, including methods that the System Governor must follow in deciding the order of allocation of places to individuals for certain classification types and service groups; and
(b) allocation of places to registered providers for certain specialist aged care programs.



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# Part 2—Eligibility for entry

## Division 1—Applying for access to funded aged care services

### 56-5 Classes of persons who may apply on behalf of individuals

For the purposes of subsection 56(1) of the Act, the following classes of persons are prescribed in relation to an individual:

(a) supporters of the individual;
(b) registered health practitioners;
(c) allied health professionals;
(d) aged care workers of registered providers;
(e) registered providers;
(f) social workers (however described);
(g) individuals employed or engaged as care finders under the care finder program funded by the Department;
(h) individuals employed or engaged as elder care supporters under the elder care support program funded by the Department;
(i) family members, friends, advocates and carers of the individual;
(j) independent aged care advocates.

### 57-5 Period for deciding whether to make eligibility determinations

For the purposes of subsection 57(2) of the Act, the period for making a decision under subsection 57(1) of the Act on an application for access to funded aged care services is 28 days after receiving the application.

### 58-5 Information to be provided for eligibility determinations

For the purposes of paragraph 58(b) of the Act, each of the following is prescribed as a kind of information relating to an individual's care needs:

(a) a declaration by the individual, made orally or in writing, that the individual has care needs;
(b) a written statement by a person in a class of persons referred to in section 56-5 of this instrument that sets out the individual's care needs;
(c) written medical records of the individual's care needs.

Note: For the definition of ***care needs***, see section 7 of the Act.



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# Division 2—Aged care needs assessments and reassessments

## Subdivision A—Aged care needs assessments

### 62-5 Assessment tool

For the purposes of subsection 62(1) of the Act, the Integrated Assessment Tool is prescribed.

## Subdivision B—Aged care needs reassessments

### 64-5 Significant changes in circumstances

For the purposes of subparagraph 64(1)(c)(i) of the Act, each of the following is prescribed as a significant change in circumstances in relation to an individual who is accessing funded aged care services other than through the service group residential care:

(a) a carer for the individual has permanently ceased to provide some or all care to the individual;

(b) the individual has experienced an event, or a decline in their condition, that is likely to mean that the individual will require:

(i) more frequent access to a funded aged care service that is covered by the individual's access approval and that the individual has been accessing; or

(ii) access to a funded aged care service that is covered by the individual's access approval but that the individual has not been accessing; or

(iii) access to a funded aged care service that is not covered by the individual's access approval.

### 64-10 Other circumstances—individuals accessing funded aged care services other than through the service group residential care—general

For the purposes of subparagraph 64(1)(c)(ii) of the Act, each of the following is prescribed as other circumstances in relation to an individual who is accessing funded aged care services other than through the service group residential care:

(a) a carer for the individual has temporarily ceased to provide some or all care to the individual;

(b) the individual has experienced an event, or a decline in their condition, that is likely to mean that the individual will require access to a funded aged care service that is not covered by the individual's access approval;

(c) all of the following apply:

(i) the individual has an access approval in effect for the classification type ongoing or short-term for the service group home support;

(ii) on the date of the application for the reassessment, the individual has a prognosis of a life expectancy of 3 months or less;



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(iii) on the date of the application for the reassessment, the individual has an AKPS score of 40 or less;

(iv) the individual has not previously had the classification level SAH end-of-life pathway in effect;

(v) a notice in the approved form containing evidence of the matters referred to in subparagraphs (ii) and (iii) has been given to the System Governor;

(d) both of the following apply:

(i) the individual has an access approval in effect for the classification type short-term for the service group home support;

(ii) the individual has, or has previously had, the classification level SAH end-of-life pathway in effect;

(e) both of the following apply:

(i) the classification level SAH end-of-life pathway for the classification type short-term for the service group home support has been established for the individual by a classification decision;

(ii) the individual will require access to funded aged care services in the service group assistive technology;

(f) both of the following apply:

(i) the classification level SAH restorative care pathway for the classification type short-term for the service group home support has been established for the individual by a classification decision;

(ii) the individual will require access to funded aged care services in the service group assistive technology or home modifications;

(g) both of the following apply:

(i) the individual has an access approval in effect for the classification type ongoing for the service group home support;

(ii) repairs or maintenance are required for an included AT-HM item or a conditionally included AT-HM item that the individual owns or has been loaned or rented under a Commonwealth aged care program (including under the old Act);

(h) both of the following apply:

(i) the individual has an access approval in effect for the classification type ongoing for the service group home support;

(ii) the individual has a condition referred to in subsection 211-10(2) of this instrument;

(i) both of the following apply:

(i) the individual has an access approval in effect for the classification type ongoing for the service group home support;

(ii) the individual has an assistance dog.



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### 64-11 Other circumstances—individuals accessing funded aged care services other than through the service group residential care—individuals wishing to access subscription services

For the purposes of subparagraph 64(1)(c)(ii) of the Act, the following are prescribed as other circumstances in relation to an individual who is accessing funded aged care services other than through the service group residential care:

(a) the individual has an access approval and a classification level in effect for the classification type ongoing for the service group home support;

(b) the individual's access approval covers (as applicable) a non-subscription service type or a non-subscription service for which there is a corresponding subscription service type or subscription service;

(c) funded aged care services are being delivered to the individual through the service group home support by a registered provider that is, or has been invited by the System Governor to apply to be, registered in the provider registration category subscription trial;

(d) the provider has explained to the individual the parameters of the access provided by (as applicable) funded aged care services in the corresponding subscription service type or the corresponding subscription service, as delivered or proposed to be delivered by the provider;

(e) the individual's application for reassessment referred to in paragraph 64(1)(b) of the Act states that the individual:

(i) wishes to access (as applicable) funded aged care services in the corresponding subscription service type or the corresponding subscription service; and

(ii) consents to the collection of the individual's personal information for the purpose of evaluating the delivery of (as applicable) funded aged care services in the corresponding subscription service type or the subscription service.

### 64-15 Information for reassessments in other circumstances

For the purposes of paragraph 64(2)(b) of the Act, each of the following is prescribed as a kind of information in relation to an individual:

(a) information in the application for the aged care needs reassessment;

(b) information provided, orally or in writing, by any of the following about the individual's need for funded aged care services:

(i) a registered health practitioner;

(ii) an allied health professional;

(iii) a registered provider delivering the funded aged care service home support care management or home support restorative care management to the individual;

(c) information provided, orally or in writing, by an approved needs assessor following a review by the assessor of the report of the most recent aged care needs assessment for the individual;

(d) information in the most recent application for a classification reassessment for the individual.



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# Division 3—Approval of access to funded aged care services

## Subdivision A—General

### 65-5 Approval of services in service types for individuals

For the purposes of subparagraph 65(2)(b)(ii) of the Act, the following service types are prescribed:

(a) allied health and therapy;
(ab) subscription allied health and therapy;
(b) therapeutic services for independent living;
(c) subscription therapeutic services for independent living.

### 65-10 Eligibility requirements—service group home support

For the purposes of paragraph 65(3)(b) of the Act, the eligibility requirement for the service group home support for an individual are that:

(a) the individual has a total score of less than 20 for the questions in the Integrated Assessment Tool with the following headings:
(i) "Climb stairs";
(ii) "Eating";
(iii) "Dressing";
(iv) "Take a bath or shower";
(v) "Grooming";
(vi) "Transfers";
(vii) "Toilet use";
(viii) "Toileting – bladder";
(ix) "Toileting – bowels";
(x) "Walk"; or
(b) the individual has a total score of less than 14 for the questions in the Integrated Assessment Tool with the following headings:
(i) "Get to places out of walking distance";
(ii) "Undertake housework (heavy/moderate)";
(iii) "Go shopping (assuming transportation)";
(iv) "Prepare meals";
(v) "Take medicine";
(vi) "Handle money";
(vii) "Use the telephone"; or
(c) the individual has a score of greater than zero for the questions in the sections of the Integrated Assessment Tool headed "Cognition" and "Medical and Medications"; or
(d) the individual has a score of greater than zero for the questions in the sections of the Integrated Assessment Tool headed "Psychological"; or



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(e) the individual has a score of greater than zero for the questions in the section of the Integrated Assessment Tool headed "Physical, Personal Health and Frailty".

### 65-15 Eligibility requirements—service groups assistive technology and home modifications

For the purposes of paragraph 65(3)(b) of the Act, the eligibility requirement for the service groups assistive technology and home modifications for an individual is that the service group home support is approved for the individual.

### 65-20 Eligibility requirements—service group residential care

For the purposes of paragraph 65(3)(b) of the Act, the eligibility requirement for the service group residential care for an individual is that the individual is not able to live in a home or community setting without support.

### 65-30 Period for making decisions

For the purposes of subsection 65(5) of the Act, the period for making decisions under subsections 65(1) and (2) of the Act for an individual is 14 days after receiving an assessment report for the individual provided under section 63 of the Act, or information relating to the individual that is provided to the System Governor in accordance with paragraph 64(2)(b) of the Act, as applicable.

## Subdivision AB—Conditions on approvals of service types or services in certain service groups

### 69-5 Conditions on approvals of subscription service types or subscription services—prescribed matters

*Application*

(1) This section applies to the inclusion, under subsection 69(1) of the Act, of conditions on the approval of a subscription service type, or a subscription service, under paragraph 65(2)(b) of the Act for an individual.

*Kind of registered provider*

(2) For the purposes of paragraph 69(2)(b) of the Act, the kind of registered provider is a registered provider that is registered in the provider registration category subscription trial.

## Subdivision B—Period of effect of approval

### 71-5 Alternative entry—when access approval takes effect—circumstances and period for making application

For the purposes of paragraph 71(3)(b) of the Act, the following circumstances and period are prescribed for an individual:



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(a) the circumstances are that a registered provider that delivers funded aged care services under the MPSP or the NATSIFACP is delivering aged care services to the individual;

(b) the period is 30 days after the first day an aged care service covered by the individual's access approval was delivered to the individual by the provider.



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# Part 3—Classification

## Division 1—Classification assessments

### 75-5 Circumstances in which classification assessment not required

For the purposes of paragraph 75(5)(a) of the Act, the circumstances in which a classification assessment otherwise required under paragraph 75(1)(b) or subsection 75(2) of the Act in relation to an individual for a classification type for a service group is not required to be undertaken are that:

(a) the classification type is ongoing or short-term for the service group residential care; and

(b) the individual is accessing funded aged care services under a specialist aged care program in an approved residential care home.

### 76-10 Assessment tools and other requirements for classification assessments required under subsection 75(1) or (2) of the Act and carried out by approved needs assessors

(1) This section is made for the purposes of subparagraphs 76(1)(a)(i) and (b)(i) of the Act.

*Service groups home support, assistive technology and home modifications*

(2) For an assessment for an individual for a classification type for the service group home support, assistive technology or home modifications:

(a) the assessment tool is the Integrated Assessment Tool; and

(b) the other requirements are that the assessment must be carried out in accordance with the Aged Care Assessment Manual.

*Classification type ongoing for service group residential care*

(3) For an assessment for an individual for the classification type ongoing for the service group residential care:

(a) the assessment tool is the AN-ACC Assessment Tool; and

(b) the other requirements are that the assessment must be carried out in accordance with the AN-ACC Reference Manual.

*Classification type short-term for service group residential care*

(4) For an assessment for an individual for the classification type short-term for the service group residential care:

(a) the assessment tool is the De Morton Mobility Index assessment item; and

(b) the other requirements are that the assessment must be carried out in accordance with the part of the AN-ACC Reference Manual that relates to that item.



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## 76-15 Circumstances and information for classification assessments required under paragraph 75(1)(a) or subsection 75(2) of the Act and carried out by the System Governor

(1) This section is made for the purposes of subparagraph 76(1)(a)(ii) of the Act for an assessment for an individual for a classification type for a service group.

*Classification type short-term for service groups home support and assistive technology—end-of-life*

(2) The following circumstances and kind of information are prescribed for the classification type short-term for the service groups home support and assistive technology:

(a) the circumstances are that:

(i) the individual has an access approval in effect for the classification type short-term for the service group home support;

(ii) on the date of the individual's application for access to funded aged care services, there were reasonable grounds to believe that the individual had a prognosis of a life expectancy of 3 months or less and an AKPS score of 40 or less;

(iii) the individual has not previously had the classification level SAH end-of-life pathway in effect;

(b) the kind of information is information that provides evidence of the matters mentioned in paragraph (a).

(3) The following circumstances and kind of information are prescribed for the classification type short-term for the service groups assistive technology and home modifications and the classification type ongoing for the service group home support:

(a) the circumstances are that:

(i) the individual has an access approval in effect for the classification type short-term for the service group home support;

(ii) the individual has previously had the classification level SAH end-of-life pathway in effect;

(b) the kind of information is information that provides evidence of the matters mentioned in paragraph (a).

(4) The following circumstances and kind of information are prescribed for the classification type short-term for the service group assistive technology:

(a) the circumstances are that:

(i) the classification level SAH end-of-life pathway for the classification type short-term for the service group home support has been established for the individual by a classification decision;

(ii) the individual will require access to funded aged care services in the service group assistive technology;

(b) the kind of information is information that provides evidence of the matters mentioned in paragraph (a).



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*Classification type short-term for service group assistive technology or home modifications—restorative care pathway*

(5) The following circumstances and kind of information are prescribed for the classification type short-term for the service group assistive technology and home modifications:

(a) the circumstances are that:

(i) the classification level SAH restorative care pathway for the classification type short-term for the service group home support has been established for the individual by a classification decision;

(ii) the individual will require access to funded aged care services in the service group assistive technology or home modifications;

(b) the kind of information is information that provides evidence of the matters mentioned in paragraph (a).

*Classification type short-term for service group assistive technology—repairs or maintenance for included and conditionally included AT-HM items*

(6) The following circumstances and kind of information are prescribed for the classification type short-term for the service group assistive technology:

(a) the circumstances are that:

(i) a classification level for the classification type ongoing for the service group home support has been established for the individual by a classification decision; and

(ii) repairs or maintenance are required for an included AT-HM item or a conditionally included AT-HM item that the individual owns or has been loaned or rented under a Commonwealth aged care program (including under the old Act);

(b) the kind of information is information that provides evidence of the matter mentioned in subparagraph (a)(ii).

*Classification type short-term for service groups assistive technology and home modifications—progressive conditions*

(7) The following circumstances and kind of information are prescribed for the classification type short-term for the service groups assistive technology and home modifications:

(a) the circumstances are that:

(i) a classification level for the classification type ongoing for the service group home support has been established for the individual by a classification decision; and

(ii) the individual has a condition referred to in subsection 211-10(2) of this instrument;

(b) the kind of information is information that provides evidence of the matter mentioned in subparagraph (a)(ii).



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*Classification type ongoing for service group assistive technology —assistance dog*

(8) The following circumstances and kind of information are prescribed for the classification type ongoing for the service group assistive technology:

(a) the circumstances are that:

(i) the individual has an access approval in effect for the classification type ongoing for the service group home support;

(ii) the individual has an assistance dog;

(b) the kind of information is information that provides evidence of the matter mentioned in subparagraph (a)(ii).

*Classification type short-term for service groups assistive technology and home modifications—aged care needs reassessments in certain circumstances*

(9) The following circumstances and kinds of information are prescribed for the classification type short-term for the service groups assistive technology and home modifications:

(a) the circumstances are that:

(i) a classification level for the classification type short-term for the service group home support has been established for the individual by a classification decision; and

(ii) the circumstances referred to in paragraph 64-10(b) of this instrument (aged care needs reassessments in certain circumstances) apply to the individual;

(b) the kinds of information are the kinds of information referred to in paragraphs 64-15(a), (b) and (c) of this instrument.

*Classification type ongoing for service group residential care—palliative care pathway*

(10) The following circumstances and kind of information are prescribed for the classification type ongoing for the service group residential care, for an individual accessing funded aged care services in the form of palliative care in an approved residential care home of a registered provider:

(a) the circumstances are that:

(i) on and after the individual's start day for the home, the provider has delivered funded aged care services in the form of palliative care to the individual in the home; and

(ii) on the individual's start day for the home, there were reasonable grounds to believe that the individual had a prognosis of a life expectancy of 3 months or less and an AKPS score of 40 or less; and

(iii) the provider gives the System Governor a notice in the approved form containing the kind of information referred to in paragraph (b) within 14 days, or such longer period as is agreed in writing between the System Governor and the provider, after the provider gives the System Governor a start notification for the individual;



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(b) the kind of information is information that provides evidence of the matters mentioned in subparagraph (a)(ii).

## 76-20 Circumstances and information for classification assessments required under paragraph 75(1)(b) of the Act (on application for classification reassessment) and carried out by the System Governor

(1) This section is made for the purposes of subparagraph 76(1)(b)(ii) of the Act for an assessment for an individual for a classification type for a service group.

*Classification type short-term for service group assistive technology—increased needs*

(2) The following circumstances and kind of information are prescribed for the classification type short-term for the service group assistive technology:

(a) the circumstances are that:

(i) a classification level for the classification type short-term for the service group assistive technology, other than the classification level AT high, has been established for the individual by a classification decision; and

(ii) the individual needs an increased amount of assistive technology to mitigate functional decline or impairment and enable them to safely live in their home and community;

(b) the kind of information is information that provides evidence of the circumstances mentioned in subparagraph (a)(ii).

*Classification type short-term for service group assistive technology—individuals with classification level AT transitional*

(3) The following circumstances and kind of information are prescribed for the classification type short-term for the service group assistive technology:

(a) the circumstances are that:

(i) the classification level AT transitional for the classification type short-term for the service group assistive technology has been established for the individual by a classification decision; and

(ii) subsection (4) applies to the individual;

(b) the kind of information is information that provides evidence of the circumstances mentioned in subparagraph (a)(ii).

(4) This subsection applies to the individual if:

(a) the individual has a condition referred to in subsection 211-10(2) of this instrument; or

(b) the System Governor has reviewed the report of the most recent aged care needs assessment for the individual and considers that the individual needs assistive technology to mitigate functional decline or impairment and enable them to safely live in their home and community (see paragraph (a) in column 2 of items 1 to 3 of the table in subsection 81-25(1) of this instrument (criteria for the classification levels AT low, AT medium and AT high)).



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*Classification type short-term for service group home modifications—increased needs*

(5) The following circumstances and kind of information are prescribed for the classification type short-term for the service group home modifications:

(a) the circumstances are that:

(i) a classification level for the classification type short-term for the service group home modifications, other than the classification level HM high, has been established for the individual by a classification decision; and

(ii) the individual needs an increased amount of home modifications to mitigate functional decline or impairment and enable them to safely live in their home and community;

(b) the kind of information is information that provides evidence of the circumstances mentioned in subparagraph (a)(ii).

*Classification type short-term for service group home modifications—individuals with classification level HM transitional*

(6) The following circumstances and kind of information are prescribed for the classification type short-term for the service group home modifications:

(a) the circumstances are that:

(i) the classification level HM transitional for the classification type short-term for the service group home modifications has been established for the individual by a classification decision; and

(ii) the System Governor has reviewed the report of the most recent aged care needs assessment for the individual and considers that the individual needs home modifications to mitigate functional decline or impairment and enable them to safely live in their home and community (see paragraph (a) in column 2 of items 1 to 3 of the table in subsection 81-30(1) of this instrument (criteria for the classification levels HM low, HM medium and HM high));

(b) the kind of information is information that provides evidence of the circumstances mentioned in subparagraph (a)(ii).

*Classification type ongoing for service group residential care—palliative care pathway*

(7) The following circumstances and kind of information are prescribed for the classification type ongoing for the service group residential care, for an individual accessing funded aged care services in the form of palliative care in an approved residential care home of a registered provider:

(a) the circumstances are that:

(i) on and after the individual's start day for the home, the provider has delivered funded aged care services in the form of palliative care to the individual in the home; and

(ii) on the individual's start day for the home, there were reasonable grounds to believe that the individual had a prognosis of a life expectancy of 3 months or less and an AKPS score of 40 or less; and



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(iii) the provider gives the System Governor a notice in the approved form containing the kind of information referred to in paragraph (b) within 14 days, or such longer period as is agreed in writing between the System Governor and the provider, after the date of the application for classification reassessment for the individual;

(b) the kind of information is information that provides evidence of the matters mentioned in subparagraph (a)(ii).

## 76-25 Classification assessments for classification type ongoing for service group residential care—skills, qualifications and other requirements for approved needs assessors

*Skills and qualifications*

(1) For the purposes of paragraph 76(2)(a) of the Act, the skills and qualifications for an approved needs assessor are that the assessor is a registered nurse, occupational therapist or physiotherapist.

*Other requirements*

(2) For the purposes of paragraph 76(2)(b) of the Act, the other requirements for an approved needs assessor are the following:

(a) the assessor has at least 5 years of clinical experience in the delivery of aged care services or related health services as a registered nurse, occupational therapist or physiotherapist (as the case requires);

(b) a police certificate issued for the assessor within the last 3 years does not record that the assessor has a serious offence conviction in Australia;

(c) if, at any time after turning 16, the assessor has been a citizen or permanent resident of a country other than Australia—the assessor has made a statutory declaration that the assessor does not have a serious offence conviction in that country.

Note: Approved needs assessors must carry and produce identity cards issued under subsection 343A(1) of the Act when performing functions or exercising powers under the Act as approved needs assessors (see section 343A-10 of this instrument).



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# Division 2—Classification decisions

## Subdivision A—Period for making classification decisions

### 78-5 Period for making classification decisions

For the purposes of subsection 78(4) of the Act, the period for making a decision under subsection 78(1) of the Act to establish a classification level for an individual for a classification type for a service group is 14 days after the occurrence of whichever of the events referred to in subsection 78(1) of the Act are applicable for the individual.

## Subdivision B—Period of effect of classification levels

### 80-5 Purpose of this Subdivision

For the purposes of subsection 80(1) of the Act, this Subdivision prescribes, subject to subsection 80(3) of the Act, the period of effect for a classification level for a classification type for a service group that has been established under section 78 of the Act for an individual.

Note: For the period of effect for classification levels taken to be established for individuals by the *Aged Care (Consequential and Transitional Provisions) Determination 2025*, see the *Aged Care (Consequential and Transitional Provisions) Rules 2025*.

### 80-10 Service group home support—classification type ongoing

*CHSP class*

(1) For the classification level CHSP class for the classification type ongoing for the service group home support, established for an individual by a classification decision, the period of effect:
  (a) starts at the start of the day the individual's access approval for the classification type for the service group was given; and
  (b) ends at the end of the earlier of the following (as applicable):
    (i) the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;
    (ii) the day the individual dies.

*SAH class 1, 2, 3, 4, 5, 6, 7 or 8*

(2) For the classification level SAH class 1, 2, 3, 4, 5, 6, 7 or 8 for the classification type ongoing for the service group home support, established for an individual by a classification decision, the period of effect:
  (a) starts at the start of the day a place allocated under subsection 92(1) of the Act to the individual for the classification type for the service group takes effect under section 92A of the Act; and
  (b) ends at the end of the earlier of the following (as applicable):



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(i) the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;
(ii) the day the individual dies.

## 80-15 Service group home support—classification type short-term

*SAH restorative care pathway*

(1) For the classification level SAH restorative care pathway for the classification type short-term for the service group home support, established for an individual by a classification decision, the period of effect:
(a) starts at the start of the day a place allocated under subsection 92(1) of the Act to the individual for the classification type for the service group takes effect under section 92A of the Act; and
(b) ends at the earliest of the following (as applicable):
(ii) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;
(iii) the end of the maximum period of effect for the classification level (see section 80-55);
(iv) the end of the day the individual dies.

*SAH end-of-life pathway*

(2) For the classification level SAH end-of-life pathway for the classification type short-term for the service group home support, established for an individual by a classification decision, the period of effect:
(a) starts at the start of the day a place allocated under subsection 92(1) of the Act to the individual for the classification type for the service group takes effect under section 92A of the Act; and
(b) ends at the earliest of the following (as applicable):
(i) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;
(ii) the end of the maximum period of effect for the classification level (see section 80-57);
(iii) the end of the day the individual dies.

## 80-20 Service group assistive technology—classification type ongoing

For the classification level assistance dogs for the classification type ongoing for the service group assistive technology, established for an individual by a classification decision, the period of effect:
(a) starts at the start of the day a place allocated under subsection 92(1) of the Act to the individual for the classification type for the service group takes effect under section 92A of the Act; and
(b) ends at the end of the earlier of the following (as applicable):
(i) the day that the individual ceases to have an assistance dog;



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(ii) the day the individual dies.

## 80-25 Service group assistive technology—classification type short-term

*AT CHSP*

(1) For the classification level AT CHSP for the classification type short-term for the service group assistive technology, established for an individual by a classification decision, the period of effect:
   (a) starts at the start of the day the individual's access approval for the classification type for the service group was given; and
   (b) ends at the end of the earlier of the following (as applicable):
      (i) the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;
      (ii) the day the individual dies.

*AT low, AT medium and AT high*

(2) For the classification level AT low, AT medium or AT high for the classification type short-term for the service group assistive technology, established for an individual by a classification decision, the period of effect:
   (a) starts at the start of the day a place allocated under subsection 92(1) of the Act to the individual for the classification type for the service group takes effect under section 92A of the Act; and
   (b) ends at the earliest of the following (as applicable):
      (i) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;
      (ii) the end of the account period for the individual's notional assistive technology account established in respect of that classification level;
      (iii) the end of the day the individual dies.

## 80-30 Service group home modifications—classification type short-term

*HM CHSP*

(1) For the classification level HM CHSP for the classification type short-term for the service group home modifications, established for an individual by a classification decision, the period of effect:
   (a) starts at the start of the day the individual's access approval for the classification type for the service group was given; and
   (b) ends at the end of the earlier of the following (as applicable):
      (i) the later of the following:
         (A) the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;



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(B) if the System Governor approves a later time than the time mentioned in sub-subparagraph (A) for the individual under subsection (3)—that later time;

(ii) the day the individual dies.

*HM low, HM medium and HM high*

(2) For the classification level HM low, HM medium or HM high for the classification type short-term for the service group home modifications, established for an individual by a classification decision, the period of effect:

(a) starts at the start of the day a place allocated under subsection 92(1) of the Act to the individual for the classification type for the service group takes effect under section 92A of the Act; and

(b) ends at the earlier of the following (as applicable):

(i) the latest of the following:

(A) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;

(B) the end of the account period for the individual's notional home modifications account established in respect of that classification level;

(C) if the System Governor approves a later time than the time mentioned in sub-subparagraph (A) or (B) for the individual under subsection (3)—that later time;

(ii) the end of the day the individual dies.

*Approval of later time for services scheduled for delivery*

(3) The System Governor must approve a time that is 12 months after the time that would otherwise apply under subparagraph (1)(b)(i) or (2)(b)(i) (as applicable) for an individual if a registered provider gives the System Governor evidence mentioned in subsection (4):

(a) in the approved form; and

(b) within the account period for the individual's notional home modifications account established in respect of that classification level.

(4) For the purposes of subsection (3), the evidence is evidence that a service in the service group home modifications to be delivered by the registered provider to the individual has been scheduled for delivery, and is in progress, but will not be delivered before the time that would otherwise apply under subparagraph (1)(b)(i) or (2)(b)(i) (as applicable).

## 80-35 Service group residential care—classification type ongoing

*Class 0*

(1) For the classification level class 0 for the classification type ongoing for the service group residential care, established in accordance with section 81-42 of this instrument for an individual to whom paragraph 78(1)(c) of the Act applies:



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(a) subject to paragraph (b), the period of effect:
  (i) starts at the start of the entry day for the classification type for the service group for the individual; and
  (ii) ends at the end of the day the individual dies; and
(b) if the classification level class 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12 or 13 (the ***higher classification level***) is in effect for the individual on a day under subsection (2), the classification level class 0 is not in effect for the individual on the day.

*Classes 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12 and 13*

(2) For the classification level class 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12 or 13 for the classification type ongoing for the service group residential care, established for an individual by a classification decision, the period of effect:
(a) starts:
  (i) unless subparagraph (ii), (iii) or (iv) applies—at the start of the entry day for the classification type for the service group for the individual; or
  (ii) unless subparagraph (iii) or (iv) applies—if, before the day the classification decision is made:
    (A) the classification level class 0 is in effect for the individual under subsection (1); and
    (B) the individual started accessing funded aged care services in an approved residential care home; and
    (C) the individual did not cease to access funded aged care services in that home for a period of more than 28 days;
  —at the start of the individual's start day for that home; or
  (iii) unless subparagraph (iv) applies—if, before the day the classification decision is made:
    (A) the classification level class 0 is in effect for the individual under subsection (1); and
    (B) the individual started accessing funded aged care services in an approved residential care home; and
    (C) the individual ceased to access funded aged care services in that home for a period of more than 28 days;
  —at the start of the day after the end of that period when the individual resumed accessing ongoing funded aged care services in an approved residential care home; or
  (iv) if the classification decision relates to a classification assessment undertaken in accordance with paragraph 75(1)(b) of the Act (that is, following an application for classification reassessment)—the day the application for classification reassessment was made; and
(b) ends at the end of the day the individual dies.



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Section 80-40

## 80-40 Service group residential care—classification type short-term

*Respite class 0*

(1) For the classification level respite class 0 for the classification type short-term for the service group residential care, established in accordance with section 81-42 of this instrument for an individual to whom paragraph 78(1)(c) of the Act applies:

(a) subject to paragraphs (b) and (c)—the period of effect consists of each day on or after the entry day for the classification type for the service group for the individual on which:

(i) a registered provider delivers funded aged care services to the individual for the classification type for the service group at an approved residential care home of the provider (other than under a specialist aged care program); and

(ii) the number of days on which the individual had previously accessed funded aged care services for the classification type for the service group at an approved residential care home (other than under a specialist aged care program) during the financial year in which the day occurred is less than the maximum period of effect for the classification level (see section 80-60); and

(b) if the classification level respite class 1, 2 or 3 (the ***higher classification level***) is in effect for the individual on a day under subsection (2), the classification level respite class 0 is not in effect for the individual on the day; and

(c) the period of effect ends at the end of the earliest of the following (as applicable):

(i) the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;

(ii) the day the individual dies.

*Respite classes 1, 2 and 3*

(2) For the classification level respite class 1, 2 or 3 for the classification type short-term for the service group residential care, established for an individual by a classification decision:

(a) subject to paragraphs (b) and (c)—the period of effect consists of each day on or after the entry day for the classification type for the service group for the individual on which:

(i) a registered provider delivers funded aged care services to the individual for the classification type for the service group at an approved residential care home of the provider (other than under a specialist aged care program); and

(ii) the number of days on which the individual had previously accessed funded aged care services for the classification type for the service group at an approved residential care home (other than under a specialist aged care program) during the financial year in which the



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day occurred is less than the maximum period of effect for the classification level (see section 80-60); and

(b) if the classification decision relates to a classification assessment undertaken in accordance with paragraph 75(1)(b) of the Act (that is, following an application for classification reassessment)—the period of effect consists of each day on or after the day the application for classification reassessment was made on which:

(i) a registered provider delivers funded aged care services to the individual for the classification type for the service group at an approved residential care home of the provider (other than under a specialist aged care program); and

(ii) the number of days on which the individual had previously accessed funded aged care services for the classification type for the service group at an approved residential care home (other than under a specialist aged care program) during the financial year in which the day occurred is less than the maximum period of effect for the classification level (see section 80-60); and

(c) the period of effect ends at the end of the earliest of the following (as applicable):

(i) the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;

(ii) the day the individual dies.

## 80-45 Service group home support, assistive technology or residential care—classification type hospital transition

*Service group home support*

(1) For the classification level HS HT class for the classification type hospital transition for the service group home support, established for an individual by a classification decision:

(a) the classification level does not take effect if a start day for the individual for the classification type for the service group does not occur within 28 days from the day the access approval that is in effect for the individual took effect; and

(b) subject to paragraph (c)—the period of effect for the classification level consists of each day on or after the start day for the individual for the classification level on which:

(i) a registered provider delivers funded aged care services to the individual for the classification type for the service group; and

(ii) the number of days on which the individual had previously accessed funded aged care services for the classification type for the service group is less than the maximum period of effect for the classification level (see section 80-65); and

(c) the period of effect for the classification level ends at the earliest of the following (as applicable):



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(i) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;

(ii) if, after the start of the period of effect and before the end of the maximum period of effect for the classification level (see section 80-65), the individual does not access funded aged care services for the classification type for the service group for a total of 7 days—the end of the most recent day on which the individual accessed funded aged care services for the classification type for the service group;

(iii) the end of the maximum period of effect for the classification level (see section 80-65);

(iv) the end of the day the individual dies.

*Service group assistive technology*

(2) For the classification level AT HT class for the classification type hospital transition for the service group assistive technology, established for an individual by a classification decision:

(a) the classification level does not take effect if a start day for the individual for the classification type for the service group does not occur within 28 days from the day the access approval that is in effect for the individual took effect; and

(b) subject to paragraph (c)—the period of effect for the classification level consists of each day on or after the start day for the individual for the classification level on which:

(i) a registered provider delivers funded aged care services to the individual for the classification type for the service group; and

(ii) the number of days on which the individual had previously accessed funded aged care services for the classification type for the service group is less than the maximum period of effect for the classification level (see section 80-65); and

(c) the period of effect for the classification level ends at the earliest of the following (as applicable):

(i) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;

(ii) if, after the start of the period of effect and before the end of the maximum period of effect for the classification level (see section 80-65), the individual does not access funded aged care services for the classification type for the service group for a total of 7 days—the end of the most recent day on which the individual accessed funded aged care services for the classification type for the service group;

(iii) the end of the maximum period of effect for the classification level (see section 80-65);

(iv) the end of the day the individual dies.



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*Service group residential care*

(3) For the classification level RC HT class for the classification type hospital transition for the service group residential care, established for an individual by a classification decision:

(a) the classification level does not take effect if a start day for the individual for the classification type for the service group does not occur within 28 days from the day the access approval that is in effect for the individual took effect; and

(b) subject to paragraph (c)—the period of effect for the classification level consists of each day on or after the start day for the individual for the classification level on which:

(i) a registered provider delivers funded aged care services to the individual for the classification type for the service group; and

(ii) the number of days on which the individual had previously accessed funded aged care services for the classification type for the service group is less than the maximum period of effect for the classification level (see section 80-65); and

(c) the period of effect for the classification level ends at the earliest of the following (as applicable):

(i) the end of the day a classification level for the classification type ongoing for the service group residential care takes effect for the individual;

(ii) if, after the start of the period of effect and before the end of the maximum period of effect for the classification level (see section 80-65), the individual does not access funded aged care services for the classification type for the service group for a total of 7 days—the end of the most recent day on which the individual accessed funded aged care services for the classification type for the service group;

(iii) the end of the maximum period of effect for the classification level (see section 80-65);

(iv) the end of the day the individual dies.

## Subdivision BA—Circumstances in which classification levels resume being in effect for a period

### 80-47 Purpose of this Subdivision

For the purposes of subsection 80(1) of the Act, this Subdivision prescribes, subject to subsection 80(3) of the Act, circumstances in which a classification level for an individual that has ceased being in effect resumes being in effect for a period.



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## 80-49 Circumstances in which classification levels resume being in effect for a period—service groups home support, assistive technology and home modifications

*Service group home support*

(1) A classification level for a classification type for the service group home support for an individual that has ceased being in effect because a classification level for the classification type ongoing for the service group residential care has taken effect for the individual:

(a) resumes being in effect if the individual notifies the System Governor that the individual needs the classification level to resume being in effect; and

(b) resumes being in effect when the individual so notifies the System Governor; and

(c) remains in effect in accordance with section 80 of the Act and Subdivision B of Division 2 of Part 3 of Chapter 2 of this instrument.

(2) Subsection (1) does not apply to the classification level CHSP class for the classification type ongoing for the service group home support.

*Classification type short-term for service group assistive technology*

(3) A classification level (the ***individual's old AT classification level***) for the classification type short-term for the service group assistive technology for an individual that has ceased being in effect because a classification level for the classification type ongoing for the service group residential care has taken effect for the individual:

(a) resumes being in effect if:

(i) the individual notifies the System Governor that the individual needs the classification level to resume being in effect; and

(ii) when the individual so notifies the System Governor, the account period for the individual's notional assistive technology account established in respect of the individual's old AT classification level has not ended; and

(b) resumes being in effect when the individual so notifies the System Governor; and

(c) remains in effect in accordance with section 80 of the Act and Subdivision B of Division 2 of Part 3 of Chapter 2 of this instrument.

*Classification type short-term for service group home modifications*

(4) A classification level (the ***individual's old HM classification level***) for the classification type short-term for the service group home modifications for an individual that has ceased being in effect because a classification level for the classification type ongoing for the service group residential care has taken effect for the individual:

(a) resumes being in effect if:

(i) the individual notifies the System Governor that the individual needs the classification level to resume being in effect; and



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(ii) when the individual so notifies the System Governor, the account period for the individual's notional home modifications account established in respect of the individual's old HM classification level has not ended; and

(b) resumes being in effect when the individual so notifies the System Governor; and

(c) remains in effect in accordance with section 80 of the Act and Subdivision B of Division 2 of Part 3 of Chapter 2 of this instrument.

## Subdivision C—Maximum period of effect of classification levels

### 80-50 Purpose of this Subdivision

For the purposes of subsection 80(1) of the Act, this Subdivision prescribes, for certain classification levels, the maximum period of effect for the classification level.

### 80-55 Service group home support—classification type short-term—classification level SAH restorative care pathway

For the classification level SAH restorative care pathway for the classification type short-term for the service group home support, the maximum period of effect is 112 days.

### 80-57 Service group home support—classification type short-term—classification level SAH end-of-life pathway

For the classification level SAH end-of-life pathway for the classification type short-term for the service group home support, the maximum period of effect is 112 days.

### 80-60 Service group residential care—classification type short-term

(1) For a classification level for the classification type short-term for the service group residential care, the maximum period of effect is:

(a) 63 days; or

(b) for an individual for whom the System Governor has increased the number of days for the classification level under subsection (5)—the number of days as so increased (or as most recently increased).

Note: The maximum period of effect for the classification type short-term for the service group residential care relates to a financial year (see section 80-40).

*Increase in number of days for classification levels for individuals*

(2) A registered provider may apply to the System Governor for a determination of an increased number of days for the classification level for an individual under subsection (5).

(3) An application under subsection (2) must be made in the approved form.



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(4) The System Governor must consider an application under subsection (2) and decide whether to determine an increased number of days under subsection (5).

(5) The System Governor may increase the number of days for the classification level for the individual by up to 21 if the System Governor is satisfied that an increase in the number of days is necessary because of any of the following:
  (a) carer stress;
  (b) severity of the individual's condition;
  (c) absence of the individual's carer;
  (d) any other relevant matter.

(6) The System Governor must give written notice to the registered provider of the System Governor's decision within 28 days after the application was made.

(7) The notice of decision must include the day the decision takes effect, which may be before the day the decision is made.

(8) An increase under subsection (5) may be made more than once.

## 80-65 Service group home support, assistive technology or residential care—classification type hospital transition

(1) For a classification level for the classification type hospital transition for the service group home support, assistive technology or residential care, the maximum period of effect is:
  (a) 84 days; or
  (b) for an individual for whom the System Governor determines one or more additional periods for the classification level under subsection (5)—84 days plus those additional periods.

*Extension of maximum period of effect for classification levels for individuals*

(2) A registered provider may apply to the System Governor for a determination of an additional period for the classification level for an individual under subsection (5).

(3) An application under subsection (2) must be made:
  (a) in the approved form; and
  (b) within:
    (i) the period referred to in paragraph (1)(a); or
    (ii) if an additional period has been determined for the individual for the classification level under subsection (5)—that additional period.

(4) The System Governor must consider an application under subsection (2) and decide whether to determine an additional period under subsection (5).

(5) The System Governor may determine one or more additional periods, together totalling not more than 42 days, for the classification level for the individual if the System Governor is satisfied that the individual requires access to funded aged care services for that level for the additional periods.



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(6) The System Governor must give written notice to the registered provider of a decision under subsection (5) within 28 days after the application was made.



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# Division 3—Classification levels and procedures

## Subdivision A—Introduction

### 81-5 Purpose of this Division

For the purposes of section 81 of the Act, this Division prescribes:

(a) classification levels for classification types for service groups; and

(b) for certain classification levels—criteria that are to be used in establishing the classification level of an individual for a classification type for a service group; and

(c) for certain classification levels—methods or procedures that the System Governor must follow in deciding classification levels for an individual for a classification type for a service group; and

(d) for the service groups home support, assistive technology and home modifications—compounding factors (based on results against relevant assessment items mentioned in subsection 77(2) of the Act) that will be used to establish an individual's classification level; and

(e) for the service group residential care:

(i) compounding factors (based on results against relevant assessment items mentioned in subsection 77(2) of the Act) that will be used to establish an individual's classification level; and

(ii) the scientific population study that will be used by the System Governor to derive a method for establishing when these compounding factors, taken together, are significant because they indicate the individual has significantly higher care needs relative to the needs of other individuals.

### 81-7 Meaning of *home support compounding factors*

The home support compounding factors, for an individual, means the individual's scores for the following groups of questions in the Integrated Assessment Tool:

(a) the questions in the sections headed "Cognition" and "Medical and Medications";

(b) the questions in the section headed "Social";

(c) the questions under the headings "Toileting - Bladder" and "Toileting - Bowels";

(d) the questions in the section headed "Carer Profile";

(e) the questions in the section headed "Psychological";

(f) the questions in the section headed "Physical, Personal Health and Frailty".

### 81-8 Meaning of *residential care compounding factors*

The following table sets out the ***residential care compounding factors*** for individuals.



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| **Residential care compounding factors for individuals** | | |
|---|---|---|
| **Item** | **Column 1**<br>**For an individual who …** | **Column 2**<br>**the residential care compounding factors are the following …** |
| 1 | is independently mobile | (a) the individual's AFM cognition score;<br>(b) the individual's agitation score;<br>(c) the individual's AKPS score;<br>(d) the individual's RUG total score;<br>(e) whether the individual requires daily injections. |
| 2 | is mobile only with assistance and has higher cognitive ability | (a) the individual's AFM motor score;<br>(b) the individual's AFM social cognition score;<br>(c) the individual's AKPS score;<br>(d) the individual's Braden activity score;<br>(e) whether the individual has fallen in the last 12 months;<br>(f) whether the individual requires daily injections. |
| 3 | is mobile only with assistance and has medium cognitive ability | (a) the individual's AFM communication score;<br>(b) the individual's disruptiveness score;<br>(c) the individual's Rockwood Frailty Score;<br>(d) the individual's RUG total score;<br>(e) whether the individual requires complex wound management;<br>(f) whether the individual requires daily injections. |
| 4 | is not mobile and has higher function | (a) the individual's AFM transfers score;<br>(b) the individual's Braden total score;<br>(c) the individual's disruptiveness score;<br>(d) whether the individual requires complex wound management;<br>(e) whether the individual requires daily injections. |
| 5 | is not mobile and has lower function and higher pressure sore risk | (a) the individual's AFM eating score;<br>(b) the individual's disruptiveness score;<br>(c) whether the individual has fallen in the last 12 months;<br>(d) whether the individual has lost more than 10% of their body weight in the last 12 months;<br>(e) whether the individual requires daily injections. |

## 81-9 Scientific population study for residential care compounding factors

The scientific population study to be used as mentioned in paragraph 81(4)(b) of the Act is the Resource Utilisation and Classification Study undertaken by the Australian Health Services Research Institute at the University of Wollongong.

Note: The reports of the Resource Utilisation and Classification Study could in 2025 be viewed on the Department's website (https://www.health.gov.au).



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## Subdivision B—Classification levels and criteria

### 81-10 Service group home support—classification type ongoing

*Non-transitional classification levels and criteria*

(1) The following table sets out the non-transitional classification levels for the classification type ongoing for the service group home support and the criteria for those classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | CHSP class | Any of the following:<br>(a) the individual:<br>(i) has a home support functional independence score of 42-48; and<br>(ii) has a home support needs met score of 35 or 36;<br>(b) the individual:<br>(i) has a home support functional independence score of 42-48; and<br>(ii) has a home support needs met score of 0-34;<br>(c) the individual:<br>(i) has a home support functional independence score of 35-41; and<br>(ii) has a home support needs met score of 31-36;<br>(d) the individual:<br>(i) has a home support functional independence score of 35-41; and<br>(ii) has a home support needs met score of 0-30; and<br>(iii) does not have significant compounding factors |
| 2 | SAH class 1 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 35-41; and<br>(ii) has a home support needs met score of 0-30; and<br>(iii) has significant compounding factors; or<br>(b) the individual:<br>(i) has a home support functional independence score of 30-34; and<br>(ii) has a home support needs met score of 30-36; and<br>(iii) does not have significant compounding factors |
| 3 | SAH class 2 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 30-34; and<br>(ii) has a home support needs met score of 30-36; and<br>(iii) has significant compounding factors; or |



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**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| | | (b) the individual:<br>(i) has a home support functional independence score of 30-34; and<br>(ii) has a home support needs met score of 0-29; and<br>(iii) does not have significant compounding factors |
| 4 | SAH class 3 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 30-34; and<br>(ii) has a home support needs met score of 0-29; and<br>(iii) has significant compounding factors; or<br>(b) the individual:<br>(i) has a home support functional independence score of 25-29; and<br>(ii) has a home support needs met score of 28-36; and<br>(iii) does not have significant compounding factors |
| 5 | SAH class 4 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 25-29; and<br>(ii) has a home support needs met score of 28-36; and<br>(iii) has significant compounding factors; or<br>(b) the individual:<br>(i) has a home support functional independence score of 25-29; and<br>(ii) has a home support needs met score of 0-27; and<br>(iii) does not have significant compounding factors |
| 6 | SAH class 5 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 25-29; and<br>(ii) has a home support needs met score of 0-27; and<br>(iii) has significant compounding factors; or<br>(b) the individual:<br>(i) has a home support functional independence score of 0-24; and<br>(ii) has a home support needs met score of 29-36; and<br>(iii) does not have significant compounding factors |
| 7 | SAH class 6 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 0-24; and<br>(ii) has a home support needs met score of 29-36; and<br>(iii) has significant compounding factors; or<br>(b) the individual: |



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**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| | | (i) has a home support functional independence score of 0-24; and<br>(ii) has a home support needs met score of 23-28; and<br>(iii) does not have significant compounding factors |
| 8 | SAH class 7 | Either:<br>(a) the individual:<br>(i) has a home support functional independence score of 0-24; and<br>(ii) has a home support needs met score of 23-28; and<br>(iii) has significant compounding factors; or<br>(b) the individual:<br>(i) has a home support functional independence score of 0-24; and<br>(ii) has a home support needs met score of 0-22; and<br>(iii) does not have significant compounding factors |
| 9 | SAH class 8 | The individual:<br>(a) has a home support functional independence score of 0-24; and<br>(b) has a home support needs met score of 0-22; and<br>(c) has significant compounding factors |

(1A) Despite subsection (1), if an individual is accessing services funded under the program known as the Disability Support for Older Australians Program, the criteria for the classification level CHSP class is that the individual is accessing those services under that program.

*Transitional classification levels*

(2) The transitional classification levels for the classification type ongoing for the service group home support are the following:

(a) HCP class 1;
(b) HCP class 2;
(c) HCP class 3;
(d) HCP class 4;
(e) CHSP transitional;
(f) HSO NATSIFACP;
(g) HSO MPSP.

Note: Transitional classification levels for the classification type ongoing for the service group home support are established for certain classes of individuals to whom subsection 2(2) of Schedule 2 to the *Aged Care (Consequential and Transitional Provisions) Act 2024* applies—see the *Aged Care (Consequential and Transitional Provisions) Determination 2025*.



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## 81-15 Service group home support—classification type short-term

*Non-transitional classification levels and criteria*

(1) The following table sets out the non-transitional classification levels for the classification type short-term for the service group home support and the criteria for those classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | SAH restorative care pathway | The individual:<br>(a) does not meet the criteria for the classification level CHSP class for the classification type ongoing for the service group home support; and<br>(b) does not meet the criteria for the classification level SAH end-of-life pathway for the classification type short-term for the service group home support; and<br>(c) is not accessing, and does not have the potential to benefit from accessing, funded aged care services delivered through the service group home support under the TCP; and<br>(d) is residing in a home or community setting; and<br>(e) needs restorative care to prevent or delay the individual from needing to access ongoing funded aged care services in the service group home support in circumstances mentioned in paragraph 64-5(b); and<br>(f) has goals in line with restorative care outcomes (for example improving function and being independent); and<br>(g) either:<br>(i) has not had the classification level SAH restorative care pathway in effect in the previous 90 days; or<br>(ii) has not had an amount determined for the individual under section 195-3 during a period of effect for the classification level SAH restorative care pathway that includes a day in the previous 12 months |
| 2 | SAH end-of-life pathway | All of the following apply to the individual:<br>(a) the individual:<br>(i) has a prognosis of a life expectancy of 3 months or less; and<br>(ii) has an AKPS score of 40 or less;<br>(b) the individual has not previously had the classification level SAH end-of-life pathway in effect;<br>(c) a notice in the approved form containing evidence of the matters referred to in paragraph (a) has been given to the System Governor |



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*Transitional classification levels*

(2) The transitional classification levels for the classification type short-term for the service group home support are the following:

(a) STRC class;

(b) HSST NATSIFACP;

(c) HSST MPSP.

Note: The transitional classification levels for the classification type short-term for the service group home support are established for certain classes of individuals to whom subsection 2(2) of Schedule 2 to the *Aged Care (Consequential and Transitional Provisions) Act 2024* applies—see the *Aged Care (Consequential and Transitional Provisions) Determination 2025*.

## 81-17 Service group home support—classification type hospital transition

The following table sets out the classification levels for the classification type hospital transition for the service group home support and the criteria for the classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | HS HT class | The individual:<br>(a) has an access approval that includes the service type assistance with transition care; and<br>(b) is in the concluding stage of a hospital episode; and<br>(c) is medically stable; and<br>(d) has the potential to benefit from accessing funded aged care services delivered through the service group home support under the TCP; and<br>(e) was admitted to a hospital at the time the aged care needs assessment was carried out |

## 81-20 Service group assistive technology—classification type ongoing

The following table sets out the classification levels for the classification type ongoing for the service group assistive technology and the criteria for the classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | Assistance dogs | The individual:<br>(a) has an assistance dog; and<br>(b) is unable to access assistance under the Assistance Dogs Australia program for people with physical disabilities |



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## 81-25 Service group assistive technology—classification type short-term

*Non-transitional classification levels and criteria*

(1) The following table sets out the non-transitional classification levels for the classification type short-term for the service group assistive technology and the criteria for those classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | AT low | The individual:<br>(a) needs low cost assistive technology to mitigate functional decline or impairment and enable them to safely live in their home and community; and<br>(b) has had any of the following classification levels established for the individual:<br>(i) for the classification type ongoing for the service group home support—SAH class 1, 2, 3, 4, 5, 6, 7 or 8;<br>(ii) for the classification type short-term for the service group home support—SAH restorative care pathway or SAH end-of-life pathway |
| 2 | AT medium | The individual:<br>(a) needs medium cost assistive technology to mitigate functional decline or impairment and enable them to safely live in their home and community; and<br>(b) has had any of the following classification levels established for the individual:<br>(i) for the classification type ongoing for the service group home support—SAH class 1, 2, 3, 4, 5, 6, 7 or 8;<br>(ii) for the classification type short-term for the service group home support—SAH restorative care pathway or SAH end-of-life pathway |
| 3 | AT high | The individual:<br>(a) needs high cost assistive technology to mitigate functional decline or impairment and enable them to safely live in their home and community; and<br>(b) has had any of the following classification levels established for the individual:<br>(i) for the classification type ongoing for the service group home support—SAH class 1, 2, 3, 4, 5, 6, 7 or 8;<br>(ii) for the classification type short-term for the service group home support—SAH restorative care pathway or SAH end-of-life pathway |
| 4 | AT CHSP | The classification level CHSP class has been established for the individual for the classification type ongoing for the service group home support |



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*Transitional classification levels*

(2) The transitional classification level for the classification type short-term for the service group assistive technology is AT transitional.

Note: The transitional classification level for the classification type short-term for the service group assistive technology is established for certain classes of individuals to whom subsection 2(2) of Schedule 2 to the *Aged Care (Consequential and Transitional Provisions) Act 2024* applies—see the *Aged Care (Consequential and Transitional Provisions) Determination 2025*.

## 81-27 Service group assistive technology—classification type hospital transition

The following table sets out the classification levels for the classification type hospital transition for the service group assistive technology and the criteria for the classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | AT HT class | The individual:<br>(a) has an access approval that includes the service type assistance with transition care; and<br>(b) is in the concluding stage of a hospital episode; and<br>(c) is medically stable; and<br>(d) has the potential to benefit from accessing funded aged care services delivered through the service group assistive technology under the TCP; and<br>(e) was admitted to a hospital at the time the aged care needs assessment was carried out |

## 81-30 Service group home modifications—classification type short-term

*Non-transitional classification levels and criteria*

(1) The following table sets out the non-transitional classification levels for the classification type short-term for the service group home modifications and the criteria for those classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | HM low | The individual:<br>(a) needs low cost home modifications to mitigate functional decline or impairment and enable them to safely live in their home and community; and<br>(b) has had any of the following classification levels established for the individual:<br>(i) for the classification type ongoing for the service group |



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**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| | | home support—SAH class 1, 2, 3, 4, 5, 6, 7 or 8;<br>(ii) for the classification type short-term for the service group home support—SAH restorative care pathway |
| 2 | HM medium | The individual:<br>(a) needs medium cost home modifications to mitigate functional decline or impairment and enable them to safely live in their home and community; and<br>(b) has had any of the following classification levels established for the individual:<br>(i) for the classification type ongoing for the service group home support—SAH class 1, 2, 3, 4, 5, 6, 7 or 8;<br>(ii) for the classification type short-term for the service group home support—SAH restorative care pathway |
| 3 | HM high | The individual:<br>(a) needs high cost home modifications to mitigate functional decline or impairment and enable them to safely live in their home and community; and<br>(b) has had any of the classification levels SAH class 1, 2, 3, 4, 5, 6, 7 or 8 for the classification type ongoing for the service group home support established for the individual |
| 4 | HM CHSP | The classification level CHSP class has been established for the individual for the classification type ongoing for the service group home support |

*Transitional classification levels*

(2) The transitional classification level for the classification type short-term for the service group home modifications is HM transitional.

Note: The transitional classification level for the classification type short-term for the service group home modifications is established for certain classes of individuals to whom subsection 2(2) of Schedule 2 to the *Aged Care (Consequential and Transitional Provisions) Act 2024* applies—see the *Aged Care (Consequential and Transitional Provisions) Determination 2025*.

## 81-35 Service group residential care—classification type ongoing

*Non-transitional classification levels and criteria*

(1) The non-transitional classification levels for the classification type ongoing for the service group residential care are as follows:
(a) class 0; and
(b) the classification levels set out in the following table.

(2) The following table sets out the criteria for the classification levels set out in the table.



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**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | Class 1 | The circumstances referred to in paragraph 76-15(10)(a) of this instrument (palliative care pathway) apply to the individual |
| 2 | Class 2 | The individual:<br>(a) is independently mobile; and<br>(b) does not have significant compounding factors |
| 3 | Class 3 | The individual:<br>(a) is independently mobile; and<br>(b) has significant compounding factors |
| 4 | Class 4 | The individual:<br>(a) is mobile only with assistance; and<br>(b) has higher cognitive ability; and<br>(c) does not have significant compounding factors |
| 5 | Class 5 | The individual:<br>(a) is mobile only with assistance; and<br>(b) has higher cognitive ability; and<br>(c) has significant compounding factors |
| 6 | Class 6 | The individual:<br>(a) is mobile only with assistance; and<br>(b) has medium cognitive ability; and<br>(c) does not have significant compounding factors |
| 7 | Class 7 | The individual:<br>(a) is mobile only with assistance; and<br>(b) has medium cognitive ability; and<br>(c) has significant compounding factors |
| 8 | Class 8 | The individual:<br>(a) is mobile only with assistance; and<br>(b) has lower cognitive ability |
| 9 | Class 9 | The individual:<br>(a) is not mobile; and<br>(b) has higher function; and<br>(c) does not have significant compounding factors |
| 10 | Class 10 | The individual:<br>(a) is not mobile; and<br>(b) has higher function; and<br>(c) has significant compounding factors |
| 11 | Class 11 | The individual:<br>(a) is not mobile; and<br>(b) has lower function; and<br>(c) has lower pressure sore risk |
| 12 | Class 12 | The individual: |



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**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| | | (a) is not mobile; and<br>(b) has lower function; and<br>(c) has higher pressure sore risk; and<br>(d) does not have significant compounding factors |
| 13 | Class 13 | The individual:<br>(a) is not mobile; and<br>(b) has lower function; and<br>(c) has higher pressure sore risk; and<br>(d) has significant compounding factors |

*Transitional classification levels and criteria*

(3) The transitional classification levels for the classification type ongoing for the service group residential care are as follows:
  (a) RCO MPSP;
  (b) RCO NATSIFACP.

Note: Transitional classification levels for the classification type ongoing for the service group residential care are established for certain classes of individuals to whom subsection 3(2) of Schedule 2 to the *Aged Care (Consequential and Transitional Provisions) Act 2024* applies—see the *Aged Care (Consequential and Transitional Provisions) Determination 2025*.

## 81-40 Service group residential care—classification type short-term

*Non-transitional classification levels and criteria*

(1) The non-transitional classification levels for the classification type short-term for the service group residential care are as follows:
  (a) respite class 0; and
  (b) the classification levels set out in the following table.

(2) The following table sets out the criteria for the classification levels set out in the table.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | Respite class 1 | The individual is independently mobile |
| 2 | Respite class 2 | The individual is mobile only with assistance |
| 3 | Respite class 3 | The individual is not mobile |



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*Transitional classification levels*

(3) The transitional classification level for the classification type short-term for the service group residential care are as follows:

(a) RCST MPSP;

(b) RCST NATSIFACP.

Note: Transitional classification levels for the classification type short-term for the service group residential care are established for certain classes of individuals to whom subsection 3(2) of Schedule 2 to the *Aged Care (Consequential and Transitional Provisions) Act 2024* applies—see the *Aged Care (Consequential and Transitional Provisions) Determination 2025*.

## 81-42 Procedure for deciding initial classification levels for individuals for the classification types ongoing and short-term for the service group residential care

*Classification type ongoing*

(1) The procedure for establishing a classification level for an individual to whom paragraph 78(1)(c) of the Act applies for the classification type ongoing for the service group residential care is to establish the classification level class 0 for the individual.

*Classification type short-term*

(2) The procedure for establishing a classification level for an individual to whom paragraph 78(1)(c) of the Act applies for the classification type short-term for the service group residential care is to establish the classification level respite class 0 for the individual.

## 81-45 Service group residential care—classification type hospital transition

The following table sets out the classification levels for the classification type hospital transition for the service group residential care and the criteria for the classification levels.

**Classification levels and criteria**

| Item | Column 1<br>Classification level | Column 2<br>Criteria |
|---|---|---|
| 1 | RC HT class | The individual:<br>(a) has an access approval that includes the service type assistance with transition care; and<br>(b) is in the concluding stage of a hospital episode; and<br>(c) is medically stable; and<br>(d) has the potential to benefit from accessing funded aged care services delivered through the service group residential care under the TCP; and<br>(e) was admitted to a hospital at the time the aged care needs assessment was carried out |



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# Part 4—Prioritisation

## Division 1—Prioritisation assessments

### 84-5 When prioritisation assessments not required for classification type ongoing for service group home support

For the purposes of paragraph 84(2)(b) of the Act, the circumstances for an individual are that the classification level CHSP class for the classification type ongoing for the service group home support has been established for the individual by a classification decision.



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# Division 2—Priority category decisions

## 86-5 All service groups—period in which priority category decisions must be made

For the purposes of subsection 86(3) of the Act, the period of 14 days after the day on which the System Governor is given a prioritisation report under section 85 of the Act for an individual is prescribed.

## 86-10 Service group residential care—priority category 1—areas

For the purposes of subparagraph 86(5)(a)(ii) of the Act, an area with a 2023 MM category known as MM 5, MM 6 or MM 7 is prescribed.



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# Division 3—Priority categories for classification types for service groups

## Subdivision A—Service group home support

### 87-5 Priority categories and eligibility criteria for classification type ongoing

(1) For the purposes of subsection 87(1) of the Act:

(a) column 1 of the following table prescribes the priority categories for the classification type ongoing for the service group home support; and

(b) column 2 of the following table prescribes eligibility criteria for each of those priority categories.

**Classification type ongoing for service group home support**

| Item | Column 1<br>Priority categories | Column 2<br>Eligibility criteria |
|---|---|---|
| 1 | Urgent | The individual has 5 or more points determined in accordance with subsection (2) |
| 2 | High | The individual has 4 points determined in accordance with subsection (2) |
| 3 | Medium | The individual has 2 or 3 points determined in accordance with subsection (2) |
| 4 | Standard | The individual has 1 point, or no points, determined in accordance with subsection (2) |

*Points*

(2) For the purposes of column 2 of the table in subsection (1), an individual's points are determined by adding together the points specified in column 2 of each item of the following table in which column 1 specifies circumstances that apply to the individual (if any).

**Circumstances and points for eligibility criteria**

| Item | Column 1<br>Circumstances | Column 2<br>Points |
|---|---|---|
| 1 | The individual lives alone | 1 |
| 2 | The individual has a cognitive impairment | 1 |
| 3 | The individual is an Aboriginal or Torres Strait Islander person | 1 |
| 4 | The individual is homeless, or at risk of homelessness | 1 |
| 5 | The individual has a need for urgent access to ongoing funded aged care services through the service group home support | 2 |
| 6 | The individual:<br>(a) has waited more than 6 months from:<br>(i) the day on which the individual applied for access to funded aged | 1 |



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**Circumstances and points for eligibility criteria**

| Item | Column 1<br>Circumstances | Column 2<br>Points |
|---|---|---|
| | care services; or<br>(ii) if the individual applied for the reassessment of the individual's need for funded aged care services—the day on which the individual applied for the reassessment; and<br>(b) resides in an area with a 2023 MM category known as MM 5, MM6 or MM 7 | |
| 7 | The individual had motor neurone disease at the time the individual's most recent aged care needs assessment was undertaken | 5 |

## Subdivision B—Service groups assistive technology and home modifications

### 87-7 Priority categories and eligibility criteria

*Classification type ongoing or short-term for service group assistive technology*

(1) For the purposes of subsection 87(1) of the Act:
(a) column 1 of the following table prescribes the priority categories for the classification types ongoing and short-term for the service group assistive technology; and
(b) column 2 of the following table prescribes eligibility criteria for each of those priority categories.

**Classification type ongoing or short-term for service group assistive technology**

| Item | Column 1<br>Priority categories | Column 2<br>Eligibility criteria |
|---|---|---|
| 1 | Immediate | Either of the following circumstances applies to the individual:<br>(a) an approved needs assessor considers the classification type short-term for the service group home support should be approved for the individual;<br>(b) the individual had motor neurone disease at the time the individual's most recent aged care needs assessment was undertaken |
| 2 | High | Two or more of the following circumstances apply to the individual:<br>(a) the individual lives alone;<br>(b) the individual has a mobility impairment;<br>(c) the individual is an Aboriginal or Torres Strait Islander person;<br>(d) the individual's current place of residence poses a moderate or severe risk to the individual's health or safety;<br>(e) both of the following apply:<br>(i) the individual has waited more than 6 months from the day on which the individual made the application to which the priority assessment relates;<br>(ii) the individual resides in an area with a 2023 MM category |



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| **Classification type ongoing or short-term for service group assistive technology** | | |
|---|---|---|
| **Item** | **Column 1**<br>**Priority categories** | **Column 2**<br>**Eligibility criteria** |
| | | known as MM 5, MM 6 or MM 7<br>Note: The application referred to in subparagraph (e)(i) could be an application for access to funded aged services under section 56 of the Act or an application for an aged care needs reassessment under section 64 of the Act. |
| 3 | Medium | One circumstance referred to in column 2 of item 2 applies to the individual |
| 4 | Standard | None of the circumstances referred to in column 2 of item 2 apply to the individual |

*Classification type short-term for service group home modifications*

(2) For the purposes of subsection 87(1) of the Act:

(a) column 1 of the following table prescribes the priority categories for the classification type short-term for the service group home modifications; and

(b) column 2 of the following table prescribes eligibility criteria for each of those priority categories.

| **Classification type short-term for service group home modifications** | | |
|---|---|---|
| **Item** | **Column 1**<br>**Priority categories** | **Column 2**<br>**Eligibility criteria** |
| 1 | Immediate | Either of the following circumstances applies to the individual:<br>(a) an approved needs assessor considers the classification type short-term for the service group home support should be approved for the individual;<br>(b) the individual had motor neurone disease at the time the individual's most recent aged care needs assessment was undertaken |
| 2 | High | Two or more of the following circumstances apply to the individual:<br>(a) the individual lives alone;<br>(b) the individual has a mobility impairment;<br>(c) the individual is an Aboriginal or Torres Strait Islander person;<br>(d) the individual's current place of residence poses a moderate or severe risk to the individual's health or safety;<br>(e) both of the following apply:<br>(i) the individual has waited more than 6 months from the day on which the individual made the application to which the priority assessment relates;<br>(ii) the individual resides in an area with a 2023 MM category known as MM 5, MM 6 or MM 7<br>Note: The application referred to in subparagraph (e)(i) could be an application for access to funded aged services under section 56 of the Act or an application for an aged care needs reassessment under section 64 of the Act. |



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Section 87-7

**Classification type short-term for service group home modifications**

| Item | Column 1<br>Priority categories | Column 2<br>Eligibility criteria |
|---|---|---|
| 3 | Medium | One circumstance referred to in column 2 of item 2 applies to the individual |
| 4 | Standard | None of the circumstances referred to in column 2 of item 2 apply to the individual |



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# Division 4—Circumstances for urgency ratings—service group residential care

## 87-10 Classification type ongoing

(1) For the purposes of subsection 87(2) of the Act, this section prescribes the circumstances in which an urgency rating of low, medium or high will apply to an individual in relation to the classification type ongoing for the service group residential care.

*High*

(2) An urgency rating of high will apply to the individual if the individual has a need for immediate access to ongoing funded aged care services delivered in an approved residential care home which, if not met, may place the individual's safety, health or wellbeing at risk.

*Medium*

(3) An urgency rating of medium will apply to the individual if:
  (a) the circumstances mentioned in subsection (2) do not apply to the individual; and
  (b) taking into account the individual's circumstances and preferences, the individual is expected to seek access to ongoing funded aged care services delivered in an approved residential care home within the next 6 months.

*Low*

(4) An urgency rating of low will apply to the individual if:
  (a) the circumstances mentioned in subsection (2) do not apply to the individual; and
  (b) the circumstances mentioned in paragraph (3)(b) do not apply to the individual.



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# Part 5—Place allocation

## Division 1—Allocation of places to individuals

### Subdivision A—Allocation of places to individuals

#### 92-5 When allocation of places to individuals does not apply for classification type ongoing for service group home support

For the purposes of paragraph 92(1A)(b) of the Act, the circumstances for an individual are that the classification level CHSP class for the classification type ongoing for the service group home support has been established for the individual by a classification decision.

#### 92A-5 When a place is in effect—period for start day—service groups home support, assistive technology and home modifications

For the purposes of subsection 92A(2) of the Act, the period for a place allocated under subsection 92(1) of the Act to an individual is:

(a) 56 days from the day the place is allocated to the individual; or

(b) if, within the period referred to in paragraph (a), the individual notifies the System Governor that the individual needs an extension to that period—84 days from the day the place is allocated to the individual.

#### 92A-10 When a place ceases to have effect—service groups home support, assistive technology and home modifications

For the purposes of subsection 92A(4) of the Act, the time when a place allocated under subsection 92(1) of the Act to an individual for a classification type for the service group home support, assistive technology or home modifications ceases to have effect is the end of the earlier of the following (as applicable):

(a) for a place allocated to an individual for whom a classification level has been established for the classification type for the service group by a classification decision—the period of effect for the classification level;

(b) the day the individual dies.

### Subdivision B—Method for allocation

#### 93-5 Purpose of this Subdivision

For the purposes of subsection 93(2) of the Act, this Subdivision prescribes methods that the System Governor must follow in deciding the order of allocation under subsection 93(1) of the Act for:

(a) the classification type ongoing for the service group home support; and

(b) the classification types ongoing and short-term for the service group assistive technology; and



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(c) the classification type short-term for the service group home modifications.

## 93-10 Classification type ongoing for the service group home support—method for deciding order of allocation

The method for deciding the order of allocation of a place to an individual for the classification type ongoing for the service group home support is as follows.

| | |
|---|---|
| Step 1: | Identify the individuals (if any) to whom the circumstances referred to in paragraph 64-10(d) of this instrument apply. |
| Step 2: | Allocate the place to the individual who has the longest current wait time. |
| Step 3. | Using the method in section 93-12, identify if a place is assigned to a priority category for the classification type for the service group. |
| Step 4. | Identify whether the place is a full place or an interim place. |
| Step 5. | Identify the individuals (if any) who have that priority category for the classification type for the service group and whether those individuals have an interim place in effect for the classification type for the service group. |
| Step 6. | If the place is a full place, allocate the place to the individual who, of the individuals to whom a place has not already been allocated under step 2, has the longest current wait time. |
| Step 7. | If the place is an interim place, allocate the place to the individual who, of the individuals who do not have an interim place in effect for the classification type for the service group and to whom a place has not already been allocated under step 2, has the longest current wait time. |

## 93-12 Classification type ongoing for the service group home support—method for assigning places to priority categories

The method for assigning a place to a priority category for the classification type ongoing for the service group home support on a day is as follows.

| | |
|---|---|
| Step 1. | For each priority category for the classification type for the service group, identify the number of waiting individuals for whom the current wait time is equal to or greater than the target priority category wait time for that priority category on the day. |
| Step 2. | Add up the numbers identified for each priority category under Step 1. |
| Step 3. | Identify the number of places that are available to be allocated to individuals for that service group on the day (as worked out in |



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accordance with the method determined under subsection 91(1) of the Act).

Step 4. If the number worked out under Step 2 is less than or equal to the number identified at Step 3, divide the number worked out under Step 2 by the number identified at Step 3.

Step 5. If the number worked out under Step 2 is greater than the number identified at Step 3, divide the number identified at Step 3 by the number worked out under Step 2.

Step 6. For each priority category:

(a) multiply the number identified under step 1 by the result of Step 4 or 5 (as applicable) and round down to the nearest whole number; and

(b) if the result under paragraph (a) is a number greater than zero, that number of places are assigned to that priority category on the day; and

(c) if the result under paragraph (a) is zero, no places are assigned to that priority category on the day.

## 93-13 Classification type ongoing for the service group home support—method for working out wait time factor

The method for working out the ***wait time factor*** is as follows.

Step 1. Reduce the target classification type wait time most recently determined by the System Governor under section 93-14 by the priority category waiting proportion for the priority category urgent.

Step 2. For each remaining priority category, multiply the priority category waiting proportion by the queue rate.

Step 3. Add up the results of step 2 for each priority category.

Step 4. Divide the result of step 1 by the result of step 3.

## 93-14 Classification type ongoing for the service group home support—method for System Governor determination of target classification type wait time

The System Governor must, from time to time, determine the ***target classification type wait time*** (in months) for the classification type ongoing for the service group home support by considering:



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(a) on the first occasion the System Governor does so—the average of the old Act wait times; or

(b) on any subsequent occasion:

(i) the weighted average of the current wait times for individuals who have been assigned priority categories for the classification type for the service group; and

(ii) what change needs to be made to the previous target classification type wait time to ensure that, on and after 1 November 2027, the weighted average mentioned in subparagraph (i) is not more than 3 months.

## 93-15 Classification types ongoing and short-term for the service group assistive technology—method for deciding order of allocation

The method for deciding the order of allocation of a place to an individual for the classification type ongoing or short-term for the service group assistive technology is as follows.

| | |
|---|---|
| Step 1. | Identify the individuals (if any) who have, or have had, the classification level SAH end-of-life pathway for the classification type short-term for the service group home support in effect. |
| Step 2: | Allocate the place to the individual who has the longest current wait time. |
| Step 3: | Identify the waiting individuals (if any) who have the priority category immediate for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 4. | Of those individuals, allocate the place to the individual who has the longest current wait time. |
| Step 5. | If no individuals are identified under step 3, identify the waiting individuals (if any) who have the priority category high for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 6. | Of those individuals, allocate the place to the individual who has the longest current wait time. |
| Step 7. | If no individuals are identified under step 5, identify the waiting individuals (if any) who have the priority category medium for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 8. | Of those individuals, allocate the place to the individual who has the longest current wait time. |



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| | |
|---|---|
| Step 9. | If no individuals are identified under step 7, identify the waiting individuals who have the priority category standard for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 8. | Of those individuals, allocate the place to the individual who has the longest current wait time. |

## 93-20 Classification type short-term for the service group home modifications—method for deciding order of allocation

The method for deciding the order of allocation of a place to an individual for the classification type short-term for the service group home modifications is as follows.

| | |
|---|---|
| Step 1: | Identify the individuals (if any) who have had the classification level SAH end-of-life pathway for the classification type short-term for the service group home support in effect. |
| Step 2: | Allocate the place to the individual who has the longest current wait time. |
| Step 3. | Identify the waiting individuals (if any) who have the priority category immediate for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 4. | Of those individuals, allocate the place to the individual who has the longest current wait time. |
| Step 5. | If no individuals are identified under step 3, identify the waiting individuals (if any) who have the priority category high for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 6. | Of those individuals, allocate the place to the individual who has the longest current wait time. |
| Step 7. | If no individuals are identified under step 5, identify the waiting individuals (if any) who have the priority category medium for the classification type for the service group and to whom a place has not already been allocated under step 2. |
| Step 8. | Of those individuals, allocate the place to the individual who has the longest current wait time. |
| Step 9. | If no individuals are identified under step 7, identify the waiting individuals who have the priority category standard for the classification type for the service group and to whom a place has not already been allocated under step 2. |



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| | |
|---|---|
| Step 10. | Of those individuals, allocate the place to the individual who has the longest current wait time. |



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# Division 2—Allocation of a place to registered providers for certain specialist aged care programs

## 95-5 Allocation of a place—System Governor may invite application for allocation of TCP place

(1) For the purposes of subsection 95(2) of the Act, the System Governor may invite an entity to apply on its own behalf or on behalf of another entity (the ***nominated entity)*** for the allocation of a place for the delivery of funded aged care services under the TCP in a State or Territory.

(2) An application under subsection 95(2) of the Act may only be made in accordance with an invitation under subsection (1).

## 95-10 Allocation of a place—application for allocation of a TCP place

(1) An application under subsection 95(2) of the Act for the allocation of a place for delivering funded aged care services under the TCP in a State or Territory must specify the following:
  (a) whether the application is being made:
    (i) on an entity's own behalf; or
    (ii) on behalf of a nominated entity to which the place is to be allocated;
  (b) the number of places applied for;
  (c) the number of those places that will be used to prioritise delivery of funded aged care services to Aboriginal or Torres Strait Islander persons;
  (d) the area or areas in which the places will be used to deliver funded aged care services.

(2) For each area specified under paragraph (1)(d), the application must also specify the following:
  (a) the estimated number of persons residing in the area or areas who are aged at least 70, other than Aboriginal or Torres Strait Islander persons;
  (b) the estimated number of Aboriginal or Torres Strait Islander persons residing in the area or areas who are aged at least 50;
  (c) the estimated number of persons mentioned in paragraph (a) or (b) who are expected to be discharged from hospital;
  (d) the estimated number of persons expected to be discharged from hospital as mentioned in paragraph (c) annually, or within another relevant time period, who are expected to benefit from funded aged care services delivered under the TCP after discharge;
  (e) if the application is made on behalf of one or more nominated entities to which the places are to be allocated—the nominated entity or entities that will deliver funded aged care services under each place applied for.

(3) For the purposes of subsection (2), the number of persons for an area specified in an application is to be expressed as a whole number per thousand of the total number of persons residing in the area.



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(4) To avoid doubt:

(a) an area specified under paragraph (1)(d) may be the whole or a part of a State or Territory; and

(b) an application made on behalf of a nominated entity is made by the nominated entity for the purposes of the Act.

Note: The System Governor may request further information, which must be given within 14 days of the request (see section 588 of the Act and section 588-5 of this instrument).

## 95-15 Allocation of a place—matters of which System Governor must be satisfied before allocating TCP place

For the purposes of subparagraph 95(4)(a)(ii) of the Act, the System Governor must be satisfied of the following before allocating a place to an entity for the delivery of funded aged care services under the TCP in a State or Territory:

(a) that the place, if allocated, will be used by the entity to which the place is allocated to deliver funded aged care services in accordance with an agreement between the Commonwealth and the State or Territory made under paragraph 247(1)(b) of the Act;

(b) that, if the place is allocated to a nominated entity, the entity was nominated having regard to the community's needs.

## 97-5 When a place is in effect—basic rules

*When a place comes into effect*

(1) For the purposes of subsection 97(1) of the Act, a place allocated under subsection 95(1) of the Act to an entity for a specialist aged care program comes into effect on the day after the first day on which the entity satisfies the following conditions:

(a) the entity is a registered provider that is registered for a registration category for one or more service groups through which the entity will deliver funded aged care services under the specialist aged care program;

(b) if the place is to be used to deliver funded aged care services under the MPSP—an agreement with the entity is in force under paragraph 247(1)(a) of the Act;

(c) if the place is to be used to deliver funded aged care services under the TCP in a State or Territory—an agreement with the State or Territory covering the arrangements for the delivery of those services is in force under paragraph 247(1)(b) of the Act;

(d) if the place is to be used to deliver funded aged care services in an approved residential care home—the System Governor is satisfied that a bed in the approved residential aged care home is ready to be used for the delivery of funded aged care services under the place.

Note: A place may, in certain circumstances, be allocated a second time under subsection 95(1) of the Act (see section 97-25).



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*Period for which a place is in effect*

(2) A place that comes into effect under subsection (1) is in effect for the period:
   (a) starting on the day on which the place comes into effect; and
   (b) ending on the day on which the place ceases to be in effect under section 97-15;

but is not in effect for any period for which it is temporarily not in effect under section 97-10.

*Place cannot come into effect if 5 year delay*

(3) A place allocated under subsection 95(1) of the Act to an entity for a specialist aged care program does not come into effect, and can never come into effect, if the place has not come into effect under subsection (1) of this section within 5 years after the day on which the place was allocated.

*Entity may be required to complete application to confirm that a bed is ready to be used*

(4) For the purposes of paragraph (1)(d), the System Governor may require an entity to complete an application process published on the Department's website.

## 97-10 When a place is in effect—temporary cessation

(1) For the purposes of subsection 97(1) of the Act, this section prescribes the circumstances in which a place that has been allocated under subsection 95(1) of the Act to an entity, and has come into effect, is temporarily not in effect.

*Entity does not have capacity to deliver funded aged care services*

(2) The place is not in effect for a period during which the System Governor and the entity agree that the entity does not have the capacity to deliver funded aged care services under the place.

Note: Section 167 of the Act, and rules made under that section, require certain registered providers to give notice of certain changes in circumstances to the System Governor.

*Suspension of entity's registration*

(3) The place is not in effect for any period of suspension, under subsection 129(1) of the Act, of the registration of the entity.

*Condition makes it impracticable to deliver funded aged care services*

(4) A place is not in effect for a period if:
   (a) a condition is placed on the entity; and
   (b) the condition makes it impracticable for the entity to deliver funded aged care services under the place for that period.

(5) A place that is not in effect for a period comes back into effect immediately after the end of the period.



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## 97-15 When a place is in effect—permanent cessation

(1) For the purposes of subsection 97(1) of the Act, a place that has been allocated under subsection 95(1) of the Act to an entity, and has come into effect, ceases to be in effect, and can never resume to have effect, if:

(a) the entity ceases to be a registered provider; or

(b) the System Governor and the entity agree, in accordance with subsection (2) of this section, that the entity may relinquish the place; or

(c) the System Governor revokes the place under subsection (3) of this section.

Note: A place allocated under subsection 95(1) of the Act may be brought back into effect for allocation to another entity in certain circumstances (see section 97-25).

(2) The System Governor must not agree to the relinquishment of a place under paragraph (1)(b) unless the System Governor is satisfied that the entity to which the place was allocated has complied with the requirements prescribed by Division 4 of Part 4 of Chapter 4 of this instrument (which deals with continuity of delivery of funded aged care services).

(3) The System Governor may revoke a place if the entity has not used the place to deliver funded aged care services for a period of 12 months or more.

## 97-25 When a place is in effect—reallocation of TCP place

(1) For the purposes of subsection 97(1) of the Act, if:

(a) the Minister determines under paragraph 94(1)(a) of the Act that a place is available for allocation for use in a specified State or Territory; and

(b) the place is allocated to an entity under subsection 95(1) of the Act; and

(c) the place would, apart from this section, cease permanently to be in effect under section 97-15;

the State or Territory specified in the determination may apply to the System Governor for the place to be allocated to another entity.

(2) If the State or Territory makes an application to the System Governor under subsection (1):

(a) the System Governor may decide to allocate the place to the other entity under subsection 95(1) of the Act; and

(b) if the System Governor so decides, the place comes into effect in accordance with section 97-5.

## 98-5 Transfer of places

(1) For the purposes of subsection 98(1) of the Act, this section prescribes circumstances in which a place allocated under subsection 95(1) of the Act to the first entity may be transferred to the second entity.

(2) One circumstance is that:

(a) the place is for delivering funded aged care services under the TCP in a State or Territory; and



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(b) the State or Territory makes an application to the System Governor for the place to be transferred to the second entity; and
(c) the second entity is a registered provider delivering funded aged care services under the TCP in the State or Territory.

(3) Another circumstance is that:
(a) the place is for delivering funded aged care services under the MPSP in a State or Territory; and
(b) the State or Territory makes an application to the System Governor for the place to be transferred to the second entity; and
(c) the second entity is a registered provider delivering funded aged care services under the MPSP in the State or Territory; and
(d) the first entity and second entity agree to the transfer.

## 99-5 Conditions that apply to an allocated place

For the purposes of paragraph 99(1)(f) of the Act, the following conditions are prescribed in relation to a place allocated by the System Governor to an entity under subsection 95(1) of the Act:

(a) the entity must notify the System Governor if the entity will not be able to, or does not intend to, use the place to deliver funded aged care services for a period of 12 months or more;
(b) for a place allocated for the MPSP:
(i) if the place is used for the delivery of funded aged care services through the service group residential care—the place must only be used by the entity at the approved residential care home specified in the notice given under subsection 96(1) of the Act in relation to the allocation of the place; and
(ii) if the place is used for the delivery of funded aged care services through a service group that is not the service group residential care—the place must only be used by the entity at a location specified in the notice given under subsection 96(1) of the Act in relation to the allocation of the place.

Note: Section 167 of the Act, and rules made under that section, have the effect that this condition of place allocation can be met by giving the Commissioner and the System Governor a notice under that section.

## 101-5 System Governor decision on whether to vary a condition—matters to which System Governor must have regard

For the purposes of paragraph 101(2)(a) of the Act, the matters to which the System Governor must have regard in considering whether to vary a condition that applies to a place allocated to an entity under subsection 95(1) of the Act are the following:

(a) the objectives of the specialist aged care program for which the place is allocated;
(b) the needs of the communities of which individual members are expected to be able to access funded aged care services delivered under the place;



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(c) the Statement of Principles;

(d) any information or documents given by the entity in relation to the variation of the condition.



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# Chapter 3—Provider registration

## Part 1—Introduction

### 104-1 Simplified outline of this Chapter

This Chapter relates to provider registration under Chapter 3 of the Act.

Part 2 of this Chapter relates to provider registration and residential care home approval process, and deals with:

(a) applications for registration as a registered provider and applications for renewal of registration; and

(b) requirements for audits or assessments of an entity's ability to conform with the Aged Care Quality Standards for a provider registration category; and

(c) applications for approval of a residential care home; and

(d) matters that must be included in a notice of a decision to register an entity as a registered provider or renew an entity's registration; and

(e) deeming certain classes of entities to be registered providers and matters that must be included in a determination that an entity is taken to be a registered provider.

Part 3 of this Chapter relates to variations, suspensions and revocations of the registration of a registered provider and variations of approvals of a residential care home, and deals with:

(a) the appointment by a registered provider of an eligible adviser to assist the provider to comply with the provider's conditions and obligations under Part 4 of Chapter 3 of the Act; and

(b) variations of approvals of residential care homes on the Commissioner's own initiative and on application by a registered provider; and

(c) matters that the Provider Register must include in relation to the registration of a registered provider.



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# Part 2—Provider registration and residential care home approval process

## Division 1—Applications for registration and registration decisions

### Subdivision A—Application fees, information for applications and decision making periods

#### 104-5 Application for registration—application fee

For the purposes of paragraph 104(2)(b) of the Act, the application fee for an application by an entity to be registered as a registered provider is the sum of:

(a) $3,270.00; and

(b) the sum of the amounts specified in column 2 of the following table for each provider registration category specified in the application as a provider registration category that the entity is applying to be registered in.

Note: Exemptions, waivers and refunds are dealt with in rules made for the purposes of section 597 of the Act.

**Amounts for provider registration categories**

| Item | Column 1<br>Provider registration category | Column 2<br>Amount ($) |
|---|---|---|
| 1 | Home and community services | 1,270.00 |
| 2 | Assistive technology and home modifications | 1,270.00 |
| 3 | Advisory and support services | 1,270.00 |
| 4 | Personal and care support in the home or community | 3,800.00 |
| 5 | Nursing and transition care | 3,800.00 |
| 6 | Residential care | 5,070.00 |

#### 104-15 Application for registration—other information

For the purposes of paragraph 104(3)(i) of the Act, the other information that must be specified in an application by an entity for registration is, for each provider registration category that the entity is applying to be registered in under which funded aged care services are delivered in a home or community setting—each local government area in which the entity intends to deliver funded aged care services.

Note: For provider registration categories, see subsection 11(3) of the Act and section 11-5 of this instrument.



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## 105-5 Application for registration—period for making decision

For the purposes of subsection 105(2) of the Act, the period within which the Commissioner must make a decision on an application by an entity for registration is 90 days from the later of the following:

(a) the day the Commissioner receives the application;

(b) if an audit finding referred to in subparagraph 109(2)(d)(i) of the Act is required in relation to one or more provider registration categories specified in the application—the day the Commissioner gives a copy of the final audit report to the entity under section 110-38 of this instrument.

Note: For provider registration categories prescribed for the purposes of paragraph 109(2)(d) of the Act, see section 109-5 of this instrument. For circumstances prescribed for the purposes of subparagraph 109(2)(d)(ii) of the Act, see section 109-10 of this instrument.

## 107-5 Application for renewal of registration—application fee

For the purposes of paragraph 107(2)(b) of the Act, the application fee for an application by a registered provider to renew the provider's registration is the sum of:

(a) $295.00; and

(b) the sum of the amounts specified in column 2 of the following table for each provider registration category specified in the application as a provider registration category that the registered provider is applying to be registered in.

Note: Exemptions, waivers and refunds are dealt with in rules made for the purposes of section 597 of the Act.

**Amounts for provider registration categories**

| Item | Column 1<br>Provider registration category | Column 2<br>Amount ($) |
|---|---|---|
| 1 | Home and community services | 770.00 |
| 2 | Assistive technology and home modifications | 770.00 |
| 3 | Advisory and support services | 770.00 |
| 4 | Personal and care support in the home or community | 4,040.00 |
| 5 | Nursing and transition care | 4,040.00 |
| 6 | Residential care | 5,380.00 |

## 108-5 Application for renewal of registration—period for making decision

For the purposes of subsection 108(2) of the Act, the period within which the Commissioner must make a decision on an application by a registered provider for renewal of registration is 90 days from the later of the following:

(a) the day the Commissioner receives the application for renewal;

(b) if an audit finding referred to in subparagraph 109(2)(d)(i) of the Act is required in relation to one or more provider registration categories specified in the application—the day the Commissioner gives a copy of the



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final audit report to the registered provider under section 110-38 of this instrument.

Note: For provider registration categories prescribed for the purposes of paragraph 109(2)(d) of the Act, see section 109-5 of this instrument. For circumstances prescribed for the purposes of subparagraph 109(2)(d)(ii) of the Act, see section 109-10 of this instrument.

## Subdivision B—Provider registration category specific requirements

### 109-5 Provider registration categories for which audit findings or prescribed circumstances are required

For the purposes of paragraph 109(2)(d) of the Act, the following provider registration categories are prescribed:

(a) personal and care support in the home or community;
(b) nursing and transition care;
(c) residential care.

### 109-10 Circumstances in which audit findings are not required—health service standards assessments

(1) For the purposes of subparagraph 109(2)(d)(ii) of the Act, the prescribed circumstances for an application by an entity for registration or renewal of registration in the following provider registration categories are:

(a) for residential care, in relation to the delivery, or proposed delivery, by the entity of funded aged care services through that provider registration category as part of an integrated service arrangement—that the entity satisfies subsection (2);
(b) for personal and care support in the home and community, and nursing or transition care, in relation to the delivery, or proposed delivery, by the entity of funded aged care services through those provider registration categories as part of an integrated service arrangement—that the entity satisfies subsection (3).

*Prescribed circumstances—residential care*

(2) An entity satisfies this subsection if:

(a) during the 3 years immediately preceding the date of the entity's application, a health service standards assessment was conducted; and
(b) the health service standards assessment assessed the entity's ability to comply with the Australian Health Service Safety and Quality Accreditation Scheme requirements for:
(i) the National Safety and Quality Health Service Standards; and
(ii) the Integrated Health and Aged Care Services Module (or a substantially equivalent module that was in force before the transition time);

that are equivalent to the Aged Care Quality Standards that apply to the provider registration category residential care, in relation to each



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residential care home in which the entity delivers, or proposes to deliver, funded aged care services as part of an integrated service arrangement; and

(c) the assessment:

(i) covers all the residential care homes in which the entity delivers, or proposes to deliver, funded aged care services as part of an integrated service arrangement; and

(ii) found that the entity is able to comply with those Standards and that Module for each residential care home.

Note: The effect of this subsection is that an audit is not required for residential care homes of an entity in an integrated service arrangement for which a health service standards assessment has been conducted and has found that the entity is able to comply with those Standards and that Module.

*Prescribed circumstances—personal and care support in the home or community and nursing or transition care*

(3) An entity satisfies this subsection if:

(a) during the 3 years immediately preceding the date of the application, a health service standards assessment was conducted; and

(b) the assessment assessed the entity's ability to comply with the Australian Health Service Safety and Quality Accreditation Scheme requirements for:

(i) the National Safety and Quality Health Service Standards; and

(ii) the Integrated Health and Aged Care Services Module (or a substantially equivalent module that was in force before the transition time);

that are equivalent to the Aged Care Quality Standards that apply to the provider registration category personal and care support in the home or community, or nursing and transition care (as applicable); and

(c) the assessment covers the provider registration categories personal and care support in the home and community, or nursing or transition care, through which the entity delivers, or proposes to deliver funded aged care services as part of an integrated service arrangement; and

(d) the assessment found that the entity is able to comply with those Standards and that Module for all of the funded aged care services delivered, or proposed to be delivered, through those provider registration categories.

Note: The effect of this subsection is that an audit is not required for funded aged care services provided through the registration categories personal and care support in the home or community, or nursing and transition care, as part of an integrated service arrangement, for which a health service standards assessment has been conducted and has found that entity is able to comply with those Standards and that Module.

*Meaning of health service standards assessment*

(4) A ***health service standards assessment*** means an assessment conducted:

(a) in accordance with Australian Health Service Safety and Quality Accreditation Scheme formulated and coordinated by the Australian Commission on Quality and Safety in Health Care; and

(b) by an accrediting agency approved by the Australian Commission on Quality and Safety in Health Care to assess health service organisations



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against the National Safety and Quality Health Service Standards and the Integrated Health and Aged Care Services Module (or a substantially equivalent module that was in force before the transition time).

## 109-15 Circumstances if audit finds nonconformance

For the purposes of subparagraph 109(2)(d)(ii) of the Act, circumstances for an application by an entity for registration or renewal of registration are that:

(a) an audit referred to in subparagraph 109(2)(d)(i) of the Act has found that the entity has not conformed with one or more of the Aged Care Quality Standards that apply to a provider registration category specified in the application; and

(b) the nonconformance:

(i) has been addressed; or

(ii) can be addressed through an update to a continuous improvement plan that outlines how the nonconformance will be addressed within a specified period; or

(iii) can be addressed by the imposition of a condition under section 143 of the Act.

## 109-25 Other requirements for registration, renewal or variation of registration—delivery of funded aged care services in certain circumstances

(1) For the purposes of paragraph 109(2)(e) of the Act, this section prescribes other requirements for an application by an entity for registration, renewal of registration or variation of registration in a provider registration category other than residential care.

*Delivery to certain individuals through the service group home support*

(2) If the entity delivers, or intends to deliver, funded aged care services to an individual:

(a) who has an access approval in effect for the classification type ongoing for the service group home support; or

(b) who has the classification level SAH end-of-life pathway in effect for the classification type short-term for the service group home support;

another requirement is that the entity:

(c) also delivers, or intends to deliver, the service type care management to the individual; and

(d) has applied for, and satisfies the requirements for, registration in the provider registration category personal and care support in the home or community.

*Delivery of services in service type restorative care management*

(3) If the entity delivers, or intends to deliver, funded aged care services in the service type restorative care management, another requirement is that the entity



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delivers, or intends to deliver, funded aged care services in the service type allied health and therapy.

*Section does not apply to specialist aged care programs*

(4) This section does not apply if the entity delivers, or intends to deliver, funded aged services only under a specialist aged care program.

*Section does not apply in respect of provider registration category subscription trial*

(5) This section does not apply to an application to the extent that the application is for renewal of registration or variation of registration in the provider registration category subscription trial.

## 109-30 Other requirements for renewal or variation of registration—provider registration category subscription trial

(1) For the purposes of paragraph 109(2)(e) of the Act, this section prescribes other requirements for an application by a registered provider under subsection 107(1) (renewal of registration) or 124(1) (variation of registration) of the Act that specifies the provider registration category subscription trial.

(2) The following paragraphs must apply to the registered provider:
   (a) the System Governor has invited the provider to be the recipient of a grant of financial assistance that would support the provider in delivering funded aged care services under the provider registration category subscription trial, and has not withdrawn the invitation;
   (b) the registered provider's registration covers each non-subscription service that corresponds to a subscription service that the registered provider delivers, or proposes to deliver.

## 109-35 Exemption from requirements for renewal or variation of registration—provider registration category subscription trial

For the purposes of subsection 109(4) of the Act, paragraphs 109(2)(a), (b) and (c) of the Act do not apply to an entity to the extent that the entity is applying for renewal of registration or variation of registration in the provider registration category subscription trial.



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# Division 2—Audit requirements

## Subdivision A—Purpose of this Division

### 110-5 Purpose of this Division

For the purposes of subsection 110(1) of the Act, this Division prescribes requirements relating to conducting an audit of an entity's ability to conform with the Aged Care Quality Standards for a provider registration category.

## Subdivision B—How audits must be conducted—general

### 110-13 Audit must be conducted

(1) An audit of an entity's ability to conform with the Aged Care Quality Standards for a provider registration category must be conducted by the Commissioner.

(2) The Commissioner must gather evidence relevant to the scope of the audit being conducted and may be assisted by persons with the technical knowledge or skill required to collect and interpret information relevant to the scope of the audit.

(3) Before the audit is undertaken, the Commissioner must give a notice to the entity that includes the following information:
  (a) a general description of the audit process;
  (b) that an audit is proposed to be conducted in accordance with the audit process;
  (c) the purpose of the audit, and the decision or decisions it will inform;
  (d) the scope of the audit, including the applicable Aged Care Quality Standards;
  (e) the period during which the audit is proposed to occur;
  (f) the applicable fees for the proposed audit;
  (g) advice on how individuals (if any) should be notified about opportunities to contribute to the audit;
  (h) a copy of the audit methodology;
  (i) any other information the Commissioner considers relevant.

## Subdivision C—How audits must be conducted—assessments of approved residential care homes

### 110-26 Assessments—approved residential care homes

(1) This section applies if an audit of a registered provider's ability to conform with the Aged Care Quality Standards relates to an application for renewal of the registered provider's registration in the provider registration category residential care.



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*Assessments of approved residential care homes*

(2) The Commissioner must, for each of the approved residential care homes included in the registered provider's registration, either:
  (a) conduct a home assessment of the residential care home; or
  (b) if the residential care home is covered by an integrated service arrangement—be satisfied that a health standards assessment has found it complies with the National Safety and Quality Health Service Standards and the Integrated Health and Aged Care Services Module (or a substantially equivalent module that was in force before the transition time).

*Scope of home assessment*

(3) A home assessment of an approved residential care home must assess whether the registered provider delivering funded aged care services in the approved residential care home conforms with the applicable Aged Care Quality Standards.

(4) A home assessment of an approved residential care home must, to the extent possible, consider the following matters as part of the home assessment:
  (a) the experience of individuals to whom funded aged care services are delivered in the approved residential care home;
  (b) documents and records relevant to the home assessment;
  (c) feedback from the following:
    (i) aged care workers of the registered provider;
    (ii) responsible persons of the registered provider;
    (iii) the governing body of the registered provider;
    (iv) third parties with relevant knowledge or experience;
  (d) observations at the approved residential care home;
  (e) care outcomes relating to the approved residential care home.

*Attendance at approved residential care homes*

(5) A home assessment of an approved residential care home must include the following:
  (a) attendance by the Commissioner at the approved residential care home;
  (b) such other means of assessment as are appropriate.

*Notice of home assessment*

(6) Before a home assessment of an approved residential care home included in the registration of a registered provider is conducted, the Commissioner must give written notice to the registered provider:
  (a) specifying each approved residential care home included in the registered provider's registration, and stating that a home assessment of each approved residential care home is to be conducted; and
  (b) requiring the registered provider to notify each individual to whom funded aged care services are delivered in each approved residential care home:



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(i) that the home assessment is going to be conducted; and

(ii) that the person or persons conducting the home assessment will attend the approved residential care home as part of the home assessment, and the day or days on which this will occur; and

(iii) that the person or persons attending the approved residential care home will seek the individual's consent before engaging with the individual, or entering a part of the approved residential care home that comprises the individual's personal space (for example, the individual's room).

*Rule if consent not obtained from individuals*

(7) If an individual to whom funded aged care services are delivered at an approved residential care home does not give consent as mentioned in subparagraph (6)(b)(iii), the persons conducting the home assessment will not enter any part of the approved residential care home that comprises the individual's personal space.

Note: This means that if none of the individuals accessing funded aged care services in the approved residential care homes included in a registered provider's registration give consent, the person or persons conducting the home assessments will not enter any parts of the approved residential care homes that comprises the personal space of an individual.

## 110-28 Preliminary assessment report of home assessment

(1) If a home assessment of an approved residential care home included in the registration of a registered provider is conducted, the Commissioner must prepare a preliminary assessment report setting out the findings of the home assessment.

(2) The preliminary assessment report must assign a grade for conformance with the Aged Care Quality Standards for the registered provider's delivery of funded aged care services in the approved residential care home as outlined in the audit methodology mentioned in paragraph 110-13(3)(h).

(3) The Commissioner must:

(a) give a copy of the preliminary assessment report to the registered provider within 14 days from the attendance at the approved residential care home for the home assessment; and

(b) give the registered provider the opportunity to respond in writing to the report within 14 days from the day the Commissioner gave the preliminary assessment report to the registered provider.

## 110-30 Final assessment report of home assessment

(1) If a home assessment of an approved residential care home included in the registration of a registered provider is conducted, the Commissioner must prepare a final assessment report setting out the findings of the home assessment.

(2) The final assessment report must assign a grade for conformance with the Aged Care Quality Standards for the registered provider's delivery of funded aged care



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services in the approved residential care home as outlined in the audit methodology mentioned in paragraph 110-13(3)(h).

(3) In preparing the final assessment report, the Commissioner must consider any response received from the registered provider as mentioned in paragraph 110-28(3)(b).

(4) The Commissioner must give a copy of the final assessment report to the registered provider within 28 days from:
  (a) if the registered provider responded in writing to the preliminary report within the period mentioned in paragraph 110-28(3)(b)—the day the Commissioner received the response; or
  (b) if the registered provider did not respond in writing to the preliminary report within the period mentioned in paragraph 110-28(3)(b)—the last day of that period.

(5) The registered provider must, within 14 days from receiving the final assessment report, notify the following of the findings of the home assessment (including the grade):
  (a) responsible persons and aged care workers of the registered provider who deliver funded aged care services in the approved residential care home;
  (b) individuals to whom funded aged care services are delivered in the approved residential care home.

## Subdivision D—How audits must be conducted—attendance at a service delivery location for home or community setting

### 110-32 Attendance at a service delivery location—home or community setting

(1) This section applies if:
  (a) an audit of a registered provider's ability to conform with the Aged Care Quality Standards relates to an application for renewal of the registered provider's registration; and
  (b) the scope of the audit includes funded aged care services delivered by the registered provider in a home or community setting.

*Notice of attendance at a service delivery location*

(2) If the scope of the audit includes attendance at a service delivery location, the Commissioner must give written notice to the registered provider:
  (a) stating that attendance at the service delivery location is to be conducted; and
  (b) stating whether or not one or more persons propose to attend individuals' residences or the premises of a community setting in conducting the attendance at the service delivery location; and
  (c) if the Commissioner proposes to attend the residence of one of more individuals—requiring the registered provider to notify the individuals:
    (i) that attendance at the service delivery location is going to be conducted; and



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(ii) that the Commissioner will attend individuals' residences as part of the attendance, and the day or days on which this will occur; and

(iii) that if the Commissioner proposes to attend the residence of a particular individual, the Commissioner will seek consent to enter the residence from the individual and any other person at their residence; and

(d) if the Commissioner proposes to attend the premises of a community setting—stating that the Commissioner will seek consent from the owner or occupier to enter the premises.

(3) If:

(a) an individual to whom funded aged care services are delivered at their residence does not give consent as mentioned in subparagraph (2)(c)(iii), attendance at the service delivery location must not include their residence; and

(b) if no such individuals give consent, attendance at the service delivery location must not be included in the scope of the audit.

(4) If it is not reasonably practicable to obtain consent from the owner or occupier of the premises where funded aged care services are delivered in a community setting, attendance at the service delivery location must not include those premises.

## Subdivision E—How audits must be conducted—final audit report

### 110-38 Final audit reports

(1) If an audit of an entity's ability to conform with the Aged Care Quality Standards is conducted, the Commissioner must prepare a final audit report.

(2) The final audit report must include the following:

(a) the period to which the audit relates;

(b) the scope of the audit;

(c) the audit outcomes;

(d) an assessment of the entity's conformance, or ability to conform, with the Aged Care Quality Standards;

(e) if the final audit report identifies nonconformance with the Aged Care Quality Standards—whether the entity has the ability to conform and the things the entity must do, or has done, to conform;

(f) the outcomes of the following (if any):

(i) each home assessment of an approved residential care home conducted in accordance with section 110-26 for the purposes of the audit;

(ii) each attendance at a service delivery location of residences or other premises conducted in accordance with section 110-32 for the purposes of the audit.

(3) The Commissioner must:



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(a) give a copy of the final audit report to the entity within 28 days of the completion of the audit; and

(b) give the entity the opportunity to respond in writing to the final audit report within 14 days from the day the final audit report was given to the entity, or such longer period as the Commissioner agrees with the entity.

(4) An entity that is a registered provider must, within 28 days from receiving the final audit report, notify the following of the outcomes of the audit:

(a) responsible persons and aged care workers of the registered provider;

(b) if the audit relates to an approved residential care home—any individuals to whom funded aged care services are delivered in the approved residential care home.

## Subdivision F—Type and scope of audits and other matters

### 110-40 Type and scope of audits for provider registration categories

*Provider registration categories*

(1) This section prescribes the types and scope of audits that can be conducted for the following provider registration categories:

(a) personal and care support in the home or community;

(b) nursing and transition care;

(c) residential care.

*Types of audit*

(2) The types of audit that may be conducted in respect of the provider registration categories referred to in subsection (1) are as follows:

(a) registration of an entity;

(b) renewal of an entity's registration;

(c) variation of an entity's registration.

*Scope of audit*

(3) The scope of an audit is to include (as applicable):

(a) whether the audit relates to registration of an entity, or the renewal or variation of an entity's registration; and

(b) the applicable Aged Care Quality Standards in respect of:

(i) the provider registration category or categories for which the audit is being conducted; and

(ii) service types to which the audit relates; and

(c) whether or not the audit will include any home assessments or attendance at any service delivery locations.

Note 1: For the provider registration category residential care, a home assessment of all approved residential care homes must be conducted for renewal of registration (see subsection 110-26(2)), except in certain circumstances specified in subsection 109-10(2), where there has been a health service standards assessment in the previous 3 years.



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Note 2: For the provider registration categories personal and care support in the home or community, and nursing and transition care, attendance at a service delivery location on renewal of registration may be conducted (see subsection 110-32(2)).

(4) The Commissioner must consider the following when determining the scope of the audit:

(a) matters or risks that are identified as requiring specific examination;

(b) in the case of renewal or variation of an entity's registration:

(i) the time that has elapsed since the most recent audit of the entity was conducted; and

(ii) the scope of any previous audits of the entity; and

(iii) the outcomes of previous audits of the entity; and

(iv) the history of compliance with the Act of the entity;

(c) any guidelines relating to audit matters made for the purposes of subsection 348(2) of the Act.

Note: In considering the scope of an audit, the Commissioner may consider an audit undertaken by another person or body, which the Commissioner recognises as equivalent to meeting the Quality Standards, despite the audit not having been undertaken by the Commissioner.

## Subdivision G—Fees payable for an audit

### 110-45 Audit fee—audit in connection with registration

(1) This section applies to an audit, for one or more of the following provider registration categories, in connection with an entity applying to be registered as a registered provider under subsection 104(1) of the Act in the provider registration category or categories:

(a) personal and care support in the home or community;

(b) nursing and transition care;

(c) residential care.

(2) The fee payable by the entity for an audit for one or more of the provider registration categories is:

(a) unless subsection (3) applies to the entity—$14,870.00; or

(b) if subsection (3) applies to the entity—$0.

(3) This subsection applies to an entity if, at the time of the application:

(a) the entity intends to deliver funded aged care services under the MPSP or the TCP; or

(b) the entity intends that 85% of the individuals that the entity is to deliver services to will be located in an area with a 2023 MM category known as MM 6 or MM7; or

(c) the entity is known as an Aboriginal Community Controlled Organisation.

### 110-47 Audit fee—audit in connection with registration renewal

(1) This section applies to an audit, for one or more of the following provider registration categories, in connection with the renewal of the registration of a



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registered provider under subsection 107(1) of the Act in the provider registration category or categories:

(a) personal and care support in the home or community;

(b) nursing and transition care;

(c) residential care.

(2) The fee payable by the registered provider for an audit for one or more of the provider registration categories is the sum of:

(a) in any case:

(i) unless subsection (3) applies to the registered provider—$7,890.00; or

(ii) if subsection (3) applies to the registered provider—$0; and

(b) for either or both of the provider registration categories personal and care support in the home or community and nursing and transition care:

(i) unless subsection (3) applies to the registered provider—the amount that applies to the registered provider under subsection (5); or

(ii) if subsection (3) applies to the registered provider—$0; and

(c) for the provider registration category residential care:

(i) unless subsection (4) applies to the registered provider—the sum of the amounts applicable under subsection (6) or (7) (as the case may be) for each of the approved residential care homes included in the registered provider's registration at the time of the application and for which home assessments must be conducted under subsection 110-26(2); or

(ii) if subsection (4) applies to the registered provider—$0.

*Circumstances in which a nil fee component is applicable*

(3) For the purposes of paragraphs (2)(a) and (b), this subsection applies to a registered provider if, on the day the Commissioner invites the provider under section 106 of the Act to renew the registration:

(a) the entity is a registered provider delivering funded aged care services only under the MPSP or the TCP; or

(b) the entity intends that all of the individuals that the entity is to deliver services to will be located in an area with a 2023 MM category known as MM 5, MM6 or MM7; or

(c) the Commissioner considers, based on the information held by the Commissioner as at that day, that the provider is in the smallest 10% of all registered providers based on the number of individuals to whom funded aged care services are delivered by the provider.

(4) For the purposes of paragraph (2)(c), this subsection applies to a registered provider if, on the day the Commissioner invites the provider under section 106 of the Act to renew the registration:

(a) the registered provider is delivering funded aged care services only under the MPSP or the TCP; or

(b) both of the following apply:



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(i) an approved residential care home included in the provider's registration is located in an area in the 2023 MM category known as MM 4, MM 5, MM 6 or MM 7;

(ii) the average number of occupied beds per day over the quarter immediately preceding the day the invitation was given was fewer than 25; or

(c) both of the following apply:

(i) an approved residential care home included in the provider's registration has specialised Aboriginal or Torres Strait Islander status or has specialised homeless status;

(ii) the average number of occupied beds per day over the quarter immediately preceding the day the invitation was given was fewer than 25.

*Personal and care support in the home or community and nursing and transition care*

(5) For the purposes of paragraph (2)(b), the amount that applies to the registered provider is:

(a) if, on the day the Commissioner invites the provider under section 106 of the Act to renew the registration, the registered provider delivers funded aged care services only to fewer than 30 individuals in a single State or Territory—$9,800.00; or

(b) if, on that day, the registered provider delivers funded aged care services only to between 30 and 342 individuals in a single State or Territory—$15,550.00; or

(c) if, on that day, the registered provider delivers funded aged care services to:

(i) 343 or more individuals in a single State or Territory; or

(ii) any number of individuals in more than one State or Territory;

$17,950.00.

*Residential care—standard amounts*

(6) For an approved residential care home to which subsection (8) does not apply, the amount is:

(a) for an approved residential care home with not more than 150 beds—$16,340.00; or

(b) for an approved residential care home with more than 150 beds but not more than 250 beds—$17,510.00; or

(c) for an approved residential care home with more than 250 beds—$18,670.00.

*Residential care—reduced amounts*

(7) For an approved residential care home to which subsection (8) applies, the amount is:



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(a) for an approved residential care home with not more than 150 beds—$8,170.00; or

(b) for an approved residential care home with more than 150 beds but not more than 250 beds—$8,760.00; or

(c) for an approved residential care home with more than 250 beds—$9,340.00.

(8) This subsection applies to an approved residential care home included in a registered provider's registration if:

(a) on the day the Commissioner invites the provider under section 106 of the Act to renew the registration, funded aged care services are delivered in the approved residential care home:

(i) in any case—to fewer than 25 individuals; or

(ii) if the approved residential care home is located in an area in the 2023 MM category known as MM category 4—to at least 25, and fewer than 30, individuals; or

(iii) if the approved residential care home is located in an area in the 2023 MM category known as MM category 5—to at least 25, and fewer than 39, individuals; or

(iv) if the approved residential care home is located in an area in the 2023 MM category known as MM category 6 or 7—to at least 25 individuals; or

(b) on the day the Commissioner invites the provider under section 106 of the Act to renew the registration:

(i) the approved residential care home has specialised Aboriginal and Torres Strait Islander status or specialised homeless status; and

(ii) funded aged care services are delivered in the approved residential care home to at least 25 individuals.

(9) For the purposes of paragraph (8)(b), the number of individuals is to be worked out by reference to the average number of occupied beds per day over the quarter immediately preceding the day the invitation was given.

## 110-49 Audit fee—audit in connection with variation of registration

(1) This section applies to an audit, for one or more of the following provider registration categories, in connection with a registered provider applying to vary the provider's registration to register the provider in the provider registration category or categories:

(a) personal and care support in the home or community;

(b) nursing and transition care;

(c) residential care.

(2) The fee payable by the registered provider for an audit for one or more of the provider registration categories is:

(a) unless subsection (3) applies to the registered provider—$18,530.00; or

(b) if subsection (3) applies to the registered provider—$0.



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(3) This subsection applies to an entity if, at the time of the application:
  (a) the entity intends to deliver funded aged care services under the MPSP or the TCP; or
  (b) the entity intends that 85% of the individuals that the entity is to deliver services to will be located in an area with a 2023 MM category known as MM 6 or MM7; or
  (c) the entity is known as an Aboriginal Community Controlled Organisation.

### 110-51 Audit fee—refunds

(1) This section prescribes circumstances in which any of the following may be refunded to an entity:
  (a) an audit fee prescribed in section 110-45 of this instrument in connection with an application under section 104 of the Act;
  (b) an audit fee prescribed in section 110-47 of this instrument in connection with an application under section 107 of the Act;
  (c) an audit fee prescribed in section 110-49 of this instrument in connection with an application under section 124 of the Act.

(2) The circumstances are that:
  (a) the entity makes the application; and
  (b) the entity pays, in connection with the application (whether before or after making the application), the audit fee; and
  (c) an audit of the entity is not conducted under section 110 of the Act.

## Subdivision H—Audit timeframes

### 110-53 Audit timeframes for provider registration categories

(1) Subsections (2) and (3) of this section prescribe the timeframes within which an audit must be conducted for the following provider registration categories:
  (a) personal and care support in the home or community;
  (b) nursing and transition care;
  (c) residential care.

(2) An audit in relation to an initial application for registration must be conducted during the period beginning on the date of the application and ending when the Commissioner makes a decision on the application.

(3) An audit in relation to a renewal of registration must be conducted during the period beginning on the date of the invitation to renew under subsection 106(1) of the Act and ending when the Commissioner makes a decision on the application.

(4) The Commissioner must not make a decision to register an entity, or renew a registration, until the final audit report is completed.



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# Division 3—Applications for approval of residential care homes

## 111-5 Application fee

For the purposes of paragraph 111(2)(b) of the Act, the application fee for an application by an entity for approval of a residential care home in relation to the entity is $3,800.00 for each residential care home specified in the application.

Note: Exemptions, waivers and refunds are dealt with in rules made for the purposes of section 597 of the Act.

## 111-10 Information for residential care homes

For the purposes of subparagraph 111(3)(b)(ii) of the Act, the other information for each residential care home specified in an application is as follows:

(a) the name of the residential care home;
(b) the street address of the residential care home;
(c) the name of the responsible person in charge of the residential care home;
(d) a copy of the certificate of occupancy or equivalent certificate (however described) for the residential care home, or any other document that specifies or relates to the number of beds in the residential care home or the total number of individuals that may occupy or reside in the residential care home;
(e) documentary evidence demonstrating that the building or buildings that comprise the residential care home are permanent and will not present a risk to the delivery of quality and safe residential care, including but not limited to:
  (i) floor plans and bed layout; and
  (ii) fixtures, furnishings and design; and
  (iii) an explanation of the design of the residential care home.

## 112-5 Period for making decision

For the purposes of subsection 112(2) of the Act, the period within which the Commissioner must make a decision on an application for approval of a residential care home in relation to an entity is:

(a) if the entity has made an application for registration under subsection 104(1) of the Act—the period that applies to that application under section 105-5 of this instrument; or
(b) if the entity is a registered provider—90 days from the day the Commissioner receives the application for approval of the residential care home.

## 113-5 Approval of residential care homes

For the purposes of subparagraph 113(b)(ii) of the Act, other requirements of which the Commissioner must be satisfied before approving a residential care home in relation to an entity are as follows:



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(a) either:
  (i) the entity owns the premises at which the residential care home is located; or
  (ii) if the entity does not own the premises at which the residential care home is located—the owner of the premises agrees to the entity using the premises as a residential care home;
(b) the building or buildings that comprise the residential care home are permanent; and
(c) the building or buildings that comprise the residential care home and its total number of beds will not present a risk to the delivery of quality and safe residential care.



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# Division 4—Notice of decisions and other provisions

## 114-5 Other matters for notices of decisions to register or renew

For the purposes of paragraph 114(3)(f) of the Act, other matters that must be included in a notice of a decision given to an entity are the details of each residential care home (if any) that is approved in relation to the entity.

## 117-5 Deemed registration—classes of entity

For the purposes of subsection 117(1) of the Act, the following classes of entity are prescribed:

(a) hospitals (whether operated by government entities or non-government entities);

(b) entities that are not registered providers, but that:

(i) deliver services similar to funded aged care services that are delivered in an approved residential care home; or

(ii) deliver services similar to funded aged care services that are delivered in a home or community setting or in a similar sector;

(c) registered providers that are not registered in all of the provider registration categories.

## 117-10 Deemed registration—other matters for determinations of deemed registration

For the purposes of paragraph 117(2)(h) of the Act, other matters that must be specified in a determination that an entity is taken to be a registered provider are as follows:

(a) the ABN of the entity;

(b) each responsible person of the entity;

(c) the business location of the entity.



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# Part 3—Variations, suspensions and revocations of registration

## Division 1—Variations, suspensions and revocations

### 124-5 Application fee—variation to add provider registration categories

For the purposes of paragraph 124(2)(b) of the Act, the application fee for an application by a registered provider to vary the provider's registration to register the provider in a new provider registration category is the sum of the amounts specified in column 2 of the following table for each new provider registration category that the provider is applying to be registered in.

Note: Exemptions, waivers and refunds are dealt with in rules made for the purposes of section 597 of the Act.

| Item | Column 1<br>Provider registration category | Column 2<br>Amount ($) |
|---|---|---|
| 1 | Home and community services | 1,270.00 |
| 2 | Assistive technology and home modifications | 1,270.00 |
| 3 | Advisory and support services | 1,270.00 |
| 4 | Personal and care support in the home or community | 3,800.00 |
| 5 | Nursing and transition care | 3,800.00 |
| 6 | Residential care | 5,070.00 |

### 124-10 Application fee—variation to remove provider registration categories

(1) For the purposes of paragraph 124(2)(b) of the Act, this section prescribes the application fee for an application by a registered provider to vary the provider's registration by removing the provider from being registered in a provider registration category.

(2) For an application to remove the provider from being registered in one or more of the following provider registration categories, the application fee is $545.00:
  (a) home and community services;
  (b) assistive technology and home modifications;
  (c) advisory and support services.
  (d) any of the following provider registration categories under which the registered provider is not delivering funded aged care services at the time of the application:
    (i) personal and care support in the home or community;
    (ii) nursing and transition care;
    (iii) residential care.



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(3) For an application to remove the provider from being registered in one or more of the following provider registration categories under which the registered provider is delivering funded aged care services at the time of the application, the application fee is $4800.00 for each such category:

(a) personal and care support in the home or community;

(b) nursing and transition care;

(c) residential care.

Note: Exemptions, waivers and refunds are dealt with in rules made for the purposes of section 597 of the Act

## 133-5 Classes of persons who must not be appointed as eligible advisers

For the purposes of paragraph 133(4)(a) of the Act, classes of persons who must not be appointed by a registered provider as an eligible adviser are as follows:

(a) persons who are not independent of the registered provider;

(b) persons who are unsuitable having regard to the matters specified in subsection 13(1) of the Act;

(c) persons who have, or may be perceived to have, a conflict of interest between their duties as an eligible adviser and any other interests or duties.

## 133-10 Matters to be taken into account in specifying period within which eligible adviser must be appointed

(1) For the purposes of paragraph 133(4)(b) of the Act, this section prescribes matters that the Commissioner must take into account in specifying a period in a notice given under subsection 133(2) of the Act for the purposes of paragraph 133(3)(a) of the Act (requiring a registered provider to appoint an eligible adviser).

(2) The matters are as follows:

(a) the location where funded aged care services are delivered by the registered provider;

(b) the nature of any contravention, or proposed contravention, of the Act by the registered provider that the Commissioner is satisfied has occurred, is occurring or may occur;

(c) whether, as a result of any such contravention or proposed contravention of the Act by the registered provider, there is an immediate and severe risk to the safety, health and well-being of individuals to whom the registered provider is delivering funded aged care services;

(d) the availability of persons who have appropriate qualifications, skills or experience to assist the registered provider to comply with the conditions and obligations that apply to the registered provider under Part 4 of Chapter 3 of the Act in relation to the matters specified in subparagraphs 133(3)(a)(i) and (ii) of the Act (which relate to funded aged care services and governance and business operations);

(e) any other relevant matters.



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# Division 2—Variations, suspensions and revocations of approvals of residential care homes

## 136-5 Variation of approval on Commissioner's own initiative—bed availability

For the purposes of paragraph 136(2)(a) of the Act, the period for which the Commissioner must be satisfied that the number of beds available in a residential care home is likely to be reduced is a period of at least 2 years.

## 137-10 Variation of approval on application by registered provider—bed availability

For the purposes of paragraph 137(3)(a) of the Act, the period for which the Commissioner must be satisfied that the number of beds available in a residential care home is likely to be reduced is a period of at least 2 years.



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## Division 3—Provider Register

### 141-10 Other matters that must be included in the Provider Register—registered providers

For the purposes of paragraph 141(3)(p) of the Act, other matters that the Provider Register must include in relation to the registration of a registered provider are as follows:

(a) the name of each parent or holding company forming part of the registered provider's business structure;

(b) the ACN of the registered provider's business entity (if relevant);

(c) whether the registered provider is known as an Aboriginal Community Controlled Organisation or Aboriginal Community Controlled Health Organisation;

(d) the name of each associated provider that delivers funded aged care services on behalf of the registered provider in any of the following provider registration categories:

(i) personal and care support in the home or community;

(ii) nursing and transition care;

(iii) residential care;

(e) if the registered provider's registration has been suspended:

(i) the date of suspension; and

(ii) the period of suspension, including the expiry date (if known); and

(iii) whether the suspension was at the request of the Commissioner or at the request of the registered provider; and

(iv) any specified condition to which the suspension relates;

(f) if the registered provider delivers funded aged care services in one or more approved residential care homes:

(i) the name of the responsible person of the registered provider who is in charge of each approved residential care home; and

(ii) the total number of beds at each approved residential care home, updated in accordance with any variations to the registered provider's registration and any notification under subsection 167(1) of the Act;

(g) if a compliance notice has been given to the registered provider under section 481 or 482 of the Act:

(i) the date of the notice; and

(ii) whether the notice was given by the Commissioner or the System Governor; and

(iii) the details of the non-compliance or possible non-compliance with the Act; and

(iv) whether the notice was given under subparagraph 481(a)(i) or 482(a)(i) of the Act (non-compliance), or subparagraph 481(a)(ii) or 482(a)(ii) of the Act (possible non-compliance); and

(v) the action the provider must take or refrain from taking in response to the notice; and



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(vi) whether the notice was varied or revoked;

(h) if a banning order against the registered provider is in force under section 497 of the Act:

(i) the date of the banning order; and

(ii) a linking electronic reference to the detail of the banning order in the banning orders register.

### 141-15 Other matters that may be included in the Provider Register—former registered providers

For the purposes of paragraph 141(4)(e) of the Act, other matters that the Provider Register may include in relation to an entity that was a registered provider are as follows:

(a) if the entity's registration lapsed—the date the registration lapsed;

(b) if the entity's registration was revoked:

(i) whether the registration was revoked at the request of the provider, or on the Commissioner's initiative; and

(ii) the date of the revocation.

### 141-20 Other matters that must be included in the Provider Register—responsible persons and aged care workers against whom banning orders are in force

For the purposes of paragraph 141(5)(d) of the Act, other matters that must be included in the Provider Register in relation to a responsible person, or an aged care worker, of a registered provider against whom a banning order is in force under section 498 are as follows:

(a) the date of the banning order;

(b) a linking electronic reference to the detail of the banning order in the banning orders register.

### 141-22 Other matters that may be included in the Provider Register—responsible persons and aged care workers against whom banning orders were in force

For the purposes of paragraph 141(6)(d) of the Act, other matters that may be included in the Provider Register in relation to a responsible person, or an aged care worker, of a registered provider against whom a banning order was in force under section 498 are as follows:

(a) the date of the banning order;

(b) a linking electronic reference to the detail of the banning order in the banning orders register.

### 141-25 Corrections of the Provider Register

(1) For the purposes of paragraph 141(8)(a) of the Act:



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(a) a person may request orally or in writing that the Commissioner make a correction to information relating to that person that is included in the Provider Register; and

(b) if the Commissioner considers information included in the Provider Register is inaccurate, incomplete, out-of-date, irrelevant or misleading, the Commissioner must correct the information (whether or not a person has made a request under paragraph (a) of this subsection).

(2) Despite subsection (1), the Commissioner must not make a correction to the Provider Register under this section if there is another process under the Act through which information included on the Provider Register can be updated or changed.

Note: For example, a change that can be made through a variation or change in circumstances notification is not a correction of the Provider Register.

## 141-30 Publication of the Provider Register

(1) For the purposes of paragraph 141(8)(b) of the Act, this section makes provision for and in relation to the publication of the Provider Register.

(2) In deciding whether to publish the Provider Register in whole or in part, or to publish specified information entered on the Provider Register on the Commission's website the Commissioner must have regard to:

(a) whether the Commissioner considers that publication would be contrary to the public interest; and

(b) whether the Commissioner considers that publication would be contrary to the interests of an individual accessing funded aged care services.



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# Chapter 4—Conditions on provider registration

## Part 1—Introduction

### 142-1 Simplified outline of this Chapter

This Chapter provides for matters relating to conditions on provider registration under Part 4 of Chapter 3 of the Act.

Part 3 of this Chapter relates to conditions relating to Rights, Principles and the Aged Care Code of Conduct.

Part 4 of this Chapter relates to the delivery of funded aged care services, and deals with conditions relating to:

(a) the Aged Care Quality Standards; and
(b) continuous improvement; and
(c) the delivery of funded aged care services; and
(d) requirements for starting and ceasing the provision of funded aged care services and continuity of those services.

Part 5 of this Chapter relates to conditions relating to financial matters.

Part 6 of this Chapter relates to aged care workers, and deals with conditions relating to:

(a) workforce and aged care worker requirements; and
(b) vaccinations.

Part 7 of this Chapter relates to information and access, and deals with conditions relating to:

(a) personal information and recordkeeping; and
(b) provision of information to individuals; and
(c) access by supporters.

Part 8 of this Chapter relates to governance, and deals with conditions relating to membership of governing bodies and advisory body requirements.

Part 9 of this Chapter relates to restrictive practices in approved residential care homes, and deals with conditions relating to:

(a) requirements relating to the use of restrictive practices; and
(b) requirements relating to behaviour support; and
(c) immunity from civil or criminal liability in relation to the use of restrictive practices in certain circumstances.

Part 10 of this Chapter relates to the management of incidents and complaints, and deals with conditions relating to:



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(a) implementing an incident management system and managing and preventing incidents; and
(b) implementing a complaints and feedback management system and managing complaints and feedback; and
(c) implementing a whistleblower system and maintaining a whistleblower policy.



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# Part 3—Rights and principles

## 144-1 Kinds of providers to which the conditions apply

For the purposes of subsections 144(1) and (2) of the Act, every kind of registered provider is prescribed.



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# Part 4—Delivery of funded aged care services

## Division 1—Aged Care Quality Standards

### 146-5 Kinds of providers that must comply with Aged Care Quality Standards

For the purposes of section 146 of the Act:

(a) a registered provider registered in any of the following provider registration categories is prescribed:

(i) personal and care support in the home or community;

(ii) nursing and transition care;

(iii) residential care; and

(b) a registered provider registered in the provider registration category subscription trial is prescribed if the provider is also registered in a provider registration category referred to in subparagraph (a)(i) or (ii) of this section.



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# Division 2—Continuous improvement

## 147-5 Kinds of providers that must demonstrate capability and commitment

For the purposes of subsection 147(1) of the Act, every kind of registered provider is prescribed.

## 147-10 Kinds of providers that must have a continuous improvement plan

For the purposes of subsection 147(2) of the Act:

(a) a registered provider registered in any of the following provider registration categories is prescribed:

(i) personal and care support in the home or community;

(ii) nursing and transition care;

(iii) residential care; and

(b) a registered provider registered in the provider registration category subscription trial is prescribed if the provider is also registered in a provider registration category referred to in subparagraph (a)(i) or (ii) of this section.



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# Division 3—Delivery of funded aged care services

## Subdivision A—Kinds of provider to which the condition applies

### 148-5 Kinds of providers to which the condition applies

For the purposes of section 148 of the Act, every kind of registered provider is prescribed.

## Subdivision B—Requirements for delivering funded aged care services

### 148-10 Purpose of this Subdivision

For the purposes of paragraph 148(a) of the Act, this Subdivision prescribes requirements for delivery of funded aged care services.

### 148-15 All registered providers—preventing damage to an individual's property

A registered provider must take reasonable steps to prevent damage being caused to an individual's property by the provider, or an aged care worker of the provider, in delivering funded aged care services to the individual.

### 148-20 Providers delivering services under certain provider registration categories—requirements for meals, snacks and drinks

(1) This section applies to a registered provider that:
  (a) is registered in any of the following provider registration categories:
    (i) home and community services;
    (ii) personal and care support in the home and community; and
  (b) delivers any of the following funded aged care services to an individual on a day through the service group home support under one of the registration categories mentioned in paragraph (a):
    (i) the service meal delivery;
    (ii) the service community and centre-based respite;
    (iii) a service in the service type community cottage respite.

(1A) This section also applies to a registered provider that:
  (a) is registered in the provider registration category subscription trial; and
  (b) is also registered in a provider registration category referred to in paragraph (1)(a); and
  (c) delivers any of the following funded aged care services to an individual on a day through the service group home support under that provider registration category:
    (i) the service subscription access to meal delivery;
    (ii) the service subscription access to community and centre-based respite.



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(2) The provider must ensure any meals, snacks and drinks delivered to an individual through a funded aged care service referred to in paragraph (1)(b) or (1A)(c) are nutritious and appetising, having regard to the individual's needs and preferences.

Note: Providers have other statutory obligations in relation to food safety under the Australia New Zealand Food Standards Code.

(3) The provider must, at least annually, have a dietitian assess the meals, snacks and drinks delivered by the provider through a funded aged care service referred to in paragraph (1)(b) or (1A)(c) to ensure that any meals, snacks and drinks:

(a) are appetising; and

(b) are appropriate for the nutrition needs of individuals accessing funded aged care services, including individuals with specialised dietary needs; and

(c) reflect evidence-based guidelines and practice.

Note Examples of specialised dietary needs include medical needs, or religious or cultural preferences.

(4) The provider must implement a quality assurance framework to continuously improve the meals, snacks and drinks delivered to individuals through a funded aged care service referred to in paragraph (1)(b) or (1A)(c) by taking into account:

(a) the satisfaction of individuals with the meals, snacks and drinks they are provided; and

(b) the assessments undertaken and any recommendations made by a dietitian based on those assessments in accordance with subsection (3).

## 148-25 Providers delivering services under certain provider registration categories—requirements for service delivery equipment

(1) This section applies to a registered provider that:

(a) is registered in any of the following provider registration categories:

(i) home and community services;

(ii) assistive technology and home modifications;

(iii) advisory and support services; and

(b) is delivering funded aged care services to an individual on a day under any of the registration categories referred to in paragraph (a).

(2) This section also applies to a registered provider that:

(a) is registered in the provider registration category subscription trial; and

(b) is also registered in a provider registration category referred to in subparagraph (1)(a)(i) or (iii); and

(c) is delivering funded aged care services to an individual on a day under the provider registration category subscription trial.

(3) If:

(a) the provider uses equipment in delivering the service; or

(b) delivery of the service involves the sourcing, supply and provision of equipment to the individual;



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the provider must ensure that the equipment is safe and meets the needs of the individual at the time the service is delivered to the individual.

## 148-30 Providers delivering services under certain provider registration categories—requirements for personal protective equipment, infection prevention and control

(1) This section applies to a registered provider that is registered in any of the following provider registration categories:
  (a) home and community services;
  (b) assistive technology and home modifications;
  (c) advisory and support services.

(1A) This section also applies to a registered provider that is registered in the provider registration category subscription trial if the provider is also registered in a provider registration category referred to in subparagraph (1)(a) or (c).

(2) When delivering funded aged care services to an individual on a day under any of the registration categories referred to in subsection (1) or (1A), the provider must ensure that:
  (a) personal protective equipment is available to the individual, the aged care workers of the provider delivering the services and any other persons who need it; and
  (b) the persons mentioned in paragraph (a) are supported to correctly use the equipment.

(3) The provider must:
  (a) have an appropriate infection prevention and control system for delivery of funded aged care services; and
  (b) ensure that aged care workers of the provider use hygienic practices and take appropriate infection prevention and control precautions when delivering funded aged care services.

## 148-33 Providers delivering services under provider registration category residential care—services that must be delivered

(1) This section applies to a registered provider that:
  (a) is registered in the provider registration category residential care; and
  (b) is delivering funded aged care services to an individual through the service group residential care.

(2) The services delivered to the individual by the provider must include the services listed and described in Division 8 (residential care service types) of Part 3 of Chapter 1.



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## 148-35 Providers delivering services under the TCP—services that must be delivered

(1) This section applies to a registered provider that is delivering funded aged care services to an individual under the TCP.

(2) The services delivered to the individual by the provider must include the service transition care management in the service type assistance with transition care.

## 148-40 Providers delivering short-term funded aged care services through the service group home support—services that must be delivered

(1) This section applies to a registered provider that is delivering short-term funded aged care services to an individual (other than under a specialist aged care program) for the classification level SAH restorative care pathway for the classification type short-term through the service group home support.

(2) The services delivered to the individual by the provider must comprise a multidisciplinary package of early intervention care that is designed to optimise the functioning and independence of the individual, and reverse or slow the individual's functional decline, to help delay the individual's need to access ongoing funded aged care services.

(3) Without limiting subsection (2), the services delivered to the individual must include the following:
  (a) the service home support restorative care management in the service type restorative care management;
  (b) other services in accordance with the individual's access approval which may include:
    (i) a variety of services in the service type allied health and therapy; and
    (ii) services in the service type nursing care.

(4) The provider must ensure an aged care worker of the provider who holds relevant health qualifications (a ***restorative care partner***) delivers the service mentioned in paragraph (3)(a) to the individual.

## 148-45 Providers delivering certain funded aged care services through the service group home support—requirements for the service home support care management

(1) This section applies to a registered provider that:
  (a) is delivering ongoing funded aged care services to an individual (other than under a specialist aged care program) through the service group home support; or
  (b) is delivering short-term funded aged care services to an individual (other than under a specialist aged care program) for the classification level SAH end-of-life pathway through the service group home support.



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(2) The provider must deliver the service home support care management in the service type care management to the individual at least once in each month where the provider also delivers other services to the individual.

## Subdivision C—Requirements for service agreements

### 148-60 Purpose of this Subdivision

For the purposes of paragraph 148(c) of the Act, this Subdivision prescribes requirements for service agreements.

### 148-65 Requirements for service agreements—general

*Entry into service agreement*

(1) A registered provider must enter into a service agreement with an individual:
   (a) unless paragraph (b) applies—on or before the individual's start day; or
   (b) if the individual is accessing funded aged care services in any of the circumstances specified in subsection 71(4) of the Act, or the registered provider is delivering funded aged care services to the individual under the MPSP or the NATSIFACP—within 28 days after the day the registered provider commences delivery of the services to the individual.

(2) A registered provider must ensure the following:
   (a) the individual is involved in the development and negotiation of the service agreement;
   (b) if requested by the individual, a supporter, family member, carer or advocate of the individual, or any other person significant to the individual, is present during the development and negotiation of the service agreement;
   (c) the service agreement is expressed in plain language and is readily understandable by the individual;
   (d) the individual is helped to understand the terms of the service agreement.

*Cooling-off period—other than for short-term funded aged care services delivered through the service group residential care*

(3) A service agreement for the delivery of funded aged care services other than short-term funded aged care services delivered through the service group residential care must provide that if:
   (a) within 14 days after the date of entry into the service agreement; and
   (b) before the individual's start day;

the individual notifies (whether verbally or in writing) the registered provider that the individual wishes to withdraw from the service agreement, then:
   (c) the service agreement has no effect; and
   (d) the registered provider must refund any fees or contributions paid by the individual that are referred to in Part 3 of Chapter 4 of the Act.



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*Cooling-off period and refunds of booking fees—short-term funded aged care services delivered through the service group residential care*

(3A) A service agreement for the delivery of short-term funded aged care services through the service group residential care must:

(a) if the registered provider charges the individual a booking fee for or in connection with the delivery of those services under subsection 283(4) of the Act—set out the effect of subsections 283-15(3) to (7) of this instrument (which deal with refunds of booking fees); and

(b) provide that if:

(i) within 14 days after the date of entry into the service agreement; and

(ii) before the individual's start day;

the individual notifies (whether verbally or in writing) the registered provider that the individual wishes to withdraw from the service agreement, then:

(iii) the service agreement has no effect; and

(iv) the registered provider must refund any fees or contributions paid by the individual that are referred to in Division 2, 2A or 3 of Part 3 of Chapter 4 of the Act (other than a booking fee referred to in paragraph (a)).

*Additional cooling-off period—ongoing aged care services delivered through the service group residential care*

(4) A service agreement for an individual to whom the registered provider is to deliver funded aged care services under the classification type ongoing through the service group residential care must provide that, if the individual notifies (whether verbally or in writing) the registered provider that the individual wishes to withdraw from the service agreement within 28 days after the date of entry into the service agreement, the registered provider must refund any amount paid by the individual under the service agreement in relation to services that had not been delivered by the termination date.

*Variation of service agreement*

(5) A service agreement must provide that the agreement may be varied:

(a) by the registered provider if:

(i) the variation is necessary to implement the *A New Tax System (Goods and Services Tax) Act 1999*; and

(ii) the provider has given reasonable notice in writing about the variation to the individual; and

(iii) the variation is not inconsistent with the *A New Tax System (Goods and Services Tax) Act 1999* or the Act; or

(b) by mutual consent of the individual and the registered provider if:

(i) there has been adequate consultation about the variation between the individual and the provider; and

(ii) the provider has obtained consent from the individual to make the variation; and



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(iii) the variation is not inconsistent with the *A New Tax System (Goods and Services Tax) Act 1999* or the Act.

*Review of service agreement*

(6) A service agreement between a registered provider and an individual accessing ongoing funded aged care services must provide that the provider will review the service agreement:
  (a) at least once every 12 months; and
  (b) upon request from the individual.

(7) Without limiting the nature of the review, the provider must:
  (a) give the individual an opportunity to participate in the review; and
  (b) consider whether any updates need to be made to the service agreement; and
  (c) if necessary, vary the service agreement in accordance with subsection (5).

## 148-70 Requirements for service agreements—contents

*All registered providers*

(1) A service agreement must not contain any provision that would have the effect of the individual being treated less favourably in relation to any matter than the individual would otherwise be treated, under any law of the Commonwealth, in relation to that matter.

(2) A service agreement entered into between a registered provider and an individual must contain the following:
  (a) a statement setting out the parties to the agreement, including the following:
    (i) the name of the individual;
    (ii) the contact details of the individual;
    (iii) the name of the provider;
    (iv) the contact details of the provider;
  (b) the contact details of the supporters of the individual (if any);
  (c) a copy of the individual's access approval;
  (d) the approved residential care home (if any) in or from which the provider will deliver funded aged care services to the individual;
  (e) the date when the service agreement commences;
  (f) the start day for the individual;
  (g) if the individual is accessing short-term or hospital transition funded aged care services:
    (i) when the provider will cease delivering short-term or hospital transition funded aged care services to the individual; and
    (ii) the date when the service agreement ends;



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(h) if the individual is accessing ongoing funded aged care services, the date the service agreement is to be reviewed in accordance with subsection 148-65(6);

(i) how the individual will be involved in decisions relating to how, when and by whom funded aged care services are delivered to the individual.

(3) Despite subsection (2), a service agreement entered into between a registered provider and an individual does not have to contain the matters specified in paragraph (2)(i) if the provider is delivering funded aged care services to the individual under the TCP.

*Certain registered providers delivering services through the service groups home support, assistive technology or home modifications (other than under a specialist aged care program)*

(4) A service agreement entered into between a registered provider (to whom subsection (5) applies) and an individual must also contain the following:

(a) the funded aged care services the provider will deliver to the individual, including the following:

(i) the name of each service as provided in the service list under section 8-5 of this instrument;

(ii) if the provider intends for an associated provider to deliver any services to the individual, which services will be delivered by the associated provider;

(b) the prices the provider will charge the individual for each of the funded aged care services the provider will deliver to the individual;

(c) if the individual is to be charged a price for delivery of a funded aged care service that is higher than the price published on the provider's website or the department's website—the reason for the higher price;

(d) a statement that the prices mentioned in paragraphs (b) and (c) may be subject to regular price increases to account for indexation and the details of such increases, including:

(i) the date the prices will increase; and

(ii) the method for working out the increase; and

(iii) the reason for the matters specified at subparagraphs (i) and (ii);

(da) a statement that the individual agrees to pay any applicable fees or contributions referred to in Division 1 of Part 3 of Chapter 4 of the Act;

(e) a statement that the provider may only cease delivery of funded aged care services to the individual in the circumstances specified in subsection 149-35(2);

(f) if either of the circumstances in paragraphs 149-35(2)(e) or (f) arise, any other requirements for notifying the provider;

(g) how and when the service agreement may be terminated;

(h) a statement that the registered provider will not charge an individual contribution to the individual if the individual has made an application for a fee reduction supplement determination under subsection 197-20(1) and the System Governor has not decided on that application;



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(i) a statement that where, in relation to an application mentioned in paragraph (h), the System Governor makes a determination that the fee reduction supplement should not apply to the individual, the registered provider can collect the unpaid individual contributions for the period beginning on the day the application was made.

(4A) If the funded aged care services contained in a service agreement under paragraph (4)(a) include a subscription service, the service agreement must also contain the following:

(a) the parameters of the access provided by the subscription service, including (as applicable) the following:

(i) the days the access will be provided;

(ii) the periods on those days during which the access will be provided;

(iii) any limits on the number of times the access can be used during those periods;

(iv) whether the access is provided on a group basis;

(b) a statement of whether the individual consents to the collection of the individual's personal information for the purpose of evaluating the delivery of the subscription service;

(c) a statement that the individual may cease to access the subscription service by giving notice in writing to the provider stating the day that the individual wishes the cessation to take effect; and

(d) a statement of the effect of subsections 11A(2) to (4) of the Act and section 11A-5 of this instrument (dealing with when non-delivery of funded aged care services is taken to be delivery of funded aged care services).

(5) This subsection applies to a registered provider that is delivering funded aged care services to an individual through the service groups home support, assistive technology or home modifications (other than under a specialist aged care program).

*Registered providers delivering services under the CHSP, MPSP or NATSIFACP*

(6) A service agreement entered into between a registered provider (to whom subsection (9)) applies and an individual must also contain:

(a) the matters mentioned in paragraphs (4)(a), (e), (f) and (g); and

(b) the fees and contribution that the registered provider will charge the individual for each of the funded aged care services the provider will deliver to the individual.

(7) A service agreement entered into between a registered provider (to whom subsection (10) applies) and an individual must also contain the matters mentioned in paragraphs (4)(a), (e), (f) and (g).

(8) A service agreement entered into between a registered provider (to whom subsection (9) or (10) of this section applies) and an individual must also contain a statement that the individual agrees to pay any applicable fees referred to in Division 3 of Part 3 of Chapter 4 of the Act.



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(9) This subsection applies to a registered provider that is delivering funded aged care services to an individual under the CHSP.

(10) This subsection applies to a registered provider that is delivering funded aged care services to an individual under the MPSP or NATSIFACP.

*Registered providers delivering services through the service group residential care*

(11) A service agreement entered into between a registered provider (to whom subsection (13) or (14) of this section applies) and an individual must also contain which fees or contributions (if any), referred to in Division 2, 2A or 3 of Part 3 of Chapter 4 of the Act, the provider will charge the individual.

(12) A service agreement entered into between a registered provider (to whom subsection (13) of this section applies) and an individual must also contain the following:

(a) that the provider may only ask the individual to leave the approved residential care home in the circumstances specified in subsection 149-60(1);

(b) the assistance the provider will provide to the individual to obtain suitable alternative accommodation if the individual is asked to leave the approved residential care home;

(c) how and when the service agreement may be terminated;

(d) for any fees or contributions that the provider will charge the individual (as set out in the agreement under subsection (11))—the policies and practices the provider will follow in setting and updating the amounts of those fees and contributions.

(13) This subsection applies to a registered provider that is delivering ongoing funded aged care services to an individual through the service group residential care.

(14) This subsection applies to a registered provider that:

(a) is delivering short-term funded aged care services to an individual through the service group residential care; or

(b) is delivering funded aged care services to an individual through the service group residential care under the TCP.

## Subdivision D—Requirements for care and services plans

### 148-75 Purpose of this Subdivision

For the purposes of paragraph 148(e) of the Act, this Subdivision prescribes requirements for care and services plans.

### 148-80 Requirements for care and services plans—general

*When a care and services plan must be developed—all registered providers*

(1) A registered provider must develop a care and services plan for an individual:



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(a) unless paragraph (b) applies—on or before the individual's start day; or
(b) if the individual is accessing funded aged care services in any of the circumstances specified in subsection 71(4) of the Act, or the registered provider is delivering funded aged care services to the individual under the MPSP or the NATSIFACP—within 28 days after the day the registered provider commences delivery of the services to the individual.

*How a care and services plan is developed—certain registered providers*

(2) A registered provider to whom subsection (5) or (6) applies must actively engage with:
(a) the individual; and
(b) supporters of the individual (if any); and
(c) any other persons involved in the care of the individual;
in developing and reviewing the individual's care and services plan through ongoing communication.

*Care and services plan to be in line with the individual's needs—certain registered providers*

(3) A registered provider to whom subsection (5) or (6) applies must ensure an individual's care and services plan:
(a) describes the current care needs, goals and preferences of the individual; and
(b) includes strategies for risk management and preventative care; and
(c) where an individual is accessing ongoing funded aged care services, is reviewed at least once every 12 months.

*Care and services plan to be accessible to individual—all registered providers*

(4) A registered provider must provide a copy of an individual's care and services plan to the individual at the following times:
(a) once the plan is developed;
(b) any time the plan is updated;
(c) upon request from the individual.

*Registered providers that are subject to subsections (2) and (3)*

(5) This subsection applies to a registered provider that is delivering funded aged care services to an individual under any of the following registration categories:
(a) home and community services;
(b) assistive technology and home modifications;
(c) advisory and support services.

(6) This subsection applies to a registered provider that is registered in the provider registration category subscription trial if the provider is also registered in a provider registration category referred to in paragraph (5)(a) or (c).

Note 1: See subsections 15-20(1) to (3) for the Aged Care Quality Standards relating to care and services plans which apply to providers registered in provider registration



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categories personal and care support in the home and community, nursing and transition care, and residential care.

Note 2: For funded aged care services relating to care and services plans, see item 1 of the table in section 8-20 and items 1 and 4 in the table in section 8-155.

## 148-81 Requirements for care and services plans—restorative care partner to be responsible for care and services plans in certain circumstances

(1) This section applies to a registered provider that is delivering short-term funded aged care services to an individual (other than under a specialist aged care program) for the classification level SAH restorative care pathway through the service group home support.

(2) The provider must ensure the restorative care partner who will deliver the service mentioned in paragraph 148-40(3)(a) to the individual develops a care and services plan for the individual in accordance with any applicable requirements set out in this Subdivision.

(3) The provider must consider exit planning as part of the care and services plan and must ensure the plan complements any ongoing services the individual is also accessing.

## 148-85 Requirements for care and services plans—contents for all individuals (other than individuals accessing services through the service group residential care)

*Application of this section*

(1) This section applies to a registered provider that is delivering funded aged care services to an individual under any of the following registration categories:
  (a) home and community services;
  (b) assistive technology and home modifications;
  (c) advisory and support services;
  (d) personal and care support in the home or community;
  (e) nursing and transition care;
  (f) subscription trial.

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(2) A care and services plan for an individual must include the following:
  (a) the funded aged care services that the provider will deliver to the individual, including:
    (i) the frequency of the services; and
    (ii) the volume or duration of the services;
  (b) when the care and services plan will be reviewed;
  (c) if the individual is accessing ongoing funded aged care services, the date the individual's service agreement is to be reviewed in accordance with subsection 148-65(6).



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# Division 4—Starting and ceasing the provision of funded aged care services and continuity of those services

## Subdivision A—Preliminary

### 149-5 Requirements relating to starting and ceasing the provision of funded aged care services and continuity of those services

For the purposes of section 149 of the Act, this Division prescribes requirements for the following:

(a) providing a start notification to the System Governor and the Commissioner;
(b) ceasing the delivery of funded aged care services to an individual;
(c) providing a cessation notification to the System Governor and the Commissioner;
(d) ceasing to deliver any funded aged care services;
(e) if the provider delivers funded aged care services in an approved residential care home—security of tenure for individuals accessing those services;
(f) continuity of funded aged care services for individuals.

Note: For requirements relating to a service agreement between an individual and a registered provider, see section 148-70 of this instrument.

## Subdivision B—Start notifications

### 149-10 Requirements for start notifications—general

(1) A registered provider must prepare a start notification for delivering funded aged care services to an individual in accordance with this section.

(2) Despite subsection (1), a registered provider does not have to prepare a start notification for an individual where the individual will access funded aged care services under a specialist aged care program other than the TCP.

(3) A start notification must be in an approved form.

*Start notification for an individual accessing funded aged care services through the service groups home support, assistive technology or home modifications*

(4) A start notification for an individual accessing funded aged care services through the service groups home support, assistive technology or home modifications must specify the following:

(a) the name of the individual;
(b) the start day for the individual;
(c) the service delivery branch through which funded aged care services will be delivered to the individual.



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*Start notification for an individual accessing funded aged care services through the classification type ongoing for the service group residential care*

(5) A start notification for an individual accessing funded aged care services through the classification type ongoing for the service group residential care must specify the following:

(a) the name of the individual;

(b) the start day for the individual;

(c) the approved residential care home at which funded aged care services will be delivered to the individual.

*Start notification for an individual accessing funded aged care services through the classification type short-term or hospital transition for the service group residential care*

(6) A start notification for an individual accessing funded aged care services through the classification type short-term or hospital transition for the service group residential care must specify the following:

(a) the name of the individual;

(b) the start day for the individual;

(c) the approved residential care home at which funded aged care services will be delivered to the individual.

### 149-15 Requirements for start notifications—provision to System Governor and Commissioner

A registered provider must give a start notification for an individual to the System Governor and the Commissioner:

(a) for an individual mentioned in subsections 149-10(4) and (5)—within 28 days after the individual's start day; or

(b) for an individual mentioned in subsection 149-10(6)—within 14 days after the individual's start day.

## Subdivision C—Cessation notifications

### 149-20 Requirements for cessation notifications—general

(1) A registered provider must prepare a cessation notification for ceasing the delivery of funded aged care services to an individual in accordance with this section.

(2) Despite subsection (1), a registered provider does not have to prepare a cessation notification for an individual where the individual is accessing funded aged care services under a specialist aged care program other than the TCP.

(3) A cessation notification must be in an approved form.

(4) A cessation notification for an individual must specify the following:



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(a) the name of the individual;
(b) the day the provider ceased to deliver funded aged care services to the individual;
(c) the reason for ceasing the delivery of funded aged care services to the individual.

### 149-25 Requirements for cessation notifications—provision to System Governor and Commissioner

A registered provider must give a cessation notification for an individual to the System Governor and the Commissioner:

(a) for an individual accessing funded aged care services through the classification type short-term for the service group residential care—within 14 days after the cessation of delivery of services through the approved residential care home of the provider; or
(b) for all other individuals—within 28 days after the cessation of delivery of services through the approved residential care home or service delivery branch of the provider.

## Subdivision D—Ceasing delivery of funded aged care services (other than services delivered in an approved residential care home)

### 149-30 Application of this Subdivision

This Subdivision applies to a registered provider who is delivering funded aged care services to an individual for a classification type for the service groups home support, assistive technology or home modifications.

### 149-35 Requirements for ceasing delivery of funded aged care services - general

(1) A registered provider must not cease to deliver funded aged care services to an individual unless:
(a) one of the circumstances in subsection (2) applies; and
(b) the provider has given notice to the individual in accordance with section 149-40.

*Circumstances where registered provider may cease delivery of funded aged care services to an individual*

(2) The circumstances in which a registered provider may cease to deliver funded aged care services to an individual are:
(a) the individual cannot be cared for in the home or community with the resources available to the provider; or
(b) the individual's condition changes to the extent that:
(i) the individual no longer needs the funded aged care services delivered by the provider; or



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(ii) the individual's needs, as assessed by an approved needs assessor, can be more appropriately met by other types of funded aged care services; or

(c) the individual has:

(i) intentionally caused serious injury to an aged care worker of the provider; or

(ii) intentionally infringed the right of an aged care worker of the provider to work in a safe environment; or

(d) the individual:

(i) has not paid to the provider, for a reason within the individual's control, any applicable fee or contribution referred to in Division 1 of Part 3 of Chapter 4 of the Act as agreed in the service agreement between the individual and the provider; and

(ii) has not negotiated an alternative arrangement with the provider for payment of the fee or contribution; and

(iii) if the individual is accessing the services other than under a specialist aged care program—has no application for the fee reduction supplement in place; or

(e) the individual notifies the provider, in writing, that the individual wishes to move to a location where funded aged care services are not delivered by the provider; or

(f) the individual notifies the provider, in writing, that the individual no longer wishes to receive funded aged care services from the provider.

Note: The circumstances in which a registered provider may cease delivering funded aged care services to an individual must be specified in a service agreement between the individual and the registered provider made under section 148-70.

## 149-40 Requirements for ceasing delivery of funded aged care services—notice to individual about cessation of services

*Notice to be given if registered provider intends to cease delivery of funded aged care services*

(1) If a registered provider intends to cease delivery of funded aged care services to an individual, the provider must give the individual a written notice which includes the following:

(a) the decision;

(b) the reasons for the decision;

(c) the date the provider intends to cease delivery of funded aged care services to the individual;

(d) the individual's rights in relation to cessation of the delivery of funded aged care services, including the right to access:

(i) the provider's complaints and feedback management system; and

(ii) any other mechanisms available to address complaints; and

(iii) independent aged care advocates.



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Note: For complaints and feedback management systems, see section 165 of the Act and Division 2 of Part 10 of Chapter 4 of this instrument.

(2) The registered provider must give the notice to the individual at least 14 days before the date the provider ceases delivery of funded aged care services to the individual.

*Notice to be given if registered provider no longer intends to cease delivery of funded aged care services*

(3) If:

(a) the decision to cease the delivery of funded aged care services was based on the individual's behaviour; and

(b) the registered provider has given the individual a notice under subsection (1); and

(c) after giving the notice, the provider has agreed with the individual that, because of a change in the behaviour, the delivery of funded aged care services will not cease;

the provider must give the individual a written notice stating that the provider no longer intends to cease delivery of funded aged care services to the individual.

## 149-45 Requirements for ceasing delivery of funded aged care services—notice to individual about unspent portions

(1) This section applies if:

(a) a registered provider holds an unspent Commonwealth portion or unspent care recipient portion for an individual to whom the provider is delivering funded aged care services; and

(b) section 226C or 226D of the Act apply.

(2) The registered provider must give a notice relating to the individual's unspent Commonwealth portion or unspent care recipient portion in accordance with this section.

Note: See paragraphs 226C(2)(a) and 226D(2)(a) of the Act for requirements to give written notice to the System Governor about the unspent Commonwealth portion.

(3) The notice must:

(a) specify the following:

(i) the day the provider ceased to deliver funded aged care services to the individual;

(ii) the available balance (including a nil amount) of the unspent Commonwealth portion for the individual;

(iii) the balance (including a nil amount) of the unspent care recipient portion for the individual;

(iv) if the amount of the unspent care recipient portion for the individual was reduced by the individual contributions charged to the individual by the provider under paragraph 273A-20(a) of this instrument—the amount of those individual contributions; and



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(b) explain:
(i) the effect of paragraphs 226C(2)(b) and 226D(2)(b) of the Act; and
(ii) the effect of subsections 226A(7) to (9) and 226E(5) and (6) of the Act; and
(iii) the effect of paragraph 273A-20(b) of this instrument.

*When notice must be given*

(4) The notice must be given within 28 days after the cessation of delivery of services.

*Who notice is given to*

(5) The notice must be given to:
(a) the individual; or
(b) if the individual has died:
(i) the individual's legal representative; or
(ii) the individual's estate.

Note: A copy of the notice may also need to be given to another registered provider under section 149-80 of this instrument or a supporter of the individual under section 29 of the Act.

## 149-46 Requirements for ceasing delivery of funded aged care services—notice to new registered provider about account balances

(1) This section applies if:
(a) a registered provider has ceased the delivery of funded aged care services to an individual through a service delivery branch of the provider; and
(b) within 60 days after the day the registered provider ceased to deliver funded aged care services to the individual, the provider is notified that the individual has entered into a service agreement with a new registered provider; and
(c) the new registered provider will deliver funded aged care services to the individual for a classification type for the service groups home support, assistive technology or home modifications through a service delivery branch of the new registered provider.

(2) The registered provider must give a notice to the individual's new registered provider specifying what the provider estimates will be the available balance of the individual's:
(a) notional ongoing home support account (if any); and
(b) notional short-term home support account (if any); and
(c) notional assistive technology account (if any); and
(d) notional home modifications account (if any); and
(e) notional home care account (if any);

after the provider gives a claim in accordance with section 251 of the Act for payment of subsidy for the delivery of funded aged care services to the



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individual for the relevant period where the provider ceased delivering services to the individual.

(3) The notice must be given within 28 days after the registered provider is notified that the individual has entered into a service agreement with the new registered provider.

### 149-48 Requirements for ceasing delivery of subscription services

A registered provider must not cease to deliver a particular subscription service to an individual unless:

(a) the provider ceases to deliver funded aged care services to the individual in accordance with section 149-35; or

(b) the individual has given the provider notice in writing in relation to the particular subscription service as referred to in paragraph 148-70(4A)(c).

## Subdivision E—Security of tenure for individuals accessing funded aged care services in an approved residential care home

### 149-50 Application of this Subdivision

This Subdivision applies to a registered provider who is delivering funded aged care services to an individual for a classification type ongoing for the service group residential care in an approved residential care home of the registered provider.

### 149-55 Security of tenure—general

A registered provider must not take action to make an individual accessing funded aged care services in an approved residential care home of the approved provider leave the home, or imply that the individual must leave the home, unless:

(a) one of the circumstances in subsection 149-60(1) apply; and

(b) suitable alternative accommodation is available for the individual that meets the requirements in subsection 149-60(2); and

(c) the provider has given notice to the individual in accordance with section 149-65.

### 149-60 Security of tenure—circumstances where registered provider may ask an individual to leave an approved residential care home

(1) A registered provider may only ask an individual accessing funded aged care services in an approved residential care home of the approved provider to leave the residential care home if:

(a) the approved residential care home is closing; or

(b) the provider can no longer provide accommodation and funded aged care services through the approved residential care home which are suitable for the individual, having regard to the individual's needs as assessed in



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accordance with subsection (4) and the provider has not agreed to deliver funded aged care services of the kind that the individual presently needs; or

(c) the individual no longer needs the funded aged care services delivered through the approved residential care home, as assessed by an approved needs assessor in accordance with subsection 64(2) of the Act; or

(d) the individual has been accessing funded aged care services under a specialist dementia care agreement and a clinical advisory committee constituted in accordance with the agreement has determined that the individual is not suitable to continue accessing those services; or

(e) the individual has not paid any agreed fee or contribution to the provider within 42 days after the day when it is payable, for a reason within the individual's control; or

(f) the individual has intentionally caused:

(i) serious damage to the approved residential care home; or

(ii) serious injury to an aged care worker of the provider, or to another individual accessing funded aged care services at the approved residential care home; or

(g) the individual is away from the approved residential care home for a continuous period of at least 7 days for a reason other than:

(i) emergency leave; or

(ii) hospital leave; or

(iii) hospital transition leave; or

(iv) extended hospital leave; or

(v) social leave.

Note: For when an individual is on leave, see section 244 of the Act.

*Suitable accommodation to be available before individual can be required to leave approved residential care home*

(2) If a registered provider intends to ask an individual to leave the approved residential care home, the provider must ensure suitable alternative accommodation is available:

(a) with an alternative registered provider that meets the individual's needs and is affordable by the individual; or

(b) in a place more suited to the individual's long term needs.

(3) Without limiting subsection (2), suitable alternative accommodation with an alternative registered provider will be available if the alternative registered provider has offered to enter into a service agreement with the individual under section 148-70 of this instrument for the delivery of funded aged care services to the individual.

Note: A registered provider is required to detail the steps taken to ensure any alternative accommodation meets the needs of the individual in a continuity of care plan for the individual prepared under section 149-75 of this instrument.



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*Assessing the individual's needs*

(4) For the purposes of paragraph (1)(b), the needs of the individual must be assessed by:
  (a) an approved needs assessor; or
  (b) at least 2 medical or other health practitioners who meet the following criteria:
    (i) one must be independent of the registered provider and the approved residential care home, and must be chosen by the individual;
    (ii) both must be competent to assess the aged care needs of the individual.

## 149-65 Security of tenure—notice requirements

*Notice to be given if individual asked to leave residential care service*

(1) If a registered provider of an approved residential care home decides to ask an individual to leave the approved residential care home, the provider must give the individual a written notice which includes the following:
  (a) the decision;
  (b) the reasons for the decision;
  (c) the date the individual is to leave;
  (d) the individual's rights in relation to being asked to leave the approved residential care home, including the right to access:
    (i) the provider's complaints and feedback management system; and
    (ii) any other mechanisms available to address complaints; and
    (iii) independent aged care advocates;
  (e) a copy of the continuity of care plan for the individual.

Note: For complaints and feedback management systems, see section 165 of the Act and Division 2 of Part 10 of Chapter 4 of this instrument.

(2) The registered provider must give the notice to the individual at least 14 days before the individual is to leave.

*Notice to be given if individual no longer required to leave approved residential care home*

(3) If:
  (a) the decision to require the individual to leave the approved residential care home was based on the individual's behaviour; and
  (b) the registered provider has given the individual a notice under subsection (1); and
  (c) after giving the notice, the provider has agreed with the individual that, because of a change in the behaviour, the individual should not be required to leave the residential care home;

the provider must give the individual a written notice stating that the individual is no longer required to leave the residential care home.



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### 149-70 Security of tenure—movement of individuals

(1) A registered provider must not move an individual from a room, or part of a room, in the approved residential care home to another room, or part of a room, in the home unless:

(a) the move is at the individual's request; or

(b) the individual agrees to the move after being fully consulted and without being subjected to any pressure; or

(c) the move is necessary on genuine medical grounds as assessed by one of the following:

(i) an approved needs assessor;

(ii) at least 2 medical or other health practitioners who meet the criteria in subparagraphs 149-60(4)(b)(i) and (ii) of this instrument; or

(d) the individual has been accessing funded aged care services under a specialist dementia care agreement and a clinical advisory committee constituted in accordance with the agreement has determined that the individual is not suitable to continue accessing those services in the individual's current room, or part of a room; or

(e) the move is necessary to carry out repairs or improvements to the home and the individual has the right to return to the room, or the part of the room, if it continues to exist as a room, or part of a room, for individuals when the repairs or improvements are finished; or

(f) the move is necessary due to an emergency, including (but not limited to) the following:

(i) during serious medical situations such as disease outbreak, viral pandemic and epidemic;

(ii) in circumstances in which the safety of the individual is compromised such as during a fire, natural disaster or flood;

(iii) the individual has been repeatedly violent towards another person in the same room or in close proximity to the individual.

## Subdivision F—Continuity of funded aged care services

### 149-75 Continuity of care plan

(1) A registered provider must prepare a plan for ensuring the continuity of funded aged care services for an individual (a ***continuity of care plan***) in accordance with this section if the provider intends to ask an individual to leave an approved residential care home of the provider in any of the circumstances prescribed in subsection 149-60(1) of this instrument.

*Continuity of care plan for individuals accessing funded aged care services in an approved residential care home*

(2) A continuity of care plan for an individual accessing funded aged care services through the classification type ongoing for the service group residential care in



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an approved residential care home of a registered provider must specify the following:

(a) details of any suitable alternative accommodation that is available that meet the requirements in subsection 149-60(2);
(b) the steps the registered provider has taken to ensure that any alternative accommodation meets the individual's needs;
(c) how any records relating to ensuring the continuity of funded aged care services for the individual will be transferred to a suitable alternative registered provider if required under section 149-80;
(d) the intended start date (if any) for the individual with a suitable alternative registered provider;
(e) where an individual will not continue accessing funded aged care services with an alternative registered provider, the reason for this;
(f) the way in which the registered provider proposes to help the individual move (with their personal possessions);
(g) the measures that the registered provider proposes to take to refund any fees or contributions to the individual as required by Part 3 of Chapter 4 of the Act;
(h) the measures that the registered provider proposes to take to deal with any refundable deposit balance paid by the individual as required by Division 7 of Part 4 of Chapter 4 of the Act.

## 149-80 Transfer of records between registered providers

(1) This section applies if:
(a) a registered provider (the ***outgoing provider***) ceases to deliver funded aged care services to an individual; and
(b) another registered provider (the ***incoming provider***) starts the delivery of funded aged care services to the individual.

*Records to be given to incoming provider on request*

(2) The incoming provider may request the outgoing provider give records relating to the individual which are necessary to ensure the continuity of funded aged care services for the individual.

(3) If a request for records is made under subsection (2), the outgoing provider must give the incoming provider the records, or copies of such records, within 28 days after the request is made.

(4) Without limiting subsections (2) and (3), the outgoing provider must give the following records:
(a) any records relating to the individual the provider is required to keep under section 7 of the *Records Principles 2014*;
(b) any records relating to the individual the provider is required to keep under section 154-1000 of this instrument.



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# Part 5—Conditions relating to financial matters

## 150A-1 Requirements for refundable deposit register

For the purposes of section 150A of the Act, a registered provider's refundable deposit register must:

(a) be able to produce the information prescribed under this Part (as applicable) in a single, itemised document; and

(b) require the collection of information prescribed under this Part that will enable the provider to provide information to the Commissioner, if required or requested to do so by the Commissioner.

## 150A-3 Kinds of providers to which the condition applies

For the purposes of section 150A of the Act, a registered provider registered in the category residential care that receives or has received payment of any of the following from an individual, wholly or partly as a lump sum:

(a) a refundable deposit;

(b) an accommodation bond;

(c) an entry contribution;

is prescribed.

## 150A-5 Refundable deposit register to include information about refundable deposit paid and refundable deposit balances held

For the purposes of section 150A of the Act, a registered provider must include the following information in the refundable deposit register for each individual from whom a refundable deposit is paid to the provider, or in respect of whom the registered provider holds a refundable deposit balance:

(a) the name of the individual;

(b) the Aged Care Management Payment System number for the individual;

(c) the date on which the individual entered the approved residential care home through which the individual is provided with funded aged care services by the registered provider;

(d) the date on which the whole or each part of the refundable deposit paid by lump sum was paid to the registered provider for entry to the approved residential care home referred to in paragraph (c);

(e) the amount of each payment referred to in paragraph (d);

(f) the amount of each deduction made from the refundable deposit authorised under section 307 or section 308 of the Act;

(g) the date when each of the deduction referred to in paragraph (f) was made;

(h) for each deduction made under paragraph (f)—the provision that the deduction was authorised under and the reason for the deduction;



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(i) the refundable deposit balance as at the end of each calendar month in relation to the individual during which the registered provider held a refundable deposit balance;

(j) any amount refunded to the individual under subsection 304-5(1) of this instrument;

(k) the date when any amount was refunded to the individual under subsection 304-5(1) of this instrument;

(l) where the registered provider began delivering funded aged care services to the individual and the amount of accommodation payment charged to the individual is higher than the maximum accommodation payment amount prescribed by rules made for the purposes of section 289 of the Act—the Independent Health and Aged Care Pricing Authority approval number;

(m) any transfer of the refundable deposit made under section 312 of the Act, including any amount, the date of that transfer and the amount of retention period remaining.

## 150A-15 Refundable deposit register to include information about refundable deposit balances refunded

For the purposes of section 150A of the Act, a registered provider must include the following information in the refundable deposit register for each individual from whom a refundable deposit is paid to the provider, or in respect of whom the registered provider holds a refundable deposit balance:

(a) if the refundable deposit balance was refunded because the individual died;

(i) the date on which the individual died; and

(ii) if applicable, the date on which the registered provider is shown the probate of the will of the individual, letters of administration of the estate of the individual or other evidence that satisfied the provider that the refundable deposit is to be refunded to a person;

(b) if the refundable deposit balance was refunded because the individual ceased to be provided with ongoing funded aged care services in the service group residential care at the approved residential care home—the date on which the individual ceased to be provided with those ongoing funded aged care services;

(c) if paragraph (b) applies and the individual notified the registered provider, before the date referred to in that paragraph, that the individual intended to enter another approved residential care home to receive ongoing funded aged care services in the service group residential care—the date of notification by the individual;

(d) if the refundable deposit was refunded because of changes to the nature of delivery of funded aged care services to the individual whilst in the care of the registered provider, and those changes result in excess deposit being held by the registered provider—a description of those changes;

(e) if the refundable accommodation contribution amount is reduced because of changes to the daily means tested amount or the accommodation supplement applicable to the individual and that reduction results in excess deposit being held by the registered provider—the date the new daily



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means tested amount or new accommodation supplement applied and the date the provider refunded the excess refundable deposit.

(f) the date on which, or by which, the registered provider was required to refund the refundable deposit to the individual as worked out in accordance with section 311 of the Act;

(g) the date on which the refundable deposit was refunded to the individual or another person;

(h) the amount of the refundable deposit refunded;

(i) the amounts (if any) of interest paid, as required by section 313-5 of this instrument and worked out under section 313-10 of this instrument, broken down into amounts worked out using the base interest rate and amounts worked out using the maximum permissible interest rate, and the date or dates on which the amounts were paid.

## 150A-20 Refundable deposit register to include information about accommodation bond balances held by the registered provider

For the purposes of section 150A of the Act, a registered provider must include the following information in the refundable deposit register in relation to an individual in respect of whom an accommodation bond was paid to the registered provider on or after 1 July 2006, or in respect of whom the registered provider holds an accommodation bond balance on or after that date:

(a) the name of the individual;

(b) the Residential Identification Number allocated by the Department in respect of the individual;

(c) the date on which the individual entered the approved residential care home in which the individual is provided with funded aged care services by the registered provider on or after 1 July 2006;

(d) if, immediately before entering the approved residential care home referred to in paragraph (c), the individual was provided with care through an approved residential care home (the ***original approved residential care home***), and an accommodation bond was paid for the individual's entry to the original approved residential care home—the date on which the individual entered the original approved residential care home;

(e) the date on which the whole or each part of an accommodation bond paid by lump sum was paid for entry to the registered provider referred to in paragraph (c);

(f) the amount of each payment referred to in paragraph (e);

(g) the amount of each deduction made from the accommodation bond on or after 1 July 2006;

(h) the date of each deduction referred to in paragraph (g);

(i) the reason for each deduction referred to in paragraph (g);

(j) the accommodation bond balance as at 1 July 2006 (if applicable);

(k) the accommodation bond balance at the end of each calendar month commencing on or after 1 July 2006 during which the registered provider held the accommodation bond balance in respect of the individual.



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## 150A-25 Refundable deposit register to include information about accommodation bond balances refunded

For the purposes of section 150A of the Act, a registered provider must include the following information in the refundable deposit register in relation to an individual in respect of whom an accommodation bond balance is refunded on or after 1 July 2006:

(a) if accommodation bond balance was refunded because the individual died;
  (i) the date on which the individual died; and
  (ii) if applicable, the date on which the registered provider is shown the probate of the will of the individual, letters of administration of the estate of the individual or other evidence that satisfied the provider that the refundable deposit is to be refunded to a person;

(b) if the accommodation bond balance was refunded because the individual ceased to be provided with ongoing services in the services group residential care at the approved residential care home—the date on which the individual ceased to be provided with that care;

(c) if paragraph (b) applies and the individual notified the registered provider, before the date referred to in that paragraph, that the individual intended to enter another approved residential care home to receive residential care—the date of notification;

(d) the date on which, or by which, the registered provider was required to refund the accommodation bond balance to the individual, worked out in accordance with section 287-102 of this instrument;

(e) the date on which the accommodation bond payment balance was refunded;

(f) the amount of the accommodation bond balance refunded;

(g) the amounts (if any) of interest paid, as required by and worked out under section 287-103A of this instrument, broken down into amounts worked out using the base interest rate and amounts worked out using the pre-2014 maximum permissible interest rate, and the date or dates on which the amounts were paid.

## 150A-30 Refundable deposit register to include information about entry contributions paid and entry contribution balances held

For the purposes of section 150A of the Act, a registered provider must include the following information in the refundable deposit register in relation to an individual who holds an entry contribution balance on or after 1 July 2006:

(a) the name of the individual;

(b) the Resident Identification Number allocated by the Department in respect of the individual;

(c) the date on which the entry contribution was paid;

(d) the amount of the entry contribution;

(e) the entry contribution balance at the end of each calendar month commencing on or after 1 July 2006 during which the registered provider held an entry contribution balance in respect of the individual.



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## 150A-35 Refundable deposit register to include information about entry contributions balances refunded

For the purposes of section 150A of the Act, a registered provider must include the following information in the refundable deposit register in relation to an individual of whom an entry contribution balance is refunded on or after 1 July 2006:

(a) the date on which the individual ceased to be provided with care;

(b) the date on which the registered provider was required to refund the entry contribution balance to the individual worked out in accordance with the formal agreement applying in respect of the entry contribution balance;

(c) the date on which the entry contribution balance was refunded;

(d) the amount of the entry contribution balance refunded;

(e) the amount (if any) of interest paid and the date when the interest was paid.



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# Part 6—Aged care workers

## Division 1—Workforce and aged care worker requirements

### Subdivision A—Kinds of provider to which the condition applies

#### 152-5 Kinds of provider to which the condition applies

For the purposes of subsection 152(1) of the Act, every kind of registered provider is prescribed.

### Subdivision B—Worker screening requirements

#### 152-10 Purpose of this Subdivision

For the purposes of paragraphs 152(1)(a) and (b) of the Act, this Subdivision prescribes worker screening requirements.

#### 152-15 Responsible persons of registered providers delivering services other than under CHSP or NATSIFACP

*Clearance requirements*

(1) A registered provider (other than a registered provider to whom subsection (5) or (6) applies) must ensure that each responsible person of the provider is:
  (a) a person to whom subsection 152-25(1) (police certificates) applies; or
  (b) a person to whom subsection (2) of this section applies; or
  (c) a person in respect of whom an NDIS clearance decision is in force.

(2) This subsection applies to a person:
  (a) in respect of whom an NDIS screening application or a police certificate is pending; and
  (b) who is subject to appropriate supervision while that application or certificate is pending; and
  (c) who has made, and given to the registered provider, a statutory declaration stating the person:
    (i) has never been convicted of murder or sexual assault; and
    (ii) has never been convicted of, and sentenced to imprisonment for, any other form of assault.

*Notification requirements*

(3) A registered provider (other than a registered provider to whom subsection (5) or (6) applies) must take reasonable measures to require each responsible person of the provider to notify the provider if:
  (a) an NDIS exclusion decision is in force in respect of the person; or
  (b) an NDIS clearance decision in respect of the person is suspended; or



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(c) the person has been:
(i) convicted of murder or sexual assault; or
(ii) convicted of, and sentenced to imprisonment for, any other form of assault.

*Continuation requirements*

(4) A registered provider (other than a registered provider to whom subsection (5) or (6) applies) must ensure that a responsible person of the provider is not allowed to continue as a responsible person if the provider is satisfied on reasonable grounds that the person has been:
(a) convicted of murder or sexual assault; or
(b) convicted of, and sentenced to imprisonment for, any other form of assault.

*Registered providers in relation to whom this section does not apply*

(5) This subsection applies to a registered provider that:
(a) delivers funded aged care services under CHSP; and
(b) is registered in any of the following provider registration categories:
(i) home and community services;
(ii) assistive technology and home modifications;
(iii) advisory and support services;
(iv) personal and care support in the home or community;
(v) nursing and transition care.

(6) This subsection applies to a registered provider that delivers funded aged care services under NATSIFACP.

## 152-16 Responsible persons of registered providers delivering services only under CHSP, NATSIFACP, or CHSP and NATSIFACP

*Clearance requirements*

(1) A registered provider to whom subsection (6), (7) or (8) applies must ensure that each responsible person of the provider is:
(a) a person to whom subsection 152-25(2), (3) or (5) (police certificates) applies; or
(b) a person to whom subsection (2) of this section applies; or
(c) a person in respect of whom an NDIS clearance decision is in force.

(2) This subsection applies to a person:
(a) in respect of whom an NDIS screening application or a police certificate is pending; and
(b) who is subject to appropriate supervision while that application or certificate is pending; and
(c) who has made, and given to the registered provider, a statutory declaration stating the person:



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(i) has never been convicted of an offence involving the death of a person; and
(ii) has never been convicted of, and sentenced to imprisonment for at least one year for, an offence referred to in subsection (3).

(3) For the purposes of subparagraph (2)(c)(ii), the offences are the following:
(a) a sex related offence, including sexual assault (whether against an adult or child), child abuse material offences, or an indecent act involving a child;
(b) an offence involving dishonesty.

*Notification requirements*

(4) A registered provider to whom subsection (6), (7) or (8) applies must take reasonable measures to require each responsible person of the provider to notify the provider if:
(a) an NDIS exclusion decision is in force in respect of the person; or
(b) an NDIS clearance decision in respect of the person is suspended; or
(c) the person is, after the commencement of this section:
(i) convicted of an offence involving the death of a person; or
(ii) convicted of, and sentenced to imprisonment for at least one year for, an offence referred to in subsection (3).

*Continuation requirements*

(5) A registered provider to whom subsection (6), (7) or (8) applies must ensure that a responsible person of the provider is not allowed to continue as a responsible person if the provider is satisfied on reasonable grounds that the responsible person is, after the commencement of this section:
(a) convicted of an offence involving the death of a person; or
(b) convicted of, and sentenced to imprisonment for at least one year for, an offence referred to in subsection (3).

*Registered providers in relation to whom this section applies*

(6) This subsection applies to a registered provider that:
(a) delivers funded aged care services only under CHSP; and
(b) is registered in any of the following provider registration categories:
(i) home and community services;
(ii) assistive technology and home modifications;
(iii) advisory and support services;
(iv) personal and care support in the home or community;
(v) nursing and transition care.

(7) This subsection applies to a registered provider that delivers funded aged care services only under NATSIFACP.

(8) This subsection applies to a registered provider that delivers funded aged care services only under CHSP and NATSIFACP.



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## 152-17 Responsible persons of other registered providers

(1) A registered provider:
  (a) to whom either or both subsection (2) or (3) applies; and
  (b) to whom subsection (4) applies;
must comply with each of the requirements mentioned in section 152-16.

(2) This subsection applies to a registered provider that:
  (a) delivers funded aged care services under CHSP; and
  (b) is registered in any of the following provider registration categories:
    (i) home and community services;
    (ii) assistive technology and home modifications;
    (iii) advisory and support services;
    (iv) personal and care support in the home or community;
    (v) nursing and transition care.

(3) This subsection applies to a registered provider that delivers funded aged care services under NATSIFACP.

(4) This subsection applies to a registered provider that delivers funded aged care services other than under CHSP or NATSIFACP.

## 152-20 Aged care workers delivering services other than under CHSP or NATSIFACP

*Clearance requirements*

(1) A registered provider must ensure that each aged care worker of the provider, other than a worker to whom subsection (5) or (6) applies, is:
  (a) a person to whom subsection 152-25(1) (police certificates) applies; or
  (b) a person to whom subsection (2) of this section applies; or
  (c) a person in respect of whom an NDIS clearance decision is in force; or
  (d) a secondary school student on a formal work experience placement with the provider and the worker is directly supervised by a person to whom paragraph (a) or (c) applies.

(2) This subsection applies to a person:
  (a) in respect of whom an NDIS screening application or a police certificate is pending; and
  (b) who is subject to appropriate supervision while that application or certificate is pending; and
  (c) who has made, and given to the registered provider, a statutory declaration stating the person:
    (i) has never been convicted of murder or sexual assault; and
    (ii) has never been convicted of, and sentenced to imprisonment for, any other form of assault.



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*Notification requirements*

(3) A registered provider must take reasonable measures to require each aged care worker of the provider, other than a worker to whom subsection (5) or (6) applies, to notify the provider if:
  (a) an NDIS exclusion decision is in force in respect of the worker; or
  (b) an NDIS clearance decision in respect of the worker is suspended; or
  (c) the worker has been:
    (i) convicted of murder or sexual assault; or
    (ii) convicted of, and sentenced to imprisonment for, any other form of assault.

*Continuation requirements*

(4) A registered provider must ensure that an aged care worker of the provider, other than a worker to whom subsection (5) or (6) applies, is not allowed to continue as an aged care worker if the provider is satisfied on reasonable grounds that the worker has been:
  (a) convicted of murder or sexual assault; or
  (b) convicted of, and sentenced to imprisonment for, any other form of assault.

*Aged care workers in relation to whom this section does not apply*

(5) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services under CHSP.

(6) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services under NATSIFACP.

## 152-21 Aged care workers delivering services only under CHSP, NATSIFACP, or CHSP and NATSIFACP

*Clearance requirements*

(1) A registered provider must ensure that each aged care worker of the provider to whom subsection (5), (6) or (7) applies is:
  (a) a person to whom subsection 152-25(2), (3) or (5) (police certificates) applies; or
  (b) a person to whom subsection (2) of this section applies; or
  (c) a person in respect of whom an NDIS clearance decision is in force; or
  (d) a secondary school student on a formal work experience placement with the provider and the worker is directly supervised by a person to whom paragraph (a) or (c) applies.

(2) This subsection applies to a person:
  (a) in respect of whom an NDIS screening application or a police certificate is pending; and
  (b) who is subject to appropriate supervision while that application or certificate is pending; and



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(c) who has made, and given to the registered provider, a statutory declaration stating the person:
(i) has never been convicted of an offence involving the death of a person; and
(ii) has never been convicted of, and sentenced to imprisonment for at least one year for, an offence referred to in subsection 152-16(3).

*Notification requirements*

(3) A registered provider must take reasonable measures to require each aged care worker of the provider to whom subsection (5), (6) or (7) applies to notify the provider if:
(a) an NDIS exclusion decision is in force in respect of the worker; or
(b) an NDIS clearance decision in respect of the worker is suspended; or
(c) the worker is, after the commencement of this section:
(i) convicted of an offence involving the death of a person; or
(ii) convicted of, and sentenced to imprisonment for at least one year for, an offence referred to in subsection 152-16(3).

*Continuation requirements*

(4) A registered provider must ensure that an aged care worker of the provider to whom subsection (5), (6) or (7) applies is not allowed to continue as an aged care worker if the provider is satisfied on reasonable grounds that the worker is, after the commencement of this section:
(a) convicted of an offence involving the death of a person; or
(b) convicted of, and sentenced to imprisonment for at least one year for, an offence referred to in subsection 152-16(3).

*Aged care workers in relation to whom this section applies*

(5) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services only under CHSP.

(6) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services only under NATSIFACP.

(7) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services only under CHSP and NATSIFACP.

## 152-22 Other aged care workers

(1) A registered provider in respect of an aged care worker of the provider:
(a) to whom either or both subsection (2) or (3) applies; and
(b) to whom subsection (4) applies;
must comply with each of the requirements mentioned in section 152-21.

(2) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services under CHSP.



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(3) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services under NATSIFACP.

(4) This subsection applies to an aged care worker of a registered provider if the worker delivers funded aged care services other than under CHSP or NATSIFACP.

## 152-25 Police certificates

*Responsible persons or aged care workers of registered providers delivering services other than under CHSP or NATSIFACP*

(1) This subsection applies to a person if:
  (a) there is for the person a police certificate that is not more than 3 years old; and
  (b) the police certificate does not record that the person has been:
    (i) convicted of murder or sexual assault; or
    (ii) convicted of, and sentenced to imprisonment for, any other form of assault; and
  (c) for a person who has been, at any time after turning 16, a citizen or permanent resident of a country other than Australia—the person has made a statutory declaration stating that the person has never been:
    (i) convicted of murder or sexual assault; or
    (ii) convicted of, and sentenced to imprisonment for, any other form of assault.

*Responsible persons or aged care workers of registered providers delivering services under CHSP*

(2) This subsection applies to a person who is a responsible person or aged care worker of a registered provider to which subsection (4) applies if:
  (a) there is for the person a police certificate that is not more than 3 years old; and
  (b) either:
    (i) the person was engaged as a responsible person or aged care worker of the registered provider on or after the commencement of this section; or
    (ii) the police certificate was issued after the commencement of this section; and
  (c) the police certificate does not record that the person has been convicted of an offence involving the death of a person; and
  (d) the police certificate does not record that in the 5 years before the date of the certificate, the person has been convicted of, and sentenced to imprisonment for at least one year for, any of the following offences:
    (i) a sex-related offence, including sexual assault (whether against an adult or child), child abuse material offences, or an indecent act involving a child;
    (ii) an offence involving dishonesty; and



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(e) for a person who has been, at any time after turning 16, a citizen or permanent resident of a country other than Australia—the person has made a statutory declaration stating that the person has never been:
(i) convicted of an offence involving the death of a person; or
(ii) convicted of, and sentenced to imprisonment for at least one year for, a sex-related offence, including sexual assault (whether against an adult or child), child abuse material offences, or an indecent act involving a child; or
(iii) convicted of, and sentenced to imprisonment for at least one year for, an offence involving dishonesty.

(3) This subsection applies to a person who is a responsible person or aged care worker of a registered provider to which subsection (4) applies if:
(a) the person was engaged as a responsible person or aged care worker of the registered provider before the commencement of this section; and
(b) there is for the person a police certificate that:
(i) was issued before the commencement of this section; and
(ii) is not more than 3 years old.

(4) This subsection applies to a registered provider that:
(a) delivers funded aged care services under CHSP; and
(b) is registered in any of the following provider registration categories:
(i) home and community services;
(ii) assistive technology and home modifications;
(iii) advisory and support services;
(iv) personal and care support in the home or community;
(v) nursing and transition care.

*Responsible persons or aged care workers of registered providers delivering services under NATSIFACP*

(5) This subsection applies to a person who is a responsible person or aged care worker of a registered provider to which subsection (6) applies if:
(a) there is for the person a police certificate that is not more than 3 years old; and
(b) the police certificate does not record that the person has been convicted of an offence involving the death of a person; and
(c) the police certificate does not record that in the 5 years before the date of the certificate, the person has been convicted of, and sentenced to imprisonment for at least one year for, any of the following offences:
(i) a sex-related offence, including sexual assault (whether against an adult or child), child abuse material offences, or an indecent act involving a child;
(ii) an offence involving dishonesty; and
(d) for a person who has been, at any time after turning 16, a citizen or permanent resident of a country other than Australia—the person has made a statutory declaration stating that the person has never been:



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(i) convicted of an offence involving the death of a person; or

(ii) convicted of, and sentenced to imprisonment for at least one year for, a sex-related offence, including sexual assault (whether against an adult or child), child abuse material offences, or an indecent act involving a child; or

(iii) convicted of, and sentenced to imprisonment for at least one year for, an offence involving dishonesty.

(6) This subsection applies to a registered provider that:

(a) delivers funded aged care services under NATSIFACP; and

(b) is registered in any of the following provider registration categories:

(i) home and community services;

(ii) assistive technology and home modifications;

(iii) advisory and support services;

(iv) personal and care support in the home or community;

(v) nursing and transition care;

(vi) residential care.

## Subdivision C—Qualifications and training requirements

### 152-35 Qualifications and training requirements

For the purposes of paragraph 152(c) of the Act, the requirements are that aged care workers of a registered provider must have appropriate qualifications, skills or experience to provide the funded aged care services that the provider delivers to individuals.



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# Division 2—Vaccination

## 153-5 Kinds of provider to which the condition applies

For the purposes of subsection 153(1) of the Act, a registered provider registered in the registration category residential care is prescribed.

## 153-10 Requirements for providing access to vaccinations to individuals

For the purposes of subsection 153(1) of the Act, the requirement in accordance with which a registered provider must provide access to the vaccinations mentioned in subsection 153(2) of the Act for free to individuals to whom the provider is delivering funded aged care services is that the provider must do so in accordance with the National Immunisation Program Schedule, published by the Department, as existing from time to time.

Note: The National Immunisation Program Schedule could in 2026 be viewed on the Department's website (https://www.health.gov.au).



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# Part 7—Information and access

## Division 1—Personal information and record keeping

### Subdivision A—Purpose of this Division

#### 154-1 Purpose of this Division

For the purposes of paragraph 154(a) of the Act, this Division prescribes:

(a) the kinds of records that registered providers must keep and retain; and

(b) requirements for keeping and retaining those records.

### Subdivision B—Information provided to an individual

#### 154-2 Requirement to keep and retain information provided to an individual

(1) A registered provider must keep records demonstrating their compliance with their obligations and conditions of registration under Chapter 3.

(2) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.

#### 154-3 Requirement to correct personal information

(1) This section applies to a registered provider that is not any of the following:

(a) an APP entity within the meaning of the *Privacy Act 1988*;

(b) a State or Territory; or

(c) a body established for a public purpose by or under a law of the State or Territory (other than a local government authority).

(2) It is a requirement that when a registered provider keeps and retains records, the registered provider must comply with Australian Privacy Principle 13 as set out in Schedule 1 of the *Privacy Act 1988*, as if it were an organisation for the purposes of that Act.

Note: A registered provider that is an APP entity remains subject to the *Privacy Act 1988*, including APP 13.

### Subdivision C—Vaccination

#### 154-5 Application of this Subdivision to certain registered providers

This Subdivision applies to a registered provider registered in the provider registration category residential care.

#### 154-10 Records about service staff—influenza vaccinations

(1) A registered provider must keep records, for each calendar year, of the following information:



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(a) the total number of service staff in relation to the approved residential care home;

(b) the number of those service staff who have informed the registered provider, whether voluntarily or as required under a law of a State or Territory, that they have received the annual seasonal influenza vaccination for that year (whether or not under the registered provider's influenza vaccination scheme (if any)).

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## 154-15 Records about service staff—COVID-19 vaccinations

(1) A registered provider must keep records, for each calendar year, of the following information:

(a) the total number of service staff in relation to the approved residential care home;

(b) the number of those service staff who have voluntarily informed the registered provider that they have received a COVID-19 vaccination in that year.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## 154-20 Records about individuals receiving residential care—influenza vaccinations

(1) A registered provider must keep records, for each calendar year, of the following information:

(a) the total number of individuals accessing funded aged care services in the approved residential care home;

(b) the number of those individuals who have voluntarily informed the registered provider that they have received the annual seasonal influenza vaccination for that year.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.



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### 154-25 Records about individuals receiving residential care—COVID-19 vaccinations

(1) A registered provider must keep records, for each calendar year, of the following information:

(a) the total number of individuals accessing funded aged care services in the approved residential care home;

(b) the number of those individuals who have voluntarily informed the registered provider that they had received one or more COVID-19 vaccinations in that year;

(c) the number of individuals covered by paragraph (b) who have informed the registered provider that they had received only one COVID-19 vaccination;

(d) the number of individuals covered by paragraph (b) who have informed the registered provider that they had received 2 COVID-19 vaccinations.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision D—Quality indicators

### 154-105 Application of Subdivision to certain registered providers

This Subdivision applies to a registered provider that is required to give a quality indicators report under section 166-110.

### 154-110 Requirements for records on quality indicators

(1) Each of the following kinds of records are prescribed kinds of records that a registered provider must keep and retain:

(a) a quality indicators report;

(b) records relating to the collection and measurements of quality indicators.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision E—Incidents

### 154-150 Records about details of incidents

A registered provider must keep and retain the details recorded in relation to an incident under section 164-25(1) of this instrument in written or electronic form for 7 years starting on the day the record is made or received.



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## Subdivision F—Complaints and feedback

### 154-200 Application of Subdivision to feedback received

This Subdivision applies to feedback received by a registered provider:

(a) that is managed in accordance with section 165-30; and

(b) raises an issue as referred to in subsection 165-30(2).

### 154-205 Requirements for records of complaints and feedback

(1) Each of the following kinds of records are prescribed kinds of records that the registered provider must keep and retain:

(a) complaints and feedback received each year;

(b) the nature of complaints and feedback;

(c) the action taken to resolve complaints and feedback;

(d) responses provided to individuals about their complaints and feedback;

(e) any improvements made by the registered provider in relation to complaints and feedback;

(f) an evaluation of the effectiveness of the actions taken by the registered provider and their related outcome in relation to each complaint and feedback;

(g) a record of the number of days taken to resolve each complaint and feedback;

(h) the education and training that has been delivered to the aged care workers and responsible persons of the registered provider in relation to each complaint and feedback.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision G—Prudential and financial

### 154-300 Requirement to keep and retain financial and prudential reports

(1) Each of the following kinds of records are prescribed kinds of records that a registered provider must keep and retain:

(a) an aged care financial report;

(b) a quarterly financial report;

(c) a general purpose financial report;

(d) an annual prudential compliance statement.

Note: See Subdivision E of Division 3 of Part 2 of Chapter 5 of this instrument for requirements relating to each record prescribed.



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(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision H—CHSP

### 154-400 Application of Subdivision

This Subdivision applies to a registered provider of a kind who is registered in one or more of the following provider registration categories:

(a) home and community services;
(b) assistive technology and home modifications;
(c) advisory and support services;
(d) personal and care support in the home or community;
(e) nursing and transition care;

and delivers funded aged care services under the CHSP.

### 154-405 Requirements for records

(1) Each of the following kinds of records are prescribed kinds of records that the registered provider must keep and retain:

(a) the amount of financial assistance the System Governor, on behalf of the Commonwealth, has granted the registered provider within a particular financial year;
(b) any individual fees and contributions collected over the financial year;
(c) expenditure for the delivery of funded aged care services by the provider under the funding agreement;
(d) any surplus and uncommitted funds from the previous financial year;
(e) service types, and the duration of services, delivered to each individual;
(f) activity and performance data.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years, starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision J—NATSIFACP

### 154-500 Application of Subdivision

This Subdivision applies to a registered provider of a kind who is registered in one or more of the following provider registration categories:

(a) home and community services;
(b) assistive technology and home modifications;



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(c) advisory and support services;
(d) personal and care support in the home or community;
(e) nursing and transition care;
(f) residential care;

and delivers funded aged care services under the NATSIFACP.

### 154-505 Requirements for records

(1) Each of the following kinds of records are prescribed kinds of records that the registered provider must keep and retain:
(a) the amount of financial assistance the System Governor, on behalf of the Commonwealth, has granted the registered provider under subsection 264(2) of the Act within a particular financial year;
(b) any individual fees and contributions collected over the financial year;
(c) expenditure for the delivery of funded aged care services by the provider under the funding agreement;
(d) any surplus and uncommitted funds from the previous financial year;
(e) service types, and the duration of services, delivered to each individual;
(f) individual care and services plans;
(g) progress in embedding a wellness and reablement approach to service delivery;
(h) individual service agreements;
(i) a summary of the activities undertaken to prevent disease outbreaks;
(j) the number of individuals who are waiting to access funded aged care services for the financial year and the reason for the waitlist.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision K—Multi-Purpose Service Program

### 154-605 Application of Subdivision to certain registered providers

This Subdivision applies to a registered provider delivering funded aged care services under the MPSP.

### 154-610 Requirements for records

(1) Each of the following kinds of records are prescribed kinds of records that the registered provider must keep and retain:
(a) records of the amount of subsidy received under Division 5 of Part 2 of Chapter 4 of the Act to deliver funded aged care services at each approved residential care home;



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(b) records of any individual fees or contributions paid to the provider under Part 3 of Chapter 4 of the Act by individuals accessing funded aged care services, in or from, each approved residential care home;

(c) records of expenditure by the provider for the delivery of funded aged care services at each approved residential care home for the financial year;

(d) records of the service types delivered for the financial year;

(e) records of the number of individuals who have accessed funded aged care services, or are waiting to access services, in, or from, the approved residential care home for the financial year, including:

(i) whether the individual had an access approval when they commenced accessing services; and

(ii) the service types the individual accessed, including the services where specified in the individual's access approval; and

(iii) the individual's classification type for a service group; and

(iv) when the individual commenced and ceased accessing services; and

(v) the reason the individual ceased accessing services;

(f) records of the demographic information about each individual who have accessed funded aged care services, or are waiting to access services for the financial year, including:

(i) the name of the individual; and

(ii) the gender of the individual; and

(iii) the date of birth of the individual; and

(iv) whether the individual is an Aboriginal or Torres Strait Islander person; and

(v) whether the individual has dementia or dementia symptoms;

(f) records of a summary of the activities undertaken to prevent disease outbreaks.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision L—Transition Care Program

### 154-650 Application of Subdivision to certain registered providers

This Subdivision applies to a registered provider delivering funded aged care services under the TCP.

### 154-665 Requirements for records

(1) Each of the following kinds of records are prescribed kinds of records that the registered provider must keep and retain:

(a) records of claims for payment of subsidy made by the provider under section 260 of the Act;



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(b) records of the amount of subsidy the Commonwealth has paid to the provider for the financial year;
(c) records of any individual fees or contributions paid to the provider under Part 3 of Chapter 4 of the Act by individuals accessing funded aged care services for the financial year;
(d) records of expenditure by the provider for the delivery of funded aged care services for the financial year;
(e) records of any System Governor approved unspent funds from previous financial years.

(2) It is a requirement that a registered provider keep a record prescribed under this section in written or electronic form for 7 years starting on the day the record is made or received.

Note: A registered provider may be required to comply with other Commonwealth, State or Territory laws in relation to the retention of records.

## Subdivision M—Governing bodies

### 154-700 Records about independent non-executive members of a governing body

(1) If, because of paragraph 157(2)(a) of the Act and section 157-5 of this instrument, a registered provider is required to ensure that a majority of the members of its governing body are independent non-executive members, the registered provider must keep and retain records of:
(a) the names of the members of its governing body; and
(b) which of those members are independent non-executive members.

(2) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.

(3) Despite subsection (2), if a record prescribed under subsection (1) relates to a specific member of the governing body, the registered provider must keep the record for a period of 7 years following the end of that member's engagement with the governing body.

### 154-705 Records about members of a governing body with clinical care provision experience

(1) If, because of paragraph 157(2)(b) of the Act and section 157-10 of this instrument, a registered provider is required to ensure that at least one member of its governing body has experience in the provision of clinical care, the registered provider must keep and retain records of:
(a) the names of the members of its governing body who have experience in the provision of clinical care; and
(b) the details of those members' experience.

(2) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.



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(3) Despite subsection (2), if a record prescribed under subsection (1) relates to a specific member of the governing body, the registered provider must keep the record for a period of 7 years following the end of that member's engagement with the governing body.

## Subdivision N—Advisory bodies

### 154-800 Records about the quality care advisory body

(1) If, because of paragraph 158(2)(a) of the Act and section 158-5 of this instrument, a registered provider is required to establish, and continue in existence, a quality care advisory body, the registered provider must keep and retain the following:

(a) records of the names of the members of the quality care advisory body and details of:

(i) the date each member was appointed to the quality care advisory body; and

(ii) the date (if any) a member resigned from the quality care advisory body;

(b) records of how the quality care advisory body satisfies the requirements of section 158-15 of this instrument (requirements for reports of quality care advisory bodies);

(c) a copy of the minutes of any meeting held by the quality care advisory body and the date on which the meeting was held;

(d) a copy of each written report given to the governing body of the provider by the quality care advisory body under subparagraph 158(2)(a)(ii) of the Act;

(e) records of any feedback given to the governing body of the provider by the quality care advisory body under subparagraph 158(2)(a)(iii) of the Act;

(f) a copy of any written advice given to the quality care advisory body by the governing body of the provider under subparagraph 158(2)(b)(ii) of the Act advising how the governing body has considered the report and feedback mentioned in paragraphs (d) and (e) of this subsection.

(2) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.

(3) Despite subsection (2), if a record prescribed under subsection (1) relates to a specific member of the quality care advisory body, the registered provider must keep the record for a period of 7 years following the end of that member's engagement with the quality care advisory body.

### 154-805 Records about the consumer advisory body

(1) If, because of paragraph 158(4)(a) of the Act and section 158-20 of this instrument, a registered provider is required to offer the opportunity to establish one or more consumer advisory bodies, the registered provider must keep and retain the following:



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(a) a copy of each written offer made to individuals to whom the provider delivers funded aged care services and the supporters of those individuals giving them the opportunity to establish a consumer advisory body;
(b) records of the date on which each offer was given to those individuals and supporters.

(2) If one or more consumer advisory bodies are established, the registered provider must also keep and retain the following in relation to each body:
(a) a copy of the minutes of each meeting of the consumer advisory body;
(b) records of any feedback given to the governing body of the provider by the consumer advisory body;
(c) a copy of any written advice given to the consumer advisory body by the governing body under subparagraph 158(4)(b)(ii) of the Act advising how the governing body has considered any such feedback.

(3) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.

## Subdivision P—Worker screening

### 154-900 Records of responsible persons

(1) A registered provider must keep, and keep up-to-date, a record for each responsible person of the provider that includes the following information and documents:
(a) the person's full name, date of birth and address;
(b) if paragraph 152-15(1)(a) or 152-16(1)(a) applies to the person—a record of the person's police certificate;
(c) if paragraph 152-15(1)(b) or 152-16(1)(b) applies to the person—a record of the person's statutory declaration;
(d) if paragraph 152-15(1)(c) or 152-16(1)(c) applies to the person—a record of the person's NDIS clearance decision.

(2) Subject to subsection (3), the registered provider must also keep a record of:
(a) the name of the person who ensured that the responsible person has a police certificate, statutory declaration or NDIS clearance decision; and
(b) the date the person did so.

(3) Subsection (2) applies if the responsible person's police certificate, statutory declaration or NDIS clearance is received by the registered provider on or after the commencement of this section.

### 154-905 Records of aged care workers

(1) A registered provider must keep, and keep up-to-date, a record for each aged care worker of the provider that includes the following information and documents:
(a) the worker's full name, date of birth and address;
(b) how the provider has ensured that the worker:



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(i) has appropriate qualifications, skills or experience to provide the funded aged care services that the registered provider delivers to individuals (see section 152-35 of this instrument); and

(ii) is given opportunities to develop their capability to provide those services (see paragraph 152(d) of the Act);

(c) if paragraph 152-20(1)(a) or 152-21(1)(a) of this instrument applies to the worker—a record of the worker's police certificate;

(d) if paragraph 152-20(1)(b) or 152-21(1)(b) of this instrument applies to the worker—a record of the worker's statutory declaration;

(e) if paragraph 152-20(1)(c) or 152-21(1)(c) of this instrument applies to the worker—a record of the worker's NDIS clearance decision.

(2) Subject to subsection (3), the registered provider must also keep a record of:

(a) the name of the person who ensured that the worker has a police certificate, statutory declaration or NDIS clearance decision; and

(b) the date the person did so.

(3) Subsection (2) applies if the worker's police certificate, statutory declaration or NDIS clearance is received by the registered provider on or after the commencement of this section.

### 154-910 How long records required by sections 154-900 and 154-905 to be kept

A registered provider must keep a record required by section 154-900 or 154-905 for 7 years starting on the later of the following:

(a) the day the record is first made;

(b) the day the latest update to the record is made.

### 154-915 Copies of records relating to allegations of misconduct

(1) This section applies if a registered provider has a copy of a record relating to:

(a) an allegation of misconduct against a responsible person or aged care worker of the provider; or

(b) any action taken, including any investigation, in response to such an allegation.

(2) The provider must keep the copy for 7 years starting on the first day the provider has the copy.

## Subdivision Q—Delivery and continuity of funded aged care services

### 154-1000 Requirement to keep and retain records relating to continuity of funded aged care services

(1) A registered provider must keep the following kinds of records, or copies of such records, for an individual to whom the provider is delivering funded aged care services:

(a) any assessment and classification records of the individual that are not provided to the System Governor in electronic form;



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(b) the service agreement between the provider and the individual;
(c) the care and services plan for the individual;
(d) the medical records, progress notes and other clinical records of the individual;
(e) where the provider is delivering funded aged care services to the individual through the service group residential care, the accounts of the individual, including:
(i) the entry (if any) in the refundable deposit register of the provider that relates to the individual; and
(ii) amounts (if any) of daily accommodation payments, daily accommodation contributions or accommodation charge paid by the individual; and
(iii) entry and leave arrangements for the individual; and
(iv) the accommodation agreement between the provider and the individual; and
(f) where the provider is delivering funded aged care services to the individual through the service groups home support, assistive technology or home modifications, the accounts of the individual, including any notices provided to the individual in accordance with section 149-45;
(g) applicability of any fee reduction supplements to the individual under sections 197-5, 231-15 or 231-20;
(h) the name and contact details of any supporter registered in relation to the individual;
(i) measurements, assessments and information about the individual obtained in accordance with sections 154-110, 166-110 and 166-112 of this instrument;
(j) monthly statements prepared in accordance with section 155-40 of this instrument.

(2) It is a requirement that a registered provider keep a record made under this section for 7 years starting on the day the record is made.

## Subdivision R—Status of service delivery branches

### 154-1100 Requirement to keep and retain records about service delivery branches

(1) A registered provider must keep records of their compliance with the reporting requirements under Subdivision M of Division 3 of Part 2 of Chapter 5, including records of the following:
(a) any reports given under section 166-910;
(b) any reports given under section 166-915;
(c) any reports given under section 166-920;
(d) any reports given under section 166-925;
(e) any reports given under section 166-930.



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(2) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.

## Subdivision S—Claims for subsidy

### 154-1200 Application of this Subdivision

(1) This Subdivision applies to a registered provider registered in any registration category.

(2) Despite subsection (1), this Subdivision does not apply to a registered provider only delivering funded aged care services under one of the following specialist aged care programs:
   (a) CHSP;
   (b) MPSP;
   (c) NATSIFACP.

### 154-1205 Requirement to keep and retain records which enable claims for payment of subsidy to be verified

(1) A kind of record that a registered provider must keep and retain under paragraph 154(a) of the Act is any record that enables claims for payments of subsidy to be properly verified.

(2) Without limiting subsection (1), a kind of record that enables claims for payments of subsidy to be properly verified includes the following:
   (a) the service agreement between the registered provider and the individual to whom the provider is delivering funded aged care services;
   (b) the medical records, progress notes and other clinical records of the individual to whom the provider is delivering funded aged care services;
   (c) invoices for the delivery of funded aged care services to an individual;
   (d) attendance records for an aged care worker of the provider.

(3) It is a requirement that a registered provider keep a record prescribed under subsection (1) for 7 years starting on the day the record is made.

## Subdivision T—Compliance

### 154-1300 Requirement to keep and retain records relating to compliance

(1) A kind of record that a registered provider must keep and retain under section 154 of the Act is any record that enables proper assessments to be made of whether a provider has complied, or is complying, with its obligations under Chapter 3 of the Act.

(2) It is a requirement that a registered provider keep a record prescribed under this section for 7 years starting on the day the record is made.



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# Division 2—Provision of information to individuals

## 155-1 Purpose of this Division

For the purposes of section 155 of the Act, this Division prescribes the requirements for the following:

(a) the records and information that a registered provider must provide and explain to individuals accessing, or seeking to access, funded aged care services;

(b) the records and information (including personal information) held by the registered provider about the individual to whom the registered provider delivers funded aged care services that a registered provider must allow and facilitate access by an individual.

## 155-5 Kind of information to be provided and explained

For the purposes of subsection 155(1) of the Act, this Division prescribes requirements for the provision and explanation of the following kinds of information to individuals:

(a) the Statement of Rights;

(b) information to assist individuals to choose, in accordance with the individual's access approval, funded aged care services that best meet their needs;

(c) clear and understandable invoices;

(d) information about a registered provider's management and use of refundable deposits;

(e) information for prospective individuals;

(f) monthly statements;

(g) individualised budget;

(h) general information for individuals accessing any funded aged care services;

(i) general information for individuals accessing funded aged care services in a home or community setting;

(j) general information for individuals accessing funded aged care services in an approved residential care home;

(k) information about the financial position of the registered provider in a provider registration category other than residential care;

(l) pricing information.

## 155-15 Information to be provided and explained—Statement of Rights

(1) For the purposes of subsection 155(1) of the Act, a registered provider must provide an individual accessing or seeking to access funded aged care services the following:



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(a) information about the individual's rights under the Statement of Rights set out in section 23 of the Act, in relation to the funded aged care services the individual accesses;

(b) a copy of the Statement of Rights.

Note: The registered provider must retain records relating to the Statement of Rights given under paragraph (1)(b) (see section 154-2).

(2) A registered provider must assist the individual to understand the information and the Statement of Rights given under subsection (1).

(3) A registered provider must comply with subsections (1) and (2) before, or when, the registered provider commences delivery of funded aged care services to that individual.

## 155-20 Information to be provided—information to assist individuals to choose funded aged care services that best meet their needs

(1) This section applies to a registered provider registered in any of the following provider registration categories:

(a) home and community services;

(b) assistive technology and home modifications;

(c) advisory and support services;

(d) personal and care support in the home or community;

(e) nursing and transition care;

(f) subscription trial.

(2) A registered provider must provide such information as is reasonably necessary to assist an individual to choose, in accordance with the individual's access approval, the funded aged care services that best meet the individual's goals and assessed needs and preferences within the limits of the resources available.

## 155-25 Information to be provided—clear and understandable invoices

For the purposes of subsection 155(1) of the Act, a registered provider must provide an individual with invoices that are clear and in a format that is understandable.

## 155-30 Information to be provided—information about a registered provider's management and use of refundable deposits

(1) This section applies to a registered provider if the registered provider is prescribed under section 150A-3 of this instrument.

*Notification by registered provider*

(2) Within 7 days after an accommodation agreement is entered into between a registered provider and an individual, the registered provider must notify the individual, in writing, that the registered provider will give the individual, within 7 days of a request by the individual, the following information and documents:



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(a) a summary of the permitted uses for which refundable deposits and accommodation bonds have been used by the registered provider during the previous financial year;

(b) if, during the 2 years prior to the request, the registered provider has given the System Governor an aged care financial report for the registered provider that included a permitted uses reconciliation—a copy of the permitted uses reconciliation included in the most recent such report;

(c) information about whether the registered provider has, during the previous financial year, complied with:

(i) section 310 of the Act; and

(ii) the Financial and Prudential Standards;

(d) information about:

(i) the number (if any) of refundable deposit balances or accommodation bond balances that, in the previous financial year, were not refunded in accordance with section 311 of the Act or section 287-102 of this instrument; and

(ii) the number (if any) of entry contribution balances that, in the previous financial year, were not refunded in accordance with a formal agreement;

(e) if the registered provider is required to implement and maintain a written investment management strategy under the Financial and Prudential Standards—the registered provider's investment objectives as recorded in the registered provider's investment management strategy;

(f) a copy of the audit opinion referred to in paragraph 166-380(g) of this instrument for the previous financial year;

(g) a copy of either:

(i) the most recent statement of the audited accounts in relation to the registered provider; or

(ii) if the registered provider is operated as part of a broader organisation—the most recent statement of the audited accounts of the organisation's aged care component;

(h) a copy of the refundable deposit register entry that relates to the individual, as at the time of the request.

(3) A registered provider that is not required to prepare annual financial reports under Part 2M.3 of Chapter 2M of the *Corporations Act 2001* is not required to comply with paragraph (2)(g) of this section.

*Disclosure of refundable deposit register entry relating to an individual*

(4) A copy of the refundable deposit register entry that relates to the individual must be provided to the individual or another person in the following circumstances:

(a) when the individual ceases to access funded aged care services through the registered provider's approved residential care home, the record must be provided to the individual, or if the individual has died, the individual's legal representative or estate;

(b) when the individual moves to another approved residential care home of another approved provider, the record must be provided to the individual.



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*Disclosure on request*

(5) If an individual requests the registered provider of an approved residential care home in which the individual is accessing funded aged care services to give the individual the information and documents referred to in subsection (2) or (4), the registered provider must give the individual the information and documents requested within 7 days after receiving the request.

(6) Subsection (5) applies in relation to an individual who is accessing funded aged care services in an approved residential care home:
  (a) whether or not the individual entered into an accommodation agreement with the registered provider of the approved residential care home; and
  (b) whether or not the individual has paid a refundable deposit; an accommodation bond or an entry contribution to that registered provider.

(7) If:
  (a) as a result of a request by an individual, a registered provider is required under subsection (5) to give the individual the summary referred to in paragraph (2)(a); and
  (b) the registered provider has given the System Governor an aged care financial report for the registered provider for the previous financial year, and that report included a permitted uses reconciliation;

the registered provider may satisfy the requirement to give the individual the summary by giving the individual a copy of that permitted uses reconciliation.

*Disclosure after end of financial year for registered provider*

(8) Within 4 months after the end of each financial year for a registered provider, the registered provider must give each individual who has paid a refundable deposit, an accommodation bond or an entry contribution to the registered provider for entry to the approved residential care home operated by the registered provider:
  (a) a copy of the refundable deposit record that relates to the individual as at the end of the financial year; and
  (b) a written statement that the registered provider will provide, within 7 days of a request by the individual, the information and documents referred to in subsection (2).

## 155-35 Information to be provided—to prospective individuals

(1) This section applies to a registered provider if the registered provider is prescribed under section 150A-3 of this instrument.

(2) Within 7 days of a request from a prospective individual, a registered provider must give the prospective individual the information and documents referred to in paragraphs 155-30(2)(a) to (g).

## 155-36 Information to be provided—statement of audited accounts

(1) Subject to subsection (2), if section 155-30 of this instrument does not apply to the registered provider that is registered in the provider registration category



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residential care, the provider must, if asked by an individual to whom the provider is delivering funded aged care services, give the individual, within 7 days of the request, a copy of either:

(a) the most recent statement of the audited accounts in relation to the registered provider; or

(b) if the registered provider is operated as part of a broader organisation—the most recent statement of the audited accounts of the organisation's aged care component; or

(c) the most recent audited general purpose financial report prepared under Subdivision E of Division 3 of Part 2 of Chapter 5 of this instrument.

(2) Despite subsection (1) of this section, a registered provider is not required to comply with this requirement if the registered provider:

(a) is not required to prepare annual financial reports under Part 2M.3 of the Chapter 2M of the *Corporations Act 2001*; and

(b) is not required to prepare a general purpose financial report under section 166-345 of this instrument.

## 155-40 Information to be provided—monthly statement

(1) This section applies to a registered provider registered in any of the following provider registration categories:

(a) home and community services;

(b) assistive technology and home modifications;

(c) advisory and support services;

(d) personal and care support in the home or community;

(e) nursing and transition care;

(f) subscription trial.

(2) The requirements of this section do not apply in respect of the delivery of funded aged care services under a specialist aged care program.

(3) A registered provider that delivers funded aged care services to an individual through a service delivery branch in a calendar month must give the individual a written statement relating to the services delivered in that calendar month no later than the last day of the following month.

*Monthly statement for an individual accessing care through the service group home support*

(4) The monthly statement must contain the following:

(a) the amount of person-centred subsidy for the individual for the quarter in their ongoing home support account or the amount of person-centred subsidy for the individual for the episode in their short-term home support account;

(b) the amount of person-centred subsidy that was available for the individual in their ongoing or short-term home support account at the beginning of the calendar month;



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(c) the amount of person-centred subsidy for the individual remaining in their ongoing or short-term home support account at the end of the calendar month;

(d) the name and amount of the primary-centred supplements (if any) for the individual in their ongoing home support account for the quarter and the name and amount of the primary-centred supplements (if any) for the individual in their short-term home support account for the episode;

(e) rollover credits received in respect of the quarter;

(g) an itemised list of:

(i) each episode of services or items delivered to the individual during the calendar month; and

(ii) any adjustments, variations or refunds from previous calendar months, including the service name and delivery date if relating to a service; and

(iii) any adjustments, variations or refunds from previous calendar months, including the service name and delivery date if relating to individual contribution amounts;

(h) for each service or item delivered under the classification type ongoing or the classification type short-term:

(i) the name of the service or item as described in the service list under section 8-5 of this instrument; and

(ii) the price or cost that the registered provider charged the individual in respect of the calendar month; and

(iii) the date on which the service or item was delivered in respect of the calendar month; and

(iv) the contribution amount for the individual in respect of each service or item delivered in the calendar month; and

(v) the total amount of contribution paid by the individual for the calendar month; and

(vi) the amount of subsidy claimed by the registered provider in respect of each service or item delivered in the calendar month; and

(vii) the number of units or hours delivered in respect of the calendar month; and

(viii) if the service or item was delivered by an associated provider of the registered provider or by a third party, that fact;

(i) for each service delivered under the classification type ongoing or short-term:

(i) identify the individual contribution rate, which must be provided each time there is a change to the contribution rate arising from a new determination made under section 314 of the Act; and

(ii) the corresponding amount for which the individual is responsible to contribute for delivery of that service;

(j) for individuals with the classification type ongoing, information relating to the total number of units or hours delivered for the service type care management delivered in the relevant calendar month;



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(k) for individuals with the classification level SAH end-of-life pathway for the classification type short-term, information relating to:
  (i) the total number of units or hours delivered for the service type care management; and
  (ii) the price and cost of the service and amount of subsidy claimed by the provider in respect of that service;
(l) for individuals with the classification level SAH restorative care pathway for the classification type short-term, information relating to:
  (i) the total number of units or hours delivered for the service type restorative care management; and
  (ii) the price and cost of the service and amount of subsidy claim by the provider in respect of that service;
(m) for each service taken to be delivered in accordance with subsection 11A(2) of the Act:
  (i) the name of the service as provided in the list referred to in subsection 8(1) of the Act; and
  (ii) the amount that the registered provider charged the individual for the service; and
  (iii) the date on which the service was taken to be delivered; and
  (iv) if the registered provider charged an individual contribution for the service, the individual contribution rate used to work out the individual contribution; and
  (v) the amount of subsidy claimed by the registered provider for the service.

*Monthly statement for an individual accessing care through the service groups assistive technology or home modifications*

(5) The monthly statement must contain the following:
(a) the amount of person-centred subsidy for the individual for the account period;
(b) the available balance of the individual's notional assistive technology or home modifications account (if any) at the beginning of the calendar month;
(c) the available balance of the individual's notional assistive technology account or home modifications account at the end of the calendar month;
(d) the available balance of the individual's notional assistive technology account or notional home modifications account after taking into account any committed funds;
(e) any amount of the available balance that is no longer available to the individual due to the ceasing of the individual's notional assistive technology account or notional home modifications account in respect of the calendar month;
(f) the end date of the account period for the individual's notional assistive technology account or notional home modifications account in the following calendar month;



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(g) the name and amount of primary person-centred supplements (if any) for the individual for the account period;

(h) any additional approved amounts as specified in the determination made under subsection 211-20(5) of this instrument;

(i) an itemised list of:

(i) assistive technology or home modification items and services (if any), including administration or coordination, delivered to the individual during the calendar month; and

(ii) any adjustments, variations or refunds from previous calendar months;

(j) for each item or service delivered:

(i) the name of the item or service; and

(ii) the price that the registered provider charged the individual for the item or service in respect of the calendar month; and

(iii) the date on which the item or service was delivered in respect of the calendar month; and

(iv) the amount of contribution (if any) paid by the individual in respect of each item or service delivered in the calendar month; and

(v) the total amount of contribution paid by the individual for the calendar month; and

(vi) the amount of subsidy claimed by the registered provider in respect of each service or item delivered in the calendar month; and

(vii) the rate and number of item units or service hours delivered in respect of the calendar month; and

(viii) if the service or item was delivered by an associated provider of the registered provider or by a third party, that fact;

(k) any commitments to assistive technology or home modifications made during the calendar month, including those items, modifications or related services which are agreed to with the individual, but which have not yet been delivered within the calendar month;

(l) for each item or service committed to but not yet delivered:

(i) the name and price for each item or service agreed to; and

(ii) the amount of contribution in respect of each item or service that remains to be paid; and

(iii) the total amount of committed funds, including a breakdown of the total amount of subsidy to be claimed and the total amount of contribution to be paid;

(m) any assistive technology or home modification items charged through the assistive technology or home modifications account or through unspent Commonwealth portion;

(n) for each service taken to be delivered in accordance with subsection 11A(2) of the Act:

(i) the name of the service as provided in the service list under section 8-5 of this instrument; and

(ii) the amount that the registered provider charged the individual for the service; and

(iii) the date on which service was taken to be delivered; and



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(iv) if the registered provider charged an individual contribution for the service, the individual contribution rate used to work out the individual contribution; and

(v) the amount of subsidy claimed by the registered provider for the service.

*Unspent portions and notional home care accounts*

(6) If an individual has a notional home care account; or the registered provider holds an unspent Commonwealth portion or an unspent care recipient portion for the individual, the monthly statement must contain the following:

(a) the available balance of the individual's notional home care account at the beginning and end of the calendar month;

(b) the available balance of the individual's unspent Commonwealth portion at the beginning and end of the calendar month;

(c) the amount of the individual's unspent care recipient portion at the beginning and end of the calendar month.

*Monthly statement for established account types*

(7) The monthly statement must only contain information regarding the notional accounts established for the individual.

*Monthly statement for a period in which no services are delivered*

(8) If no service or item was delivered by registered provider to the individual during the calendar month, a statement must still be provided no later than the last day of the following calendar month.

*When statement must be given*

(9) A registered provider must give the statement to the individual no later than the last day of the following calendar month.

*Informing the individual of, and helping the individual to understand, statement*

(10) A registered provider must inform the individual of and help the individual to understand the monthly statement.

## 155-41 Information to be provided with monthly statement—subscription services

(1) This section applies to a registered provider in respect of a calendar month if:

(a) the provider is registered in the provider registration category subscription trial; and

(b) the provider is required under subsection 155-40(3) to give an individual a written statement relating to the funded aged care services delivered to the individual by the provider in the month; and

(c) the services included a subscription service.



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(2) The registered provider must give the individual a statement, for each subscription service delivered to the individual in the month, of the use that the individual made of the service in the month, including the following:

(a) each day that the individual accessed the service made available to the individual by the subscription service;

(b) for each day referred to in paragraph (a):

(i) the number of hours on the day that the individual accessed the service made available to the individual by the subscription service; and

(ii) whether the service made available to the individual by the subscription service was delivered by an associated provider of the registered provider;

(c) for the month—the total number of hours that the individual accessed the service made available to the individual by the subscription service.

(3) The registered provider must give the statement under subsection (2) at the same time that the provider gives the statement under subsection 155-40(3).

## 155-45 Information to be given—final monthly statement

(1) This section applies to a registered provider registered in any of the following provider registration categories:

(a) home and community services;

(b) assistive technology and home modifications;

(c) advisory and support services;

(d) personal and care support in the home or community;

(e) nursing and transition care;

(f) subscription trial.

(2) The requirements of this section do not apply in respect of the delivery of funded aged care services under a specialist aged care program.

(3) If a registered provider ceases to deliver funded aged care services to the individual, the registered provider must give the individual or their supporter, after the final claim is made for the individual, a final monthly statement.

(4) The final monthly statement must be prepared in accordance with subsection 155-40(4) or (5).

(6) A registered provider must give the final monthly statement to the individual or their supporter no later than the last day of the calendar month after the last claim is made.

## 155-50 Information to be provided—individualised budget

(1) This section applies to a registered provider registered in any of the following provider registration categories:

(a) home and community services;

(b) assistive technology and home modifications;



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(c) advisory and support services;
(d) personal and care support in the home or community;
(e) nursing and transition care;
(f) subscription trial.

(2) The requirements of this section do not apply in respect of the delivery of funded aged care services under a specialist aged care program.

(3) A registered provider that delivers funded aged care services to an individual through one or more of the service groups home support, assistive technology and home modifications must give the individual a written individualised budget.

(4) The individualised budget for the individual must:
(a) be prepared in partnership with the individual; and
(b) cover a period agreed with the individual (the ***budget period***); and
(c) be prepared having regard to the individual's preferences, goals and assessed needs, the resources available and the funded aged care services selected by the individual for the budget period; and
(d) set out an itemised budget for the funded aged care services to be delivered to the individual in the budget period, as set out in the individual's care and services plan, including, for each service to be delivered to the individual on a day:
(i) if an individual contribution rate determination for the individual is in effect—the individual contribution rate for the service for the individual; and
(ii) if an individual contribution rate determination for the individual is not in effect—the maximum individual contribution rate that could apply to the service for the individual (when determined for each means testing category for the individual under section 314 of the Act); and
(iii) if known—the cost for the delivery of the service; and
(iv) if known—the amount that will be the individual contribution for the delivery of the service to the individual on the day; and
(v) if known—the amount of subsidy for which the provider will be eligible for the delivery of the service to the individual on the day; and
(e) for the service group assistive technology, the itemised budget must also specify the following (if known):
(i) the description and cost for assistive technology products and equipment;
(ii) the description and cost for assistive technology product and equipment repair or maintenance;
(iii) the prescription cost (if applicable) for assistive technology;
(iv) the description and cost of wraparound activities for assistive technology;
(v) administrative costs charged by the registered provider for assistive technology; and



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(f) for the service group home modifications, the itemised budget must also specify the following:
  (i) the description and cost of home modification supplies and services;
  (ii) the prescription cost (if applicable) for home modifications;
  (iii) the description and cost of wraparound activities for home modifications;
  (iv) coordination costs charged by the registered provider for home modifications; and
(g) if known—set out the amount of subsidy payable to the registered provider for the individual in respect of the period agreed between the individual and the registered provider.

(5) A registered provider must give the individualised budget to the individual as soon as practicable after the registered provider has all the necessary information to complete it.

(6) A registered provider must review and, if necessary, revise the individualised budget for the individual if:
(a) a change to the funded aged care services mentioned in paragraph (4)(d) is proposed; or
(b) there is a change to the costs for delivering those services; or
(c) the individual contribution rate for the individual changes; or
(d) the individual requests the registered provider to do so.

(7) If a registered provider reviews and revises the individualised budget for an individual, the registered provider must give the individual a copy of the revised individualised budget:
(a) if the review and revision was for a reason referred to in paragraph (6)(a), (b) or (c)—as soon as practicable after the registered provider has all the necessary information to complete it; or
(b) if the review and revision was in response to a request referred to in paragraph (6)(d)—within 14 days of the request being made.

(8) The registered provider must inform the individual of, and help the individual to understand, the individualised budget for the individual.

## 155-55 Information to be provided—general information for individuals accessing any funded aged care services

(1) For the purposes of subsection 155(1) of the Act, a registered provider must give an individual accessing, or seeking to access, funded aged care services the following:
(a) a copy of the document mentioned in paragraph 165-20(1)(f) of this instrument, relating to giving complaints and feedback;
(b) an explanation of the effect of section 168 of the Act (which deals with the protection of personal information);
(c) a copy of the Aged Care Code of Conduct.



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(2) A registered provider must assist the individual to understand the information given under subsection (1) of this section.

(3) A registered provider must comply with subsections (1) and (2) before, or when, the registered provider commences delivery of funded aged care services to that individual.

## 155-60 Information to be provided—general information for individuals accessing funded aged care services in a home or community setting

(1) This section applies to a registered provider registered in any of the following provider registration categories:
   (a) home and community services;
   (b) assistive technology and home modifications;
   (c) advisory and support services;
   (d) personal and care support in the home or community;
   (e) nursing and transition care;
   (f) subscription trial.

(2) For the purposes of subsection 155(1) of the Act, a registered provider must give an individual accessing, or seeking to access, funded aged care services in a home or community setting the following:
   (a) an explanation of the effect of Division 1 of Part 5 of Chapter 4 of the Act and Division 1 of Part 2 of Chapter 10 of this instrument (which deals with means testing in a home or community setting);
   (b) information about the circumstances in which the provider may cease delivery of funded aged care services to the individual as specified in subsection 149-35(2) of this instrument, including an explanation of the effect of section 149-40 (which deals with notice requirements applicable to the provider);
   (c) an explanation of the effect of subsection 148-80(1) of this instrument (which deals with the development of a care and services plan for the individual);
   (d) information that the provider will give the individual a monthly statement in accordance with sections 155-40 and 155-45 of this instrument;
   (e) for individuals accessing funded aged care services through the service group home support, other than under a specialist aged care program—a copy of the most recent version of the Support at Home Service List published by the Department.

Note: The Support at Home Service List could in 2026 be viewed on the Department's website (https://www.health.gov.au).

(3) A registered provider must assist the individual to understand the information given under subsection (2) of this section.

(4) A registered provider must comply with subsections (2) and (3) before, or when, the registered provider commences delivery of funded aged care services to that individual.



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(5) Despite subsection (2) of this section, the requirements of paragraphs (2)(a) and (2)(d) of this section do not apply in respect of the delivery of funded aged care services delivered under any specialist aged care program.

## 155-65 Information to be provided—general information for individuals accessing funded aged care services in an approved residential care home

(1) This section applies to a registered provider registered in the provider registration category residential care.

(2) For the purposes of subsection 155(1) of the Act, a registered provider must give an individual accessing, or seeking to access, funded aged care services in an approved residential care home the following:
   (a) information about the circumstances in which the individual may be asked to leave the approved residential care home as specified in subsection 149-60(1) of this instrument, including an explanation of the effect of section 149-65 (which deals with notice requirements applicable to the provider);
   (b) information about any policies or protocols of the approved residential care home that are relevant to the individual.

(3) A registered provider must assist the individual to understand the information given under subsection (2) of this section.

(4) A registered provider must comply with subsections (2) and (3) before, or when, the registered provider commences delivery of funded aged care services to that individual.

## 155-70 Information to be provided—information about the financial position of a registered provider registered in a provider registration category other than residential care

(1) Subject to subsection (2), this section applies to a registered provider registered in any of the following provider registration categories:
   (a) home and community services;
   (b) assistive technology and home modifications;
   (c) advisory and support services;
   (d) personal and care support in the home or community;
   (e) nursing and transition care;
   (f) subscription trial.

(2) This section does not apply to registered providers of services in the registration categories listed in subsection (1) which are government entities.

(3) For the purposes of subsection 155(1) of the Act, before or when a registered provider starts delivering funded aged care services to an individual the registered provider must notify the individual, in writing, that the registered



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provider will give the individual, within 7 days of a request by the individual, the following information and documents:

(a) a clear and simple presentation of the financial position of the registered provider;

(b) a copy of the most recent statement of the audited accounts of the service delivery branch or, if the service delivery branch is operated as part of a broader organisation, the most recent statement of the audited accounts of the organisation's aged care component (that includes the service delivery branch).

(4) If an individual requests the registered provider to give the individual the information and documents referred to in subsection (3), the registered provider must give the individual the information and documents requested within 7 days after receiving the request.

(5) A registered provider that is not required to prepare annual financial reports under Part 2M.3 of Chapter 2M of the *Corporations Act 2001* is not required to comply with paragraph (3)(b) of this section.

## 155-80 Information to be provided—pricing information (by way of publication)

(1) This section applies to a registered provider registered in any of the following provider registration categories:

(a) home and community services;

(b) assistive technology and home modifications;

(c) advisory and support services;

(d) personal and care support in the home or community;

(e) nursing and transition care.

*Service group home support*

(2) A registered provider delivering funded aged care services through the service group home support must publish on the registered provider's website the most common price that the registered provider charges individuals for each service in the following service types:

(a) allied health and therapy;

(b) care management;

(c) domestic assistance;

(d) home maintenance and repairs;

(e) home or community general respite;

(f) meals;

(g) nursing care;

(h) personal care;

(i) restorative care management;

(j) social support and community engagement;

(k) therapeutic services for independent living;

(l) transport.



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(3) The pricing information published for the purposes of subsection (2) of this section must:

(a) specify the most common price for delivery of the service during standard business hours; and

(b) specify the most common price for delivery of the service on weekdays outside standard business hours; and

(c) specify the most common price for delivery of the service on Saturdays; and

(d) specify the most common price for delivery of the service on Sundays; and

(e) specify the most common price for delivery of the service on public holidays.

(4) In this section:

***most common price*** means the price for a service that the registered provider has most frequently charged during the previous 2 calendar months, calculated at the end of the following:

(a) August;

(b) October;

(c) December;

(d) February;

(e) April;

(f) June.

(5) A registered provider must update the pricing information on the registered provider's website for a service where the most common price for the service changes, within 30 days of the end of the calculation period mentioned in subsection (4).

(6) A registered provider is not required to publish a price for a service under subsection (2) if the registered provider is not currently delivering the service and has not delivered the service in the last 12 months.

(7) The requirements of this section do not apply to a registered provider in respect of the delivery of funded aged care services under a specialist aged care program.

*Service group assistive technology*

(8) A registered provider registered in the provider registration category assistive technology and home modifications delivering funded aged care services through the service group assistive technology must publish on the registered provider's website:

(a) whether the registered provider charges a provider administration fee for the provision of funded aged care services; and

(b) if the registered provider charges a provider administration fee, the amount of the provider administration fee as a percentage of the price for the provision of services.



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(9) A registered provider must update the pricing information on the registered provider's website where the information published under subsection (8) changes, within 30 days of the change.

*Service group home modifications*

(10) A registered provider registered in the provider registration category assistive technology and home modifications delivering funded aged care services through the service group home modifications must publish on the registered provider's website:

(a) whether the registered provider charges a provider coordination fee for the provision of funded aged care; and

(b) if the registered provider charges a provider coordination fee, the amount of the provider coordination fee as a percentage of the price for the provision of services.

(11) A registered provider must update the pricing information on the registered provider's website where the information published under subsection (10) changes, within 30 days of the change.

## 155-85 Requirements for allowing and facilitating access to information held about an individual

(1) For the purposes of subsection 155(2) of the Act, a registered provider that is not any of the following is prescribed:

(a) an APP entity within the meaning of the *Privacy Act 1988*;

(b) a State or Territory;

(c) body established for a public purpose by or under a law of the State or Territory (other than a local government authority).

(2) For the purposes of paragraph 155(2)(b) of the Act, it is a requirement when a registered provider is allowing and facilitating access by an individual to whom the registered provider delivers funded aged care services to records and information (including personal information) held by the registered provider about the individual, that the registered provider must comply with Australian Privacy Principle 12 as set out in Schedule 1 of the *Privacy Act 1988*, as if it were an organisation for the purposes of that Act.

Note: A registered provider that is an APP entity remains subject to the *Privacy Act 1988*, including APP 12.



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# Division 3—Access by supporters etc.

## 156-5 Access to individuals

(1) For the purposes of subsection 156(1) of the Act, every kind of registered provider is prescribed.

*Supporters*

(2) For the purposes of subsection 156(1) of the Act, it is a requirement that a registered provider must allow and facilitate access (whether physically, by visual link or other reasonable means requested by the individual) by a supporter of an individual to whom the provider delivers funded aged care services to the individual at any time requested, or consented to, by the individual.

Note: See also subsection 156(4) of the Act, which relates to providers delivering services to an individual in a home or community setting, and access requirements only applying in relation to times during which those services are being delivered.

*Legal advisors etc.*

(3) For the purposes of subsection 156(1) of the Act, it is a requirement that a registered provider must allow and facilitate access (whether physically, by visual link or other reasonable means requested by the individual) to an individual to whom the provider delivers funded aged care services by a relevantly qualified person providing legal advice or another legal service to the individual at any time requested, or consented to, by the individual.

Note: See also subsection 156(4) of the Act, which relates to providers delivering services to an individual in a home or community setting, and access requirements only applying in relation to times during which those services are being delivered.

*Independent aged care advocates*

(4) For the purposes of subsection 156(1) of the Act, it is a requirement that a registered provider must allow and facilitate access (whether physically, by visual link or other reasonable means requested by the individual) by an independent aged care advocate to an individual to whom the provider delivers funded aged care services:

(a) unless paragraph (b) applies—at any time requested, or consented to, by the individual; or

(b) if the individual is unable to request or consent to the access—at any time.

Note: See also subsection 156(4) of the Act, which relates to providers delivering services to an individual in a home or community setting, and access requirements only applying in relation to times during which those services are being delivered.

*Aged care volunteer visitor*

(5) For the purposes of subsection 156(1) of the Act, it is a requirement that a registered provider must allow and facilitate access (whether physically, by visual link or other reasonable means requested by the individual) by an aged



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care volunteer visitor to an individual to whom the provider delivers funded aged care services at any time requested, or consented to, by the individual.

Note: See also subsection 156(4) of the Act, which relates to providers delivering services to an individual in a home or community setting, and access requirements only applying in relation to times during which those services are being delivered.

## 156-10 Access to settings

(1) For the purposes of subsection 156(2) of the Act:

(a) every kind of registered provider is prescribed; and

(b) it is a prescribed requirement that, if:

(i) a registered provider delivers funded aged care services to an individual in a setting; and

(ii) the individual requests, or consents to, access by an independent aged care advocate; and

(iii) the purpose of the access by the advocate is to provide information and education to the individual;

the registered provider must allow and facilitate the access to the individual, and any other individual who requests or consents to the access, in the setting.

Note 1: See also subsection 156(4) of the Act, which relates to providers delivering services to an individual in a home or community setting, and access requirements only applying in relation to times during which those services are being delivered.

Note 2: See also subsections 156(6) and (7) of the Act, which relate to access to settings.

(2) Without limiting paragraph (1)(b), a setting may include the following:

(a) a residential care home;

(b) a home or community setting.



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# Part 8—Governance

## Division 1—Membership of governing bodies

### 157-5 Kinds of provider to which the independent non-executive members requirement applies

(1) For the purposes of paragraph 157(2)(a) of the Act, a registered provider registered in any of the following provider registration categories is prescribed, unless the provider is covered by subsection (2):
  (a) nursing and transition care;
  (b) residential care.

(2) This subsection covers a registered provider that is:
  (a) a kind of body known as an Aboriginal Community Controlled Organisation; or
  (b) a body that is registered under the Co-operatives National Law or the *Co-operatives Act 2009* (WA) as a co-operative and has clauses in its rules to the effect that:
    (i) an objective or primary activity of the registered provider is delivering quality funded aged care services to individuals in accordance with the Aged Care Quality Standards; and
    (ii) distribution of assets to members upon winding-up are limited to the nominal value of the member's contribution to the co-operative, provided that such a limitation complies with the legislation under which the co-operative is registered.

### 157-10 Kinds of provider to which the clinical care provision experience requirement applies

For the purposes of paragraph 157(2)(b) of the Act, a registered provider is prescribed if the provider:
  (a) is registered in any of the following provider registration categories:
    (i) nursing and transition care;
    (ii) residential care; and
  (b) is not a kind of body known as an Aboriginal Community Controlled Organisation.

### 157-15 Kinds of providers to which other governing body requirements apply

For the purposes of subsection 157(5) of the Act, a registered provider that is covered by paragraph 157-5(2)(b) of this instrument must ensure that the members of the provider's governing body undertake training on governance focused on co-operatives or includes course material on co-operatives within 6 months of the member's appointment as a member of the governing body.



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## 157-20 Application for determination that certain conditions do not apply—other matters that the Commissioner may take into account

For the purposes of paragraph 159(4)(h) of the Act, the matters are the following:

(a) subject to paragraph (b) of this section, the registered provider's history of compliance with the governance conditions set out in paragraphs 157(2)(a) and 157(2)(b) of the Act;

(b) any circumstances surrounding the registered provider's history of non-compliance with the governance conditions set out in paragraphs 157(2)(a) and 157(2)(b) of the Act, including steps (if any) that the provider has taken to become compliant;

(c) any other matter the Commissioner considers relevant.

Note: For paragraph (b), examples of circumstances include the following:

(a) where a member of a governing body has unexpectedly resigned, and recruitment of a new member is ongoing, which renders a provider non-compliant with a governance condition;

(b) where a person has accepted a role as a member of the governing body but is yet to commence in that role.



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# Division 2—Advisory body requirements

## 158-5 Kinds of provider to which the quality care advisory body condition applies

For the purposes of subsection 158(2) of the Act, a registered provider registered in any of the following provider registration categories is prescribed:

(a) nursing and transition care;

(b) residential care.

## 158-10 Requirements for membership of quality care advisory body

For the purposes of subparagraph 158(2)(a)(i) of the Act, the requirements are that the membership of the quality care advisory body of a registered provider must include the following:

(a) a responsible person of the registered provider who has appropriate experience in the delivery of funded aged care services;

(b) if the registered provider is registered in the provider registration category residential care—a person who is directly involved in the provider's delivery of funded aged care services;

(c) if the registered provider is registered in the provider registration category nursing and transition care and delivers funded aged care services in the service type nursing care—a person who is directly involved in the delivery of those services;

(d) a person who represents the interests of individuals to whom the provider delivers funded aged care services.

Note: For paragraph (d), examples of such persons include the following:

(a) an individual to whom the provider is delivering funded aged care services;

(b) a member of a consumer advisory body (if established);

(c) a member of an organised consumer advisory service.

## 158-15 Requirements for reports of quality care advisory body

For the purposes of subparagraph 158(2)(a)(ii) of the Act, the requirements for a written report given by the quality care advisory body of a registered provider to the governing body of the provider are that the report must include any concerns that the body has about the quality of funded aged care services delivered by the provider in the period covered by the report (the ***report period***), taking into account the following:

(a) feedback provided (if any) about the quality of funded aged care services delivered by the provider in the report period by:

(i) individuals to whom the provider delivered funded aged care services; and

(ii) responsible persons and aged care workers of the provider;

(b) complaints received (if any) in the report period by the provider about the quality of funded aged care services delivered by the provider and action taken by the provider to address the complaints;



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(c) the use of regulatory mechanisms (if any) under Chapter 6 of the Act by the Commissioner in relation to the quality of funded aged care services delivered by the provider;

(d) progress made in the report period in relation to the provider's continuous improvement plan, particularly improvements made in the delivery of funded aged care services by the provider;

(e) the results of any audits performed by the Commissioner under section 110-38;

(f) staffing arrangements of the provider during the report period, including details of the following, as applicable:

(i) the availability of allied health professionals or registered health practitioners;

(ii) the availability of registered nurses;

(iii) turnover of the aged care workers of the provider;

(g) reportable incidents (if any) in connection with the delivery of funded aged care services to an individual by the provider that occurred in the report period and any action taken by the provider in response to the reportable incidents;

(h) if the provider delivers funded aged care services in an approved residential care home:

(i) feedback received (if any) in the report period from individuals to whom the provider delivers funded aged care services about the quality of food provided by the provider; and

(ii) changes (if any) in the report period in the quality of food provided, and the food preparation model used, by the provider; and

(iii) menu assessments (if any) conducted by an accredited practising dietitian in the report period in relation to food and nutrition provided by the provider; and

(iv) information compiled or derived from a measurement or other assessment made by the provider in the report period in relation to the Quality Indicators in Subdivision B of Division 3 of Part 2 of Chapter 5 of this instrument.

## 158-20 Kinds of provider to which the consumer advisory bodies condition applies

For the purposes of subsection 158(4) of the Act, a registered provider registered in any of the following provider registration categories is prescribed:

(a) nursing and transition care;

(b) residential care.



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# Part 9—Restrictive practices—approved residential care homes

## Division 1—Preliminary

### 162-5 Kinds of provider to which the condition applies

For the purposes of section 162 of the Act, a registered provider registered in the provider registration category residential care is prescribed.

### 162-10 Requirements relating to the use of restrictive practices

For the purposes of section 162 of the Act, this Part prescribes requirements relating to the use of restrictive practices in relation to an individual to whom a registered provider is delivering funded aged care services in an approved residential care home.

Note: See also sections 17 and 18 of the Act and Division 2 of Part 7 of Chapter 1 of this instrument.



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## Division 2—Requirements relating to the use of restrictive practices

### 162-15 Requirements for the use of any restrictive practice

(1) The following requirements apply to the use of any restrictive practice in relation to an individual:

(a) the restrictive practice is used only:

(i) as a last resort to prevent harm to the individual or other persons; and

(ii) after consideration of the likely impact of the use of the restrictive practice on the individual;

(b) to the extent possible, best practice alternative strategies are used before the restrictive practice is used;

(c) the alternative strategies that have been considered or used have been documented in the behaviour support plan for the individual;

(d) the restrictive practice is used only to the extent that it is necessary and in proportion to the risk of harm to the individual or other persons;

(e) the restrictive practice is used in the least restrictive form, and for the shortest time, necessary to prevent harm to the individual or other persons;

(f) informed consent to the use of the restrictive practice, and how it is to be used (including its duration, frequency and intended outcome), has been given by:

(i) the individual; or

(ii) if the individual lacks the capacity to give that consent—the restrictive practices substitute decision-maker for the restrictive practice;

(g) the use of the restrictive practice is in accordance with the informed consent mentioned in paragraph (f);

(h) the use of the restrictive practice complies with any provisions of the behaviour support plan for the individual that relate to the use of the restrictive practice;

(i) the use of the restrictive practice complies with the Aged Care Quality Standards and the Aged Care Code of Conduct;

(j) the use of the restrictive practice is not inconsistent with the Statement of Rights;

(k) the use of the restrictive practice meets the requirements (if any) of the law of the State or Territory in which the restrictive practice is used.

(2) However, the requirements set out in paragraphs (1)(a), (b), (c), (f), (g) and (h) do not apply to the use of a restrictive practice in relation to an individual if the use of the restrictive practice in relation to the individual is necessary in an emergency.

(3) Subsection (2) applies only while the emergency exists.

Note: See section 162-35 for other responsibilities of registered providers that apply if the use of a restrictive practice in relation to an individual is necessary in an emergency.



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## 162-20 Additional requirements for the use of restrictive practices other than chemical restraint

(1) The following requirements apply to the use of a restrictive practice in relation to an individual that is not chemical restraint:

(a) an approved health practitioner who has day-to-day knowledge of the individual has:

(i) assessed the individual as posing a risk of harm to the individual or any other person; and

(ii) assessed that the use of the restrictive practice is necessary;

(b) the following matters have been documented in the behaviour support plan for the individual:

(i) the assessments;

(ii) a description of any engagement with persons other than the approved health practitioner in relation to the assessments;

(iii) a description of any engagement with external support services (for example, dementia support specialists) in relation to the assessments.

(2) However, the requirement set out in paragraph (1)(b) does not apply to the use of a restrictive practice in relation to an individual if the use of the restrictive practice in relation to the individual is necessary in an emergency.

(3) Subsection (2) applies only while the emergency exists.

Note: See section 162-35 for other responsibilities of registered providers that apply if the use of a restrictive practice in relation to an individual is necessary in an emergency.

## 162-25 Additional requirements for the use of restrictive practices that are chemical restraint

(1) The following requirements apply to the use of a restrictive practice in relation to an individual that is chemical restraint:

(a) the registered provider is satisfied that a medical practitioner or nurse practitioner has:

(i) assessed the individual as posing a risk of harm to the individual or any other person; and

(ii) assessed that the use of the chemical restraint is necessary; and

(iii) prescribed medication for the purpose of using the chemical restraint; and

(iv) obtained informed consent to the prescribing of the medication for the purpose of using the chemical restraint;

(b) the following matters have been documented in the behaviour support plan for the individual:

(i) the assessments;

(ii) the practitioner's decision to use the chemical restraint;

(iii) the individual's behaviours that are relevant to the need for the chemical restraint;

(iv) the reasons the chemical restraint is necessary;



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(v) the information (if any) provided by the registered provider to the practitioner that informed the decision to prescribe the medication for the purpose of using the chemical restraint;

(vi) that the registered provider is satisfied that the practitioner obtained informed consent to the prescribing of the medication;

(vii) the details of the prescription for the prescribed medication, including its name, dosage and when it may be used;

(viii) a description of any engagement with persons other than the practitioner in relation to the use of the chemical restraint;

(ix) a description of any engagement with external support services (for example, dementia support specialists) in relation to the assessments;

(c) the use of the medication for the purpose of using the chemical restraint is in accordance with the prescription mentioned in subparagraph (b)(vii).

Note: Codes of appropriate professional practice for medical practitioners and nurse practitioners provide for the practitioners to obtain informed consent before prescribing medications. Those codes are approved under the National Law and are:

(a) for medical practitioners—*Good medical practice: a code of conduct for doctors in Australia* (which in 2025 could be viewed on the website of the Medical Board of Australia (www.medicalboard.gov.au)); and

(b) for nurse practitioners—*Code of conduct for nurses* (which in 2025 could be viewed on the website of the Nursing and Midwifery Board of Australia (www.nursingmidwiferyboard.gov.au)).

(2) However, the requirements set out in subparagraph (1)(a)(iv) and paragraph (1)(b) do not apply to the use of a restrictive practice in relation to an individual if the use of the restrictive practice in relation to the individual is necessary in an emergency.

(3) Subsection (2) applies only while the emergency exists.

Note: See section 162-35 for other responsibilities of registered providers that apply if the use of a restrictive practice in relation to an individual is necessary in an emergency.

## 162-30 Requirements while restrictive practice being used

If a registered provider uses a restrictive practice in relation to an individual, the registered provider must ensure that while the restrictive practice is being used:

(a) the individual is monitored for the following:

(i) signs of distress or harm;

(ii) side effects and adverse events;

(iii) changes in mood or behaviour;

(iv) changes in well-being, including the individual's ability to engage in activities that enhance quality of life and are meaningful and pleasurable;

(v) changes in the individual's ability to maintain independent function (to the extent possible);

(vi) changes in the individual's ability to engage in activities of daily living (to the extent possible); and

(b) the necessity for the use of the restrictive practice is regularly monitored, reviewed and documented; and



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(c) the effectiveness of the use of the restrictive practice, and the effect of changes in the use of the restrictive practice, are monitored; and

(d) to the extent possible, changes are made to the individual's environment to reduce or remove the need for the use of the restrictive practice; and

(e) if the restrictive practice is chemical restraint—information about the effects and use of the chemical restraint is provided to the medical practitioner or nurse practitioner who prescribed the medication for the purpose of using the chemical restraint as mentioned in paragraph 162-25(1)(a).

## 162-35 Requirements following emergency use of restrictive practice

If a registered provider uses a restrictive practice in relation to an individual and the use of the restrictive practice in relation to the individual is necessary in an emergency, the registered provider must, as soon as practicable after the restrictive practice starts to be used:

(a) if the individual lacked capacity to consent to the use of the restrictive practice—inform the restrictive practices substitute decision-maker for the restrictive practice about the use of the restrictive practice; and

(b) ensure that the following matters are documented in the behaviour support plan for the individual:

(i) the individual's behaviours that were relevant to the need for the use of the restrictive practice;

(ii) the alternative strategies that were considered or used (if any) before the use of the restrictive practice;

(iii) the reasons the use of the restrictive practice was necessary;

(iv) the care to be provided to the individual in relation to the individual's behaviour;

(v) if the restrictive practices substitute decision-maker for the restrictive practice was informed about the use of the restrictive practice under paragraph (a)—a record of the restrictive practices substitute decision-maker being so informed; and

(c) if the restrictive practice is not chemical restraint—ensure that the assessments mentioned in paragraph 162-20(1)(a) are documented in the behaviour support plan for the individual; and

(d) if the restrictive practice is chemical restraint—ensure that the matters mentioned in subparagraphs 162-25(1)(b)(i) to (v) and (vii) to (ix) are documented in the behaviour support plan for the individual.

## 162-40 Requirements relating to nominations of restrictive practices nominees

*Preventing coercion and duress*

(1) A registered provider must take reasonable steps to ensure that:

(a) an individual to whom the registered provider delivers funded aged care services is not subject to coercion or duress in making, varying or revoking a nomination under section 6-15; and



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(b) an individual nominated under section 6-15 (whether as an individual or as a member of a group) is not subject to coercion or duress in agreeing as mentioned in paragraph 6-15(2)(b) or (3)(b), or in withdrawing that agreement.

*Assisting care individuals*

(2) If an individual nominates an individual under section 6-15 (whether as an individual or as a member of a group), the registered provider delivering funded aged care services to the individual must assist the individual to:
   (a) notify the individual of the nomination; and
   (b) give the individual a copy of the nomination; and
   (c) seek the individual's agreement as mentioned in paragraph 6-15(2)(b) or (3)(b).

*Keeping records*

(3) If an individual nominates an individual under section 6-15 (whether as an individual or as a member of a group), the registered provider delivering funded aged care services to the individual must keep a record of:
   (a) the nomination; and
   (b) whether the individual has agreed as mentioned in paragraph 6-15(2)(b) or (3)(b); and
   (c) if the individual has agreed as mentioned in paragraph 6-15(2)(b) or (3)(b)—whether the individual has withdrawn that agreement.



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# Division 3—Requirements relating to behaviour support

## 162-45 Requirement for behaviour support plans

(1) If:
  (a) a registered provider delivers funded aged care services to an individual; and
  (b) behaviour support is needed for the individual;
the registered provider must ensure that a behaviour support plan for the individual is included in the care and services plan for the individual.

(2) The registered provider must ensure that the behaviour support plan:
  (a) is prepared, reviewed and revised in accordance with this Division; and
  (b) sets out the matters required by this Division and Division 2.

(3) In preparing the behaviour support plan, the registered provider must take into account any previous assessment relating to the individual that is available to the registered provider.

## 162-50 Requirements for behaviour support plans—alternative strategies for addressing behaviours of concern

A behaviour support plan for an individual must set out the following matters:
  (a) information about the individual that helps the registered provider to understand the individual and the individual's behaviour (such as information about the individual's past experience and background);
  (b) any assessment of the individual that is relevant to understanding the individual's behaviour;
  (c) information about behaviours of concern for which the individual may need support;
  (d) the following information about each occurrence of behaviours of concern for which the individual has needed support:
    (i) the date, time and duration of the occurrence;
    (ii) any adverse consequences for the individual or other persons;
    (iii) any related incidents;
    (iv) any warning signs for, or triggers or causes of, the occurrence (including trauma, injury, illness or unmet needs such as pain, boredom or loneliness);
  (e) alternative strategies for addressing the behaviours of concern that:
    (i) are best practice alternatives to the use of restrictive practices in relation to the individual; and
    (ii) take into account the individual's preferences (including preferences in relation to care delivery) and matters that might be meaningful or of interest to the individual; and
    (iii) aim to improve the individual's quality of life and engagement;



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(f) any alternative strategies that have been considered for use, or have been used, in relation to the individual;

(g) for any alternative strategy that has been used in relation to the individual:

(i) the effectiveness of the strategy in addressing the behaviours of concern; and

(ii) records of the monitoring and evaluation of the strategies;

(h) a description of the registered provider's consultation about the use of alternative strategies in relation to the individual with the individual or a supporter of the individual (if any).

## 162-55 Requirements for behaviour support plans—if use of restrictive practice assessed as necessary

If the use of a restrictive practice in relation to an individual is assessed as necessary as mentioned in section 162-20 or 162-25, the behaviour support plan for the individual must set out the following matters:

(a) the individual's behaviours of concern that are relevant to the need for the use of the restrictive practice;

(b) the restrictive practice and how it is to be used, including its duration, frequency and intended outcome;

(c) the best practice alternative strategies that must be used (to the extent possible) before using the restrictive practice;

(d) how the use of the restrictive practice is to be monitored, including how the monitoring will be escalated if required, taking into account the nature of the restrictive practice and any care needs that arise from the use of the restrictive practice;

(e) how the use of the restrictive practice is to be reviewed, including consideration of the following:

(i) the outcome of its use and whether the intended outcome was achieved;

(ii) whether an alternative strategy could be used to address the individual's behaviours of concern;

(iii) whether a less restrictive form of the restrictive practice could be used to address the individual's behaviours of concern;

(iv) whether there is an ongoing need for its use;

(v) if the restrictive practice is chemical restraint—whether the medication prescribed for the purpose of using the chemical restraint can or should be reduced or stopped;

(f) if the individual lacks the capacity to give informed consent to the use of the restrictive practice:

(i) whether subsection 6-20(1), or an item of the table in subsection 6-20(2), applies for the restrictive practice in relation to the individual, and why that subsection or item applies; and

(ii) the name of the restrictive practices substitute decision-maker for the restrictive practice in relation to the individual;



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(g) a description of the registered provider's consultation about the use of the restrictive practice with:
  (i) the individual; or
  (ii) if the individual lacks the capacity to give informed consent to the use of the restrictive practice—the restrictive practices substitute decision-maker for the restrictive practice;

(h) a record of the giving of informed consent to the use of the restrictive practice, and how it is to be used (including its duration, frequency and intended outcome), by:
  (i) the individual; or
  (ii) if the individual lacks the capacity to give that consent—the restrictive practices substitute decision-maker for the restrictive practice.

Note: Sections 162-20 and 162-25 also require other matters to be documented in the behaviour support plan.

## 162-60 Requirements for behaviour support plans—if restrictive practice used

If a restrictive practice is used in relation to an individual, the behaviour support plan for the individual must set out the following matters:

(a) the restrictive practice and how it was used, including the following:
  (i) when it began to be used;
  (ii) the duration of each use;
  (iii) the frequency of its use;
  (iv) the outcome of its use and whether the intended outcome was achieved;
  (v) whether its use was in accordance with the informed consent set out under paragraph 162-55(h);

(b) if, under the plan, the restrictive practice is to be used only on an as-needed basis in response to particular behaviour, or in particular circumstances:
  (i) the individual's behaviours of concern that led to the use of the restrictive practice; and
  (ii) the actions (if any) taken leading up to the use of the restrictive practice, including any alternative strategies that were used before the restrictive practice was used;

(c) the details of the persons involved in the use of the restrictive practice;

(d) a description of any engagement with external support services (for example, dementia support specialists) in relation to the use of the restrictive practice;

(e) details of the monitoring of the use of the restrictive practice as required by the plan;

(f) the outcome of the review of the use of the restrictive practice as required by the plan.

Note 1: For paragraphs (e) and (f), see paragraphs 162-55(d) and (e) for the requirements for a behaviour support plan for an individual to require monitoring and review of the use of a restrictive practice in relation to the individual.



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Note 2: If the use of a restrictive practice in relation to an individual is necessary in an emergency, other matters must also be documented in the behaviour support plan for the individual (see section 162-35).

## 162-65 Requirements for behaviour support plans—if need for ongoing use of restrictive practice indicated

If a review of the use of a restrictive practice in relation to an individual (as required by the behaviour support plan for the individual) indicates a need for the ongoing use of the restrictive practice, the behaviour support plan for the individual must set out the following matters:

(a) the restrictive practice and how it is to be used, including its duration, frequency and intended outcome;

(b) how the ongoing use of the restrictive practice is to be monitored, including how the monitoring will be escalated if required, taking into account the nature of the restrictive practice and any care needs that arise from the use of the restrictive practice;

(c) how the ongoing use of the restrictive practice is to be reviewed, including consideration of the following:

(i) the outcome of the ongoing use of the restrictive practice and whether the intended outcome is being achieved;

(ii) whether an alternative strategy could be used to address the individual's behaviours of concern;

(iii) whether a less restrictive form of the restrictive practice could be used to address the individual's behaviours of concern;

(iv) whether there continues to be need for the ongoing use of the restrictive practice;

(v) if the restrictive practice is chemical restraint—whether the medication prescribed for the purpose of using the chemical restraint can or should be reduced or stopped;

(d) a description of the registered provider's consultation about the ongoing use of the restrictive practice, and how it is to be used (including its duration, frequency and intended outcome), with:

(i) the individual; or

(ii) if the individual lacks the capacity to give informed consent to the ongoing use of the restrictive practice—the restrictive practices substitute decision-maker for the restrictive practice;

(e) a record of the giving of informed consent to the ongoing use of the restrictive practice by:

(i) the individual; or

(ii) if the individual lacks capacity to give that consent—the restrictive practices substitute decision-maker for the restrictive practice.

## 162-70 Requirement to review and revise behaviour support plans

A registered provider must review a behaviour support plan for an individual and make any necessary revisions:

(a) on a regular basis; and



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(b) as soon as practicable after any change in the individual's circumstances.

## 162-75 Requirement to consult on behaviour support plans

(1) In preparing, reviewing or revising a behaviour support plan for an individual, a registered provider must consult the following:
   (a) if the individual has the capacity to be consulted—the individual and a supporter of the individual (if any);
   (b) if the individual lacks the capacity to be consulted—a person or body who, under the law of the State or Territory in which the individual accesses funded aged care services, can make decisions about that care;
   (c) health practitioners with expertise relevant to the individual's behaviours of concern.

(2) If the use of a restrictive practice in relation to the individual is assessed as necessary as mentioned in section 162-20 or 162-25, the registered provider must also consult the following in preparing, reviewing or revising the behaviour support plan:
   (a) the approved health practitioner who made the assessment;
   (b) if the individual lacks the capacity to be consulted—the restrictive practices substitute decision-maker for the restrictive practice.

(3) In consulting under this section, the registered provider must provide the plan or revised plan, and any associated information, in an appropriately accessible format.



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# Division 4—Immunity from civil or criminal liability in relation to the use of a restrictive practice in certain circumstances

## 163-5 Giving of informed consent by certain persons or bodies

For the purposes of paragraph 163(2)(a) of the Act (which refers to the giving of informed consent to the use of a restrictive practice in relation to an individual), a person or body that is a restrictive practices substitute decision-maker for the restrictive practice in relation to the individual is prescribed.



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# Part 10—Management of incidents and complaints

## Division 1—Incident management

### Subdivision A—Preliminary

#### 164-1 Kinds of provider to which the condition applies

For the purposes of section 164 of the Act, a registered provider registered in any of the following provider registration categories is prescribed:

(a) home and community services;
(b) advisory and support services;
(c) personal and care support in the home or community;
(d) nursing and transition care;
(e) residential care;
(f) subscription trial.

#### 164-5 Requirements relating to incident management

For the purposes of section 164 of the Act, this Division prescribes:

(a) requirements for implementing and maintaining an incident management system; and
(b) requirements for managing, and taking reasonable steps to prevent, incidents.

Note: For requirements for reporting reportable incidents to the Commissioner, see Division 2 of Part 2 of Chapter 5 of this instrument.

#### 164-10 Incidents to which this Division applies

(1) This Division applies to incidents that consist of acts, omissions, events or circumstances that:

(a) occur, are alleged to have occurred, or are suspected of having occurred, in connection with the delivery of funded aged care services to an individual by a registered provider; and
(b) either:
(i) have caused harm to the individual or another person; or
(ii) could reasonably have been expected to have caused harm to an individual or another person.

(2) This Division also applies to incidents not covered by subsection (1) that consist of acts, omissions, events or circumstances that:

(a) a registered provider becomes aware of in connection with the delivery of funded aged care services to an individual in a residential care home of the registered provider; and
(b) have caused harm to the individual.



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## Subdivision B—Implementing and maintaining an incident management system

### 164-15 Requirements for system—objects

The objects of a registered provider's incident management system must include the following:

(a) to promote the safety, health, well-being and quality of life of individuals to whom the provider delivers funded aged care services by:
  (i) detecting, addressing and remediating incidents; and
  (ii) preventing incidents; and
  (iii) ensuring the provider's incident management system facilitates the open disclosure and resolution of incidents between individuals and the provider;

(b) to promote continuous improvement of:
  (i) the provider's management and prevention of incidents; and
  (ii) the provider's delivery of funded aged care services.

### 164-20 Requirements for system—general

A registered provider's incident management system must:

(a) be able to identify, record, assess, respond to and report on incidents; and

(b) specify procedures for identifying, recording, assessing, responding to and reporting on incidents; and

(c) require that appropriate support and assistance (including access to advocates and language services) is provided to persons affected by an incident to ensure their safety, health, well-being and quality of life; and

(d) specify how persons affected by an incident will be appropriately involved in the management and resolution of the incident; and

(e) specify the roles and responsibilities of aged care workers and responsible persons of the provider in managing and responding to incidents; and

(f) specify the roles and responsibilities of aged care workers and responsible persons of the provider in notifying reportable incidents to the Commissioner; and

(g) require an aged care worker of the provider who becomes aware of a reportable incident to notify one of the following of that fact as soon as possible:
  (i) a responsible person of the provider;
  (ii) a supervisor or manager of the aged care worker;
  (iii) a person specified for the purposes of paragraph (e) or (f); and

(h) require reportable incidents to be reported to the Commissioner in accordance with Division 2 of Part 2 of Chapter 5 of this instrument; and

(i) specify when an investigation by the provider is required to establish:
  (i) the causes of a particular incident; and
  (ii) the harm caused by the incident; and



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(iii) any operational issues that may have contributed to the incident occurring; and

(j) specify the nature of investigations mentioned in paragraph (i); and

(k) specify when remedial action is required and the nature of that action; and

(l) set out procedures for ensuring that the requirements of sections 164-40 and 164-45 are complied with.

## 164-25 Requirements for system—recording details of incidents

(1) A registered provider's incident management system must require the following details, as a minimum, to be recorded in relation to each incident:

(a) a description of the incident, including:

(i) the harm that was caused, or that could reasonably have been expected to have been caused, to each person affected by the incident; and

(ii) if known—the consequences of that harm;

(b) whether the incident is a reportable incident;

(c) if known—the time, date and place at which the incident occurred or was alleged or suspected to have occurred;

(d) the time and date the incident was identified;

(e) the names and contact details of the persons directly involved in the incident;

(f) the names and contact details of any witnesses to the incident;

(g) details of the assessments undertaken in accordance with subparagraph 164-40(1)(b)(i) and subsection 164-45(1);

(h) the actions taken in response to the incident, including actions taken under sections 164-40 or 164-45;

(i) any consultations undertaken with the persons affected by the incident;

(j) whether persons affected by the incident have been provided with any reports or findings regarding the incident;

(k) if an investigation is undertaken by the provider in relation to the incident—the details and outcomes of the investigation;

(l) the name and contact details of the person recording the details of the incident;

(m) if the incident has been reported to the police—the details included in that report.

(2) A registered provider's incident management system must require details recorded in relation to an incident to be retained for 7 years after the date the record was made or received, in accordance with section 154-150 of this instrument.

## 164-30 Requirements for system—data collection and analysis

A registered provider's incident management system must:

(a) require the collection of data relating to incidents that will enable the provider to:



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(i) identify occurrences, or alleged or suspected occurrences, of similar incidents; and

(ii) identify and address systemic issues in the quality of funded aged care services delivered by the provider; and

(iii) provide feedback and training to the provider's aged care workers and responsible persons about managing and preventing incidents; and

(iv) provide information to the Commissioner, if required or requested to do so by the Commissioner; and

(v) provide information to the provider's quality care advisory body (if any) and consumer advisory bodies (if any) to assist the body or bodies to prepare reports or feedback about the quality of the funded aged care services delivered by the provider; and

(vi) continuously improve the provider's management and prevention of incidents; and

(b) require the regular analysis and review of data mentioned in paragraph (a) to assess:

(i) the effectiveness of the provider's management and prevention of incidents; and

(ii) what (if any) actions could be taken to improve the provider's management and prevention of incidents.

## 164-35 Requirements for registered providers

(1) A registered provider must:

(a) prepare and keep up to date documents detailing:

(i) the provider's incident management system, including the objects mentioned in section 164-15 and the requirements mentioned in sections 164-20 to 164-30; and

(ii) the roles and responsibilities in the system of the provider's aged care workers and responsible persons in identifying, managing and resolving incidents and in preventing incidents from occurring; and

(b) give the documents to:

(i) the provider's aged care workers and responsible persons; and

(ii) to the Commissioner, if required or requested to do so by the Commissioner; and

(c) ensure that the provider's aged care workers and responsible persons are aware of, and understand, their roles and responsibilities in the system; and

(d) require the provider's aged care workers and responsible persons to comply with the system; and

(e) provide appropriate training to the provider's aged care workers and responsible persons on how the system works, including:

(i) how to recognise, respond to and report incidents; and

(ii) their roles and responsibilities in the system; and

(f) make the documents mentioned in paragraph (a) available, in an accessible form, to the following persons:



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(i) the individuals to whom the provider is delivering funded aged care services;

(ii) supporters, family members, carers and advocates of the individuals to whom the provider is delivering funded aged care services, and any other person significant to those individuals; and

(g) assist persons referred to in paragraph (f) to understand how the system works.

(2) Without limiting paragraph (1)(e), a provider provides appropriate training to a person who is an aged care worker or responsible person of the provider if the training is provided:

(a) at least annually; and

(b) at the following times:

(i) when the person becomes an aged care worker or responsible person of the provider;

(ii) when there is a change to how the system works that affects the person's roles and responsibilities in the system;

(iii) when there is a change to the person's role that affects the person's roles and responsibilities in the system.

## Subdivision C—Managing and preventing incidents

### 164-40 Requirements for managing incidents

*General*

(1) A registered provider must manage an incident:

(a) in accordance with the provider's incident management system; and

(b) by doing the following:

(i) assessing the support and assistance required to ensure the safety, health, well-being and quality of life of persons affected by the incident;

(ii) providing that support and assistance to those persons;

(iii) assessing how to appropriately involve each person affected by the incident, or a supporter or advocate of the person, in the management and resolution of the incident;

(iv) involving each person or supporter or advocate in that way;

(v) using an open disclosure process.

*Notifying police of incident where reasonable grounds to do so*

(2) If there are reasonable grounds to report the incident to police, the provider must notify a police officer of the incident within 24 hours of becoming aware of the incident.

(3) If the provider later becomes aware of reasonable grounds to report the incident to police, the provider must notify a police officer of the incident within 24 hours of becoming aware of those grounds.



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## 164-45 Requirements for improving management of incidents and taking reasonable steps to prevent incidents

(1) The provider must assess the incident in relation to the following, taking into account the views of persons affected by the incident:
  (a) whether the incident could have been prevented;
  (b) what (if any) remedial action needs to be undertaken to prevent further similar incidents from occurring, or to minimise their harm;
  (c) how well the incident was managed and resolved;
  (d) what (if any) actions could be taken to improve the provider's management and resolution of similar incidents;
  (e) whether other persons or bodies should be notified of the incident.

(2) The provider must notify the persons and bodies determined under paragraph (1)(e).

(3) The provider must:
  (a) take any actions determined under paragraph (1)(b); and
  (b) take any actions determined under paragraph (1)(d) of this section or subparagraph 164-30(b)(ii) that are reasonable in the circumstances.



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# Division 2—Complaints, feedback and whistleblowers

## Subdivision A—Preliminary

### 165-5 Purpose of this Part

For the purposes of section 165 of the Act, this Part prescribes requirements for the following:

(a) implementing and maintaining a complaints and feedback management system;

(b) managing complaints and feedback;

(c) implementing and maintaining a whistleblower system and maintaining a whistleblower policy;

(d) managing disclosures that qualify for protection under section 547 of the Act (whistleblower protections).

## Subdivision B—Implementing and maintaining a complaints and feedback management system

### 165-10 Objects of this Subdivision

The objects of this Subdivision are:

(a) to promote quality care and the safety of individuals to whom a registered provider delivers funded aged care services by ensuring the provider's complaints and feedback management system facilitates the open disclosure and resolution of complaints between individuals and the provider; and

(b) to ensure that the provider's complaints and feedback management system acknowledges, assesses, manages and resolves matters relating to the provider's delivery of funded aged care services in a fair, transparent, accessible, safe, culturally safe and timely manner; and

(c) to ensure that:

(i) individuals to whom the provider delivers funded aged care services; and

(ii) supporters of those individuals, and other persons supporting those individuals; and

(iii) the provider's aged care workers; and

(iv) any other persons;

are encouraged and supported to make complaints and give feedback about the provider's delivery of funded aged care services; and

(d) to ensure that complaints and feedback contribute to the continuous improvement of the provider's delivery of funded aged care services.

### 165-15 Requirements for system—general

(1) A registered provider's complaints and feedback management system must:



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(a) be able to:
  (i) receive, record, assess, acknowledge, respond to and report on complaints and feedback; and
  (ii) securely store information relating to complaints and feedback; and
(b) enable any person to:
  (i) make a complaint or give feedback; and
  (ii) withdraw a complaint or feedback that the person has made or given; and
(c) enable complaints to be made and withdrawn, and feedback to be given and withdrawn, orally and in writing; and
(d) enable complaints to be made, and feedback to be given, anonymously; and
(e) require that there are to be no costs charged by the provider for making, withdrawing or managing a complaint or giving, withdrawing or managing feedback; and
(f) require that the process for making and resolving complaints and giving and responding to feedback is accessible to any person who wishes to make or give, or has made or given, a complaint or feedback; and
(g) require that appropriate support and assistance (including access to advocates and language services) is provided to:
  (i) any person who wishes to make or give, or has made or given, a complaint or feedback; and
  (ii) each individual to whom the provider delivers funded aged care services who is directly affected by an issue raised in a complaint or feedback; and
(h) require that the role of the independent aged care advocate is acknowledged and supported; and
(i) enable cooperation with, and facilitate arrangements for, the independent aged care advocate to support the following persons in making a complaint, giving feedback, or otherwise in relation to a complaint made or feedback given:
  (i) individuals to whom the provider delivers funded aged care services;
  (ii) supporters of those individuals, and other persons supporting those individuals; and
(j) require that, if a person who has made a complaint (other than anonymously), or an individual to whom the provider delivers funded aged care services who is directly affected by an issue raised in the complaint, wishes to be involved in the resolution of the complaint, the person or individual is:
  (i) involved in an appropriate way in the resolution of the complaint; and
  (ii) kept informed in an appropriate way of the progress and outcome of the complaint; and
(k) require that there are no reprisals for a person who makes a complaint or gives feedback; and
(l) require that information provided in a complaint or feedback is kept confidential and only disclosed if required by law or if the disclosure is otherwise appropriate in the circumstances; and



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(m) require that the provider must, in responding to a complaint or feedback, afford procedural fairness to:
  (i) the person who made the complaint or gave the feedback (other than anonymously); and
  (ii) the person (if any) against whom the complaint is made or who is the subject of the feedback; and
(n) require that the provider practise open disclosure and prioritise restorative practices; and
(o) ensure that any requirements for the referral or notification of complaints under Commonwealth, State or Territory laws (as applicable) are met; and
(p) provide for the system to be reviewed as required, and at least annually, to ensure that it is meeting the requirements mentioned in paragraphs (a) to (o).

(2) Without limiting paragraph (1)(l), a disclosure is appropriate if the person about whom the information relates has consented to the disclosure.

## 165-20 Requirements for registered providers

(1) A registered provider must:
(a) prepare and keep up to date documents detailing:
  (i) the provider's complaints and feedback management system, including the requirements mentioned in section 165-15; and
  (ii) the roles and responsibilities in the system of the provider's aged care workers and responsible persons in relation to complaints and feedback; and
(b) give the documents to the provider's aged care workers and responsible persons; and
(c) ensure that the provider's aged care workers and responsible persons are aware of, and understand, their roles and responsibilities in the system; and
(d) require the provider's aged care workers and responsible persons to comply with the system; and
(e) provide appropriate training to the provider's aged care workers and responsible persons on how the system works, including:
  (i) how to handle personal information and data; and
  (ii) how to recognise and respond to complaints and feedback; and
  (iii) managing relationships and clearly communicating with persons making complaints or giving feedback; and
  (iv) when and how to escalate complaints and feedback in the system; and
  (v) their roles and responsibilities in the system; and
  (vi) the roles and functions of independent aged care advocates in the system; and
(f) publish an accessible document that:
  (i) describes how a complaint can be made, or feedback can be given, to the provider; and



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(ii) describes what a person who makes a complaint or gives feedback to the provider can expect in relation to the provider's management of the complaint or feedback; and

(iii) includes information about how a complaint can be made, or feedback can be given, to the Complaints Commissioner; and

(iv) explains that the provider will not victimise or discriminate against anyone for making a complaint or giving feedback to the provider or the Complaints Commissioner; and

(v) includes information about how to contact an independent aged care advocate; and

(g) give the document mentioned in paragraph (f) to the following:

(i) individuals to whom the provider delivers funded aged care services;

(ii) any other person who requests the document; and

(h) if it is necessary, to enable a person to whom the document mentioned in paragraph (f) must be given under subparagraph (g)(i) to understand the document, to translate the document into another language or present the document in an alternative appropriate format—translate the document into that language, or present the document in that format, and give the translation or reformatted document to the person; and

(i) help the persons mentioned in subparagraph (g)(i) to understand how the provider's complaints and feedback management system works; and

(j) communicate regularly, and at least monthly, to the persons mentioned in paragraph (b) and subparagraph (g)(i) that complaints and feedback are welcome; and

(k) review the system as mentioned in paragraph 165-15(p).

(2) Without limiting paragraph (1)(e), the provider provides appropriate training to a person who is an aged care worker or responsible person of the provider if the training is provided:

(a) at regular intervals, which must be at least annually; and

(b) at the following times:

(i) when the person becomes an aged care worker or responsible person of the provider;

(ii) when there is a change to how the system works that affects the person's roles and responsibilities in the system;

(iii) when there is a change to the person's role that affects the person's roles and responsibilities in the system.

## Subdivision C—Managing complaints and feedback

### 165-25 Requirements for complaints management and resolution

If a registered provider receives a complaint, the provider must:

(a) manage the complaint in accordance with the provider's complaints and feedback management system; and



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(b) as soon as practicable after receiving the complaint, resolve each issue raised in the complaint by taking appropriate action in relation to the issue; and

(c) in resolving each issue raised in the complaint, use a resolution approach that:

(i) is consistent with the Statement of Rights; and

(ii) is appropriate given the nature of the issue; and

(iii) is centred around each individual to whom the provider delivers funded aged care services who is directly affected by the issue; and

(iv) seeks to address the issue as raised in the complaint; and

(v) will contribute to the continuous improvement of the provider's delivery of funded aged care services; and

(d) unless the complaint was made anonymously—take reasonable steps to notify the following persons of the outcome of the complaint, and the reasons for the outcome:

(i) the person who made the complaint;

(ii) if the complaint was made on behalf of an individual to whom the provider is delivering funded aged care services—the individual; and

(e) unless the complaint was made anonymously—take reasonable steps to tell the following persons how the complaint (the ***initial complaint***) can also be made to the Complaints Commissioner:

(i) the person who made the complaint;

(ii) if the complaint was made on behalf of an individual to whom the provider is delivering funded aged care services—the individual; and

(f) take reasonable steps to tell any other person involved in the resolution of the initial complaint how the complaint can also be made to the Complaints Commissioner; and

(g) take reasonable steps to tell the persons mentioned in paragraphs (d) and (e):

(i) how a new complaint about the resolution of the initial complaint can be made to the Complaints Commissioner;

(ii) information about the availability of independent aged care advocates to assist with making a new complaint to the Complaints Commissioner.

Note: See section 29 of the Act for provisions relating to giving information and documents to supporters.

## 165-30 Requirements for feedback management and resolution

(1) If a registered provider receives feedback, the provider must manage the feedback in accordance with the provider's complaints and feedback management system.

(2) In resolving any issues raised in the feedback, the provider must:

(a) use a resolution approach that will contribute to the continuous improvement of the provider's delivery of funded aged care services; and

(b) unless the feedback was given anonymously—consult with:



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(i) the person who gave the feedback; and
(ii) if the feedback was given on behalf of an individual to whom the provider is delivering funded aged care services—the individual; and
(iii) if the person who gave the feedback is an individual to whom the provider is delivering funded aged care services and the individual has consented to the consultation—the individual's supporter (if any); and
(iv) if the feedback was given on behalf of an individual to whom the provider is delivering funded aged care services and the individual has consented to the consultation—the individual's supporter (if any).

(3) The provider must:
(a) unless the feedback was given anonymously—take reasonable steps to tell the persons mentioned in paragraph (2)(b) how the feedback (the ***initial feedback***) can also be given to the Complaints Commissioner; and
(b) take reasonable steps to tell any other person involved in the resolution of any issue raised by the initial feedback how the feedback can also be given to the Complaints Commissioner; and
(c) take reasonable steps to tell the persons mentioned in paragraph (2)(b) and paragraph (b) of this subsection how a complaint or new feedback about the resolution of any issue raised by the initial feedback can be made or given to the Complaints Commissioner.

## 165-35 Other requirements relating to complaints and feedback

(1) If a registered provider receives a complaint or feedback, the provider must:
(a) provide appropriate support and assistance (including access to advocates and language services), in relation to contacting the Complaints Commissioner, to the following persons:
(i) the person who made the complaint or gave the feedback (other than anonymously);
(ii) each individual to whom the provider delivers funded aged care services who is directly affected by an issue raised in the complaint or feedback; and
(b) take reasonable steps to ensure that:
(i) the person who made the complaint or gave the feedback, or on whose behalf the complaint was made or the feedback was given, is not adversely affected as a result of the making of the complaint or the giving of the feedback; and
(ii) no individual to whom the provider delivers funded aged care services who is affected by an issue raised in the complaint or feedback suffers any detriment, victimisation or reprisal as a result of the making of the complaint or the giving of the feedback.

(2) A registered provider must use an open disclosure process:
(a) in relation to a matter that is the subject of a complaint or feedback; and
(b) if things go wrong in managing a complaint or feedback or resolving an issue raised in a complaint or feedback.



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*Handling whistleblower disclosures as complaints or feedback*

(3) An individual who discloses information that qualifies for protection under section 547 of the Act (whistleblower protections) to a provider may elect to have the disclosure managed as a complaint or feedback under paragraph 165(1)(b) of the Act and this Division of this instrument.

(4) If an individual makes an election under subsection (3), the provider must manage the disclosure as a complaint or feedback in accordance with this Subdivision rather than in accordance with Subdivision E.

## Subdivision D—Implementing and maintaining a whistleblower system and maintaining a whistleblower policy

### 165-40 Objects of this Subdivision

The objects of this Subdivision are:

(a) to promote quality care and the safety of individuals to whom a registered provider delivers funded aged care services by ensuring the provider's whistleblower system facilitates certain disclosures of information by individuals without fear of persecution, retribution or personal detriment; and

(b) to ensure that the confidentiality of such disclosures is maintained and, where relevant, the anonymity of the individual making the disclosure, and any other specified individual, is protected; and

(c) to ensure that the provider's whistleblower system acknowledges, assesses, manages and responds to concerns raised in such disclosures in a fair, transparent, accessible, safe, culturally safe and timely manner; and

(d) to ensure that such disclosures contribute to the continuous improvement of the provider's delivery of funded aged care services; and

(e) to ensure that aged care workers and any other persons are encouraged and supported to raise concerns about the provider's delivery of funded aged care services.

### 165-45 Requirements for system—general

A registered provider's whistleblower system must:

(a) support the operation of Part 5 of Chapter 7 of the Act (whistleblower protections); and

(b) enable individuals to disclose information to the following:

(i) the provider;

(ii) a responsible person of the provider;

(iii) an aged care worker of the provider; and

(c) enable disclosures of information mentioned in paragraph (b) to be made:

(i) orally and in writing; and

(ii) anonymously; and



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(d) provide for the system to be reviewed as required, and at least annually, to ensure that it is meeting the requirements mentioned in paragraphs (a) to (c).

## 165-50 Requirements for registered providers—general

(1) A registered provider must:

(a) prepare and keep up to date documents detailing:

(i) the provider's whistleblower system, including the requirements mentioned in section 165-45; and

(ii) the roles and responsibilities in the system of the provider's aged care workers and responsible persons; and

(b) give the documents to the provider's aged care workers and responsible persons; and

(c) ensure that the provider's aged care workers and responsible persons are aware of, and understand, their roles and responsibilities in the system; and

(d) require the provider's aged care workers and responsible persons to comply with the system; and

(e) provide appropriate training to the provider's aged care workers and responsible persons on how the system works, including:

(i) how to handle personal information and data; and

(ii) how to recognise and respond to disclosures that qualify for protection under section 547 of the Act; and

(iii) managing relationships and communicating with disclosers; and

(iv) when and how to escalate disclosures in the system; and

(v) their roles and responsibilities in the system; and

(vi) the penalties for contravening subsection 550(1) of the Act (confidentiality of identity of disclosers); and

(f) communicate regularly, and at least monthly, to the provider's aged care workers and responsible persons that disclosures that qualify for protection under section 547 of the Act are welcome; and

(g) review the system as mentioned in paragraph 165-45(d).

(2) Without limiting paragraph (1)(e), the provider provides appropriate training to a person who is an aged care worker or responsible person of the provider if the training is provided:

(a) at regular intervals, which must be at least annually; and

(b) at the following times:

(i) when the person becomes an aged care worker or responsible person of the provider;

(ii) when there is a change to how the system works that affects the person's roles and responsibilities in the system;

(iii) when there is a change to the person's role that affects the person's roles and responsibilities in the system.



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Section 165-55

## 165-55 Requirements for registered providers—whistleblower policy

A registered provider must:

(a) prepare and keep up to date a whistleblower policy that sets out the following:
  (i) the effect of Part 5 of Chapter 7 of the Act;
  (ii) that individuals may disclose information to the provider or another person mentioned in section 547 of the Act, and how such disclosures may be made;
  (iii) how the provider will manage disclosures that qualify for protection under section 547 of the Act that are made to the provider or a responsible person or aged care worker of the provider;
  (iv) that individuals may also disclose information to the entities mentioned in subparagraphs 547(a)(i), (ii), (vi) and (vii) of the Act, and how such disclosures may be made;
  (v) how the provider will investigate disclosures;
  (vi) how the provider will comply with the requirements mentioned in paragraphs 165-60(c) and (d) of this instrument;
  (vii) how the provider will comply with the obligations in section 553 of the Act;
  (viii) what an individual who has made a disclosure that qualifies for protection under section 547 of the Act can do if they suspect that there has been a contravention of a provision of Part 5 of Chapter 7 of the Act; and
(b) publish the policy in an accessible document; and
(c) give the policy to the provider's aged care workers and responsible persons; and
(d) give the policy to the following:
  (i) individuals to whom the provider delivers funded aged care services;
  (ii) a person who requests the policy and is a supporter of an individual to whom the provider delivers funded aged care services;
  (iii) any other person who requests the policy; and
(e) if it is necessary, to enable a person to whom the policy must be given under subparagraph (d)(i) or (ii) to understand the policy, to translate the policy into another language or present the policy in an alternative appropriate format—translate the policy into that language, or present the policy in that format, and give the translation or reformatted policy to the person; and
(f) help the persons mentioned in subparagraphs (d)(i) and (ii) to understand how the whistleblower system works; and
(g) communicate regularly, and at least monthly, to the persons mentioned in paragraph (c) and subparagraphs (d)(i) and (ii) that disclosures that qualify for protection under section 547 of the Act are welcome.



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## Subdivision E—Managing disclosures that qualify for protection under section 547 of the Act

### 165-60 Requirements for managing disclosures

If an individual makes a disclosure that qualifies for protection under section 547 of the Act to a registered provider or a responsible person or aged care worker of the provider, the provider must:

(a) manage the disclosure in accordance with the provider's whistleblower system; and

(b) as soon as practicable after the disclosure is made, take appropriate action in relation to the disclosure; and

(c) support:

(i) the individual (the ***first individual***) who made the disclosure; and

(ii) any other individual, or an entity, that employs or is otherwise associated with the first individual, and to which detriment might be caused, or a threat of detriment might be made, because of the disclosure; and

(d) ensure fair treatment of any responsible person or aged care worker of the provider who is mentioned in the disclosure or to whom the disclosure relates.

Note: See section 551 of the Act for civil penalties for victimisation relating to a disclosure that qualifies for protection under section 547 of the Act.



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Section 165A-1

# Chapter 5—Registered provider, responsible person and aged care worker obligations

## Part 1—Introduction

### 165A-1 Simplified outline of this Chapter

This Chapter provides for matters relating to registered provider, responsible person and aged care worker obligations under Part 4 of Chapter 3 of the Act.

Part 2 of this Chapter relates to obligations relating to reporting, notifications and information, and deals with requirements for reporting by certain registered providers to particular persons specified in subsection 166(2) of the Act on:

(a) vaccinations; and
(b) quality indicators; and
(c) complaints and feedback management; and
(d) complaints and feedback information on request; and
(e) prudential and financial matters; and
(f) reportable incidents; and
(g) the specialist aged care program CHSP; and
(h) the specialist aged care program NATSIFACP; and
(i) governing bodies; and
(j) registered nurses; and
(k) the status of service delivery branches; and
(l) the Multi-Purpose Service Program; and
(m) the transition care program; and
(n) pricing information.

Part 3 of this Chapter relates to provider obligations in relation to notifying of changes in circumstances, and deals with:

(a) certain kinds of changes in relation to which certain kinds of registered providers must give notice to the Commissioner; and
(b) circumstances in relation to which a notice must also be given to the System Governor; and
(c) information that must be included in a notice given to the Commissioner or the System Governor.

Part 4 of this Chapter deals with the kinds of registered providers to which the obligation to notify of changes of circumstances relating to suitability applies.

Part 5 of this Chapter relates to obligations relating to suitability of responsible persons, and deals with the kind of registered providers to which the obligations apply and requirements for records of suitability matters.



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Part 6 of this Chapter relates to obligations relating to aged care workers, and deals with registered nurses and the delivery of direct care.

Part 7 of this Chapter relates to other obligations, and deals with requirements relating to cooperation by registered providers with the Pricing Authority.



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# Part 2—Obligations relating to reporting, notifications and information

## Division 1—Preliminary

### 165A-2 No limitation on other requests

Nothing in this Part limits or affects the System Governor, the Commissioner or the Complaints Commissioner from requesting information from a registered provider under any other provision of the Rules or the Act.



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## Division 2—Reportable incidents

### 165A-5 Purpose of this Subdivision

For the purposes of paragraph 165A(2)(b) of the Act, this Subdivision prescribes requirements for reporting reportable incidents to the Commissioner.

### 165A-10 Application of Subdivision to registered providers

For the purposes of subsection 165A(1) of the Act, every kind of registered provider is prescribed.

### 165A-15 Registered provider must notify reportable incidents in accordance with this Subdivision

A registered provider must take all reasonable steps to ensure that reportable incidents are notified to the Commissioner in accordance with this Subdivision.

### 165A-20 Registered provider must ensure that aged care workers notify reportable incidents

A registered provider must ensure that an aged care worker of the provider who becomes aware of a reportable incident notifies one of the following of that fact as soon as possible:

(a) one of the provider's responsible persons;
(b) a supervisor or manager of the aged care worker;
(c) a person specified for the purposes of paragraph 164-20(e).

### 165A-25 Priority 1 notice must be given within 24 hours

(1) If:

(a) a registered provider becomes aware of a reportable incident; and
(b) the provider has reasonable grounds to believe that the incident is a priority 1 reportable incident;

the provider must give the Commissioner a notice (a ***priority 1 notice***) in accordance with subsection (4) within 24 hours of becoming aware of the reportable incident.

Note: Notice about certain reportable incidents is not required to be given: see section 165A-35.

(2) A ***priority 1 reportable incident*** is a reportable incident:

(a) that has caused an individual physical or psychological injury or discomfort that requires medical or psychological treatment; or
(b) where there are reasonable grounds to report the incident to police; or
(c) of the kind covered by paragraph 16(1)(b) of the Act (about unlawful sexual contact or inappropriate sexual conduct, inflicted on an individual); or



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(d) of the kind covered by paragraph 16(1)(d) or (h) of the Act (about unexpected death or unexplained absence).

(3) For the purposes of paragraph (2)(a), in considering whether a reportable incident has caused an individual who has an impairment that directly affects their ability to recognise or communicate physical or psychological injury or discomfort (an ***impairment***), physical or psychological injury or discomfort that requires medical or psychological treatment, a registered provider:

(a) must not consider the impairment as:

(i) preventing the individual from being caused physical or psychological injury or discomfort; or

(ii) reducing the degree of physical or psychological injury or discomfort caused; and

(b) must recognise and consider that each individual's experience of an incident will be unique and that an impairment may contribute to causing the individual physical or psychological injury or discomfort.

*Information to be included in notice*

(4) Subject to subsection (5), the priority 1 notice must include the following information about the reportable incident:

(a) the name and contact details of the registered provider;

(b) a description of the reportable incident including:

(i) the kind of reportable incident; and

(ii) the harm that was caused, or that could reasonably have been expected to have been caused, to each person affected by the incident; and

(iii) if known—the consequences of that harm;

(c) the immediate actions taken in response to the reportable incident, including:

(i) actions taken to ensure the safety, health and well-being of each individual affected by the incident or the supporter of the individual; and

(ii) whether the incident has been reported to police or any other body;

(d) any further actions proposed to be taken in response to the reportable incident;

(e) the name, position and contact details of the person giving the notice;

(f) if known—the time, date and place at which the reportable incident occurred or was alleged or suspected to have occurred;

(g) the names of the persons directly involved in the reportable incident;

(h) if known—the level of cognition of the affected individuals directly involved in the reportable incident.

(5) The registered provider is not required to include information in the priority 1 notice if that information is not available within the 24 hours.



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*Additional information*

(6) The registered provider must give the Commissioner a notice including the following information about the reportable incident within 5 days after the start of the 24 hours, or within such other period as the Commissioner determines:
  (a) any information required by subsection (4) not provided in the priority 1 notice;
  (b) any further information specified by the Commissioner that is required to deal with the reportable incident.

(7) However, the registered provider is not required to give a notice under subsection (6) if the Commissioner decides otherwise.

*Form of notices*

(8) A notice given under this section must:
  (a) be in writing; and
  (b) be in the approved form.

Note: The Commissioner may approve forms for the purposes of this Division: see section 585 of the Act.

## 165A-30 Priority 2 notice must be given within 30 days

(1) If:
  (a) a registered provider becomes aware of a reportable incident; and
  (b) the provider has not given a notice under section 165A-25 about the incident;

the provider must give the Commissioner a notice (a ***priority 2 notice***) in accordance with subsection (2) within 30 days of becoming aware of the incident.

Note: Notice about certain reportable incidents is not required to be given: see section 165A-35.

(2) The priority 2 notice must include the following information about the reportable incident:
  (a) the name and contact details of the registered provider;
  (b) a description of the reportable incident including:
    (i) the kind of reportable incident; and
    (ii) the harm that was caused, or that could reasonably have been expected to have been caused to each person affected by the incident; and
    (iii) if known—the consequences of that harm;
  (c) the actions taken in response to the reportable incident, including:
    (i) actions taken to ensure the safety, health and well-being of each individual affected by the incident or the supporter of the individual; and
    (ii) whether the incident has been reported to police or any other body;



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(d) any further actions proposed to be taken in response to the reportable incident;
(e) the name, position and contact details of the person giving the notice;
(f) if known—the time, date and place at which the reportable incident occurred or was alleged or suspected to have occurred;
(g) the names of the persons directly involved in the reportable incident;
(h) if known—the level of cognition of the affected individuals directly involved in the reportable incident.

*Additional information*

(3) If the Commissioner requires the registered provider to give a notice including specified further information about the reportable incident within a specified period, the provider must give the Commissioner a notice including that information with the specified period.

*Form of notices*

(4) A notice given under this section must:
(a) be in writing; and
(b) be in the approved form.

Note: The Commissioner may approve forms for the purposes of this Division: see section 585 of the Act.

## 165A-35 Reporting not required in certain circumstances

The Commissioner may decide that a registered provider is not required to give a notice under section 165A-25 or 165A-30 about a reportable incident if the Commissioner is satisfied that:
(a) the same incident has been repeatedly alleged by an individual accessing funded aged care services to have occurred; and
(b) the allegation is the result of a delusion of the individual.

## 165A-40 Significant new information must be notified

(1) A registered provider must notify the Commissioner of significant new information relating to a reportable incident as soon as reasonably practicable after becoming aware of the information if:
(a) the provider notifies the Commissioner of the reportable incident under section 165A-25 or 165A-30; and
(b) the provider later becomes aware of the significant new information.

(2) The notification must:
(a) be in writing; and
(b) be in the approved form.

Note: The Commissioner may approve forms for the purposes of this Division: see section 585 of the Act.



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## 165A-45 Final report about reportable incident must be given if required

(1) If required by the Commissioner, a registered provider must give the Commissioner a final report that includes specified information about a reportable incident.

(2) The final report must be given:
  (a) within 84 days of the day a notice about the incident was first given to the Commissioner under section 165A-25 or 165A-30; or
  (b) within such other period as is specified by the Commissioner.

(3) The final report must:
  (a) be in writing; and
  (b) be in the approved form; and
  (c) contain the information specified by the Commissioner under subsection (1).

Note: The Commissioner may approve forms for the purposes of this Division: see section 585 of the Act.



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# Division 3—Provider obligation—reporting to particular persons

## Subdivision A—Vaccinations

### 166-5 Application of Subdivision to certain registered providers

This Subdivision applies to a registered provider registered in the provider registration category residential care.

### 166-10 Reports about service staff—influenza vaccinations

(1) For the purposes of paragraphs 166(1)(a) and (d) of the Act, this section prescribes that a registered provider must, on request by the System Governor or the Commissioner, give the System Governor or the Commissioner a report, in a form approved by the System Governor, that sets out the following information as at the reporting day specified in the request:

(a) the total number of service staff in relation to the approved residential care home;

(b) the number of those service staff who have informed the registered provider, whether voluntarily or as required under a law of a State or Territory, that they have received the annual seasonal influenza vaccination for the calendar year that includes the reporting day (whether or not under the registered provider's influenza vaccination scheme (if any)).

(2) The System Governor or the Commissioner may, at any time, request a registered provider to give to the System Governor or the Commissioner a report under subsection (1).

(3) A request under subsection (2) must:

(a) be in writing; and

(b) specify a reporting day that is not more than 3 years before the request is made.

(4) A registered provider must comply with a request under subsection (2) within 7 days after the request is made, or such longer period as is agreed, in writing, between the System Governor or the Commissioner and the registered provider.

### 166-15 Reports about service staff—COVID-19 vaccinations

(1) For the purposes of paragraphs 166(1)(a) and (d) of the Act, this section prescribes that a registered provider must, on request by the System Governor or the Commissioner, give the System Governor or the Commissioner a report, in a form approved by the System Governor, that sets out the following information as at the reporting day specified in the request:

(a) the total number of service staff in relation to the approved residential care home;



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(b) the number of those service staff who have voluntarily informed the registered provider that they have, in the period specified in the request before the reporting day, received a COVID-19 vaccination.

(2) The System Governor or the Commissioner may, at any time, request a registered provider to give to the System Governor or the Commissioner a report under subsection (1).

(3) A request under subsection (2) must:
(a) be in writing; and
(b) specify a reporting day that is not more than 3 years before the request is made.

(4) A registered provider must comply with a request under subsection (2) within 7 days after the request is made, or such longer period as is agreed, in writing, between the System Governor or the Commissioner and the registered provider.

## 166-25 Reports about individuals receiving residential care—COVID-19 vaccinations

(1) For the purposes of paragraphs 166(1)(a) and (d) of the Act, this section prescribes that a registered provider must, on request by the System Governor or the Commissioner, give the System Governor or the Commissioner a report, in a form approved by the System Governor, that sets out the following information, as at the reporting day specified in the request:
(a) the total number of individuals accessing funded aged care services in an approved residential care home;
(b) the number of those individuals who have voluntarily informed the registered provider, in the period specified in the request before the reporting day, that they had received one or more COVID-19 vaccinations;
(c) the number of individuals covered by paragraph (b) who have informed the registered provider that they had received only one COVID-19 vaccination;
(d) the number of individuals covered by paragraph (b) who have informed the registered provider that they had received 2 COVID-19 vaccinations.

(2) The System Governor or the Commissioner may, at any time, request a registered provider to give to the System Governor or the Commissioner a report under subsection (1).

(3) A request under subsection (2) must:
(a) be in writing; and
(b) specify a reporting day that is not more than 3 years before the request is made.

(4) A registered provider must comply with a request under subsection (2) within 7 days after the request is made, or such longer period as is agreed, in writing, between the System Governor or the Commissioner and the registered provider.



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## Subdivision B—Quality indicators

### 166-105 Application of Subdivision to certain registered providers

(1) Subject to subsection (2), this Subdivision applies to a registered provider registered in the provider registration category residential care.

(2) The requirements of this Subdivision do not apply to a registered provider in respect of the delivery of funded aged care services under a specialist aged care program.

### 166-110 Requirement to provide a quality indicators report

(1) For the purposes of paragraph 166(1)(d) of the Act, a registered provider must give a report (***the quality indicators report)*** on the quality indicators provided in this Subdivision to the System Governor each reporting period.

Note: For the reporting period for a quality indicators report, see section 166-115.

(2) To avoid doubt, if any individuals are excluded for any reason under this Subdivision, a registered provider is required to include information about the exclusion of those individuals in the report where required.

Example: If an individual is absent from the approved residential care home throughout the reporting period, as referred to in paragraph 166-135(1)(b) of this instrument, the registered provider is not required to include that individual in an assessment for falls and falls resulting in major injury, as referred to in paragraph 166-135(1)(a), but must include information about the exclusion of the individual under paragraph 166-135(1)(b).

### 166-112 Requirements for the collection of information for quality indicators reports—measurements and assessments

(1) For the purposes of paragraph 166(4)(a) of the Act, a registered provider must:
  (a) make measurements or other assessments relevant to indicating the quality of residential care; and
  (b) compile or otherwise derive information from those measurements and assessments mentioned in paragraph (a); and
  (c) give the information to the System Governor for the purposes of a quality indicator report.

(2) For paragraph (1)(a), measurements or assessments may include the collection or use of personal information and health information, within the meaning of the *Privacy Act 1988*, about the individual accessing funded aged care services.

(3) For paragraph (1)(b), the compiling or deriving of information must not include personal information about the individual accessing funded aged care services.

### 166-115 Timeframes for reporting under this Subdivision

For the purposes of subsection 166(4) of the Act:



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(a) the reporting period for a quality indicators report given to the System Governor under this Subdivision is a quarter, being a period of 3 months, beginning at the start of a financial year; and

(b) a registered provider must give a quality indicators report to the System Governor within 21 days after the end of the reporting period.

## 166-120 Quality indicator—pressure injuries

(1) A quality indicators report must include the following information on pressure injuries and the number of individuals accessing funded aged care services that:

(a) were assessed for pressure injuries;

(b) were excluded because of withholding consent to undergo an observational assessment for pressure injuries throughout the reporting period;

(c) were excluded because of an absence from accessing funded aged care services throughout the reporting period;

(d) have one or more pressure injuries;

(e) have one or more pressure injuries with each injury measured and reported against the following sub-categories:

(i) Stage 1 Pressure Injury;

(ii) Stage 2 Pressure Injury;

(iii) Stage 3 Pressure Injury;

(iv) Stage 4 Pressure Injury;

(v) Unstageable Pressure Injury;

(vi) Suspected Deep Tissue Pressure Injury;

(f) have one or more pressure injuries acquired outside the approved residential care home during the reporting period;

(g) have one or more pressure injuries acquired outside the approved residential care home during the reporting period with each injury measured and reported against the following sub-categories:

(i) Stage 1 Pressure Injury;

(ii) Stage 2 Pressure Injury;

(iii) Stage 3 Pressure Injury;

(iv) Stage 4 Pressure Injury;

(v) Unstageable Pressure Injury;

(vi) Suspected Deep Tissue Pressure Injury.

Note: For paragraphs (e) and (g), the relevant sub-category of pressure injury is determined by reference to the ICD-10 Australian Modified Pressure Injury Classification System.

(2) Registered providers, in preparing a quality indicators report for the purposes of subsection (1), must:

(a) identify a date once every reporting period when assessment for pressure injuries is to be carried out for each individual residing at the approved residential care home; and

(b) collect data relating to pressure injures for each individual during a single observation assessment at approximately the same time every reporting period; and



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(c) carry out assessment on or around the same time each reporting period and complete assessment as part of the individual's routine personal care (such as bathing and toileting).

(3) Registered providers, in assessing and reporting pressure injuries acquired by individuals, must inform the individual about the proposed assessment and must seek consent from the individual before the assessment takes place.

## 166-125 Quality indicator—restrictive practices

(1) A quality indicators report must include the following information on restrictive practices:

(a) the collection date in the reporting period;

(b) the number of individuals whose records were assessed for the use of restrictive practices other than chemical restraint throughout the collection period;

(c) the number of individuals excluded because of an absence from accessing funded aged care services throughout the collection period;

(d) the number of individuals subjected to the use of restrictive practices other than chemical restraint throughout the collection period:

(i) on any occasion;

(ii) on any occasion only in a secured area.

Note: See section 17-5 (Practices and interventions that are restrictive practices in relation to individuals). Restrictive practices covered by this section include environmental restraint, mechanical restraint, physical restraint and seclusion as defined in section 17-5.

(2) For the purposes of subsection (1), a collection period for this quality indicator is a 3-day period during the reporting period.

(3) Registered providers, in preparing a quality indicators report for the purposes of subsection (1), must:

(a) identify and record a collection date which is to take place during each reporting period; and

(b) ensure that the collection date is varied and unpredictable to staff directly involved in the care of an individual; and

(c) ensure that the 3-day collection period consists of the selected collection date and the 2 days before that date; and

(d) ensure that the 3-day collection period is the same for all individuals receiving funded aged care services at the approved residential care home.

(4) Registered providers, in preparing a quality indicators report for the purposes of subsection (1), must review an individual's existing record over the 3- day collection period and assess whether the individual was subject to the use of restrictive practices other than chemical restraint at any stage during the collection period.



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## 166-130 Quality indicator—unplanned weight loss

(1) A quality indicators report must include information on unplanned weight loss, with information reported under the following categories:
  (a) significant unplanned weight loss;
  (b) consecutive unplanned weight loss.

(2) For the purposes of paragraph (1)(a), a quality indicators report must include the following information on significant unplanned weight loss and the number of individuals accessing funded aged care services that:
  (a) were assessed for significant unplanned weight loss;
  (b) were excluded because of withholding consent to be weighed on the finishing weight collection date;
  (c) were excluded because of receiving end-of-life care;
  (d) were excluded because the previous or finishing weights were not recorded, including comments on why any such weights were not recorded;
  (e) experienced a 5% or higher decrease in weight between the finishing weight and the previous weight.

(3) Registered providers, in preparing the quality indicators report for the purposes of paragraph (1)(a), must:
  (a) identify each individual's previous weight using the approved residential care home's weight records of individuals in the residential care home; and
  (b) collect and record the finishing weight for each individual residing at the approved residential care home in the final month of the current reporting period using a calibrated scale; and
  (c) seek the consent of the individuals concerned before making the assessment required and must record any refusal to provide consent by an individual being assessed for their finishing weight; and
  (d) weigh individuals at or around the same time each month and at around the same time of the day and ensure that the individual is wearing clothing of a similar weight; and
  (e) for each individual who provided their consent for the making of the assessment, compare their finishing weight from the current reporting period with their previous weight and calculate the percentage weight loss.

(4) For the purposes of paragraph (1)(b), a quality indicators report must include the following information on consecutive unplanned weight loss and the number of individuals accessing funded aged care services that:
  (a) were assessed for consecutive unplanned weight loss;
  (b) were excluded because of withholding consent to be weighed on any weight collection date;
  (c) were excluded because of receiving end-of-life care;
  (d) were excluded because any of the required weights were not recorded, including comments on why any such weights were not recorded;
  (e) experienced any decrease in weight between the previous weight, starting weight, middle weight and finishing weight.



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(5) Registered providers, in preparing the quality indicators report for the purposes of paragraph (1)(b), must:

(a) measure consecutive unplanned weight loss as the weight loss of any amount measured every month over 3 consecutive months of a reporting period, and the 3 monthly weights are compared against each other and against the finishing weight from the previous reporting period; and

(b) use their approved residential care home's weight records in identifying each individual's previous weight; and

(c) in the first month of the reporting period, collect and record the starting weight of each individual residing at the approved residential care home using a calibrated scale; and

(d) in the second month of the reporting period collect and record the middle weight of each individual residing at the approved residential care home using a calibrated scale; and

(e) in the third and final month of the reporting period collect and record the finishing weight of each individual at the approved residential care home using a calibrated scale; and

(f) where previous weights and finishing weights may already have been collected and recorded for the individual as part of assessments and measurements made for significant unplanned weight loss, the previous weight and finishing weights recorded and collected can be used for assessing consecutive unplanned weight loss and do not need to be collected again; and

(g) seek the consent of the individuals concerned before making the assessment required and must record any refusal to provide consent by an individual being assessed for their finishing weight; and

(h) weigh individuals at or around the same time each month and at around the same time of the day and ensure that the individual is wearing clothing of a similar weight; and

(i) for each individual who provided their consent for the making of the assessment, compare their previous, starting, middle and finishing weight to determine whether there has been weight loss in every month over 3 consecutive months of a reporting period.

(6) If an individual is assessed under one category of unplanned weight loss, this does not limit the ability to be assessed and reported under the other category.

## 166-135 Quality indicator—falls and major injury

(1) A quality indicators report must include the following information on falls and falls resulting in major injury, and the number of individuals accessing funded aged care services:

(a) whose records were assessed for falls and falls resulting in major injury;

(b) who were excluded because of an absence from accessing funded aged care services at the approved residential care home throughout the reporting period;

(c) who experienced one or more falls at the approved residential care home during the reporting period;



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(d) who experienced one or more falls at the approved residential care home resulting in major injury during the reporting period.

(2) Registered providers, in preparing a quality indicators report for the purposes of subsection (1), must:
(a) identify and record a collection date for the reporting period; and
(b) ensure that the collection date is in the 21 days after the end of the reporting period.

## 166-140 Quality indicator—medication management

(1) A quality indicators report must include information on medication management, with information reported under the following categories:
(a) polypharmacy;
(b) antipsychotics.

(2) For the purposes of paragraph (1)(a), a quality indicators report must include the following information on polypharmacy:
(a) the collection date in the reporting period;
(b) the number of individuals assessed for polypharmacy;
(c) the number of individuals excluded because they were admitted in hospital on the collection date;
(d) the number of individuals prescribed 9 or more medications based on a review of any of the following taken on the collection date:
(i) the individual's medication chart;
(ii) the individual's administration record.

(3) For the purposes of paragraph (1)(b), a quality indicators report must include the following information on antipsychotics:
(a) the collection date in the reporting period;
(b) the number of individuals assessed for antipsychotic medications;
(c) the number of individuals excluded because of they were admitted in hospital for at least 6 days prior to the collection date;
(d) the number of individuals that received an antipsychotic medication based on a review of any of the following taken on the collection date:
(i) the individual's medication chart;
(ii) the individual's administration record;
(e) the number of individuals that received an antipsychotic medication for a medically diagnosed condition of psychosis based on a review of any of the following taken on the collection date:
(i) the individual's medication chart;
(ii) the individual's administration record.

(4) For the purposes of paragraph (2)(a), the collection date is any date during the reporting period and the collection of data must involve a single review of medication charts and administration records for each individual on a particular collection date during the reporting period.



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(5) Registered providers, in preparing a quality indicators report for the purposes of paragraph (1)(a), must:
  (a) identify and record a collection date for the reporting period;
  (b) use each individual's medication charts and administration records as at the identified collection date.

(6) For the purposes of paragraph (3)(a), the collection date is the final day in the collection period which for that subsection is a 7-day period during the reporting period.

(7) Registered providers, in preparing a quality indicators report for the purposes of paragraph (1)(b), must:
  (a) identify a collection date during the reporting period which is between the second week and the end of the reporting period;
  (b) assess and review all individual's medication charts and administration records during the assessment period which covers the collection date and the 6 days prior to that collection date;
  (c) ensure that the collection date is varied between reporting periods and must not be identified to staff directly involved in the care of individuals accessing funded aged care services;
  (d) ensure the assessment and review is not conducted by staff directly involved in the care of individuals receiving funded aged care services.

## 166-145 Quality indicator—activities of daily living

(1) A quality indicators report must include the following information on activities of daily living and the number of individuals receiving funded aged care services that:
  (a) were assessed for activities of daily living function;
  (b) were excluded because of receiving end-of-life care;
  (c) were excluded because of an absence from accessing funded aged care services throughout the entire reporting period;
  (d) were excluded because an assessment for activities of daily living function was not recorded for the previous reporting period, including comments on why any such previous assessment was not recorded;
  (e) were assessed for activities of daily living function and received a total score of zero in the previous reporting period;
  (f) were assessed for activities of daily living function and experienced a decline in the total score by one or more points.

(2) Registered providers, in preparing a quality indicators report for the purposes of subsection (1), must ensure the collection of information involves a single assessment of each individual, completed around the same time every reporting period, comparing the information to the activities of daily living assessment total score in the previous reporting period to determine decline.

(3) For the purposes of subsection (1), registered providers must:



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(a) identify each individual's activities for daily living assessment total score from the previous reporting period using their approved residential care home's care records;

(b) conduct activities for daily living assessment for each individual by completing the Barthel Index;

(c) compare the previous reporting period assessment total score with the current reporting period assessment total score in relation to an individual in order to determine if the individual experienced a decline of one or more points.

Note: Without limiting this section, activities of daily living includes fundamental skills typically needed to manage basic physical needs in the following areas:

(a) grooming and personal hygiene, such as oral care;
(b) dressing;
(c) toileting and continence;
(d) ambulating and movement;
(e) eating.

## 166-150 Quality indicator—incontinence care

(1) A quality indicators report must include the following information on incontinence care and the number of individuals receiving funded aged care services that:

(a) were assessed for incontinence care;

(b) were excluded because of an absence from accessing funded aged care services throughout the reporting period;

(c) were excluded from an Incontinence Associated Dermatitis assessment because they did not have incontinence;

(d) have incontinence and Incontinence Associated Dermatitis measured and reported against the following Ghent Global Incontinence Associated Dermatitis Categorisation Tool sub-categories:

(i) 1A: persistent redness without clinical signs of infection;
(ii) 1B: persistent redness with clinical signs of infection;
(iii) 2A: skin loss without clinical signs of infection;
(iv) 2B: skin loss with clinical signs of infection;

(e) have incontinence;

(f) have incontinence and Incontinence Associated Dermatitis.

(2) Registered providers, in preparing a quality indicators report for the purposes of subsection (1), must:

(a) identify a date once every reporting period to assess each individual residing at the approved residential care home for incontinence as part of routine care and carry out that assessment on or around the same time each reporting period; and

(b) ensure that the assessment of Incontinence Associated Dermatitis is conducted by staff who understand the Ghent Global Incontinence Associated Dermatitis Categorisation Tool and have the necessary skills and experience to do so accurately and safely; and



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(c) if personal care workers observe or identify signs of redness or skin loss during routine personal care, escalate to appropriately trained staff for further assessment; and

(d) consult with a suitably qualified health practitioner if there is uncertainty about the presence or severity of Incontinence Associated Dermatitis.

(3) For the purposes of subsection (1), registered providers must ensure that the collection of information involves a single assessment for each individual as part of routine care around the same time every reporting period.

## 166-155 Quality indicator—hospitalisation

(1) A quality indicators report must include the following information on hospitalisation and the number of individuals receiving funded aged care services that:

(a) were assessed for hospitalisation;

(b) were excluded because of an absence from accessing funded aged care services throughout the reporting period;

(c) had one or more emergency department presentations during the reporting period;

(d) had one or more emergency department presentations or hospital admissions during the reporting period.

(2) In preparing a quality indicators report for the purposes of subsection (1), registered providers must:

(a) ensure that the collection of information involves a single review of the care records for each individual for the entire reporting period;

(b) view the care records within 21 days after the end of the reporting period.

## 166-160 Quality indicator—workforce

(1) A quality indicators report must include information on the workforce of a registered provider, reported against the following sub-categories of staff delivering funded aged care services:

(a) service managers;

(b) nurse practitioners and registered nurses;

(c) enrolled nurses;

(d) personal care workers and nursing assistants.

(2) A quality indicators report must include the following information reported against each sub-category specified in subsection (1):

(a) on the number of staff that have worked any number of hours in the previous reporting period;

(b) on the number of staff that:

(i) were employed at the start of the reporting period; and

(ii) have worked for at least 120 hours in the previous reporting period;

(c) on the number of staff that:

(i) were employed at the start of the reporting period; and



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(ii) did not work for at least 60 consecutive days in the reporting period.

(3) For the purposes of subsection (1), registered providers must review the records within 21 days after the end of the reporting period.

## 166-165 Quality indicator—Consumer Experience Assessment

(1) A quality indicators report must include the following information on an individual's experience obtained through a Consumer Experience Assessment and the number of individuals accessing funded aged care services that:

(a) were excluded because of an absence from accessing funded aged care services throughout the reporting period;

(b) were excluded because of choosing not to complete the Consumer Experience Assessment in the reporting period;

(c) were offered a Consumer Experience Assessment during the reporting period through any of the following means:

(i) a self-completion assessment;

(ii) an interviewer facilitated assessment;

(iii) a proxy assessment;

(d) undertook the Consumer Experience Assessment during the reporting period, the number of individuals who reported against the following sub-categories and the means of assessment mentioned in paragraph (a) for the individuals in each sub-category:

(i) excellent: for individuals who score between 22 and 24;

(ii) good: for individuals who score between 19 and 21;

(iii) moderate: for individuals who score between 14 and 18;

(iv) poor: for individuals who score between 8 and 13;

(v) very poor: for individuals who score between 0 and 7;

(2) In preparing a quality indicators report for the purposes of subsection (1), registered providers must ensure that the collection of information for consumer experience assessment uses the Quality-of-Care Experience questions.

(3) In preparing a quality indicators report for the purposes of subsection (1), registered providers must collect information from each individual, once every reporting period.

(4) In preparing a quality indicators report for the purposes of subsection (1), registered providers must:

(a) in order to collect information, offer a copy of the QCE-ACC Self Complete Version document for self-completion by suitable individuals at the approved residential care home at around the same time every reporting period; and

(b) allow an individual with no or mild cognitive impairment to self-complete the QCE-ACC Self Complete Version document; and

(c) arrange interviewer facilitated completion for all individuals requiring assistance to complete the QCE-ACC document (such as where the individual requires support with reading the questions or writing their



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responses) at around the same time every reporting period using the QCE-ACC Interviewer Facilitated Version; and

(d) arrange proxy-completion for all individuals who cannot complete the QCE-ACC through self-completion or interviewer facilitated completion such as in circumstances where the individual has moderate or cognitive impairment at around the same time every reporting period using the QCE-ACC Proxy Version.

## 166-170 Quality indicator—Quality of Life Assessment

(1) A quality indicators report must include the following information on an individual's quality of life obtained through a Quality of Life Assessment and the number of individuals accessing funded aged care services that:

(a) were offered a Quality of Life Assessment during the reporting period through any of the following means:

(i) a self-completion assessment;

(ii) an interviewer facilitated assessment;

(iii) a proxy assessment;

(b) were excluded because of an absence from receiving funded aged care services throughout the reporting period;

(c) were excluded because of choosing not to complete the Quality of Life Assessment in the reporting period;

(d) undertook the Quality of Life Assessment, the number of individuals who reported against the following sub-categories and the means of assessment mentioned in paragraph (a) for the individuals in each sub-category:

(i) excellent: for individuals who score between 22 and 24;

(ii) good: for individuals who score between 19 and 21;

(iii) moderate: for individuals who score between 14 and 18;

(iv) poor: for individuals who score between 8 and 13;

(v) very poor: for individuals who score between 0 and 7.

(2) For the purposes of subsection (1), registered providers must ensure that the collection and assessment of information relating to quality of life must use the Quality of Life questions.

(3) For the purposes of subsection (1), registered providers must collect information from each individual, once every reporting period.

(4) In preparing a quality indicators report for the purposes of subsection (1), registered providers must:

(a) in order to collect information, offer a copy of the QOL-ACC Self Complete Version document for self-completion by suitable individuals at the approved residential care home at around the same time every reporting period; and

(b) allow an individual with no or mild cognitive impairment to self-complete the QOL-ACC Self Complete Version document; and

(c) arrange interviewer facilitated completion for all individuals requiring assistance to complete the QOL-ACC document (such as where the



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individual requires support with reading the questions or writing their responses) at around the same time every reporting period using the QOL-ACC Interviewer Facilitated Version; and

(d) arrange proxy-completion for all individuals who cannot complete the QOL-ACC through self-completion or interviewer facilitated completion such as in circumstances where the individual has moderate or cognitive impairment at around the same time every reporting period using the QOL-ACC Proxy Version.

## 166-175 Quality indicator—allied health

(1) A quality indicators report must include the following information on allied health:

(a) the number of individuals accessing funded aged care services in the residential care home that were assessed for services delivered by an allied health professional during the reporting period;

(b) the number of individuals who were excluded because of an absence from accessing funded aged care services throughout the reporting period;

(c) the number of funded aged care services in the service type allied health and therapy that were recommended to be delivered by an allied health professional (including through a care and services plan under paragraph 148(e) of the Act), reported against the sub-categories set out in subsection (3);

(d) the number of funded aged care services in the service type allied health and therapy that were recommended to be delivered by an allied health professional (including through a care and services plan under paragraph 148(e) of the Act) and which were received, reported against the sub-categories set out in subsection (3).

Note: See section 148 of the Act (Delivery of funded aged care services).

(2) A quality indicators report must include the number of allied health professional labour hours (represented as minutes of care and services delivered to individuals each day) and reported separately as:

(a) allied health professional employee labour hours;

(b) allied health professional agency labour hours.

(3) For the purposes of paragraphs (1)(c) and (d) and subsection (2), a registered provider must report against each of the following sub-categories of allied health professionals;

(a) physiotherapist;

(b) occupational therapist;

(c) speech pathologist;

(d) podiatrist;

(e) dietitian;

(f) allied health assistant;

(g) other allied health professionals.



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(4) For the purposes of paragraph (3)(g) other allied health professionals include the following:
  (a) art therapists;
  (b) audiologists;
  (c) exercise physiologists;
  (d) music therapists;
  (e) chiropractors;
  (f) counsellors;
  (g) osteopaths;
  (h) psychologists;
  (i) social workers.

(5) In preparing a quality indicators report for the purposes of subsection (1), registered providers must:
  (a) collect information as to recommended funded aged care services in the service type allied health and therapy by undertaking a single review of all individual's care records for the entire reporting period; and
  (b) only report once per allied health discipline.

## 166-180 Quality indicator—lifestyle officers

(1) A quality indicators report must include the following information on lifestyle officers delivering funded aged care services:
  (a) the total labour hours worked in care minutes by lifestyle officers;
  (b) the total labour hours worked as agency staff by lifestyle officers.

Note: For subsection (1), a lifestyle officer includes any of the following roles:
  (a) diversional officer;
  (b) recreation officer;
  (c) activities officer.

(2) For the purposes of a quality indicators report and the quality indicator on lifestyle officers, a registered provider must also provide information on the number of occupied bed days for a provider for the reporting period.

## 166-185 Quality indicator—enrolled nursing

(1) A quality indicators report must include the following information on enrolled nursing with respect to direct care staff members that deliver funded aged care services:
  (a) enrolled nursing total number of care minutes (in hours);
  (b) registered nursing total number of care minutes (in hours);
  (c) personal care workers and nursing assistants total number of care minutes (in hours).

(2) For the purposes of subsection (1), direct care staff members covered under this section are those specified in paragraphs (a) to (d) of the definition of ***direct care staff member*** in section 7 of the Act.



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## Subdivision C—Complaints and feedback management report

### 166-205 Application of Subdivision

(1) This Subdivision applies to registered providers registered in one or more of the following provider registration categories:
  (a) personal and care support in the home or community;
  (b) nursing and transition care;
  (c) residential care.

(1A) This Subdivision also applies to a registered provider registered in the provider registration category subscription trial if the provider is also registered in a provider registration category referred to in paragraph (1)(a) or (b).

(2) This Subdivision applies to feedback received by a registered provider:
  (a) that is managed in accordance with section 165-30; and
  (b) raises an issue as referred to in subsection 165-30(2).

### 166-210 Requirements for reporting information relating to complaints and feedback management

(1) For the purposes of paragraph 166(1)(a) of the Act this section prescribes that a registered provider to whom this Subdivision applies must give a report about the management of complaints and feedback (the ***complaints and feedback management report***) to the Commissioner within 4 months after the end of the reporting period for the registered provider.

(2) The reporting period for a registered provider is:
  (a) the period of 12 months starting on 1 July of a year; or
  (b) another 12 month period that starts on the first day of a month of a year that is determined for the registered provider by the System Governor in accordance with the rules.

(3) The report must:
  (a) be in the approved form; and
  (b) be signed by a governing body of the registered provider; and
  (c) include the information prescribed by subsection (4) of this section that is a summary of the management of complaints and feedback.

(4) For the purposes of paragraph 166(1)(a) of the Act, the following information is prescribed:
  (a) information about complaints and feedback received, including the number of complaints and feedback and the nature of the complaints and feedback;
  (b) information about the action taken to resolve complaints or in response to feedback received, including any subsequent improvements made by the registered provider in relation to complaints and feedback;
  (c) information about an evaluation of the effectiveness of the actions taken and their related outcome in relation to each complaint and feedback;



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(d) information about the number of days taken to resolve each complaint and feedback;
(e) information about the education and training that has been delivered to the staff of the registered provider in relation to each complaint and feedback;
(f) an analysis of the patterns of, and underlying reasons for, complaints.

## Subdivision D—Complaints and feedback information on request

### 166-215 Application of Subdivision to all registered providers

(1) This Subdivision applies to every kind of registered provider.

(2) This Division applies to feedback received by a registered provider:
(a) that is managed in accordance with section 165-30; and
(b) raises an issue as referred to in subsection 165-30(2).

### 166-220 Requirements for reporting information on request relating to complaints and feedback management

(1) For the purposes of paragraphs 166(1)(a) and (d) of the Act this section prescribes that a registered provider to whom this Subdivision applies must on request by the System Governor or the Commissioner, give a report about the management of complaints and feedback (the ***complaints and feedback management report***) to the System Governor and the Commissioner.

(2) The System Governor or the Commissioner may, at any time, request a registered provider to give the System Governor and the Commissioner a report under subsection (1).

(3) A registered provider must comply with a request under subsection (1) within 14 days after the request is made, or such longer period as specified in the request.

(4) The report must:
(a) be in the approved form; and
(b) be signed by a governing body of the registered provider; and
(c) include the information prescribed by subsection (5) of this section.

(5) For the purposes of paragraphs 166(1)(a) and (d) of the Act, the following information is prescribed:
(a) information about complaints and feedback received, including the number of complaints and feedback and the nature of the complaints and feedback;
(b) information about the action taken to resolve complaints or in response to feedback received including any subsequent improvements made by the registered provider in relation to complaints and feedback;
(c) information about an evaluation of the effectiveness of the actions taken and their related outcome in relation to each complaint and feedback;
(d) information about the number of days taken to resolve each complaint and feedback;



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(e) information about the education and training that has been delivered to the staff of the registered provider in relation to each complaint and feedback;

(f) an analysis of the patterns of, and underlying reasons for, complaints.

## Subdivision E—Prudential and financial

### 166-310 Aged care financial report—general

*Scope of this section*

(1) Subject to subsection (2), for the purposes of paragraph 166(1)(d) of the Act:

(a) every kind of registered provider is prescribed; and

(b) a registered provider must give a report about financial and prudential matters (the ***aged care financial report***) to the System Governor each reporting period for the registered provider.

Note: For the reporting period for an aged care financial report, see section 166-355 of this instrument.

(2) The requirements of this section do not apply in respect of the delivery of funded aged care services under any of the following specialist aged care programs:

(a) CHSP;

(b) NATSIFACP;

(c) TCP.

Note: For requirements relating to an annual prudential compliance statement, see section 166-360 of this instrument.

*Requirements for an aged care financial report*

(3) The aged care financial report must:

(a) be in the approved report form; and

(b) be signed by a member of the governing body of the registered provider; and

(c) be given to the System Governor within 4 months after the end of each reporting period for the registered provider; and

(d) if section 166-315 of this instrument applies—include a financial support statement; and

(e) if section 166-345 of this instrument applies—include a general purpose financial report; and

(f) if section 166-360 of this instrument applies—include an annual prudential compliance statement.

Note: The term ***governing body*** is defined in section 7 of the Act. The effect of this definition is that for registered providers who are not a body corporate with a board of directors (for example, State and Territory governments), the Aged Care Financial Report can be signed by a person or group of persons responsible for the executive decisions of the registered provider.

(4) To avoid doubt, the System Governor may, for the purposes of paragraph (3)(a), approve different forms in relation to specified provider registration categories or specified kinds of registered providers.



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*Additional requirements if registered in provider registration category residential care*

(5) For a registered provider registered in the provider registration category residential care, in addition to the requirements in subsection (3), an aged care financial report must also include the following:

(a) subject to section 166-335 of this instrument, a care minutes performance statement;

(b) the amount of accommodation payments and accommodation contributions paid to the registered provider;

(c) the amounts of those accommodation payments and accommodation contributions paid as refundable deposits and daily payments;

(d) the amounts of accommodation bonds and accommodation charges paid to the registered provider;

(e) the extent of building, upgrading and refurbishment of residential care homes.

*Requirement to comply with a notice for further information*

(6) A registered provider must comply with a notice under subsection (8) within the period specified in the notice or, if no period is specified in the notice, within 28 days after the day when the notice is given.

(7) A registered provider complies with a notice under subsection (8) only if the provider gives the information in a form (if any) approved by the System Governor for the purposes of that subsection.

*System Governor may request further information*

(8) The System Governor may, by notice in writing, require a registered provider to give the System Governor information about a matter, as specified in the notice, if the information is about a matter required under this section.

(9) A notice under subsection (8) may require a registered provider to give updated information about a matter in relation to a period that is:

(a) the same as the reporting period to which a report under this section relates; or

(b) different to the reporting period to which a report under this section relates.

*Compliance with this section for part of reporting period*

(10) If the registered provider:

(a) is registered as a registered provider for part of the reporting period for the provider; or

(b) expands its services to a new registration category for part of the reporting period;

the provider is taken to have complied with this section if the provider complies with this section for that part of the reporting period.



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## 166-315 Aged care financial report—provision of a financial support statement

*Scope of this section*

(1) Subject to subsection (2), for the purposes of paragraph 166(1)(d) of the Act, a registered provider is prescribed if the provider:
  (a) is registered in the provider registration category residential care; and
  (b) is part of or is financially reliant on a related corporate entity; and
  (c) is not a government entity or a local government authority; and
  (d) is required under section 166-310 of this instrument to give the System Governor an aged care financial report for the provider for a reporting period.

(2) This section does not apply to registered providers that deliver funded aged care services under the MPSP.

*Financial support statement to be given with aged care financial report*

(3) If a registered provider is required to give the System Governor an aged care financial report for the provider for a reporting period, then the provider must, when giving the report to the System Governor, also give the System Governor a financial support statement for the provider signed within the period of 4 months starting on the day after the end of the reporting period.

(4) However, subsection (3) does not apply to a registered provider in relation to a reporting period if the aged care financial report for the provider for the year includes an explanation of why the provider has not complied with subsection (3) in relation to that year.

*Financial support statement to be given on request*

(5) The System Governor may at any time, by notice in writing, require a registered provider to give the System Governor a financial support statement for the provider signed within the period for signing specified in the notice.

Note: For who must sign a financial support statement, see subsection 166-320(4) of this instrument.

(6) A registered provider must comply with a notice under subsection (5) within the period for complying specified in the notice or, if no such period is specified in the notice, within 28 days after the day when the notice is given.

(7) However, subsection (6) does not apply to a registered provider in relation to a notice under subsection (5) if at or before the end of the period within which the provider would (but for this subsection) be required to comply with the notice, the provider gives the System Governor a written explanation of why the provider is not able to comply with the notice.



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*System Governor may request further information and documents*

(8) The System Governor may at any time, by notice in writing, require a registered provider to give the System Governor information about a matter, as specified in the notice, if the information is about a matter required under this section.

(9) A registered provider must comply with a notice under subsection (8) within the period for complying specified in the notice or, if no such period is specified in the notice, within 28 days after the day when the notice is given.

## 166-320 Aged care financial report—what is a financial support statement

(1) A ***financial support statement***, for a registered provider of a kind prescribed under subsection 166-315(1) of this instrument, is a written statement by the ultimate holding company in relation to the provider that satisfies the requirements in subsections (2), (4) and (5).

(2) The statement must either:
   (a) state that the ultimate holding company is willing and able, while the provider remains a registered provider, to provide any financial support to the provider that is needed in order to enable the provider to pay the debts of the provider specified under subsection (3) in relation to the statement; or
   (b) state that the ultimate holding company is not willing and able, while the provider remains a registered provider, to provide such financial support to the provider.

(3) For the purposes of paragraph (2)(a), the following debts of the provider are specified in relation to the statement (whether or not the debts relate to the provision of aged care services by the provider):
   (a) any debts of the provider that are outstanding immediately before the start of the day (the ***giving day***) when the statement is given to the System Governor;
   (b) any debts of the provider that:
      (i) are debts that become due during the period that starts on the giving day and ends immediately before the start of the first day after the giving day when the provider gives the System Governor another financial support statement for the provider; or
      (ii) if the provider never gives the System Governor another financial support statement for the provider after the giving day—are debts that become due on or after the giving day.

(4) A financial support statement must be signed by:
   (a) if the ultimate holding company is a body corporate that is incorporated, or taken to be incorporated, under the *Corporations Act 2001*—a director of the body corporate for the purposes of that Act; or
   (b) otherwise—a member of the ultimate holding company's governing body.

(5) The statement must be in a form (if any) approved by the System Governor for the purposes of this subsection.



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## 166-325 Aged care financial report—permitted uses reconciliation

(1) A form approved by the System Governor for an aged care financial report for a registered provider may require such a report to include a statement (a ***permitted uses reconciliation***) that sets out information about reportable uses of funds by the registered provider during a reporting period or reporting periods for the registered provider.

(2) For the purposes of subsection (1), a reportable use of funds is any of the following:
  (a) a permitted use of refundable deposits or accommodation bonds;
  (b) a use of funds (other than refundable deposits or accommodation bonds) which was such that, if the funds had been refundable deposits or accommodation bonds, the use would have been a permitted use of refundable deposits or accommodation bonds.

Note: For the permitted uses of refundable deposits, see section 310 of the Act. For the permitted uses of accommodation bonds, see section 287-101 of this instrument.

(3) To avoid doubt, the System Governor may, for the purposes of subsection (1), approve different forms in relation to specified provider registration categories or specified kinds of registered providers.

## 166-335 Aged care financial report—care minutes performance statement

*Scope of this section*

(1) Subject to subsection (2), for the purposes of paragraph 166(1)(d) of the Act, a registered provider is prescribed if the provider:
  (a) is registered in the provider registration category residential care; and
  (b) is required under section 166-310 of this instrument to give the System Governor an aged care financial report for the provider for a reporting period.

(2) This section does not apply to registered providers that deliver funded aged care services under the MPSP.

(3) A registered provider of the kind prescribed under this section must provide a written statement that satisfies the requirements in subsections (4), (5) and (6) (a ***care minutes performance statement***).

*Care minutes performance statement to be given with aged care financial report*

(4) A registered provider must, when giving the aged care financial report to the System Governor, also give the System Governor a care minutes performance statement for the provider in accordance with the requirements prescribed by subsection (5).

*Requirements for a care minutes performance statement*

(5) A care minutes performance statement for a reporting period for a registered provider must:



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(a) be in writing; and
(b) be in a form approved by the System Governor; and
(c) not contain false or misleading information; and
(d) include the total number of care minutes (in hours) for each reporting period quarter as delivered by:
(i) registered nurses; and
(ii) enrolled nurses; and
(iii) personal care workers and nursing assistants; and
(e) include the total expenses for care minutes for each reporting period quarter as delivered by:
(i) registered nurses; and
(ii) enrolled nurses; and
(iii) personal care workers and nursing assistants; and
(f) include 24/7 registered nursing coverage across the reporting period by calendar month and expressed as a percentage; and
(g) if there is a variance in the data submitted under paragraph (f) throughout the reporting period—the registered provider must include an explanation for the variance; and
(h) if there is a variance between the care minutes reported in the care minutes performance statement and the care minutes reported in the previous quarterly financial reports—the registered provider must include an explanation for the variance; and
(i) subject to subsection (6), include a signed audit report for the submitted care minutes performance statement by an independent auditor.

*Auditing of care minutes performance statement*

(6) A care minutes performance statement must be audited in accordance with the following requirements:
(a) be audited by:
(i) a registered company auditor within the meaning of the *Corporations Act 2001*; or
(ii) a person approved by the System Governor under subsection (7);
(b) be audited in accordance with the Assurance Engagements Other than Audits or Reviews of Historical Financial Information Standard (ASAE 3000).

Note: For the purposes of paragraph (b), the Assurance Engagements Other than Audits or Reviews of Historical Financial Information Standard (ASAE 3000) is published by the Australian Auditing and Assurance Standards Board.

(7) The System Governor may approve a person to audit a care minutes performance statement if the System Governor is satisfied that the person has appropriate qualifications and experience.

(8) The System Governor may revoke an approval of a person under subsection (7) if the System Governor is satisfied that the person is no longer a fit and proper person to audit a care minutes performance statement.



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## 166-340 Quarterly financial report

*Scope of this section*

(1) Subject to subsections (2) and (3), for the purposes of paragraph 166(1)(d) of the Act, every kind of registered providers is prescribed.

(2) The requirements of this section do not apply in respect to the delivery of funded aged care services under any of the following specialist aged care programs:
  (a) CHSP;
  (b) TCP.

(3) The requirements of this section do not apply in respect of the delivery of funded aged care services under the NATSIFACP where the registered provider is not registered in the provider registration category residential care.

(4) A registered provider of the kind prescribed under this section must give a report about matters provided in subsection (5) (the ***quarterly financial report***) to the System Governor for each quarter of a reporting period for the registered provider.

Note: For the reporting period for a quarterly financial report, see section 166-355 of this instrument.

*Requirements for a quarterly financial report*

(5) The quarterly financial report must:
  (a) be in the approved report form; and
  (b) be signed by a member of the governing body of the registered provider.

Note: The term ***governing body*** is defined in section 7 of the Act.

(6) To avoid doubt, the System Governor may, for the purposes of paragraph (5)(a), approve different forms in relation to specified provider registration categories or specified kinds of registered providers.

*Timeframes to give System Governor a quarterly financial report*

(7) The quarterly financial report for a quarter of a reporting period for the registered provider must be given to the System Governor:
  (a) for a quarter ending at the end of 31 December—within 45 days after the end of the quarter; and
  (b) for any other quarter—within 35 days after the end of the quarter.

(8) Each of the following is a quarter of a reporting period for the registered provider:
  (a) the period of 3 months beginning on the first day of the reporting period for the registered provider;
  (b) each successive period of 3 months that occurs during the reporting period for the registered provider after the end of the period mentioned in paragraph (a).



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*Requirement to comply with a notice for further information*

(9) A registered provider must comply with a notice under subsection (11) within the period specified in the notice or, if no period is specified in the notice, within 28 days after the day when the notice is given.

(10) A registered provider complies with a notice under subsection (11) only if the provider gives the information in a form (if any) approved by the System Governor for the purposes of that subsection.

*System Governor may request further information*

(11) The System Governor may, by notice in writing, require a registered provider to give the System Governor information about a matter, as specified in the notice, if the information is about a matter required under this section.

(12) A notice under subsection (11) may require a registered provider to give updated information about a matter in relation to a period that is:
(a) the same as the reporting period to which a report under this section relates; or
(b) different to the reporting period to which a report under this section relates.

*Compliance with this section for part of reporting period*

(13) If the registered provider is registered as a registered provider for part of the reporting period for the registered provider or the registered provider expands its services to a new registration category for part of the reporting period, the registered provider is taken to have complied with this section if the registered provider complies with this section for that part of the reporting period.

## 166-345 Aged care financial report—general purpose financial report

*Scope of this section*

(1) Subject to subsection (2), for the purposes of paragraph 166(1)(d) of the Act, a registered provider is prescribed if the provider is registered in the provider registration category residential care.

(2) The requirements of this section do not apply to a registered provider if the registered provider:
(a) is a government entity or a local government authority; or
(b) is delivering funded aged care services under a specialist aged care program.

(3) A registered provider of a kind prescribed under this section must give a report on matters provided in subsection (4) (the ***general purpose financial report***) to the System Governor each reporting period for the registered provider.

Note: For the reporting period for a general purpose financial report, see section 166-355 of this instrument.



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*Requirements for a general purpose financial report*

(4) The general purpose financial report must:
  (a) be a general purpose financial report within the meaning given by section 6 of the Statement of Accounting Concepts SAC 1; and
  (b) be in accordance with the Australian Accounting Standards in force at the time the report is prepared; and
  (c) give a true and fair view of the financial position and performance of the registered provider for the reporting period for each residential care home approved in relation to the registered provider; and
  (d) be written as if the registered provider were, so far as it provided those services, a distinct reporting entity within the meaning of the Statement of Accounting Concepts SAC 1; and
  (e) be given to the System Governor at the same time the registered provider gives an aged care financial report under section 166-310 of this instrument to the System Governor.

(5) If a general purpose financial report deals with a matter other than funded aged care services delivered through the service group residential care, the report must be prepared as if the funded aged care services delivered through the service group residential care it relates to were a reportable segment for the purposes of the Australian accounting standards related to segment reporting in force at the time the report is prepared.

(6) Despite subsections (4) and (5), if all the information about each residential care home approved in relation to the registered provider is included in the provider's aged care financial report for the reporting period, none of that information need be included in the general purpose financial report for the reporting period.

(7) A registered provider must give a copy of its most recently audited general purpose financial report to each person who asks for a copy and is:
  (a) an individual accessing funded aged care services at the approved residential care home; or
  (b) an individual who has an access approval in effect for a residential care home and is considering accessing funded aged care services through the residential care home; or
  (c) a supporter of an individual to whom paragraph (a) or (b) applies.

*Requirement to comply with a notice for further information*

(8) A registered provider must comply with a notice under subsection (10) within the period specified in the notice or, if no period is specified in the notice, within 28 days after the day when the notice is given.

(9) A registered provider complies with a notice under subsection (10) only if the provider gives the information in a form (if any) approved by the System Governor for the purposes of that subsection.



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*System Governor may request further information*

(10) The System Governor may, by notice in writing, require a registered provider to give the System Governor information about a matter, as specified in the notice, if the information is about a matter required under this section.

(11) A notice under subsection (10) may require a registered provider to give updated information about a matter in relation to a period that is:
  (a) the same as the reporting period to which a report under this section relates; or
  (b) different to the reporting period to which a report under this section relates.

*Compliance with this section for part of reporting period*

(12) If the registered provider is registered as a registered provider for part of the reporting period or the registered provider expands its services to a new registration category for part of the reporting period, the registered provider is taken to have complied with this section if the registered provider complies with this section for that part of the reporting period.

*Requirement to publish reports*

(13) A registered provider must ensure that a general purpose financial report for a reporting period is published, in a way that makes the report readily accessible to members of the public, on:
  (a) the registered provider's website; or
  (b) if the registered provider does not have a website—another website;

for a period of at least 3 years starting no later than 5 months after the end of the reporting period.

## 166-350 Aged care financial report—general purpose financial report audit requirements

(1) A general purpose financial report must:
  (a) be audited by:
    (i) a registered company auditor within the meaning of the *Corporations Act 2001*; or
    (ii) a person approved by the System Governor under subsection (2); and
  (b) include each of the following:
    (i) an audit opinion about the general purpose financial report from a registered company auditor or a person approved under subsection (2);
    (ii) a statement from the registered company auditor or the person approved under subsection (2) as to whether the report complies with paragraphs 166-345(4)(b) and (c) of this instrument.

(2) The System Governor may approve a person to audit a general purpose financial report if the System Governor is satisfied that the person has appropriate qualifications and experience.



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(3) The System Governor may revoke an approval of a person under subsection (2) if the System Governor is satisfied that the person is no longer a fit and proper person to audit a general purpose financial report.

## 166-355 Financial and prudential reports—reporting period

(1) For the purposes of subsection 166(4) of the Act, the reporting period for all reports given to the System Governor by a registered provider under this Subdivision means:
  (a) the financial year (the 12 month period beginning on 1 July and ending on 30 June); or
  (b) if under subsection (3), the System Governor determines another period of 12 months (being a period that begins on the first day of a month)—that other period.

(2) A registered provider may apply to the System Governor to determine a period of 12 months, other than the financial year, to be the registered provider's reporting period.

(3) If the System Governor receives an application from a registered provider for a determination under subsection (2), the System Governor must:
  (a) make, or refuse to make, the determination; and
  (b) notify the registered provider, in writing, of the System Governor's decision:
    (i) within 28 days; or
    (ii) if the Secretary has requested further information in relation to the application—within 28 days, excluding the period within which the information is requested and received.

(4) The System Governor may determine another period to be the registered provider's reporting period under subsection (3) only if the System Governor is satisfied, on reasonable grounds, that it would be impracticable for the registered provider to comply with the requirements of this Subdivision in relation to a financial year.

(5) If the System Governor refuses to make a determination for the registered provider under subsection (3), the System Governor must also give the registered provider a written statement of the reasons for the decision.

## 166-360 Aged care financial report—annual prudential compliance statement—general

*Scope of this section*

(1) For the purposes of paragraph 166(1)(d) of the Act, a registered provider is prescribed if the registered provider:
  (a) is registered in the provider registration category residential care; and
  (b) receives or has received payment of any of the following from an individual, wholly or partly as a lump sum:



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(i) a refundable deposit;
(ii) an accommodation bond;
(iii) an entry contribution.

(2) A registered provider of the kind prescribed under this section must give the System Governor a statement (the ***annual prudential compliance statement***) for a reporting period for the registered provider that includes the following:
(a) if the lump sum is a refundable deposit—information about refundable deposits and refundable deposit balances referred to in section 166-365 of this instrument;
(b) if the lump sum is an accommodation bond—information about the accommodation bond and accommodation bond balance referred to in section 166-365 of this instrument;
(c) if the lump sum is an entry contribution—information about entry contributions referred to in section 166-365 of this instrument;
(d) information about other fees referred to in section 166-375 of this instrument;
(e) the statements and other information referred to in section 166-380 of this instrument.

*Requirements for an annual prudential compliance statement*

(3) An annual prudential compliance statement for a reporting period for a registered provider must:
(a) be in writing; and
(b) be in a form approved by the System Governor; and
(c) further to subsection (2), include the information specified in that subsection; and
(d) not contain false or misleading information; and
(e) include a declaration that the registered provider has only charged the amount of refundable deposit the registered provider is permitted to charge; and
(f) if the registered provider has not complied with paragraph (e), include the number of times the provider has not complied with this requirement and the reasons for the non-compliance; and
(g) if the registered provider is required to prepare an aged care financial report for the reporting period—be included in the aged care financial report for the registered provider for the reporting period; and
(h) if paragraph (g) does not apply to the registered provider:
(i) be signed by a person who is one of the registered provider's responsible persons and is authorised by the registered provider to sign the statement; and
(ii) be given to the System Governor within 4 months after the end of the reporting period for the registered provider.

Note: The annual prudential compliance statement must be supported by an independent audit (see section 166-385 of this instrument).



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## 166-365 Annual prudential compliance statement—information about refundable deposits, accommodation bonds and entry contributions that must be included

(1) Further to section 166-360 of this instrument, this section prescribes the information to be included in an annual prudential compliance statement if the registered provider receives a refundable deposit, an accommodation bond or entry contribution from an individual.

(2) The information about a refundable deposit, an accommodation bond or entry contribution, a refundable deposit balance or an accommodation bond balance or entry contribution balance that must be included in a registered provider's annual prudential compliance statement for a reporting period is as follows:

(a) the total number of refundable deposit balances, or accommodation bond balances or entry contribution balances held by the registered provider as at the end of the reporting period;

(b) the total value of refundable deposit balances, or accommodation bond balances or entry contribution balances held by the registered provider as at the end of the reporting period;

(c) the total value of refundable deposits, accommodation bonds or entry contributions received by the registered provider during the reporting period;

(d) the total amount deducted by the registered provider during the reporting period from refundable deposit balances, accommodation bond balances or entry contribution balances, including:

(i) for refundable deposit balances or accommodation bond balances—any general deductions under section 307 of the Act or section 287-90 of this instrument; and

(ii) for refundable deposit balances only—any retention amounts deducted under section 308 of the Act by the registered provider during the reporting period from refundable deposit balances;

(iii) for accommodation bond balances only—any accommodation bond retention amounts deducted under section 287-95 of this instrument by the registered provider during the reporting period from accommodation bond balances;

(e) the total amount deducted by the registered provider during the reporting period from refundable deposits or accommodation bonds that were received during the year, including:

(i) for refundable deposit balances or accommodation bond balances—any general deductions under section 307 of the Act or section 287-90 of this instrument; and

(ii) for refundable deposit balances only—any retention amounts deducted under section 308 of the Act by the registered provider during the reporting period from refundable deposit balances received during the reporting period;

(iii) for accommodation bond balances only—any accommodation bond retention amounts deducted under section 287-95 of this instrument



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by the registered provider during the reporting period from accommodation bond balances;

(f) any transfer of a refundable deposit made under section 312 of the Act, a transfer of an accommodation bond made under section 287-103 of this instrument, or transfer of entry contribution balances, including the amount, the date of transfer and the amount left on retention;

(g) the total value of refundable deposit balances, accommodation bond balances and entry contribution balances refunded by the registered provider during the reporting period;

(h) if, during the reporting period, refundable deposit balances or accommodation bond balances or entry contribution balances were not refunded in accordance with subsection 311(3) of the Act, subsection 287-102(2) of this instrument or section 287-160 of this instrument (other than a refundable deposit balance, or accommodation bond balance or entry contribution balance in relation to which the registered provider has made an agreement as referred to in section 311-20(3) of this instrument)—the following information:

(i) the total number of refundable deposit balances, accommodation bond balances or entry contribution balances that were not refunded in accordance with subsection 311(3) of the Act, subsection 287-102(2) of this instrument or section 287-160 of this instrument;

(ii) where entry contribution balances were not refunded in accordance with an applicable formal agreement with an individual—the number of entry contributions that were refunded under the formal agreement applying in respect of the relevant entry contribution balance;

(iii) the reason or reasons for the delay in refunding the refundable deposit balances, accommodation bond balances or entry contribution balances;

(iv) in respect of each reason provided—the total number of instances of delay attributable to the reason;

(i) if, for the whole or a part of the reporting period, the registered provider was not permitted to charge a refundable deposit or accommodation bond for entry by an individual to any aged care service that the registered provider is responsible for operating:

(i) the period or periods during which the registered provider was not permitted to charge a refundable deposit or accommodation bond; and

(ii) the funded aged care service in respect of which each period specified applies;

(j) the use of refundable deposits and accommodation bonds by the registered provider during the reporting period;

(k) whether any use of refundable deposits, accommodation bonds by the registered provider during the reporting period was not permitted under section 310 of the Act or section 287-101 of this instrument;

(l) the total amount expended by the registered provider (whether or not obtained from refundable deposits or accommodation bonds) during the reporting period on capital expenditure for which use of a refundable



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deposit or an accommodation bond was permitted under section 310 of the Act or section 287-101 of this instrument;

(m) the total amount expended by the registered provider (whether or not obtained from refundable deposits or accommodation bonds) during the reporting period on investment in financial products for which use of a refundable deposit or an accommodation bond was permitted under section 310 of the Act or section 287-101 of this instrument;

(n) the total amount expended by the registered provider (whether or not obtained from refundable deposits or accommodation bonds) during the reporting period on loans for which use of a refundable deposit or an accommodation bond was permitted under section 310 of the Act or section 287-101 of this instrument;

(o) the total amount expended by the registered provider (whether or not obtained from refundable deposits or accommodation bonds) during the reporting period on repaying debt accrued for the purposes of:

(i) capital expenditure of the kind described in paragraph (l); or

(ii) refunding refundable deposit balances, accommodation bond balances or entry contribution balances;

(p) the total amount expended by the registered provider (whether or not obtained from refundable deposits or accommodation bonds) during the reporting period on repaying debt that accrued before 1 October 2011 if the debt was accrued for the purpose of delivering funded aged care services to individuals;

(q) the total amount expended by the registered provider (whether or not obtained from refundable deposits or accommodation bonds) during the reporting period on each of the uses of refundable deposits or accommodation bonds permitted under section 310 of the Act or section 287-101 of this instrument;

(r) the amount that has been returned to the registered provider during the reporting period from the sale, disposal or redemption of financial products covered by subsection 310(3) of the Act, or subsection 287-101(4) of this instrument, that the registered provider invested in after 1 October 2011, whether or not the investment was obtained from refundable deposits or accommodation bonds;

(s) for each accommodation agreement entered for a room priced above the maximum accommodation payment amount, whether the provider ensured they were permitted to charge the price, including having a current approval from the Pricing Authority at the time the individual agreed to the payment of the higher room price;

(t) where there is a decrease or reduction in the payable amount from each resident's daily accommodation contribution and refundable contribution held, information that the provider refunded the overpaid amount within 28 days of becoming aware of the reduced amount payable or paid by the individual;

(u) information regarding the overpayment of accommodation payments or contributions during prior reporting period that the provider has become aware of during the reporting period.



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Note: Paragraph (h) does not apply to a registered provider that has transferred their refundable deposit balance to another registered provider under section 312 of the Act.

## 166-375 Annual prudential compliance statement—information about other fees that must be included

The information about other fees that must be included in a registered provider's annual prudential compliance statement for a reporting period is as follows:

(a) the fees (if any) other than individual contributions, accommodation payments and accommodation contributions that the registered provider charged to individuals during the reporting period;

(b) the total value of each such fee charged by the registered provider during the reporting period;

(c) what each such fee purports to cover;

(d) for individuals accessing ongoing funded aged care services, whether the provider has conducted, in accordance with subsection 284-11(4) of this instrument, an annual review of each individual's higher everyday living agreement during the reporting period.

## 166-380 Annual prudential compliance statement—Financial and Prudential Standards

The statements and other information that must be included in a registered provider's annual prudential compliance statement for a reporting period are as follows:

(a) a statement about whether the registered provider has, during the reporting period, complied with the following:

(i) the Financial and Prudential Standards;

(ii) section 287-20 of this instrument and subsection 293(1), subsections 311(1) and (2) and section 313 of the Act;

(iii) Division 3 of Part 4 of Chapter 4 of the Act;

(iv) section 287-101 of this instrument subsection 310(1) of the Act;

(b) if the registered provider has not complied with any of the Standards in the Financial and Prudential Standards—a statement about why the registered provider has not complied with the Standard;

(ba) if the registered provider is of a kind prescribed by section 150A-3 of this instrument:

(i) a statement about whether the registered provider has, during the reporting period, complied with the condition under section 150A of the Act; and

(ii) if the provider has not complied with the condition—a statement about why the registered provider has not complied with the condition;

(c) if the registered provider has not complied with the conditions applicable to disclosure in section 155-30, 155-35, 166-365, 166-375, and 166-385 of this instrument—the following information:



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(i) the total number of occasions on which the registered provider did not comply;
(ii) the reason or reasons for the registered provider's failure to comply;
(iii) in respect of each reason provided—the total number of occasions of non-compliance attributable to the reason;

(d) the amount set out in the registered provider's liquidity management strategy, as at the end of the reporting period, as the registered provider's minimum liquidity amount for the end of the most recent quarter;

(e) the date on which the registered provider's liquidity management strategy was last reviewed and assessed;

(ea) the date on which the registered provider's investment management strategy was last reviewed and assessed;

(f) the date on which the registered provider's financial and prudential management system was last reviewed and assessed;

(g) an audit opinion, provided by the person who provides the independent audit referred to in section 166-385 of this instrument, on whether the registered provider has complied with the following in the reporting period:
(i) the Financial and Prudential Standards;
(ii) the condition under section 150A of the Act;
(iii) sections 155-30 and 155-35 of this instrument;
(iv) sections 166-360 to 166-385 of this instrument.

Note: The annual prudential compliance statement must be supported by an independent audit (see section 166-385 of this instrument).

### 166-385 Annual prudential compliance statement—audit requirements

(1) An annual prudential compliance statement must be supported by an independent audit provided by:
(a) a registered company auditor within the meaning of the *Corporations Act 2001*; or
(b) a person approved by the System Governor under subsection (2).

(2) The System Governor may approve a person to audit an annual prudential compliance statement if the System Governor is satisfied that the person has appropriate qualifications and experience.

(3) The System Governor may revoke an approval of a person under subsection (2) if the System Governor is satisfied that the person is no longer a fit and proper person to audit an annual prudential compliance statement.

## Subdivision F—CHSP

### 166-600 Application of Subdivision

This Subdivision applies to a registered provider of a kind who is registered in one or more of the following provider registration categories:
(a) home and community services;
(b) assistive technology and home modifications;



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(c) advisory and support services;
(d) personal and care support in the home or community;
(e) nursing and transition care;

and delivers funded aged care services under the CHSP.

## 166-605 Annual financial declaration statement

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give the System Governor a statement (the ***annual financial declaration statement***) each financial year for the registered provider.

(2) The annual financial declaration statement for a financial year for the registered provider must be given to the System Governor by 31 August of that year.

(3) The annual financial declaration statement must be in a report form approved by the System Governor.

(4) The annual financial declaration statement must include the following information for the financial year:
(a) certification that all financial assistance granted by the System Governor was spent for the purposes specified in the grant agreement;
(b) the amount of financial assistance the System Governor has granted the registered provider under the grant agreement, excluding GST;
(c) the registered provider's expenditure for the delivery of funded aged care services, excluding GST;
(d) any surplus and uncommitted funds in the financial year;
(e) any surplus and uncommitted funds from the previous financial year.

*Statement of compliance to be given with annual financial declaration statement*

(5) If a registered provider is required under subsection (1) to give the System Governor an annual financial declaration statement for the financial year, then the provider must, when giving the report to the System Governor, also give the System Governor a statement of compliance that the financial assistance granted under the grant agreement was only spent on individuals with an assessment approval for funded aged care services in one or more service groups for the individuals and the classification types for the service groups under section 65 of the Act.

(6) The annual financial declaration statement must be certified by the registered provider's governing body.

(7) For the purposes of subsection (1), the annual financial declaration statement must not include information about the registered provider's:
(a) own funds; or
(b) funds from another aged care program.



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## 166-610 Monthly performance report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give the System Governor a report about activity and performance data matters (the ***monthly performance report***) each month.

(2) The monthly performance report must be in an approved report form approved by the System Governor.

(3) Without limiting subsection (2), the monthly performance report must include:
  (a) client level data and service delivery information for all activities described in Item B of the grant agreement, including:
    (i) the output measure as per the service type; and
    (ii) the My Aged Care ID for each individual; and
  (b) any individual fees or contributions paid to the registered provider under Part 3 of Chapter 4 of the Act.

(4) The monthly performance report for a registered provider must be given to the System Governor within 14 days after the end of the month.

## 166-615 Annual wellness and reablement report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give the System Governor a report about the registered provider's progress in embedding wellness and reablement in its service delivery (the ***annual wellness and reablement report***).

(2) The System Governor may, at any time, request a registered provider to give the System Governor a report under subsection (1).

(3) A registered provider must comply with a request under subsection (1) within 28 days after the request is made.

(4) The report must be in a report form approved by the System Governor.

## 166-620 Compliance report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give the System Governor a report about any matters related to the registered provider's management of a grant agreement under section 264 of the Act (the ***compliance report***).

(2) The System Governor may, at any time, request a registered provider to give the System Governor a report under subsection (1).

(3) The compliance report for a registered provider must be given to the System Governor within 14 days after the request is made.

(4) The report must be in a report form approved by the System Governor.



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## 166-625 Service delivery report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must, on request by the System Governor, give a report about service delivery (the ***service delivery report)***.

(2) The System Governor may, at any time, request a registered provider to give the System Governor a report under subsection (1).

(3) A registered provider must comply with a request under subsection (1) within 21 days after the request is made, or such longer period as specified in the request.

(4) The report must be in a report form approved by the System Governor.

## 166-627 Exemption process for certain reports

(1) For the purposes of paragraph 166(4)(b) of the Act, this section prescribes the exemption process in relation to the submission of the following:
  (a) annual financial declaration statement;
  (b) monthly performance report;
  (c) compliance report;
  (d) annual wellness and reablement report;

(2) A registered provider may apply to the System Governor for an exemption from subsections 166-605(1), 166-610(1), 166-615(1) or 166-620(1), if the provider is affected by extenuating circumstances.

(3) The application:
  (a) may be in a form approved by the System Governor; and
  (b) must be made to the System Governor at least 14 days before the due date for the report listed in subsection (1).

(4) The System Governor must decide to grant or refuse an exemption within:
  (a) if the registered provider is given a notice under subsection (5) and gives the System Governor the further information or documents requested within the 14-day period, or such longer period specified in the notice—14 days after receiving the further information or documents; or
  (b) if the registered provider is given a notice under subsection (5) but does not give the System Governor the further information or documents requested within the 14-day period, or such longer period specified in the notice—14 days after the end of that period; or
  (c) otherwise—14 days after receiving the request.

(5) If the System Governor receives an application under subsection (2) from a registered provider, the System Governor may, by notice in writing given to the provider, request further information or documents specified in the notice for the purposes of considering the application.

(6) If the registered provider does not provide the requested information or documents within 14 days after the day when the notice is given, or within such



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longer period specified in the notice, the application is taken to have been withdrawn. The notice must contain a statement setting out the effect of this subsection.

(7) If the System Governor receives an application under subsection (2) from a registered provider for an exemption under subsections 166-605(1), 166-610(1), 166-615(1) or 166-620(1), the System Governor may grant the exemption only if the System Governor is satisfied that the provider is affected by extenuating circumstances.

(8) If the System Governor decides to grant an exemption, the System Governor must decide the period for which the exemption is to be in force.

(9) The period may:
  (a) be for the financial year; or
  (b) be a duration no longer than 12 months.

(10) If the System Governor grants an exemption from subsections 166-605(1), 166-610(1), 166-615(1) or 166-620(1), to a registered provider, the System Governor must give the provider notice in writing of the decision that:
  (a) states an exemption has been granted; and
  (b) states the period for which the exemption is in force; and
  (c) the reasons for the decision.

(11) If the System Governor refuses to grant an exemption from subsections 166-605(1), 166-610(1), 166-615(1) or 166-620(1), to a registered provider, the System Governor must give the provider notice in writing of the decision that:
  (a) states an exemption has not been granted; and
  (b) states the due date for the report; and
  (c) the reasons for the decision.

## 166-628 Child safety compliance statement

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report on matters provided for in subsection (3) (the ***child safety compliance statement***) to the System Governor each year.

(2) The child safety compliance statement for the registered provider must be given to the System Governor by 31 March each year.

(3) The child safety compliance statement must include the following information:
  (a) a declaration that the registered provider has delivered grant activities consistent with the Child Safety supplementary term in the registered provider's grant agreement;
  (b) the activity name;
  (c) the activity ID;
  (d) the statement type in relation to the incidental contract term in the grant agreement;



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(e) the reporting start date;
(f) the reporting end date.

(4) If the registered provider has not complied with the Child Safety supplementary term in the registered provider's grant agreement, the registered provider must provide a description of the non-compliance and set out the reasons for the non-compliance in the child safety compliance statement.

(5) The child safety compliance statement must be in a report form approved by the System Governor.

## Subdivision G—NATSIFACP

### 166-630 Application of Subdivision

This Subdivision applies to a registered provider of a kind who is registered in one or more of the following provider registration categories:
(a) home and community services;
(b) assistive technology and home modifications;
(c) personal and care support in the home or community;
(d) nursing and transition care;
(e) residential care;
(f) advisory and support services;
and delivers funded aged care services under the NATSIFACP.

### 166-635 Annual financial declaration statement

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give the System Governor a statement (the ***annual financial declaration statement***) each financial year for the registered provider.

(2) The annual financial declaration statement for a financial year for the registered provider must be given to the System Governor:
(a) annually; or
(b) at such other time as agreed between the System Governor and the registered provider.

(3) The annual financial declaration statement must be in a report form approved by the System Governor.

(4) The annual financial declaration statement must include the following information for the financial year:
(a) certification that the provider, in relation to the use of financial assistance granted by the System Governor under subsection 264(2) of the Act, complied with the statutory funding condition under subsection 267(1) of that Act;
(b) the amount of financial assistance the System Governor, on behalf of the Commonwealth, granted the registered provider;



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(c) the amount of expenditure for the delivery of funded aged care services by the provider;
(d) the amount, if any, of unspent grant funding in the financial year;
(e) any unspent funds approved by the System Governor from previous financial year.

Note: It is a statutory funding condition that the financial assistance granted to a registered provider is used only for the purpose of delivering funded aged care services to individuals in the circumstances mentioned in subsection 267(1) of the Act.

(5) The annual financial declaration statement must be signed by a member of the registered provider's governing body.

## 166-640 Audited income and expenditure report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report about financial matters (the ***audited income and expenditure report***) to the System Governor each financial year for the registered provider.

(2) The audited income and expenditure report must be prepared by:
(a) a registered company auditor within the meaning of the *Corporations Act 2001*; or
(b) a member of:
(i) the Institute of Public Accountants; or
(ii) the Institute of Chartered Accountants in Australia.

*Audited income and expenditure statement to be given with audited income and expenditure report*

(3) If a registered provider is required under this section to give the System Governor an audited income and expenditure report for the financial year, then the provider must, when giving the report to the System Governor, also give the System Governor an audited income and expenditure statement.

(4) The audited income and expenditure statement must:
(a) be in accordance with the Australian accounting standards in force at the time the report is prepared; and
(b) be based on proper accounts and records of the registered provider; and
(c) verify that the provider, in relation to the use of financial assistance granted by the System Governor under subsection 264(2) of the Act, complied with the statutory funding condition under subsection 267(1) of that Act; and
(d) include other matters as specified in the funding agreement; and
(e) include the audit opinion; and
(f) include the amount of financial assistance the System Governor, on behalf of the Commonwealth, has granted the registered provider under subsection 264(2) of the Act for the financial year; and
(g) include the amount of individual fees and contributions paid to the provider by individuals for the financial year; and



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(h) include salary expenditure; and
(i) include rent or lease expenditure; and
(j) include outbreak management expenditure; and
(k) include vehicle lease and maintenance cost; and
(l) include other expenditure; and
(m) include total expenditure.

## 166-645 Service activity report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report about the registered provider's progress in providing funded aged care services in accordance with the grant agreement (the ***service activity report***) to the System Governor every 6 months.

(2) The service activity report must be given to the System Governor:
(a) on 27 January for the 6 month period ending on 31 December; and
(b) on 27 July for the 6 month period ending on 30 June.

(3) The service activity report must:
(a) be in a report form approved by the System Governor; and
(b) include information about labour worked hours data for registered nurses, enrolled nurses, personal care workers and allied health professionals for the reporting period; and
(c) include information about labour cost for registered nurses, enrolled nurses, personal care workers and allied health professionals for the reporting period; and
(d) include information about the number of occupied beds for the reporting period; and
(e) include de-identified profiles of each aged care worker of the provider; and
(f) include the number of individuals accessing funded aged care services by service type; and
(g) include the number and type of health profession visits during the reporting period; and
(h) include the number and type of traditional or cultural events individuals accessing funded aged care services were supported to engage in during the reporting period.

## 166-646 Child safety compliance statement

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report on matters provided for in subsection (3) (***child safety compliance statement***) to the System Governor each year.

(2) The child safety compliance statement for the registered provider must be given to the System Governor by 31 March each year.



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(3) The child safety compliance statement must include the following information:
  (a) a declaration that the registered provider has delivered grant activities consistent with the Child Safety supplementary term in the registered provider's grant agreement;
  (b) the activity name;
  (c) the activity ID;
  (d) the statement type in relation to the incidental contract term in the grant agreement;
  (e) the reporting start date;
  (f) the reporting end date.

(4) If the registered provider has not complied with the Child Safety supplementary term in the registered provider's grant agreement, the registered provider must provide a description of the non-compliance and set out the reasons for the non-compliance in the child safety compliance statement.

(5) The child safety compliance statement must be in a report form approved by the System Governor.

## Subdivision H—Multi-Purpose Service Program

### 166-720 Application of Subdivision to certain registered providers

This Subdivision applies to a registered provider delivering funded aged care services under the MPSP.

### 166-725 Annual activity report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report on matters provided for in subsection (4) (the ***annual activity report***) to the System Governor each financial year.

(2) The annual activity report for a financial year for the provider must be given to the System Governor:
  (a) by 31 July each year; or
  (b) at such other time as agreed between the System Governor and the registered provider.

(3) The annual activity report must be in a report form approved by the System Governor.

(4) The information that must be included in the annual activity report for the financial year is as follows:
  (a) the number of individuals who have accessed funded aged care services in, or from, the approved residential care home;
  (b) the number of individuals who commenced accessing funded aged care services without an access approval and were later approved through the alternative entry pathway under subsection 71(2) of the Act;



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(c) the number of individuals waiting to access such funded aged care services;
(d) any fees or contributions charged to individuals who accessed funded aged care services delivered in, or from, an approved residential care home;
(e) the service types delivered;
(f) the activities undertaken by the residential care home to prevent and manage disease outbreaks.

(5) A registered provider must provide the information prescribed in subsection (4) in respect of each residential care home operated by the registered provider.

## 166-730 Annual statement of financial compliance and income and expenditure

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report on matters provided for in subsection (4) (the ***annual statement of financial compliance and income and expenditure***) to the System Governor each financial year.

(2) The annual statement of financial compliance and income and expenditure must be given to the System Governor:
(a) by 31 October each year; or
(b) at such other time as agreed between the System Governor and the registered provider.

(3) The annual statement of financial compliance and income and expenditure must be in a report form approved by the System Governor.

(4) The information that must be included in the annual statement of financial compliance and income and expenditure for the financial year is as follows:
(a) amount of subsidy received under Division 5 of Part 2 of Chapter 4 of the Act to deliver funded aged care services at each approved residential care home;
(b) individual fees or contributions paid to the provider under Part 3 of Chapter 4 of the Act by individuals accessing funded aged care services at each approved residential care home;
(c) the amount of expenditure on each of the following:
(i) salaries or wages of aged care workers and responsible persons;
(ii) any labour costs in addition to salaries in wages such as superannuation benefits, leave loadings, payroll tax, workers compensation and other liability insurance, cost of subsidised services to employees and training costs;
(iii) non-salary related other expenditure;
(iv) capital expenditure;
(v) disease outbreak management activities.

## 166-735 Service demographics report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a registered provider to whom this Subdivision applies must give a report on



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matters provided for in subsections (4) and (5) (***service demographics report***) to the System Governor each financial year.

(2) The service demographics report must be given to the System Governor:
  (a) by 31 July each year; or
  (b) at such other time as agreed between the System Governor and the registered provider.

(3) The service demographics report must be in a report form approved by the System Governor.

(4) The service demographics report must include a list of individuals who accessed funded aged care services delivered by the registered provider for the financial year.

(5) A report prepared for the purposes of subsection (1) of this section must also include the following information:
  (a) for each individual who accessed funded aged care service delivered by the provider, the service types, including the services where specified in the individual's access approval, delivered;
  (b) the classification type for the service group for each individual;
  (c) demographic information where requested in the report form approved by the System Governor, including:
    (i) the name of the individual; and
    (ii) the gender of the individual; and
    (iii) the date of birth of the individual; and
    (iv) whether the individual is an Aboriginal or Torres Strait Islander Person; and
    (v) whether the individual has been diagnosed with dementia or has suspected dementia symptoms;
  (d) whether the individual had an access approval when they commenced accessing services;
  (e) the date on which the individual commenced accessing funded aged care services;
  (f) the date on which the individual ceased accessing funded aged care services;
  (g) the reason for ceasing any funded aged care services.

(6) A registered provider must give the System Governor a service demographics report for each approved residential care home operated by the registered provider.

## Subdivision J—Transition Care Program

### 166-740 Application of Subdivision to certain registered providers

This Subdivision applies to a registered provider delivering funded aged care services under the TCP.



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Section 166-745

## 166-745 Annual accountability report

(1) For the purposes of paragraph 166(1)(d) of the Act, this section prescribes that a provider to whom this Subdivision applies must give a report on matters provided for in subsection (4) (***annual accountability report***) to the System Governor each financial year.

(2) The annual accountability report for a financial year for the provider must be given to the System Governor:
  (a) annually; or
  (b) at such other time as agreed between the System Governor and the registered provider.

(3) The annual accountability report must be in a report form approved by the System Governor.

(4) The annual accountability report for the financial year must include the following information for the provider:
  (a) any subsidy received under section 249 of the Act;
  (b) any income derived from any individual fees and contributions under Part 3 or Chapter 4 of the Act;
  (c) State or Territory direct funding;
  (d) State or Territory in-kind contributions;
  (e) other income including;
    (i) donations; and
    (ii) interest; and
    (iii) expenditure recoveries and reimbursements; and
    (iv) client fees; and
    (v) any other income not specified in subparagraph (i) and (ii) of this paragraph or paragraph (a) to (d) of this subsection;
  (f) total income;
  (g) salary expenditure;
  (h) non-salary expenditure including:
    (i) office costs; and
    (ii) operating costs; and
    (iii) service agreements; and
    (iv) consumables and equipment for client use; and
    (v) travel; and
    (vi) capital costs;
  (i) other expenditure including:
    (i) asset related expenses including depreciation; and
    (ii) any other expenditure not specified in subparagraph (i) of this paragraph or paragraph (g) and (h) of this subsection;
  (j) total expenditure;
  (k) TCP activity for the financial year including;
    (i) total clients; and



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(ii) total care days; and
(iii) total residential care days; and
(iv) total home-based care days;

(l) surplus analysis and itemisation of surplus by income source including;
(i) Commonwealth government; and
(ii) State government; and
(iii) Territory government; and
(iv) client contributions; and
(v) any income specified in paragraph (e) of this subsection;

(m) the Commonwealth surplus expressed as a number of care days;

(n) any explanatory notes or commentary on the following;
(i) income;
(ii) expenditure;
(iii) activity for the financial year;
(iv) surplus analysis;
(v) additional activity;

(o) feedback, complaints and improvements including;
(i) the most common kinds of positive feedback; and
(ii) the most common kinds of complaints; and
(iii) improvements made by the provider;

(p) information in respect of whether the Fair Work Commission wage increase component of the Commonwealth subsidy has been passed on to any eligible workers;

(q) certification that the information provided in the report is true and correct.

*Reporting period*

(5) The reporting period for the provider is the period specified for a service activity report under section 166-645.

## Subdivision K—Governing bodies

### 166-800 Application of this Subdivision

(1) Subject to subsection (2), this Subdivision applies to registered providers registered in one or more of the following provider registration categories:
(a) nursing and transition care;
(b) residential care.

(2) This Subdivision does not apply to the delivery of funded aged care services provided under any of the following specialist aged care programs:
(a) MPSP;
(b) NATSIFACP.



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Section 166-805

## 166-805 Governing bodies must prepare and provide statements

(1) Subject to subsection (2), for the purposes of paragraph 166(1)(d) of the Act, it is prescribed that a registered provider to whom this Subdivision applies must give a report of a kind referred to in this section to the System Governor within 4 months after the end of the reporting period.

(2) This section does not apply to registered providers that deliver funded aged care services under the MPSP.

(3) The reporting period for a registered provider to whom this Subdivision applies is:
  (a) the period of 12 months starting on 1 July of a year; or
  (b) another 12 month period that starts on the first day of a month of a year that is determined for the registered provider by the System Governor in accordance with the rules.

(4) The report must:
  (a) be in the approved form; and
  (b) be signed by a member of the provider's governing body on behalf of all members of the governing body; and
  (c) include the information prescribed by subsection (5) of this section; and
  (d) include any other statements or information required by the approved form.

(5) The following information is prescribed:
  (a) whether the governing body of the provider believes that the provider has complied with the conditions, obligations and requirements of the provider under the Act;
  (b) if the governing body of the provider believes that the provider has failed to comply with one or more conditions, obligations or requirements of the provider under the Act—the details of:
    (i) each condition, obligation or requirement that the governing body believes that the provider has failed to comply with; and
    (ii) the reasons why the provider has failed to comply with the condition, obligation or requirement; and
    (iii) the actions that the provider has taken, has started to take or will take to rectify the non-compliance.

Example: For subparagraph (b)(iii), under Chapter 6 of the Act a registered provider may:
  (a) give an enforceable undertaking about remedying non-compliance (see Part 8 of Chapter 6 of the Act); or
  (b) agree to certain matters if revocation of registration is being considered (see section 133 of the Act).

## 166-810 Requirements for certain registered providers to give information relating to reporting periods

(1) For the purposes of paragraph 166(1)(d) of the Act, it is prescribed that a registered provider to whom this Subdivision applies must give the following additional information:



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(a) information about the kind of feedback and complaints received by the registered provider in the reporting period;
(b) information about improvements made by the registered provider in the reporting period in relation to quality of care;
(c) information about initiatives that the registered provider has implemented in the reporting period to support a diverse and inclusive environment for individuals accessing funded aged care services and aged care workers;
(d) information about the representation of different demographic groups in the membership of the governing body of the provider (but, for any group, only if a member of the governing body who is a member of that group consents to that information being provided);
(e) information on whether the registered provider was, in the reporting period, a government entity or a local government authority;
(f) for registered providers that are not government entities or local government authorities, whether the registered provider is a registered provider of a kind prescribed by the rules for the purposes of subsection 157(2) of the Act;
(g) for registered providers that are subject to the requirements of subsection 157(2) of the Act, whether the registered provider has complied during the reporting period with the requirements of subsection 157(2);
(h) whether subsection 157(3) or (4) of the Act applied to the registered provider during the reporting period.

(2) A registered provider registered in the registration category nursing and transition care must provide the information prescribed in subsections (1)(a) to (c) in respect of each service delivery branch operated by the registered provider.

(3) A registered provider registered in the registration category residential care must provide the information prescribed in subsections (1)(a) to (c) in respect of each residential care home operated by the registered provider.

### 166-815 Service provided during part only of reporting period

If a registered provider registered in the residential care category was responsible for the operations of a residential care home during part of a reporting period for the registered provider, the registered provider is taken to have complied with sections 166-805 and 166-810 in relation to the home for the reporting period if the registered provider complied with 166-805 and 166-810 in relation to the home and that part of the reporting period.

## Subdivision L—Registered nurses

### 166-850 Application of this Subdivision

(1) Subject to subsection (2), this Subdivision applies to a registered provider registered in the provider registration category residential care.

(2) The requirements of this Subdivision do not apply to a registered provider only delivering funded aged care services under a specialist aged care program.



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Section 166-855

### 166-855 Reporting requirements relating to registered nurses

(1) For the purposes of paragraph 166(1)(d) of the Act, a registered provider must give the System Governor a report about the obligation in subsection 175(1) of the Act in relation to each approved residential care home of the provider within 7 days after the end of each calendar month, or within a longer period specified by the System Governor by notice in writing given to the provider.

(2) A registered provider is not required to provide a report under subsection (1) of this section in relation to an approved residential care home for a calendar month if, during that calendar month, the registered provider did not deliver funded aged care services to at least one individual in the home.

(3) To avoid doubt, it is a requirement that a registered provider provide a report under subsection (1) of this section in relation to an approved residential care home regardless of whether the registered provider has been granted an exemption under paragraph 175(2)(a) of the Act from the obligation in subsection 175(1) of the Act in relation to that home.

(4) The report must:
  (a) be in a form approved, in writing, by the System Governor; and
  (b) specify in relation to each approved residential care home, whether a registered nurse was on site and on duty at all times for each day during the calendar month on which at least one individual received funded aged care services in the home; and
  (c) if a registered nurse was not on site and on duty at all times on any such day—specify the following information:
    (i) each period of 30 minutes or more that a registered nurse was not on site and on duty at the home;
    (ii) for each such period, the reasons why a registered nurse was not on site or on duty (or both) during that period;
    (iii) for each such period, the alternative arrangements that were made for the period to ensure the clinical needs of the individuals in the home were met, or a statement that no alternative arrangements were made.

## Subdivision M—Status of service delivery branches

### 166-900 Application of this Subdivision

This Subdivision applies to a registered provider registered in one or more of the following categories:
  (a) home and community services;
  (b) assistive technology and home modifications;
  (c) advisory and support services;
  (d) personal and care support in the home or community;
  (e) nursing and transition care.



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## 166-905 Reporting requirements relating to service delivery branches

For the purposes of paragraph 166(1)(d) of the Act, this Subdivision prescribes:

(a) the reports a registered provider must give to the System Governor in relation to a service delivery branch of the provider; and

(b) the requirements relating to those reports.

## 166-910 Report for opening of a service delivery branch

(1) A registered provider must give a report, in an approved form, to the System Governor in accordance with this section if:

(a) the provider intends to begin delivering funded aged care services through a service delivery branch; and

(b) either:

(i) the provider has not previously delivered any funded aged care services through this service delivery branch; or

(ii) the provider has previously given a report to the System Governor in accordance with section 166-925 in relation to closure of the service delivery branch.

Note: A separate notification must be given to the System Governor to establish a service delivery branch account for the service delivery branch: see subsection 203(1) of the Act.

*Information to be included in the report*

(2) The report must include the following information:

(a) the name of the service delivery branch;

(b) the date the provider proposes to begin delivering funded aged care services through the service delivery branch;

(c) the address of the service delivery branch;

(d) the contact details for the service delivery branch.

*When report must be given*

(3) The report must be given no later than the day the provider begins delivering funded aged care services through the service delivery branch.

## 166-915 Report for change to a service delivery branch

(1) A registered provider must give a report, in an approved form, to the System Governor in accordance with this section if:

(a) the provider has given a report to the System Governor in accordance with section 166-910 for a service delivery branch of the provider; and

(b) there has been a change to any information relating to the service delivery branch that was included in the report given to the System Governor in accordance with section 166-910.



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*Information to be included in the report*

(2) The report must include the following:
  (a) the name of the service delivery branch;
  (b) any updated information relating to the service delivery branch that was included in the report given under section 166-910.

*When report must be given*

(3) The report must be given within 28 days after the day the change in paragraph (1)(b) has occurred.

## 166-920 Report for merger of service delivery branches

(1) A registered provider must give a report, in an approved form, to the System Governor in accordance with this section if:
  (a) the provider has given a report to the System Governor in accordance with section 166-910 for 2 or more service delivery branches of the provider; and
  (b) the provider intends to merge 2 or more of these service delivery branches into a single service delivery branch of the provider.

*Information to be included in the report*

(2) The report must include the following information:
  (a) the names of the service delivery branches that the provider proposes to merge, specifying;
    (i) the name of the service delivery branch through which the provider proposes to continue delivering funded aged care services; and
    (ii) the name of each service delivery branch the provider intends to close;
  (b) the date the provider proposes to merge the service delivery branches;
  (c) in relation to the individuals (if any) accessing funded aged care services through the service delivery branches that the provider proposes to merge:
    (i) whether the provider has notified the individuals of the proposed merger; and
    (ii) whether the provider has given a cessation notification to the System Governor and the Commissioner for each individual accessing funded aged care services through each service delivery branch mentioned in subparagraph (a)(ii).

*When report must be given*

(3) The report must be given at least 90 days, or such smaller number of days as is agreed in writing between the System Governor and the registered provider, before the date of the proposed merger.



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## 166-925 Report for closure of a service delivery branch

(1) A registered provider must give a report, in an approved form, to the System Governor in accordance with this section if:
  (a) the provider has given a report to the System Governor in accordance with section 166-910 for a service delivery branch of the provider; and
  (b) the provider intends to close the service delivery branch.

(2) Despite subsection (1), a registered provider does not have to give a report in accordance with this section in relation to a service delivery branch if:
  (a) the provider intends to merge the service delivery branch with one or more other service delivery branches of the provider; and
  (b) a report under section 166-920 has been given in relation to the service delivery branch.

*Information to be included in the report*

(3) The report must include the following information:
  (a) the name of the service delivery branch;
  (b) the date the provider proposes to close the service delivery branch;
  (c) in relation to the individuals (if any) accessing funded aged care services through the service delivery branch:
    (i) whether the provider has notified the individuals of the proposed closure; and
    (ii) whether the provider has given a cessation notification to the System Governor and the Commissioner for each individual.

*When report must be given*

(4) The report must be given at least 28 days before the date of the proposed closure.

## 166-930 Report if provider no longer intends to open, merge or close service delivery branch

(1) A registered provider must give a report, in an approved form, to the System Governor in accordance with this section if:
  (a) the provider has given a report (the ***original report***) to the System Governor in accordance with sections 166-910, 166-920 or 166-925; and
  (b) the provider no longer intends to proceed with the opening, merger or closure described in the original report.

*Information to be included in the report*

(2) The report must include the following:
  (a) a description of the original report, including:
    (i) the name of each service delivery branch included in the original report; and
    (ii) whether the original report was in relation to an opening, merger or closure; and



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(iii) the proposed date of the opening, merger or closure that was included in the original report;

(b) a statement that the provider no longer intends to proceed with the opening, merger or closure described in the original report;

(c) the reason why the provider no longer intends to proceed with the opening, merger or closure described in the original report.

*When report must be given*

(3) The report must be given no later than:

(a) for an opening or closure—the proposed date of the opening or closure; and

(b) for a merger—28 days before the proposed date of the merger.

## Subdivision N—Pricing information

### 166-1000 Application of this Subdivision

(1) Subject to subsection (2), this Subdivision applies to a registered provider registered in one or more of the following provider registration categories:

(a) home and community services;

(b) assistive technology and home modifications;

(c) advisory and support services;

(d) personal and care support in the home or community;

(e) nursing and transition care.

(2) The requirements of this Subdivision do not apply in respect of the delivery of funded aged care services delivered under any specialist aged care program.

### 166-1005 Pricing information

(1) For the purposes of paragraph 166(1)(d) of the Act, a registered provider must:

(a) give a report about the information specified in subsection (3) to the System Governor for each reporting period; and

(b) do so within 30 days of the end of the relevant reporting period; and

(c) do so in the approved form.

*Reporting period*

(2) For the purposes of this section, a ***reporting period*** is a period of 2 months commencing on 1 January, 1 March, 1 May, 1 July, 1 September or 1 November of a year.

*Information to be included*

(3) The information that must be included in a report given in accordance with subsection (1) is the most frequently charged price during standard business hours for the relevant reporting period that the provider charges individuals for each service in the following service types:



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(a) allied health and therapy;
(b) care management;
(c) domestic assistance;
(d) home maintenance and repairs;
(e) home or community general respite;
(f) meals;
(g) nursing care;
(h) personal care;
(i) restorative care management;
(j) social support and community engagement;
(k) therapeutic service for independent living;
(l) transport.

(4) A registered provider is not required to report on a price for a service in accordance with subsection (1) if:
(a) the registered provider is not currently delivering the service and has not delivered the service in the last 12 months; or
(b) the service is delivered under a specialist aged care program.



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# Part 3—Provider obligation—notifying of change in circumstances

## Division 1—Obligation to notify

### 167-5 Purpose of this Part

This Part is made for the purposes of section 167 of the Act and prescribes:

(a) changes of a kind in relation to which prescribed kinds of registered providers must give notice to the Commissioner under that section; and

(b) circumstances in relation to which a notice must also be given to the System Governor under that section; and

(c) information that must be included in a notice given to the Commissioner or the System Governor under that section.

### 167-10 Notifying the Commissioner—kinds of registered providers and changes

The following table prescribes the kinds of registered providers and kinds of changes that those providers must give notice to the Commissioner under subsection 167(1) of the Act.

**Kinds of registered providers and changes**

| Item | Column 1<br>Kind of registered providers | Column 2<br>Kinds of changes |
|---|---|---|
| 1 | every kind of registered provider | a change referred to in section 167-20 relating to the provider's suitability to be a registered provider |
| 2 | every kind of registered provider | a change referred to in section 167-25 relating to the suitability of a responsible person of the provider |
| 3 | every kind of registered provider | a change referred to in section 167-30 of responsible persons of the provider |
| 4 | every kind of registered provider | a significant change referred to in section 167-35 relating to the organisation arrangements of the provider |
| 5 | a registered provider to whom section 157 of the Act applies | a change referred to in section 167-40 relating to the governance arrangements of the provider |
| 6 | every kind of registered provider | a change referred to in section 167-45 relating to the scale of operations of the provider |
| 7 | a registered provider registered in any of the following provider registration categories:<br>(a) home and community services; | a change referred to in section 167-50 relating to intended service types |



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**Kinds of registered providers and changes**

| Item | Column 1<br>Kind of registered providers | Column 2<br>Kinds of changes |
|---|---|---|
| | (b) assistive technology and home modifications;<br>(c) advisory and support services;<br>(d) personal and care support in the home or community;<br>(e) nursing and transition care;<br>(f) subscription trial. | |
| 8 | a registered provider registered in any of the following provider registration categories:<br>(a) personal and care support in the home or community;<br>(b) nursing and transition care;<br>(c) residential care | a change referred to in section 167-55 relating to associated providers |
| 8A | a registered provider registered in the provider registration category subscription trial that delivers funded aged care services in any of the following service types:<br>(a) subscription allied health and therapy;<br>(b) subscription home or community general respite;<br>(c) subscription personal care;<br>(d) subscription therapeutic services for independent living;<br>(e) subscription nursing care | a change referred to in section 167-55 relating to associated providers |
| 9 | every kind of registered provider, except government entities | a change referred to in section 167-60 relating to financial and prudential matters, except a change prescribed in item 10 of this table |
| 10 | a registered provider that:<br>(a) is registered in the provider registration category residential care; and<br>(b) is not a government entity or a local government authority; and<br>(c) is not delivering aged care services through the specialised aged care program NATSIFACP | a change referred to in section 167-65 relating to liquidity |
| 11 | a registered provider registered in the provider registration category residential care | a change referred to in section 167-70 relating to approved residential care homes |

## 167-15 Notifying the System Governor—circumstances

For the purposes of subsection 167(2) of the Act, the circumstances in which a notice must also be given to the System Governor under that subsection are the



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circumstances in which a notice must be given to the Commissioner under subsection 167(1) of the Act relating to the following:

(a) the circumstances prescribed by item 4 of the table in section 167-10, to the extent that the circumstances relate to entering into administration;

(b) the circumstances prescribed by item 9 of the table in section 167-10 (relating to financial and prudential matters);

(c) the circumstances prescribed by item 10 of the table in section 167-10 (relating to liquidity);

(d) the circumstances prescribed by item 11 of the table in section 167-10 (relating to approved residential care homes).



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## Division 2—Changes in circumstances

### 167-20 Suitability of a registered provider

(1) The change prescribed in item 1 of the table in section 167-10 is any change in circumstances for a registered provider that materially affects, or may materially affect, the provider's suitability to be a registered provider, taking into account the matters referred to in paragraph 109(1)(b) of the Act (which deals with suitability of registered providers).

Note: See section 167-10, which relates to notifying the Commissioner.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) the date on which a responsible person for the provider first became aware of the change of circumstances that materially affects the provider's suitability to be a registered provider; and

(b) how the provider became aware of the change of circumstances; and

(c) details about the change of circumstances.

### 167-25 Suitability of a responsible person of a registered provider

(1) The change prescribed in item 2 of the table in section 167-10 is any change in circumstances relating to a responsible person of a registered provider that materially affects, or could materially affect, the responsible person's suitability to be a responsible person of the provider, having regard to the matters referred to in subsection 13(1) of the Act (which deals with suitability matters in relation to an individual).

Note: See section 167-10, which relates to notifying the Commissioner.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) the name of the responsible person; and

(b) the date the provider first became aware of the change in circumstances; and

(c) how the provider became aware of the change in circumstances; and

(d) details of the change of circumstances; and

(e) whether, after considering those matters, the provider is reasonably satisfied that the responsible person continues to be suitable to be a responsible person of the provider; and

(f) what, if any, action the provider has taken, or proposes to take, in relation to the responsible person.

### 167-30 Change of responsible persons of a registered provider

(1) The change prescribed in item 3 of the table in section 167-10 is any change of responsible persons of a registered provider, including:

(a) an individual becoming a responsible person of the provider; and



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(b) an individual ceasing to be a responsible person of the provider.

Note: See section 167-10, which relates to notifying the Commissioner.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) for a change relating to an individual becoming a responsible person of the provider:

(i) the name and contact details of the individual; and

(ii) a description of the individual's responsibilities in their capacity as a responsible person of the provider; and

(iii) a statement to the effect that the provider has had regard to the suitability matters in relation to the individual, as referred to in subsection 13(1) of the Act, and that the provider is satisfied that the individual is suitable to be involved in the delivery of funded aged care services; and

(b) for a change relating to an individual ceasing to be a responsible person of the provider:

(i) the name of the individual ceasing to be a responsible person; and

(ii) the reason that the individual has ceased to be a responsible person of the provider; and

(iii) the name and contact details of another individual (if any) who is carrying out, or will carry out, the responsibilities of the individual ceasing to be a responsible person.

## 167-35 Organisation arrangements of a registered provider

(1) The change prescribed in item 4 of the table in section 167-10 is any change of the following kind to the organisation of a registered provider:

(a) if the provider is not a government entity—any significant change to the provider's legal and business structure, including any of the following:

(i) a restructure of the organisation of the provider;

(ii) a sale, acquisition, or merger relating to the provider;

(b) for any registered provider (including a government entity)—the entering into of an agreement by the provider with another entity in which the other entity agrees to deliver services that support the management or governance function of the provider;

(c) for any registered provider (including a government entity)—any significant change to an agreement (including the cessation of a such an agreement) by the provider with another entity in which the other entity agrees to deliver services that support the management or governance of the provider;

(d) if the provider is not a government entity—any of the following events:

(i) the provider enters into administration;

(ii) the provider appoints a restructuring practitioner (within the meaning of the *Corporations Act 2001*);



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(iii) an insolvency event (within the meaning of the *Aged Care (Accommodation Payment Security) Act 2006*) occurs in relation to the provider.

Note 1: See section 167-10, which relates to notifying the Commissioner.

Note 2: See section 167-15, which relates to notifying the System Governor of matters relating to entering administration.

(2) To avoid doubt, the circumstances referred to in paragraph (1)(b) relating to the provider entering into an agreement do not include the entering into of an agreement for the delivery of funded aged care services.

(3) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) for a change to the provider's legal or business structure—details about the change to the provider's legal or business structure; and

(b) for an event referred to in paragraph (1)(b) or (c) (which relate to agreements):

(i) a detailed statement about the services that the other entity is to provide to the registered provider; and

(ii) the nature and duration of the agreement; and

(iii) if the notification is about the cessation of the agreement—the reason and date of cessation; and

(c) for an event set out in paragraph (1)(d) of this section (which relates to administration, restructuring and insolvency):

(i) the date on which the event occurred; and

(ii) the type of administration (where applicable); and

(iii) the name of the administrator or restructuring practitioner (where applicable).

## 167-40 Governance arrangements of a registered provider

(1) The change prescribed in item 5 of the table in section 167-10 is any change to the governance of a registered provider that would result in the registered provider no longer complying with subsection 157(2) of the Act.

Note 1: See sections 157-5 and 157-10, which prescribe the kinds of registered providers to whom section 157 of the Act applies.

Note 2: See section 167-10, which relates to notifying the Commissioner.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) a statement that the provider no longer complies with a condition of registration set out in paragraph 157(2)(a) or (b) of the Act (whichever is relevant to the provider); and

(b) the names of incoming and outgoing members in relation to the non-compliance; and

(c) information on steps being taken to return to compliance, including whether an application under subsection 159(1) of the Act (which relates to



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determinations that certain conditions relating to the governing body of a registered provider do not apply) has been submitted, or will be submitted.

## 167-45 Change relating to the scale of operations of a provider

(1) The change prescribed in item 6 of the table in section 167-10 is any of the following changes:

(a) for a registered provider other than a registered provider registered in the provider registration category residential care—a change to the local government area in which the provider delivers a funded aged care service;

(b) for a registered provider (including a registered provider registered in the provider registration category residential care)—a significant increase or decrease in the number of aged care workers of the provider that materially affects the provider's ability to deliver funded aged care services.

Note: See section 167-10, which relates to notifying the Commissioner, and section 167-15, which relates to notifying the System Governor.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) details of the change; and

(b) the aged care funded services that will be, or are likely to be, affected by the change; and

(c) any other anticipated effects of the change; and

(d) the reason for the change; and

(e) any actions that the provider has taken or will take to manage the effect on services, or other anticipated effects.

## 167-50 Changes relating to intended service types

(1) The change prescribed in item 7 of the table in section 167-10 is any change relating to a registered provider's intended service types.

Note 1: See section 167-10, which relates to notifying the Commissioner, and section 167-15, which relates to notifying the System Governor.

Note 2: See Division 4 of Part 4 of Chapter 4 of this instrument for provisions relating to starting and ceasing the provision of funded aged care services to a particular individual and continuity of those services.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) the relevant service type; and

(b) the date on which the provider expects the change to take effect.

## 167-55 Changes relating to arrangements with associated providers

(1) Subject to subsections (2) and (2A), for the purposes of items 8 and 8A of the table in section 167-10, the changes are the following:



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(a) the commencement of a new arrangement between the registered provider and an associated provider for the delivery of funded aged care services by the associated provider on behalf of the registered provider;
(b) the variation or extension of an existing such arrangement between the registered provider and an associated provider;
(c) the cessation of an existing such arrangement between the registered provider and an associated provider.

Note: See section 167-10, which relates to notifying the Commissioner.

(2) Subsection (1) does not apply to circumstances where the registered provider has entered into an arrangement with an associated provider for:
(a) the supply of labour under a labour hire arrangement; or
(b) the delivery of services that support the management or governance function of the registered provider.

(2A) Subsection (1) applies only to arrangements for the delivery of the following services:
(a) services in service types that can be delivered under any of the following provider registration categories:
(i) residential care;
(ii) personal and care support in the home or community;
(iii) nursing and transition care;
(b) services in the following service types:
(i) subscription allied health and therapy;
(ii) subscription home or community general respite;
(iii) subscription personal care;
(iv) subscription therapeutic services for independent living;
(v) subscription nursing care.

(3) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:
(a) in all circumstances—information about the associated provider, including:
(i) the associated provider's business name; and
(ii) the associated provider's ABN; and
(iii) the associated provider's ACN (if any); and
(iv) the associated provider's business location; and
(b) in circumstances relating to the delivery of funded aged care services in a residential care home—information about the residential care home, including:
(i) the name of the residential care home; and
(ii) the address of the residential care home; and
(c) in circumstances relating to the commencement of a new arrangement:
(i) details of the services to be delivered under the new arrangement; and
(ii) the reasons for the new arrangement; and
(iii) the time period for the new arrangement (including the commencement date and expiry date of the arrangement); and



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(d) in circumstances relating to the variation or extension of an arrangement:
(i) details of the variation or extension, including any new commencement dates or expiry dates for the arrangement; and
(ii) the reason for the variation or extension; and
(e) in circumstances relating to the cessation of an arrangement:
(i) the date on which the arrangement ceased or will cease; and
(ii) the reason for the cessation.

## 167-60 Changes relating to financial and prudential matters

(1) The change prescribed in item 9 of the table in section 167-10 is any change to a registered provider's capacity to deliver funded aged care services that the provider is registered to deliver, including the following:
(a) the inability to pay the entitlements of the provider's aged care workers;
(b) any change that materially affects an existing or anticipated revenue source;
(c) any instance in which the provider is unable to pay the provider's debts as and when those debts become due and payable.

Note: See section 167-10, which relates to notifying the Commissioner, and section 167-15, which relates to notifying the System Governor.

(2) Subsection (1) does not apply to circumstances set out in section 167-65 (which relates to liquidity).

(3) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:
(a) details about the change; and
(b) the impact that the provider expects the change to have on the provider's ability to deliver funded aged care services; and
(c) details about any mitigating strategies the provider has attempted, or will attempt, to manage this impact.

## 167-65 Changes relating to liquidity

(1) The change prescribed in item 10 of the table in section 167-10 is any circumstance where the registered provider is not maintaining, or is at risk of not maintaining, the provider's default minimum liquidity amount for a quarter or evaluated minimum liquidity amount for a quarter.

Note: See section 167-10, which relates to notifying the Commissioner, and section 167-15, which relates to notifying the System Governor.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:
(a) the provider's current and expected liquidity for the relevant quarter; and
(b) the reason for the reduction, or expected reduction in liquidity; and
(c) the provider's Liquidity Management Strategy; and
(d) the provider's plan to increase liquidity to a level that is sufficient for the purposes set out in subsection (1).



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Note: The standards in relation to financial and prudential matters are set out in the Financial and Prudential Standards made by the Commissioner under subsection 376(1) of the Act.

## 167-70 Changes relating to approved residential care homes

(1) The change prescribed in item 11 of the table in section 167-10 is any of the following changes relating to an approved residential care home of a registered provider:

(a) a change that may affect whether the approved residential care home of the provider continues to meet the definition of residential care home in the Act;

(b) a change to a building or the premises that comprise the approved residential care home that presents a risk to the delivery of quality and safe residential care;

(c) a planned construction or renovation activity relating to the approved residential care home that the provider reasonably expects will affect the delivery of funded aged care services at the residential care home;

(d) an unplanned event relating to the approved residential care home that the provider reasonably expects will cause sustained disruption to the delivery of funded aged care services at the residential care home;

(e) a change in the ownership of the premises at which the residential care home is located or a change to any agreement between the owner of the premises and the registered provider to use the premises as a residential care home;

(f) for any provider other than a provider referred to in paragraph (g)—a reduction in the availability of the total number of beds covered by the approval of the approved residential care home that the provider expects will continue for a continuous period of at least 3 months, except where the approved residential care home only delivers funded aged care services under a specialist aged care program;

(g) for a registered provider in the 2023 MM category known as MM 6 or MM 7—a reduction in the availability of the total number of beds covered by the approval of the approved residential care home for any period of time, except where the approved residential care home only delivers funded aged care services under a specialist aged care program;

(h) a change in circumstances that causes beds that were unavailable as referred to in paragraph (1)(f) or (g) to become available.

Note 1: See section 167-10, which relates to notifying the Commissioner, and section 167-15, which relates to notifying the System Governor.

Note 2: See section 136 of the Act, which relates to varying the approval of a residential care home of a registered provider.

(2) For the purposes of paragraph 167(4)(c) of the Act, the information that must be included in a notice given in relation to a change to which this section applies is:

(a) the name of the approved residential care home; and

(b) the street address of the residential care home; and

(c) a description of the change of circumstances; and



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(d) for circumstances referred to in paragraph (1)(f) or (g):

(i) the number of beds in respect of which the registered provider is, or will be, unable to provide residential care; and

(ii) the period during which the registered provider is, or will be, unable to provide residential care for those beds; and

(iii) the reason for the change to the availability of beds; and

(e) details of the change in ownership, and in circumstances where the registered provider is not the owner of the premises used for the provision of services and there is a change in ownership, the information must include the date and details of the type of agreement under which permission to use the premises is given.



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# Part 4—Responsible person obligation—change in circumstances relating to suitability

## 169-5 Kinds of registered provider to which the obligation applies

For the purposes of paragraph 169(1)(a) of the Act, every kind of registered provider is prescribed except a provider that is a sole trader.



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# Part 5—Obligations relating to suitability of responsible persons

## 172-5 Kinds of registered provider to which the obligation applies

For the purposes of subsection 172(1) of the Act, every kind of registered provider is prescribed.

## 172-10 Requirements for records of suitability matters

For the purposes of paragraph 172(1)(b) of the Act, a registered provider must keep a record of its consideration of suitability matters in relation to a person that includes the following:

(a) the name of the person in relation to whom the suitability matters were considered;

(b) the date or dates on which the suitability matters were considered in relation to the person;

(c) the outcome of the provider's consideration of each suitability matter in relation to the person;

(d) the reasons for reaching that outcome.



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# Part 6—Obligations relating to aged care workers etc.

## Division 1—Registered nurses

### Subdivision A—Preliminary

#### 175-5 Exception to registered nurse requirement—approved residential care homes at which funded aged care services delivered only under TCP

For the purposes of paragraph 175(2)(b) of the Act, the TCP is prescribed.

#### 175-10 Purpose of this Division

(1) For the purposes of subsection 175(3) of the Act, this Division provides for:
  (a) the circumstances in which an exemption from subsection 175(1) of the Act may be granted to a registered provider in relation to an approved residential care home; and
  (b) the period for which an exemption may be in force; and
  (c) the conditions that apply to that exemption.

(2) This Division also provides for the revocation of exemptions.

### Subdivision B—Process for granting exemptions

#### 175-15 Application for exemption

(1) A registered provider may apply to the System Governor for an exemption from subsection 175(1) of the Act in relation to an approved residential care home at which the provider delivers funded aged care services.

(2) An application under subsection (1) must be made in the approved form.

#### 175-25 Decision whether to grant exemption

*Criteria for granting exemption—approved residential care homes at which funded aged care services are not delivered only under the MPSP*

(1) If the System Governor receives an application under section 175-15 of this instrument from a registered provider for an exemption from subsection 175(1) of the Act in relation to an approved residential care home, other than a home at which the provider delivers funded aged care services only under the MPSP, the System Governor may grant the exemption only if:
  (a) the home is located in the 2023 MM category known as MM 5, MM 6 or MM7; and
  (b) there are no more than 30 operational beds in the home on the day of the System Governor's decision; and



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(c) the System Governor is satisfied that the provider has taken reasonable steps to ensure that the clinical care needs of the individuals in the home will be met during the period for which the exemption is in force; and

(d) the registered provider has given to the System Governor the reports required under section 166-855 of this instrument in relation to the home for each calendar month.

*Criteria for granting exemption—approved residential care homes at which funded aged care services are delivered only under the MPSP*

(1A) If the System Governor receives an application under section 175-15 of this instrument from a registered provider for an exemption from subsection 175(1) of the Act in relation to an approved residential care home at which the provider delivers funded aged care services only under the MPSP, the System Governor may grant the exemption only if:

(a) the home is located in the 2023 MM category known as MM 5, MM 6 or MM7; and

(b) no more than 30 residential care places for the home are in effect under section 97-5 of this instrument on the day of the System Governor's decision; and

(c) the System Governor is satisfied that the provider has taken reasonable steps to ensure that the clinical care needs of the individuals in the home will be met during the period for which the exemption is in force; and

(d) for an application made on or after 1 November 2026—the registered provider has given to the System Governor the reports required under section 166-855 of this instrument in relation to the home for each calendar month beginning on or after 1 October 2026.

(2) In deciding whether to grant the exemption, the System Governor must have regard to:

(a) any variation to the registration of the registered provider by the Commissioner under paragraph 123(1)(a) of the Act to vary a condition to which the registration is subject to under section 143 of the Act; and

(b) any variation to the registration of the registered provider by the Commissioner under paragraph 123(1)(b) of the Act; and

(c) any notice given to the registered provider by the System Governor or Commissioner under:

(i) Division 2 of Part 10 of Chapter 6 of the Act; or

(ii) Part 11 of Chapter 6 of the Act; and

(d) any notice given to the registered provider under Part 5 of the Regulatory Powers Act (as applied by section 448 of the Act); and

(e) if the registered provider has given an undertaking under section 114 of the Regulatory Powers Act (as applied by section 458 of the Act); and

(f) if the registered provider has given an undertaking under section 463 of the Act; and

(g) whether the registered provider has been convicted of an offence against the Act; and



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(h) whether the registered provider has been found liable to pay a civil penalty under this Act.

(3) The System Governor may grant an exemption to a registered provider in relation to an approved residential care home for which the registered provider has previously been granted an exemption.

*Period of exemption*

(4) If the System Governor decides to grant an exemption, the System Governor must decide the period for which the exemption is to be in force.

(5) The period:
(a) must not be longer than 12 months; and
(b) must not begin before the day on which the System Governor grants the exemption.

*Conditions that apply to exemption*

(6) The following conditions apply to an exemption:
(a) the registered provider must give the System Governor notice in writing of any material change to the information given to the System Governor:
(i) in the application for the exemption; or
(ii) in response to a request by the System Governor under section 588 of the Act;
(b) any additional conditions that the System Governor decides to impose on the exemption.

## 175-30 Notice of decision

(1) If the System Governor grants an exemption from subsection 175(1) of the Act to a registered provider in relation to an approved residential care home, the System Governor must give the provider notice in writing of the decision that:
(a) states the period for which the exemption is in force; and
(b) states the conditions that apply to the exemption.

(2) If the System Governor refuses to grant an exemption from subsection 175(1) of the Act to a registered provider in relation to an approved residential care home, the System Governor must give the provider notice in writing of the decision, including:
(a) the reasons for the decision; and
(b) the date of the decision; and
(c) how the registered provider may apply for reconsideration of the decision.



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## Subdivision C—Revocation of exemptions

### 175-45 Revocation on request

(1) The System Governor must revoke a registered provider's exemption from subsection 175(1) of the Act in relation to a residential care home if the registered provider requests the revocation in writing.

(2) The System Governor must give the registered provider notice in writing of the date that the exemption ceases to have effect, which may be the date of the notice or a later date.

### 175-50 Revocation on System Governor's initiative

*Grounds for revocation*

(1) The System Governor may revoke a registered provider's exemption from subsection 175(1) of the Act in relation to an approved residential care home if:
  (a) the System Governor is satisfied the registered provider has breached a condition of the exemption; or
  (b) the System Governor is not satisfied that the clinical care needs of individuals in the home:
    (i) are being met; or
    (ii) will be met during the period the exemption would otherwise be in force; or
  (c) the registration of the registered provider has been varied by the Commissioner under paragraph 123(1)(a) of the Act to vary a condition to which the registration is subject to under section 143 of the Act; or
  (d) the registration of the registered provider has been varied by the Commissioner under paragraph 123(1)(b) of the Act; or
  (e) in relation to a home other than a home at which the provider delivers funded aged care services only under the MPSP—the System Governor becomes aware there are more than 30 operational beds in the home; or
  (f) in relation to a home at which the provider delivers funded aged care services only under the MPSP—the System Governor becomes aware there are more than 30 residential care places for the home in effect under section 97-5 of this instrument.

*Notice before revocation*

(2) Before the System Governor revokes a registered provider's exemption under this section, the System Governor must give the provider written notice that the System Governor is considering revoking the exemption.

(3) The notice must:
  (a) set out the reasons why the System Governor is considering revoking the exemption, and what the effect of the revocation would be; and



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(b) invite the provider to make a submission, in writing, to the System Governor in relation to the matter within the period specified in the notice (which must not be less than 28 days after giving the notice); and

(c) inform the provider that the System Governor may decide to revoke the exemption:

(i) if no submission is made within the specified period; or

(ii) after considering any submission made by the provider within the specified period.

*Request for further information*

(4) The System Governor may also request, in the notice given under subsection (2), that the provider give the System Governor the information specified in the notice, within the period specified in the notice for the purposes of paragraph (3)(b), to assist the System Governor to decide whether to revoke the exemption.

Note: The provider is not obliged to give the information.

(5) If the provider makes a submission in accordance with the invitation under paragraph (3)(b), the System Governor may, by written notice, request the provider to give the System Governor further information, within the period specified in the notice (which must not be less than 28 days after giving the notice), to assist the System Governor to decide whether to revoke the exemption.

Note 1: The System Governor may request further information under this subsection regardless of whether information was requested or given under subsection (4).

Note 2: The provider is not obliged to give the information.

(6) The System Governor may, at the request of the provider, extend the period specified for paragraph (3)(b) or subsection (5).

*Deciding whether to revoke exemption*

(7) The System Governor must:

(a) consider any submission made in accordance with the invitation under paragraph (3)(b) and any further information given in accordance with a request under subsection (4) or (5); and

(b) make a decision whether to revoke the exemption within 28 days from:

(i) if no submission is made in accordance with the invitation in the notice under paragraph (3)(b)—the end of the period specified in the notice (including that period as extended (if applicable) under subsection (6)); or

(ii) the later of:

(A) if a submission is made in accordance with the invitation in the notice under paragraph (3)(b) and no further information is requested under subsection (5)—the day the submission is made; and



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(B) if a submission is made in accordance with the invitation in the notice under paragraph (3)(b) and further information is requested under subsection (5)—the day the provider gives the System Governor the further information.

## 175-55 Notice of decision

If the System Governor decides under section 175-50 of this instrument to revoke a registered provider's exemption from subsection 175(1) of the Act in relation to an approved residential care home, the System Governor must give the registered provider notice in writing of the decision, including:

(a) the reasons for the decision; and

(b) the date that the exemption ceases to have effect, which may be the date of the decision or a later date; and

(c) how the registered provider may apply for reconsideration of the decision.



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# Division 2—Delivery of direct care

## Subdivision A—Delivery of direct care—mainstream providers

### 176-5 Kinds of provider to which the obligation applies, and requirements

For the purposes of section 176 of the Act:

(a) a registered provider registered in the provider registration category residential care is prescribed; and

(b) this Subdivision sets out the requirements for delivering direct care.

### 176-10 Application of this Subdivision

The requirements of this Subdivision do not apply in respect to the delivery of funded aged care services provided under any of the following specialist aged care programs:

(a) TCP;

(b) MPSP;

(c) NATSIFACP.

### 176-15 Care minutes that must be delivered

(1) This section applies if the provider is delivering funded aged care services in an approved residential care home in a quarter.

*Requirement—care minutes delivered by direct care staff members*

(2) The provider must ensure that the average number of care minutes delivered in the home by direct care staff members of the provider per counted mainstream individual per day is at least the required combined staff average number of care minutes per individual per day worked out under subsection 176-20(1) in respect of the home for the quarter.

Note: Direct care staff members of the provider include registered nurse staff members of the provider (see the definition of ***direct care staff member*** in section 7 of the Act).

*Requirement—care minutes delivered by registered nurse staff members*

(3) The provider must ensure that the average number of care minutes delivered in the home by registered nurse staff members of the provider per counted mainstream individual per day is at least 90% of the required registered nurse average number of care minutes per individual per day worked out under subsection 176-20(2) in respect of the home for the quarter.

*Responsibility—care minutes delivered by registered nurse staff members and enrolled nurse staff members*

(4) The registered provider must ensure that the average number of care minutes delivered in the home by registered nurse staff members and enrolled nurse staff



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members of the provider per counted mainstream individual per day is at least the required registered nurse average number of care minutes per individual per day worked out under subsection 176-20(2) in respect of the home for the quarter.

*Counted individuals*

(5) An individual accessing funded aged care services in an approved residential care home on a day is a ***counted mainstream individual*** on the day unless:
  (a) the individual accesses the funded aged care services through a specialist aged care program referred to in section 176-10; or
  (b) the individual is on extended hospital leave, and the day is on or after the 29th day of the individual's leave.

## 176-20 Average numbers of care minutes

*Required combined staff average number of care minutes*

(1) The ***required combined staff average number of care minutes*** per individual per day in respect of an approved residential care home for a quarter is worked out by:
  (a) starting with the sum of the combined staff daily amounts for all of the days of recognised residential care provided in respect of individuals in the home during the reference period for the quarter; and
  (b) dividing that sum by the total number of days of recognised residential care provided in respect of individuals in the home during the reference period for the quarter; and
  (c) rounding the result of that division to 2 decimal places (rounding up if the third decimal place is 5 or more).

*Required registered nurse average number of care minutes*

(2) The ***required registered nurse average number of care minutes*** per individual per day in respect of an approved residential care home for a quarter is worked out by:
  (a) starting with the sum of the registered nurse daily amounts for all of the days of recognised residential care provided in respect of individuals in the home during the reference period for the quarter; and
  (b) dividing that sum by the total number of days of recognised residential care provided in respect of individuals in the home during the reference period for the quarter; and
  (c) rounding the result of that division to 2 decimal places (rounding up if the third decimal place is 5 or more).

*Daily amounts*

(3) The following table sets out, for a day of recognised residential care provided in respect of an individual in an approved residential care home:
  (a) the combined staff daily amount for the day for the individual; and



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(b) the registered nurse daily amount for the day for the individual.

**Daily amounts**

| Item | Column 1<br>For an individual classified as … | Column 2<br>the combined staff daily amount is … (minutes) | Column 3<br>and the registered nurse daily amount is … (minutes) |
|---|---|---|---|
| 1 | Class 1 | 268 | 51 |
| 2 | Class 2 | 128 | 27 |
| 3 | Class 3 | 178 | 36 |
| 4 | Class 4 | 150 | 32 |
| 5 | Class 5 | 185 | 41 |
| 6 | Class 6 | 176 | 37 |
| 7 | Class 7 | 215 | 46 |
| 8 | Class 8 | 232 | 47 |
| 9 | Class 9 | 214 | 44 |
| 10 | Class 10 | 229 | 44 |
| 11 | Class 11 | 253 | 48 |
| 12 | Class 12 | 247 | 47 |
| 13 | Class 13 | 268 | 51 |
| 14 | Respite Class 1 | 176 | 37 |
| 15 | Respite Class 2 | 223 | 48 |
| 16 | Respite Class 3 | 262 | 51 |

*Day of recognised residential care*

(4) A ***day of recognised residential care*** is provided in respect of an individual in an approved residential care home if funded aged care services are delivered to the individual in the home through the residential care service category on that day.

(5) Despite subsection (4), a ***day of recognised residential care*** does not include a day where:
  (a) funded aged care services are delivered through a specialist aged care program referred to in section 176-10; or
  (b) funded aged care services are provided to an individual classified as:
    (i) Class 0; or
    (ii) Respite Class 0.

*Reference period*

(6) The ***reference period*** for a quarter is the period of 3 months beginning on the day that is 4 months before the first day of the quarter.



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*Information to be disregarded for calculations*

(7) For the purposes of a calculation under this section for a quarter:
  (a) information about an individual entering or exiting an approved residential care home during the reference period for the quarter is to be disregarded if it is given to the System Governor on or after the calculation day for the quarter; and
  (b) a change to a classification decision for an individual that is made on or after the calculation day for the quarter but takes effect before the calculation day is to be disregarded.

*Calculation if no days of recognised residential care provided during reference period*

(8) If no days of recognised residential care were provided in respect of individuals in an approved residential care home during the reference period for a quarter, the result of the calculations in subsections (1) and (2) in relation to the quarter is taken to be zero minutes per individual per day.

Note: This subsection is to avoid an undefined result when dividing by zero.

## Subdivision B—Delivery of direct care—NATSIFACP providers

### 176-25 Kinds of provider to which the obligation applies, and requirements

For the purposes of section 176 of the Act:
  (a) a registered provider registered in the provider registration category residential care is prescribed; and
  (b) this Subdivision sets out the requirements for delivering direct care.

### 176-30 Application of this Subdivision

The requirements of this Subdivision do not apply in respect to the delivery of funded aged care services provided under any of the following specialist aged care programs:
  (a) TCP;
  (b) MPSP.

### 176-35 Care minutes that must be delivered—NATSIFACP providers

(1) This section applies if the registered provider is delivering funded aged care services in an approved residential care home in a quarter under the specialist aged care program NATSIFACP.

*Requirement—care minutes delivered by direct care staff members*

(2) The provider must ensure that the average number of care minutes delivered in the home by direct care staff members of the provider per counted NATSIFACP individual per day is at least 215 minutes per individual per day in respect of the home for the reporting period for the provider.



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Note: Direct care staff members of the provider include registered nurse staff members of the provider (see the definition of ***direct care staff member*** in section 7 of the Act).

*Requirement—care minutes delivered by registered nurse staff members*

(3) The provider must ensure that the average number of care minutes delivered in the home by registered nurse staff members of the provider per counted NATSIFACP individual per day is at least 39.6 minutes per individual per day in respect of the home for the reporting period for the provider.

*Responsibility—care minutes delivered by registered nurse staff members and enrolled nurse staff members*

(4) The provider must ensure that the average number of care minutes delivered in the home by registered nurse staff members and enrolled nurse staff members of the provider per counted NATSIFACP individual per day is at least 44 minutes per individual per day in respect of the home for the reporting period for the provider.

*Counted individuals*

(5) An individual accessing funded aged care services in an approved residential care home on a day under the specialist aged care program NATSIFACP is a ***counted NATSIFACP individual*** on the day unless:
  (a) the individual is on extended hospital leave; and
  (b) the day is on or after the 29th day of the individual's leave.

*Reporting period*

(6) The reporting period for the provider is the period specified for a service activity report under section 166-645.



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# Part 7—Other obligations—cooperation with other persons

## 177-10 Giving data or records to the Pricing Authority

*Application of this section*

(1) Subject to subsection (2), this section applies to every kind of registered provider.

(2) The requirements of this section do not apply to a registered provider in respect of the delivery of funded aged care services under the following specialist aged care programs:
  (a) CHSP;
  (b) TCP.

*Requirement to comply with notice*

(3) For the purposes of subsection 177(2) of the Act, it is a requirement that a registered provider must comply with a request made under subsection (4) or (6).

*Notice to provide data or records*

(4) The Pricing Authority may request, by written notice given to a registered provider, that the provider:
  (a) give the Pricing Authority data or records held by the provider, or copies of that data or records, that are necessary for the conduct of a Pricing Authority advice activity; and
  (b) do so in a form and manner specified in the notice; and
  (c) do so before or on a day specified in the notice.

(5) A notice given under subsection (4) must:
  (a) set out that the request is for the purposes of the Pricing Authority performing the function mentioned in paragraph 131A(1)(a) of the *National Health Reform Act 2011*; and
  (b) specify the details of the data or records that the registered provider is requested to give; and
  (c) specify a day by which the data or records must be given, which must be at least 14 days after the day on which the notice is given to the provider; and
  (d) set out the effect of subsection 177(4) of the Act.

*Request for further data or records*

(6) If:
  (a) a provider responds to a request to provide data or records made under subsection (4); and



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(b) the Pricing Authority considers that additional data or records are required before the Pricing Authority can carry out a function mentioned in paragraph 131A(1)(a) of the *National Health Reform Act 2011*;

the Pricing Authority may, by written notice given to the applicant, request the provider to give that additional data or records, or copies of that additional data or records, to the Pricing Authority.

## 177-15 Allowing access by the Pricing Authority to certain persons

*Application of this section*

(1) Subject to subsection (2), this section applies to every kind of registered provider.

(2) The requirements of this section do not apply to a registered provider in respect of the delivery of funded aged care services under the following specialist aged care programs:
(a) CHSP;
(b) TCP.

*Requirement to comply with notice*

(3) For the purposes of subsection 177(2) of the Act, it is a requirement that a registered provider must comply with a request made under subsection (4).

*Notice to allow access to certain persons*

(4) The Pricing Authority may request, by written notice given to a registered provider, that the provider:
(a) allow and facilitate access by an official of the Pricing Authority to a person referred to in subsection (6) for the purposes of undertaking a Pricing Authority advice activity; and
(b) make arrangements before or on a day specified in the notice to allow and facilitate the access within a reasonable timeframe.

(5) A notice given under subsection (4) must:
(a) specify the form of the access, which may be in person, by audio link, or by audio-visual link; and
(b) specify a day by which the provider must make arrangements for the access, which must be at least 14 days after the day on which the notice is given to the provider; and
(c) set out the effect of subsection 177(4) of the Act.

(6) For the purposes of paragraph (4)(a), the persons are the following:
(a) a specified responsible person of the provider;
(b) any responsible person of the provider who is responsible for a specified matter for the provider;
(c) a specified aged care worker of the provider;



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(d) any aged care worker of the provider who is responsible for a specified matter for the provider.

## 177-20 Allowing access by the Pricing Authority to residential care homes

*Application of this section*

(1) Subject to subsection (2), this section applies to a registered provider in the provider registration category residential care.

(2) The requirements of this section do not apply to a registered provider in respect of the delivery of funded aged care services under the following specialist aged care programs:
(a) CHSP;
(b) TCP.

*Requirement to comply with notice*

(3) For the purposes of subsection 177(2) of the Act, it is a requirement that a registered provider must comply with a request made under subsection (4).

*Notice to allow access to residential care homes*

(4) The Pricing Authority may request, by written notice given to a registered provider, that the provider:
(a) allow and facilitate access by an official of the Pricing Authority to a residential care home of the provider for the purposes of undertaking a Pricing Authority advice activity; and
(b) do so on a day specified in the notice.

(5) A notice given under subsection (4) must:
(a) specify a day on which the access must be facilitated, which must be at least 14 days after the day on which the notice is given to the provider; and
(b) set out the effect of subsection 177(4) of the Act.

(6) For the purposes of subsection 177(2) of the Act, it is a requirement that reasonable facilities and assistance provided by the registered provider to whom a notice has been given under subsection (4) must include:
(a) providing a site orientation to the official of the Pricing Authority undertaking the Pricing Authority advice activity to which the notice relates, including guidance on how to safely navigate the residential care home; and
(b) providing information in relation to the activity to any of the following:
(i) aged care workers of the provider;
(ii) individuals to whom the provider delivers funded aged care services;
(iii) supporters of the individuals; and
(c) providing the official with access to aged care workers of the provider who are at the residential care home; and



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(d) providing the official with access to individuals to whom the provider delivers funded aged care services who have consented to the access; and

(e) providing the official with access to all areas of the residential care home, excluding rooms and part rooms of individuals who have not consented to the access and private bathroom facilities; and

(f) providing the official with facilities (including a suitable workspace) for the purpose of undertaking the Pricing Authority advice activity; and

(g) providing the official with access to records held by the provider; and

(h) providing the official with instructions about how to access records held by the provider; and

(i) allowing the official to make copies of those records.



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